

Solicitation: 75D301-26-R-73374

This is a combined synopsis/solicitation for commercial products or commercial services prepared in accordance with part 12. This announcement constitutes the only solicitation. Offers are being requested and a separate written solicitation will not be issued.

The solicitation number for this requirement is **75D301-26-R-73374** and is hereby issued as a Request for Proposal (RFP).

This acquisition is not set-aside for small business concerns. This solicitation incorporates provisions and clauses by reference. The full text of provisions and clauses may be accessed electronically at www.acquisition.gov.

The Centers for Disease Control and Prevention intend to award a hybrid (firm-fixed-price, time and materials) contract for the World Trade Center Health Program National Program Administrator in accordance with solicitation Section E – World Trade Center Program National Program Administrator Performance Work Statement.

See Section B- Schedule of the solicitation for pricing information.

For a list of applicable contract clauses and provisions see Section C, F, and Section G of the solicitation to follow.

For detailed proposal preparation instructions and basis of evaluation see Section H & I.

Questions regarding this solicitation must be submitted via email and received by both Megan Kelly at xvp9@cdc.gov AND Liubov Kriel at vyhl@cdc.gov on or before **10:00 am EST, Monday, January 5, 2026**. Please submit questions in accordance with the template provided in ATT025_NPA_QuestionsandAnswersTemplate found in Section D.

Proposals must be submitted via email and received by the undersigned by **10:00 am EST, January 30, 2026**.

When submitting your proposal, please include in the Subject Line: Solicitation #75D301-26-R-73374.

PoC for this requirement is:

Megan Kelly

404-498-4805

Xvp9@cdc.gov

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SOLICITATION/CONTRACT/ORDER FOR COMMERCIAL ITEMS <i>OFFEROR TO COMPLETE BLOCKS 12, 17, 23, 24, & 30</i>				1. REQUISITION NUMBER		PAGE 2 OF 159	
2. CONTRACT NO.		3. AWARD/EFFECTIVE DATE		4. ORDER NUMBER		5. SOLICITATION NUMBER 75D301-26-R-73374	
7. FOR SOLICITATION INFORMATION CALL:		a. NAME Megan Kelly				b. TELEPHONE NUMBER (No collect calls) 440-498-4805	
9. ISSUED BY Centers for Disease Control and Prevention Chamblee Campus Building 108 4770 Buford Hwy Chamblee, GA 30341				CODE 8219		10. THIS ACQUISITION IS X UNRESTRICTED SET ASIDE: % FOR SMALL BUSINESS SMALL DISADV. BUSINESS	
						11. DELIVERY FOR FOB DESTINATION UNLESS BLOCK IS MARKED X SEE SCHEDULE 13a. THIS CONTRACT IS A RATED ORDER UNDER DPAS (15 CFR 700) 13b. RATING	
15. DELIVER TO Centers for Disease Control and Prevention Chamblee Campus Building 108 4770 Buford Hwy Chamblee, GA 30341				CODE 532		16. ADMINISTERED BY Centers for Disease Control and Prevention Chamblee Campus Building 102, MS S102-4 4770 Buford Hwy Atlanta, GA 30341-3717	
17a. CONTRACTOR/OFFEROR TELEPHONE NO.				CODE		18a. PAYMENT WILL BE MADE BY Centers for Disease Control and Prevention (FMO) PO Box 15580 404-718-8100	
				FACILITY CODE		CODE 434	
17b. CHECK IF REMITTANCE IS DIFFERENT AND PUT SUCH ADDRESS IN OFFER				18b. SUBMIT INVOICES TO ADDRESS SHOWN IN BLOCK 18a UNLESS BLOCK BELOW IS CHECKED SEE ADDENDUM			
19. ITEM NO.		20. SCHEDULE OF SUPPLIES/SERVICES		21. QUANTITY		22. UNIT	
						23. UNIT PRICE	
						24. AMOUNT	
		See Section B (Attach Additional Sheets as Necessary)					
25. ACCOUNTING AND APPROPRIATION DATA						26. TOTAL AWARD AMOUNT (For Govt. Use Only)	
X 27a. SOLICITATION INCORPORATES BY REFERENCE FAR 52.212-1, 52.212-4, FAR 52.212-3 AND 52.212-5 ARE ATTACHED. ADDENDA ARE X ARE NOT ATTACHED. 27b. CONTRACT/PURCHASE ORDER INCORPORATES BY REFERENCE FAR 52.212-4. FAR 52.212-5 IS ATTACHED. ADDENDA ARE X ARE NOT ATTACHED.							
28. CONTRACTOR IS REQUIRED TO SIGN THIS DOCUMENT AND RETURN 1 COPIES TO ISSUING OFFICE. CONTRACTOR AGREES TO FURNISH AND DELIVER ALL ITEMS SET FORTH OR OTHERWISE IDENTIFIED ABOVE AND ON ANY ADDITIONAL SHEETS SUBJECT TO THE TERMS AND CONDITIONS SPECIFIED HEREIN. X				29. AWARD OF CONTRACT: REFERENCE OFFER DATED . YOUR OFFER ON SOLICITATION (BLOCK 5), INCLUDING ANY ADDITIONS OR CHANGES WHICH ARE SET FORTH HEREIN, IS ACCEPTED AS TO ITEMS:			
30a. SIGNATURE OF OFFEROR/CONTRACTOR				31a. UNITED STATES OF AMERICA (Signature of Contracting Officer)			
30b. NAME AND TITLE OF SIGNER (Type or print)		30c. DATE SIGNED		31b. NAME OF CONTRACTING OFFICER (Type or print)		31c. DATE SIGNED	
32a. QUANTITY IN COLUMN 21 HAS BEEN RECEIVED INSPECTED ACCEPTED, AND CONFORMS TO THE CONTRACT, EXCEPT AS NOTED				33. SHIP NUMBER		34. VOUCHER NUMBER	
				PARTIAL FINAL		35. AMOUNT VERIFIED CORRECT FOR	
32b. SIGNATURE OF AUTHORIZED GOV'T REPRESENTATIVE		32c. DATE		36. PAYMENT COMPLETE PARTIAL FINAL		37. CHECK NUMBER	
				38. S/R ACCOUNT NUMBER		39. S/R VOUCHER NUMBER	
						40. PAID BY	
41a. I CERTIFY THIS AMOUNT IS CORRECT AND PROPER FOR PAYMENT				42a. RECEIVED BY (Print)			
41b. SIGNATURE AND TITLE OF CERTIFYING OFFICER				42b. RECEIVED AT (Location)			
41c. DATE							

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	Points of Contact: Contracting Officer: Megan Kelly, xvp9@cdc.gov, 404-498-4805 Contracting Officer: Liubov Kriel, vyh1@cdc.gov , 770-488-2856	

SECTION B - Schedule

Base Period: March 18, 2026 – March 17, 2027

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
0001	Transition In The contractor shall provide services as described in Section E, the Performance Work Statement Months 1-8: PWS Transition-In Sections 4.1.1 through 4.1.4 Months 9-12: PWS Handover Section 4.1.5 See B.1 Payment Schedule Non- Severable Services Firm-Fixed-Price March 18, 2026 to March 17, 2027	1 Job	\$ _____	\$ _____
0002	Sustainment– National Program Administrator The contractor shall provide Sustainment services as described in Section E, the Performance Work Statement. PWS Section: 4.2 through 4.6 Severable Services November 18, 2026 to March 17, 2027 Times & Materials	1 Job	\$ _____	\$ _____ Not-To-Exceed
0003	Travel (non-conference) All travel in accordance with Federal Travel Regulation March 18, 2026 to March 17, 2027 All travel expenses require prior written approval from the COR Time and Materials	1 Lot	\$ _____	\$ _____ Not-To-Exceed

Base Period Total: \$ _____

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Option Period 1: March 18, 2027 – March 17, 2028

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1001	Sustainment– National Program Administrator The contractor shall provide Sustainment services as described in Section E, the Performance Work Statement. PWS Section: 4.2 through 4.6 Severable Services March 18, 2027 to March 17, 2028 Times & Materials	1 Job	\$ _____	\$ _____ Not-To-Exceed
1002	Travel (non-conference) All travel will be in accordance with the Federal Travel Regulation. March 18, 2027 to March 17, 2028 All travel expenses require prior written approval from the COR Time and Materials	1 Lot	\$ _____	\$ _____ Not-To-Exceed

Option Period 1 Total:

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Option Period 2: March 18, 2028 – March 17, 2029

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
2001	Sustainment– National Program Administrator The contractor shall provide Sustainment services as described in Section E, the Performance Work Statement. PWS Section: 4.2 through 4.6 Severable Services March 18, 2028 to March 17, 2029 Times & Materials	1 Job	\$ _____	\$ _____ Not-To-Exceed
2002	Travel (non-conference) All travel will be in accordance with the Federal Travel Regulation. March 18, 2028 to March 17, 2029 All travel expenses require prior written approval from the COR Time and Materials	1 Lot	\$ _____	\$ _____ Not-To-Exceed

Option Period 2 Total:

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Option Period 3: March 18, 2029 – March 17, 2030

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
3001	Sustainment– National Program Administrator The contractor shall provide Sustainment services as described in Section E, the Performance Work Statement. PWS Section: 4.2 through 4.6 Severable Services March 18, 2029 to March 17, 2030 Times & Materials	1 Job	\$ _____	\$ _____ Not-To-Exceed
3002	Travel (non-conference) All travel will be in accordance with the Federal Travel Regulation March 18, 2029 to March 17, 2030 All travel expenses require prior written approval from the COR Time and Materials	1 Lot	\$ _____	\$ _____ Not-To-Exceed

Option Period 3 Total:

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Option Period 4: March 18, 2030 – March 17, 2031

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
4001	Sustainment– National Program Administrator The contractor shall provide Sustainment services as described in Section E, the Performance Work Statement. PWS Section: 4.2 through 4.6 Severable Services March 18, 2030 to March 17, 2031 Times & Materials	1 Job	\$ _____	\$ _____ Not-To-Exceed
4002	Travel (non-conference) All travel will be in accordance with the Federal Travel Regulation. March 18, 2030 to March 17, 2031 All travel expenses require prior written approval from the COR Time and Materials	1 Lot	\$ _____	\$ _____ Not-To-Exceed
4003	Transition Out The contractor shall provide the services as described in Section E, the Performance Work Statement Transition Out Tasks Include: Section 4.5 See Payment Schedule Non-Severable Services Firm-Fixed-Price March 18, 2030 to March 17, 2031	1 Job	\$ _____	\$ _____

Option Period 4 Total:

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Base Year Value:	
Option Period 1 Value:	
Option Period 2 Value:	
Option Period 3 Value:	
Option Period 4 Value:	
Total Contract Value:	

B.1 Payment Schedule**Milestone Payment Schedule for Firm-Fixed-Price CLINs, CLIN 0001 & 4003, ONLY**

The contractor is invited to propose a Milestone Payment Schedule for *CLINs 0001 and 4003*, along with its proposed payment percentage allocations for each scheduled payment below based upon your total proposed price for each *CLIN 0001 and 4005*. Milestones should be tied to the completion of specific, measurable deliverables that provide identifiable benefit to the Government, rather than simply dividing the 12-month period of performance into monthly increments. During negotiations, CDC may suggest alternative milestones and payment percentages. The final Milestone Payment Schedule will be negotiated and incorporated into the award.

Example Payment Schedule -**CLIN 0001: March 18, 2026 – March 17, 2027**

Milestones	Milestone Criteria	% Payment	Amount
Milestone 1 –		__%	\$ _____
Milestone 2 –		__%	\$ _____
Milestone 3 –		__%	\$ _____
Milestone 4 –		__%	\$ _____
Total		100%	\$ _____

CLIN 4005: March 18, 2030 – March 17, 2031

Milestones	Milestone Criteria	% Payment	Amount
Milestone 1 –		__%	\$ _____
Milestone 2 –		__%	\$ _____
Milestone 3 –		__%	\$ _____
Milestone 4 –		__%	\$ _____
Total		100%	\$ _____

B.2 Level of Effort/Labor Categories and ODCs

To be included within the Offeror's Volume II Business Proposal. Information will form the basis of invoicing for T&M CLINs.

**ATTACHMENT ATT024
FULLY BURDENED LABOR RATES
TO BE INCLUDED AS PART OF VOLUME II – BUSINESS PROPSAL**

The following labor categories and labor rates shall be used for all CLINs issued under this contract.

ESTIMATED HOURS & FIXED HOURLY RATES ARE AS FOLLOWS:

Base Period: Performance Period: 03/18/2026-03/17/2027		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 1 Period: Performance Period: 03/18/2027-03/17/2028		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 2 Period: Performance Period: 03/18/2028-03/17/2029		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 3 Period: Performance Period: 03/18/2029-03/17/2030		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 4 Period: Performance Period: 03/18/2030-03/17/2031		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

NOTATION REGARDING LABOR HOUR VARIANCE: Performance under this Time and Materials Contract is in accordance with FAR 52.232-7, "Payments under Time and Materials and Labor Hour Contracts," incorporated by reference in Section C, which requires the vendor to manage to the ceiling price in the contract and the ceiling price of the line items. The number of hours per labor category are estimates. The COR must be notified of any variance from the estimated hours shown in the Level of Effort/Labor Categories chart. If the variance in hours for a specific labor category line in the Level of Effort/Labor Categories chart above is 10% or more, the Contracting Officer must be notified.

SECTION C - CONTRACT CLAUSES**FAR 52.252-2 Clauses Incorporated by Reference (Feb 1998)**

This contract incorporates one or more clauses by reference, with the same force and effect as if they were given in full text. Upon request, the Contracting Officer will make their full text available. Also, the full text of a clause may be accessed electronically at this/these address(es):

<https://www.acquisition.gov/>

FAR SOURCE	TITLE AND DATE
FAR 52.203-6, Alternate I (Nov 2021)	Restrictions on Subcontractor Sales to the Government. (JUN 2020)
FAR 52.203-13	Contractor Code of Business Ethics and Conduct (NOV 2021)
FAR 52.203-17	Contractor Employee Whistleblower Rights (NOV 2023)
FAR 52.203-19	Prohibition on Requiring Certain Internal Confidentiality Agreements or Statements (JAN 2017)
FAR 52.204-13	System for Award Management Maintenance (DEVIATION) (RFO AUG 2025)
FAR 52.204-14	Service Contract Reporting Requirements. (DEVIATION) (RFO AUG 2025)
FAR 52.204-19	Incorporation by Reference of Representations and Certifications (DEC 2014)
FAR 52.209-6	Protecting the Government's Interest When Subcontracting with Contractors Debarred, Suspended, Proposed for Debarment, or Voluntarily Excluded (DEVIATION) (RFO AUG 2025)
FAR 52.209-9	Updates of Publicly Available Information Regarding Responsibility Matters (DEVIATION) (RFO AUG 2025)
FAR 52.209-10	Prohibition on Contracting with Inverted Domestic Corporations (DEVIATION) (RFO AUG 2025)
FAR 52.212-4 w/ Alt I	Contract Terms and Conditions—Commercial Products and Commercial Services (DEVIATION) (RFO AUG 2025)
FAR 52.219-4	Notice of Price Evaluation Preference for HUBZone Small Business Concerns ((DEVIATION) (RFO AUG 2025)
FAR 52.219-8	Utilization of Small Business Concerns (DEVIATION) (RFO AUG 2025)
FAR 52.219-9	Small Business Subcontracting Plan (DEVIATION) (RFO AUG 2025)
FAR 52.219-16	Liquidated Damages—Subcontracting Plan (DEVIATION) (RFO AUG 2025)
FAR 52.222-3	Convict Labor (DEVIATION) (RFO AUG 2025)
FAR 52.222-35	Equal Opportunity for Veterans (DEVIATION) (RFO AUG 2025)
FAR 52.222-36	Equal Opportunity for Workers with Disabilities (DEVIATION) (RFO AUG 2025)
FAR 52.222-37	Employment Reports on Veterans (DEVIATION) (RFO AUG 2025)
FAR 52.222-40	Notification of Employee Rights Under the National Labor Relations Act (DEVIATION) (RFO AUG 2025)
FAR 52.222-41	Service Contract Labor Standards (DEVIATION) (RFO AUG 2025)
FAR 52.222-43	Fair Labor Standards Act and Service Contract Labor Standards – Price Adjustment (Multiple Year and Option Contracts)
FAR 52.222-50	Combating Trafficking in Persons (DEVIATION) (RFO AUG 2025)
FAR 52.222-54	Employment Eligibility Verification (DEVIATION) (RFO AUG 2025)
FAR 52.222-55	Minimum Wages for Contractor Workers Under Executive Order 14026 (DEVIATION) (RFO AUG 2025)
FAR 52.222-62	Paid Sick Leave Under Executive Order 13706 ((DEVIATION) (RFO AUG 2025)
FAR 52.224-1	Privacy Act Notification (APR 1984)
FAR 52.224-2	Privacy Act (APR 1984)
FAR 52.224-3	Privacy Training (JAN 2017)

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FAR 52.226-8	Encouraging Contractor Policies to Ban Text Messaging While Driving (MAY 2024)
FAR 52.227-14	Rights in Data-General (MAY 2014)
FAR 52.229-12	Tax on Certain Foreign Procurements (FEB 2021)
FAR 52.232-32	Performance-Based Payments (APR 2012)
FAR 52.232-33	Payment by Electronic Funds Transfer-System for Award Management (OCT 2018)
FAR 52.232-37	Payments Under Time-and-Materials and Labor-Hour Contracts (NOV 2021)
FAR 52.232-39	Unenforceability of Unauthorized Obligations (JUN 2013)
FAR 52.232-40	Providing Accelerated Payments to Small Business Subcontractors. (MAR 2023)
FAR 52.233-3	Protest after Award (DEVIATION) (RFO AUG 2025)
FAR 52.233-4	Applicable Law for Breach of Contract Claim (DEVIATION) (RFO AUG 2025)
FAR 52.237-3	Continuity of Services (JAN 1991)
FAR 52.240-91	Security Prohibitions and Exclusions (DEVIATION) (RFO AUG 2025)
FAR 52.240-93	Basic Safeguarding of Covered Contractor Information Systems (DEVIATION) (RFO AUG 2025)
FAR 52.242-5	Payments to Small Business Subcontractors (JAN 2017)
HHSAR SOURCE	TITLE AND DATE
HHSAR 352.203-70	Anti-Lobbying (DEC 2015)
HHSAR 352.208-70	Printing and Duplication (DEC 2015)
HHSAR 352.219-71	Mentor-Protégé Program Reporting Requirements (JAN 2010)
HHSAR 352.231-70 ¹	Salary Rate Limitation (DEC 2015)
HHSAR 352.232-71	Electronic submission of payment requests (FEB 2022)
HHSAR 352.224-70	Privacy Act (DEC 2015)

FAR clauses incorporated in full text:**Addendum to FAR 52.212-4 and FAR 52.212-4 Alt I**

Paragraph (s) of FAR 52.212-4 and FAR 52.212-4 Alt I is revised as shown below in the [*emphasized text*]:

(s) *Order of precedence*. Any inconsistencies in this solicitation or contract shall be resolved by giving precedence in the following order:

- (1) The schedule of supplies/services.
- (2) The Assignments, Disputes, Payments, Invoice, Other Compliances, Compliance with Laws Unique to Government Contracts, and Unauthorized Obligations paragraphs of this clause;
- (3) Addenda to this solicitation or contract, including any license agreements for computer software.
- (4) Solicitation provisions if this is a solicitation.
- (5) Other paragraphs of this clause.
- (6) The [Standard Form 1449](#).
- (7) Other documents, exhibits, and attachments. [*The following order of precedence applies to this contract: The Performance Work Statement, followed by, in order of most recent publication date, the Technical*

¹ See <https://www.hhs.gov/grants-contracts/contracts/contract-policies-regulations/salary-rate-limitation/index.html> for guidance on the application of this clause.

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Guidance Documents, Program Administrative Manual, other attachments in Section D and all other Program materials.]

(8) The specification.

FAR 52.217-8 Option to Extend Services (NOV 1999)

The Government may require continued performance of any services within the limits and at the rates specified in the contract. These rates may be adjusted only as a result of revisions to prevailing labor rates provided by the Secretary of Labor. The option provision may be exercised more than once, but the total extension of performance hereunder shall not exceed 6 months. The Contracting Officer may exercise the option by written notice to the Contractor within 60 days of expiration of contract.

(End of clause)

FAR 52.217-9 Option to Extend the Term of the Contract (MAR 2000)

(a) The Government may extend the term of this contract by written notice to the Contractor within 15 days of expiration of term; provided that the Government gives the Contractor a preliminary written notice of its intent to extend at least 60 days before the contract expires. The preliminary notice does not commit the Government to an extension.

(b) If the Government exercises this option, the extended contract shall be considered to include this option clause.

(c) The total duration of this contract, including the exercise of any options under this clause, shall not exceed five years.

(End of clause)

52.222-42 Statement of Equivalent Rates for Federal Hires. (MAY 2014)

In compliance with the Service Contract Labor Standards statute and the regulations of the Secretary of Labor (29 CFR Part 4), this clause identifies the classes of service employees expected to be employed under the contract and states the wages and fringe benefits payable to each if they were employed by the contracting agency subject to the provisions of [5 U.S.C.5341](#) or 5332.

This Statement is for Information Only: It is not a Wage Determination

Employee Class	Monetary Wage-Fringe Benefits
01020 Administrative Assistant	\$35.34
01041 Customer Service Representative I	\$16.61
01042 Customer Service Representative II	\$18.13
01043 Customer Service Representative III	\$20.35

(End of Clause)

FAR 52.223-23 Sustainable Products and Services (MAY 2024) [(DEVIATION FEB 2025)]

(a) *Definitions.* As used in this clause—

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Biobased product means a product determined by the U.S. Department of Agriculture (USDA) to be a commercial product or industrial product (other than food or feed) that is composed, in whole or in significant part, of biological products, including renewable domestic agricultural materials and forestry materials, or that is an intermediate ingredient or feedstock. The term includes, with respect to forestry materials, forest products that meet biobased content requirements, notwithstanding the market share the product holds, the age of the product, or whether the market for the product is new or emerging. (7 U.S.C. 8101) (7 CFR part 4270.2).

Recovered material means waste materials and by-products recovered or diverted from solid waste, but the term does not include those materials and by-products generated from, and commonly reused within, an original manufacturing process. (42 U.S.C. 6903).

Sustainable products and services means:

(1) Products containing recovered material designated by the U.S. Environmental Protection Agency (EPA) under the Comprehensive Procurement Guidelines (42 U.S.C. 6962) (40 CFR part 247) (<https://www.epa.gov/smm/comprehensive-procurement-guideline-cpg-program#products>).

(2) Energy- and water-efficient products that are ENERGY STAR® certified or Federal Energy Management Program (FEMP)-designated products (42 U.S.C. 8259b) (10 CFR part 436, subpart C) (<https://www.energy.gov/eere/femp/search-energy-efficient-products> and <https://www.energystar.gov/products?s=mega>).

(3) Biobased products meeting the content requirement of the USDA under the BioPreferred® program (7 U.S.C. 8101) (7 CFR part 4270.2) (<https://www.biopreferred.gov>).

(4) Acceptable chemicals, products, and manufacturing processes listed under EPA's Significant New Alternatives Policy (SNAP) program, which ensures a safe and smooth transition away from substances that contribute to the depletion of stratospheric ozone (42 U.S.C. 76711) (40 CFR part 82, subpart G) (<https://www.epa.gov/snap>).

(b) Requirements.

(1) The sustainable products and services, including the purchasing program and type of product or service, that are applicable to this contract, and any products or services that are not subject to this clause, will be set forth in the statement of work or elsewhere in the contract.

(2) The Contractor shall ensure that the sustainable products and services required by this contract are—

(i) Delivered to the Government;

(ii) Furnished for use by the Government;

(iii) Incorporated into the construction of a public building or public work; and

(iv) Furnished for use in performing services under this contract, where the cost of the products is a direct cost to this contract (versus costs which are normally applied to the Contractor's general and administrative expenses or indirect costs). This includes services performed by contractors performing management and operation of Government-owned facilities to the same extent that, at the time of award, an agency would be required to comply if an agency operated or supported the facility.

(c) Resource. The Green Procurement Compilation (GPC) available at <https://sftool.gov/greenprocurement> provides a comprehensive list of sustainable products and services and sustainable acquisition guidance. The Contractor should review the GPC when determining which statutory purchasing programs apply to a specific product or service.

(End of clause)

HHSAR 352.204-71 Information and Information Systems Security (FEB 2024) (DEVIATION)

(a) *Definitions.* As used in this clause—

Breach means the loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where—

(1) A person other than an authorized user accesses or potentially accesses personally identifiable information, or

(2) An authorized user accesses personally identifiable information for an other than authorized purpose.

Business associate (see 45 CFR 160.103), except as provided in paragraph (2) of this definition, business associate means, with respect to a covered entity, a person who -

(1) On behalf of such covered entity or of an organized health care arrangement (as defined in this clause) in which the covered entity participates, but other than in the capacity of a member of the workforce of such covered entity or arrangement, creates, receives, maintains, or transmits protected health information for a function or activity regulated by this contract or agreement, including claims processing or administration, data analysis, processing or administration, utilization review, quality assurance, patient safety activities listed at 42 CFR 3.20, billing, benefit management, practice management, and repricing; or

(2) Provides, other than in the capacity of a member of the workforce of such covered entity, legal, actuarial, accounting, consulting, data aggregation (as defined in 45 CFR section 164.501), management, administrative, accreditation, or financial services to or for such covered entity, or to or for an organized health care arrangement in which the covered entity participates, where the provision of the service involves the disclosure of protected health information from such covered entity or arrangement, or from another business associate of such covered entity or arrangement, to the person.

(3) A covered entity may be a business associate of another covered entity.

(4) *Business associate* includes the following:

(i) A Health Information Organization, E-prescribing Gateway, or other person that provides data transmission services with respect to protected health information to a covered entity and that requires access on a routine basis to such protected health information.

(ii) A person that offers a personal health record to one or more individuals on behalf of a covered entity.

(iii) A subcontractor that creates, receives, maintains, or transmits protected health information on behalf of the business associate.

(5) *Business associate* does not include:

(i) A health care provider, with respect to disclosures by a covered entity to the health care provider concerning the treatment of the individual.

(ii) A plan sponsor, with respect to disclosures by a group health plan (or by a health insurance issuer or HMO with respect to a group health plan) to the plan sponsor, to the extent that the requirements of 45 CFR 164.504(f) apply and are met.

(iii) A government agency, with respect to determining eligibility for, or enrollment in, a government health plan that provides public benefits and is administered by another government agency, or collecting protected health information for such purposes, to the extent such activities are authorized by law.

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(iv) A covered entity participating in an organized health care arrangement that performs a function or activity as described by paragraph (1)(i) of this definition for or on behalf of such organized health care arrangement, or that provides a service as described in paragraph (1)(ii) of this definition to or for such organized health care arrangement by virtue of such activities or services.

Business associate agreement means the agreement, or other arrangement, as dictated by the HIPAA Privacy Rule (45 CFR 160), between an HHS covered entity and a business associate, which must be entered into in addition to the underlying contract for services and before any disclosure (see 45 CFR 160.103) of PHI can be made to the business associate, in order for the business associate to perform certain functions or activities on behalf of an HHS entity.

Controlled unclassified information (CUI) means information that laws, regulations, or Government-wide policies require to have safeguarding or dissemination controls, excluding classified information.

Healthcare component means a component or combination of components of a hybrid entity designated by the hybrid entity in accordance with 45 CFR 164.105(a)(2)(iii)(D) (see 45 CFR 164.103). The Secretary of HHS has designated HHS as a covered entity (further designated as a “hybrid entity”), and has also designated four HHS divisions as healthcare components under HIPAA, including —

(1) The Centers for Medicare and Medicaid Services (CMS), insofar as it operates the fee-for-service Medicare program;

(2) The Program Support Center (PSC), Division of Commissioned Personnel, insofar as it operates a health plan for Commissioned Corps officers;

(3) The World Trade Center (WTC) Health Program; and,

(4) The Indian Health Service (IHS), insofar as it operates a health plan and a program providing healthcare that uses electronic transactions.

HHS Information Technology General Rules of Behavior means a set of HHS rules that describes the responsibilities and expected behavior of users of HHS information or information systems.

HHS sensitive information means all HHS data, on any storage media or in any form or format, which requires confidentiality, integrity, and availability protection due to the risk of harm that could result to interests of HHS, other agencies or entities, or individuals from inadvertent or deliberate disclosure, alteration, or destruction of the information. The term includes—

(1) Information where the improper use or disclosure could adversely affect the ability of HHS to accomplish its mission, i.e., HHS proprietary information;

(2) Records about individuals requiring protection under laws and regulations such as the E-Government Act, Privacy Act and the HIPAA Privacy Rule, or based on a data use agreement or a promise or assurance of confidentiality; and

(3) Information that would be exempt from disclosure if requested under the Freedom of Information Act. Examples of HHS sensitive information include—

(i) Individually-identifiable medical, benefits, and personnel information;

(ii) Financial, budgetary, research, quality assurance, confidential commercial, critical infrastructure, security-sensitive, procurement-sensitive, investigatory, and law enforcement information;

(iii) Controlled unclassified information;

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(iv) Information that would be confidential and privileged in litigation such as information protected by the deliberative process privilege, attorney work-product privilege, and the attorney-client privilege; and

(v) Other information which, if released, could result in a violation of law or agreement, could cause harm or unfairness to any individual or group, or could adversely affect the national interest or the conduct of Federal programs.

HIPAA Rules means the Privacy, Security, Breach Notification, and Enforcement Rules at 45 CFR Part 160 and part 164.

Incident means an occurrence that (A) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information systems; or (B) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable policies.

Information system means a discrete set of information resources organized for the collection, processing, maintenance, use, sharing, dissemination, or disposition of information.

Information system security plan means a formal document that provides an overview of the security requirements for an information system or an information security program and describes the security controls in place or planned for meeting those requirements.

Information technology (see FAR 2.101) also means Information and Communication Technology (ICT).

Information technology-related contracts means those contracts that include services (including support services), and related resources for information technology.

Organized health care arrangement (see 45 CFR 160.103) means:

(1) A clinically integrated care setting in which individuals typically receive health care from more than one health care provider.

(2) An organized system of health care in which more than one covered entity participates and in which the participating covered entities:

(i) Hold themselves out to the public as participating in a joint arrangement; and

(ii) Participate in joint activities that include at least one of the following:

(A) Utilization review, in which health care decisions by participating covered entities are reviewed by other participating covered entities or by a third party on their behalf;

(B) Quality assessment and improvement activities, in which treatment provided by participating covered entities is assessed by other participating covered entities or by a third party on their behalf; or

(C) Payment activities, if the financial risk for delivering health care is shared, in part or in whole, by participating covered entities through the joint arrangement and if protected health information created or received by a covered entity is reviewed by other participating covered entities or by a third party on their behalf for the purpose of administering the sharing of financial risk.

(3) A group health plan and a health insurance issuer or HMO with respect to such group health plan, but only with respect to protected health information created or received by such health insurance issuer or HMO that relates to individuals who are or who have been participants or beneficiaries in such group health plan;

(4) A group health plan and one or more other group health plans each of which are maintained by the same plan sponsor; or

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(5) The group health plans described in paragraph (4) of this definition and health insurance issuers or HMOs with respect to such group health plans, but only with respect to protected health information created or received by such health insurance issuers or HMOs that relates to individuals who are or have been participants or beneficiaries in any of such group health plans.

Privacy officer means the HHS official(s) with responsibility for implementing and oversight of privacy related policies and practices that impact a given HHS acquisition.

(b) *General.* Contractors, subcontractors, their employees, third-parties, and business associates with access to HHS information, information systems, or information technology (IT) or providing and accessing IT-related goods and services, shall adhere to the HHS Cybersecurity Program and the directives and handbooks, complete HHS security training prior to accessing HHS information (including HHS sensitive information and information systems security and privacy) and on an annual basis thereafter, as well as those set forth in the contract specifications, statement of work, or performance work statement. These include, but are not limited to, HHS *Personnel Security and Suitability Program*, which establishes HHS procedures, responsibilities, and processes for complying with current Federal law, Executive Orders, policies, regulations, standards, and guidance for protecting HHS information, information systems (see 302.101, Definitions) security and privacy, and adhering to personnel security requirements when accessing HHS information or information systems.

(c) *Access to HHS information and HHS information systems.*

(1) Contractors are limited in their request for logical or physical access to HHS information or HHS information systems for their employees, subcontractors, third parties and business associates to the extent necessary to perform the services or provide the goods as specified in the contracts, agreements, task, delivery, or purchase orders.

(2) All Contractors, subcontractors, third parties, and business associates working with HHS information are subject to the same investigative requirements as those of HHS appointees or employees who have access to the same types of information. The level and process of background security investigations for Contractors to access HHS information and HHS information systems shall be in accordance with HHS *Personnel Security and Suitability Program*.

(3) Contractors, subcontractors, third parties, and business associates who require access to national security programs must have a valid security clearance.

(4) The Contractor (and/or any subcontractor) must comply with Executive Order 13556, *Controlled Unclassified Information*, (implemented at 3 CFR, part 2002) when handling CUI. 32 C.F.R. 2002.4(aa) As implemented the term "handling" refers to "...any use of CUI, including but not limited to marking, safeguarding, transporting, disseminating, re-using, and disposing of the information." 81 Fed. Reg. 63323. The requirements below apply only to nonfederal systems that process, store, or transmit CUI, or that provide security protection for such components. All sensitive information that has been identified as CUI by a regulation or statute, handled by this solicitation/contract, must be –

(i) Marked appropriately;

(ii) Disclosed to authorized personnel on a need-to-know basis;

(iii) Protected in accordance with NIST SP 800-53, *Security and Privacy Controls for Information Systems and Organizations* applicable baseline if handled by a Contractor system operated on behalf of the agency, or NIST SP 800-171, *Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations* if handled by internal Contractor system; and

(iv) Returned to HHS control, destroyed when no longer needed, or held until otherwise directed. Information and/or data must be disposed of in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*.

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(5) *HIPAA business associate agreements.* Under the HIPAA Privacy and Security Rules (see 45 CFR 164), pursuant to 45 CFR 164.502(e)(1), a covered entity may disclose protected health information to a business associate and may allow a business associate to create, receive, maintain, or transmit protected health information on its behalf, if the covered entity obtains satisfactory assurance that the business associate will appropriately safeguard the information. A covered entity is not required to obtain such satisfactory assurances from a business associate that is a subcontractor of a covered entity's business associate. Additionally, a business associate may disclose protected health information to a business associate that is a subcontractor and may allow the subcontractor to create, receive, maintain, or transmit protected health information on its behalf, if the business associate obtains satisfactory assurances, in accordance with 45 CFR 164.504(e)(1)(i), that the subcontractor will appropriately safeguard the information. The satisfactory assurances required by 45 CFR 164.504(e)(1) of this section shall be documented through a written contract or other written agreement or arrangement with the business associate that meets the applicable requirements of 45 CFR 164.504(e). The contracts shall also include breach reporting policies and procedures for suspected or confirmed breaches of protected health information. The contract shall impose a duty to cooperate with the healthcare component and/or HHS breach investigation and response and must require all subcontractors to comply with the same HIPAA Rules requirements as a condition of receiving government data.

(i) *Contractors or entities required to execute business associate agreements for contracts and other agreements become HHS business associates.* Business associate agreements are issued by HHS or may be issued by other HHS programs in support of HHS. The HIPAA Privacy Rule requires HHS to execute compliant business associate agreements with persons or entities that create, receive, maintain, or transmit HHS PHI or that will store, generate, access, exchange, process, or utilize such PHI in order to perform certain activities, functions or services to, for, or on behalf of HHS. There may be other HHS components or staff offices which also provide certain services and support to HHS and must receive PHI in order to do so. If these components award contracts or enter into other agreements, purchase/delivery orders, modifications and issue governmentwide purchase card transactions to help in the delivery of these services to HHS, they will also fall within the requirement to obtain a satisfactory assurance from these contractors by executing a business associate agreements.

(ii) *Business associate agreement flow down to subcontractors.* A prime contractor required to execute a business associate agreement shall also obtain a satisfactory assurance, in the form of a business associate agreement, of its subcontractors who will also create, receive, maintain, or transmit PHI or that will store, generate, access, exchange, process, or utilize such PHI will comply with HIPAA Rules requirements to the same degree as the Contractor. A contractor employing a subcontractor who creates, receives, maintains, or transmits PHI or that will store, generate, access, exchange, process, or utilize such PHI under a contract or agreement is required to execute a business associate agreement with each of its subcontractors which also obligates the subcontractor (i.e., also a business associate) to provide the same protections and safeguards and agree to the same disclosure restrictions to PHI that is required of the covered entity and the prime contractor.

(d) *Contractor operations required to be in United States.* Custom software development and outsourced operations must be located in the U.S. to the maximum extent practicable. If such services are proposed to be performed outside the continental United States, and are not otherwise disallowed by other Federal law, regulations or policy, or other HHS policy or other mandates as stated in the contract, specifications, statement of work or performance work statement (including applicable business associate agreements), the Contractor/subcontractor must state in its proposal where all non-U.S. services are provided. At a minimum, the Contractor/subcontractor must include a detailed Information System Security Plan, for review and approval by the Contracting Officer, specifically to address mitigation of the resulting problems of communication, control, and data protection.

(e) *Roster of employees.* Contractors and subcontractors shall provide a roster containing the name, position, e-mail address, phone number, and responsibilities of each employee, including subcontractors, performing work under the contract to develop, have the ability to access, or host and/or maintain a government information system(s). The roster must be submitted to Contracting Officer by the effective date of the contract. Revisions to the roster as a result of staffing changes must be submitted within the number of days of the change provided by the Contracting Officer. The Contracting Officer, or the Contracting Officer's Representative (COR), will notify the Contractor of the appropriate level of investigation required for each staff member based on the information provided on the roster. If an employee is filling a new position, the Contractor must provide a position description and the Government will determine the appropriate suitability level.

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(f) *Contractor/subcontractor employee reassignment and termination notification.* Contractors and subcontractors shall provide written notification to the Contracting Officer and COR immediately, and not later than four (4) hours, when an employee working on an HHS information system or with access to HHS information is reassigned or leaves the Contractor or subcontractor's employment on the cognizant HHS contract. The Contracting Officer and COR must also be notified immediately by the Contractor or subcontractor prior to an unfriendly termination.

(g) *Non-disclosure agreement.* The Contractor and subcontractors shall submit completed non-disclosure agreements, as provided by the Contracting Officer, for each employee having access to non-public government information under this contract. The non-disclosure agreements shall be submitted to the Contracting Officer prior to the performance of work.

(h) *HHS information custodial requirements.*

(1) *Release, publication, and use of data.* Information made available to a Contractor or subcontractor by HHS for the performance or administration of a contract or information developed by the Contractor/subcontractor in performance or administration of a contract shall be used only for the stated contract purpose and shall not be used in any other way without HHS prior written approval. This clause expressly limits the Contractor's/subcontractor's rights to use data as described in 52.227-14, Rights in Data—General, paragraph (d).

(2) *Media sanitization.* HHS information shall not be co-mingled with any other data on the Contractors/subcontractor's information systems or media storage systems in order to ensure federal and HHS requirements related to data protection, information segregation, classification requirements, and media sanitization can be met (see HHS Cybersecurity Program). HHS reserves the right to conduct scheduled or unscheduled on-site inspections, assessments, or audits of Contractor and subcontractor IT resources, information systems and assets to ensure data security and privacy controls, separation of data and job duties, and destruction/media sanitization procedures are in compliance with Federal and HHS requirements. The Contractor and subcontractor will provide all necessary access and support to HHS and/or GAO staff during periodic control assessments or audits.

(3) *Data retention, destruction and contractor self-certification.* The Contractor and its subcontractors are responsible for collecting and destroying any HHS data provided, created, or stored under the terms of this contract, to a point where HHS data or materials are no longer readable or reconstructable to any degree, in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*, or subsequent directive. Prior to termination or completion of this contract, the Contractor/subcontractor must provide its plan for destruction or return of all HHS data in its possession accordance with contract requirements or Contracting Officer instructions for disposition, including compliance with National Institute of Standards and Technology (NIST) SP 800-88, *Guidelines for Media Sanitization*, for the purposes of media sanitization on all IT equipment. The Contractor must certify in writing to the Contracting Officer within 30 days of termination of the contract that the data destruction requirements in this paragraph have been met.

(4) *Return of HHS data and information.* When information, data, documentary material, records and/or equipment is no longer required, it shall be returned to the HHS (as stipulated by the Contracting Officer or the COR) or the Contractor/subcontractor must hold it until otherwise directed. Items returned will be hand carried, securely mailed, emailed, or securely electronically transmitted to the Contracting Officer or to the address as provided in the contract or by the assigned COR, and/or accompanying business associate agreement. Depending on the method of return, Contractor/subcontractor must store, transport, or transmit HHS sensitive information, when permitted by the contract using HHS-approved encryption tools that are, at a minimum, validated under Federal Information Processing Standards (FIPS) 140-3 (or its successor). If mailed, Contractor/subcontractor must send via a trackable method (USPS, UPS, Federal Express, etc.) and immediately provide the Contracting Officer with the tracking information. No information, data, documentary material, records or equipment will be destroyed unless done in accordance with the terms of this contract and the HHS Agency Records Control Schedules (2019).

(5) *Use of HHS data and information.* The Contractor/subcontractor must receive, gather, store, back up, maintain, use, disclose and dispose of HHS information only in compliance with the terms of the contract and applicable Federal and HHS information confidentiality and security laws, regulations, and policies. If Federal or HHS information confidentiality and security laws, regulations, and policies become applicable to the HHS

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information or information systems after execution of the contract, or if the NIST issues or updates applicable FIPS or Special Publications (SP) after execution of this contract, the parties agree to negotiate in good faith to implement the information confidentiality and security laws, regulations and policies for this contract as a result of any updates, if required.

(6) *Copying HHS data or information.* The Contractor/subcontractor shall not make copies of HHS information except as authorized and necessary to perform the terms of the contract or to preserve electronic information stored on Contractor/subcontractor electronic storage media for restoration in case any electronic equipment or data used by the Contractor/subcontractor needs to be restored to an operating state. If copies are made for restoration purposes, after the restoration is complete, the copies must be appropriately destroyed.

(7) *Violation of information custodial requirements.* If HHS determines that the Contractor has violated any of HHS information confidentiality, privacy, or security provisions, it shall be sufficient grounds for HHS to withhold payment to the Contractor or third-party or terminate the contract for default in accordance with FAR part 49 or terminate for cause in accordance with FAR 12.403.

(8) *Encryption.* The Contractor/subcontractor must store, transport, or transmit HHS sensitive information, when permitted by the contract, using cryptography, HHS encryption policies, and HHS-approved encryption tools that are, at a minimum, validated under FIPS 140-3 (or its successor).

(9) *Firewall and web services security controls.* The Contractor/subcontractor's firewall and Web services security controls, if applicable, shall meet or exceed HHS minimum requirements. HHS Configuration Standards Guidelines are available upon request.

(10) *Disclosure of HHS data and information.* Except for uses and disclosures of HHS information authorized in a cognizant contract for performance of the contract, the Contractor/subcontractor may use and disclose HHS information only in two other situations: (i) subject to paragraph 10 of this section, in response to a court order from a court of competent jurisdiction, or (ii) with HHS prior written approval. The Contractor/subcontractor must refer all requests for, demands for production of, or inquiries about, HHS information and information systems to the Contracting Officer for response. If the Contractor/subcontractor is in receipt of a court order or other request or believes it has a legal requirement to disclose HHS information, that Contractor/subcontractor shall immediately refer such court order or other request to the Contracting Officer for response. If the Contractor or subcontractor discloses information on behalf of HHS, the Contractor and/or subcontractor must maintain an accounting of disclosures. Accounting of Disclosures documentation maintained by the Contractor/subcontractor will include the name of the individual to whom the information pertains, the date of each disclosure, the nature or description of the information disclosed, a brief statement of the purpose of each disclosure or, in lieu of such statement, a copy of a written request for a disclosure, and the name and address of the person or agency to whom the disclosure was made. The Contractor/subcontractor will provide its Accounting of Disclosures upon request and within 15 calendar days to the assigned COR and Privacy Officer. Accounting of disclosures should be provided electronically via encrypted email to the COR and designated HHS facility Privacy Officer as provided in the contract, business associate agreement, or by the Contracting Officer. If providing the Accounting of disclosures electronically cannot be done securely, the Contractor/subcontractor will provide copies via trackable methods (UPS, USPS, Federal Express, etc.) immediately, providing the designated COR and Privacy Officer with the tracking information.

(11) *Compliance with privacy statutes and applicable regulations.* The Contractor/subcontractor shall not disclose HHS information protected by any of HHS privacy statutes or applicable regulations including, but not limited to, the Privacy Act of 1974 or the HIPAA Rules. If the Contractor/subcontractor is in receipt of a court order or other requests for HHS information or has questions if it can disclose information protected under the above-mentioned confidentiality statutes because it is required by law, that Contractor/subcontractor shall immediately refer such court order or other request to the Contracting Officer for response.

(i) *Compliance with identification policies.* Contractors shall comply with the Homeland Security Presidential Directive (HSPD)-12, Policy for a Common Identification Standard for Federal Employees and Contractors; OMB M-05-24; OMB M-19-17; FIPS 201, Personal Identity Verification (PIV) of Federal Employees and Contractors;

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HHS HSPD-12 policy; HHS Policy for Information Security and Privacy Protection (IS2P) Control Catalog, and Executive Order 13467, Part 1, section 1.2.

(j) *Report of known or suspected incident or breach.* The Contractor, subcontractor, third-party affiliate or business associate, and its employees shall notify HHS immediately via the Contracting Officer and the COR or within one (1) hour of a known or suspected incident or breach. The initial notification may first be made verbally but must be followed up in writing within one (1) hour. Report all actual or suspected incident and breach information to the Contracting Officer and the COR as identified in the contract or as directed in the contract, within one hour of discovery or suspicion.

(1) Such issues shall be remediated as quickly as is practical, but within the timeframes based on the CDC Cybersecurity Weakness Management Standards that will be provide after award. The Contractor shall notify the Contracting Officer in writing.

(2) When the security fixes involve installing third party patched (e.g., Microsoft OS patches or Adobe Acrobat), the Contractor will provide written notice to HHS that the patch has been validated as not affecting the systems within 10 working days. When the Contractor is responsible for operations or maintenance of the systems, they shall apply the security fixes within the timeframes based on the CDC Cybersecurity Weakness Management Standards that will be provide after award.

(3) All other vulnerabilities shall be remediated in a timely manner based on risk, in accordance with the timelines specified in the HHS Policy for Vulnerability Management, and the HHS Standard for Plan of Action and Milestones (POAM) Management and Reporting. Contractors shall notify the Contracting Officer, and COR within 2 business days after remediation of the identified vulnerability. Exceptions to this paragraph (e.g., for the convenience of HHS) must be requested by the Contractor through the COR and shall only be granted with approval of the Contracting Officer and the Office of the Chief Information Officer (OCIO). These exceptions will be tracked by the Contractor in concert with the Government in accordance with HHS Policy for IT Procurements–Security and Privacy Language.

(k) *Incident and breach investigation.*

(1) The Contractor/ subcontractor shall immediately notify the Contracting Officer and COR for the contract of any known or suspected incident or breach (see definitions, paragraph (a)), or any other unauthorized disclosure of sensitive information, including that contained in system(s) to which the Contractor/subcontractor has access.

(2) To the extent known by the Contractor/subcontractor, the Contractor/ subcontractor's notice to HHS shall identify the information involved, an estimate of the number of potentially impacted individuals, the circumstances surrounding the incident (including to whom, how, when, and where the HHS information or assets were placed at risk or compromised), and any other information that the Contractor/subcontractor considers relevant.

(3) With respect to unsecured protected health information, the business associate is deemed to have discovered an incident as defined above when the business associate either knew, or by exercising reasonable diligence should have been known to an employee of the business associate. Upon discovery, the business associate must notify HHS of the incident immediately within one hour of discovery or suspicion as agreed to in the business associate agreement.

(4) In instances of theft or break-in or other criminal activity, the Contractor/subcontractor must concurrently report the incident to the appropriate law enforcement entity (or entities) of jurisdiction. The Contractor, its employees, and its subcontractors and their employees shall cooperate with HHS and any law enforcement authority responsible for the investigation and prosecution of any possible criminal law violation(s) associated with any incident. The Contractor/subcontractor shall cooperate with HHS in any civil litigation to recover HHS information, obtain monetary or other compensation from a third party for damages arising from any incident, or obtain injunctive relief against any third party arising from, or related to, the incident.

(l) *Incident and breach notification requirements.*

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(1) The Contractor/subcontractor shall provide notice to HHS of an incident as set forth in the incident and breach investigation section of this clause. The Contractor shall fully cooperate with HHS or third- party entity performing an independent risk analysis on behalf of HHS. Failure to cooperate may be deemed a material incident or breach and grounds for contract termination.

(2) The Contractor/subcontractor shall fully cooperate with the HHS Computer Security Incident Response Center (CSIRC), HHS Breach Response Team, Operating Divisions (OPDIVs), Staff Divisions (STAFFDIVs), other stakeholders or any Government agency conducting an analysis regarding any notice of an incident or breach, potential incident or breach, or incident which may require the Contractor to provide information to the Government or third-party performing a risk analysis for HHS, and shall address all relevant information concerning the incident or breach, including the following:

- (i) Nature of the event (loss, theft, unauthorized access).
- (ii) Description of the event, including:
 - (A) Date of occurrence.
 - (B) Date of incident or breach detection.
 - (C) Data elements involved, including any PII, such as full name, social security number, date of birth, home address, account number, disability code.
 - (D) Number of individuals affected or potentially affected.
 - (E) Names of individuals or groups affected or potentially affected.
 - (F) Ease of logical data access to the lost, stolen or improperly accessed data in light of the degree of protection for the data, e.g., unencrypted, plain text.
 - (G) Amount of time the data has been out of HHS control.
 - (H) The likelihood that the sensitive information will or has been compromised (made accessible to and usable by unauthorized persons).
 - (I) Known misuses of data containing sensitive information, if any.
 - (J) Assessment of the potential harm to the affected individuals.
 - (K) Incident or breach analysis as outlined in the HHS Breach Response Policy and Plan, as appropriate.
 - (L) Whether credit protection services may assist record subjects in avoiding or mitigating the results of identity theft based on the sensitive information that may have been compromised.
 - (M) Steps taken in response to mitigate or prevent a repetition of the incident.

(m) *Training.*

(1) All Contractor employees and subcontractor employees requiring access to HHS information or HHS information systems shall complete the following before being granted access to HHS information and its systems:

- (i) On an annual basis, successfully complete the HHS Privacy and Information Security Awareness and HHS Information Security Rules of Behavior training.

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(ii) On an annual basis, sign and acknowledge (either manually or electronically) understanding of and responsibilities for compliance with the HHS Information Security Rules of Behavior, relating to access to HHS information and information systems.

(iii) Successfully complete any additional cyber security or privacy training, as required for HHS personnel with equivalent information system access.

(2) The Contractor shall provide to the Contracting Officer and/or the COR a copy of the training certificates and affirmation that HHS Information Security Rules of Behavior signed by each applicable employee have been completed and submitted within five (5) days of the initiation of the contract and annually thereafter, as required.

(3) Failure to complete the mandatory annual training and acknowledgement of the HHS Information Security Rules of Behavior, within the timeframe required, is grounds for suspension or termination of all physical or electronic access privileges and removal from work on the contract until such time as the training and documents are complete.

(n) *Subcontract flow down.* The Contractor shall include the substance of this clause, including this paragraph (k), in subcontracts, third-party agreements, and business associate agreements, of any amount and in which subcontractor employees, third-party servicers/employees, and business associates will perform functions where they will have access to HHS information (including HHS sensitive information), information systems, information technology (IT) or providing and accessing information technology-related contract services, support services, and related resources (see HHSAR 302.101 definition of information technology-related contracts.)

(End of clause)

HHSAR 352.204-72 Records management (FEB 2024) (DEVIATION)

(a) *Applicability.* This clause applies to contracts that include Federal records, as defined in paragraph (b).

(b) *Definition.* As used in this clause—

Federal record means all recorded information, regardless of form or characteristics, made or received by a Federal agency under Federal law or in connection with the transaction of public business and preserved or appropriate for preservation by that agency or its legitimate successor as evidence of the organization, functions, policies, decisions, procedures, operations, or other activities of the United States Government or because of the informational value of data in them. See 44 U.S.C. 3301.

(1) The term Federal record—

(i) Includes HHS records;

(ii) Does not include personal materials;

(iii) Applies to records created, received, or maintained by Contractors pursuant to their contract; and

(iv) May include deliverables and documentation associated with deliverables.

(2) *Recorded information* means all traditional forms of records, regardless of physical form or characteristics, including information created, manipulated, communicated, or stored in digital or electronic form. (See 44 U.S.C. 3301.)

(3) *Personal materials* means documentary materials belonging to an individual that are not used to conduct agency business. Personal files are excluded from the definition of Federal records and are not owned by the Government. (See 36 CFR 1220.18.)

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(c) Requirements.

(1) The Contractor shall comply with all applicable records management laws and regulations, as well as National Archives and Records Administration (NARA) records policies, including but not limited to the Federal Records Act (44 U.S.C. chapters 21, 29, 31, 33), NARA regulations at 36 CFR chapter XII subchapter B, and those policies associated with the safeguarding of records covered by the Privacy Act of 1974 (5 U.S.C. 552a). These policies include the preservation of all Federal records, regardless of form or characteristics, mode of transmission, or state of completion.

(2) In accordance with 36 CFR 1222.32, all data created for Government use and delivered to, or falling under the legal control of, the Government are Federal records subject to the provisions of 44 U.S.C. chapters 21, 29, 31, and 33, the Freedom of Information Act (FOIA) (5 U.S.C. 552), and the Privacy Act of 1974 (5 U.S.C. 552a), and must be managed and scheduled for disposition only as permitted by statute or regulation.

(3) In accordance with 36 CFR 1222.32, the Contractor shall maintain all Federal records created for Government use or created in the course of performing the contract and/or delivered to, or under the legal control of the Government and must be managed in accordance with Federal law. Electronic records and associated metadata must be accompanied by sufficient technical documentation to permit understanding and use of the records and data.

(4) The Contractor is responsible for preventing the alienation or unauthorized destruction of Federal records, including all forms of mutilation. Federal records may not be removed from the legal custody of HHS or destroyed except for in accordance with the provisions of the agency records schedules and with the written concurrence of the Contracting Officer. Willful and unlawful destruction, damage or alienation of Federal records is subject to the fines and penalties imposed by 18 U.S.C. 2701. The Contractor shall report to the Contracting Officer any unlawful or accidental removal, defacing, alteration, or destruction of Federal records.

(5) The Contractor shall immediately notify the Contracting Officer upon discovery of any inadvertent or unauthorized disclosures of information, data, documentary materials, records or equipment. Disclosure of non-public information is limited to authorized personnel with a need-to-know as described in the contract. The Contractor shall ensure that appropriate personnel are trained to adhere to these contract requirements, and that applicable, administrative, technical, and physical safeguards are established to ensure the security and confidentiality of information, data, documentary material, Federal records and/or equipment is properly protected. The Contractor shall not remove Federal Records from Government facilities or systems, or facilities or systems operated or maintained on the Government's behalf, without the express written permission of the Contracting Officer. When information, data, documentary material, Federal records and/or equipment are no longer required, it shall be returned to HHS control or the Contractor must hold it until otherwise directed. Items returned to the Government shall be hand carried, mailed, emailed, or securely electronically transmitted to the Contracting Officer or as otherwise directed by the Contracting Officer. Destruction of Federal records is expressly prohibited unless in accordance with paragraph (c)(4).

(6) The Contractor shall only use Government information technology equipment for purposes specifically authorized by the contract and in accordance with HHS policy.

(7) The Contractor shall not create or maintain any Federal records containing any non-public HHS information that are not specifically authorized by the contract.

(8) The Contractor shall not retain, use, sell, or disseminate copies of any deliverable that contains information covered by the Privacy Act of 1974 or that which is generally protected from public disclosure by an exemption to the Freedom of Information Act.

(9) All Contractor employees assigned to this contract handle Federal records are required to take HHS-provided records management training. The Contractor is responsible for confirming training has been completed according to agency policies, including initial training and any annual or refresher training.

(d) *Subcontract flowdown.* The Contractor shall incorporate the substance of this clause, its terms and requirements including this paragraph, in all subcontracts under this contract.

HHSAR 352.224-70 Notification of System of Records Notice (FEB 2024) (DEVIATION)

(a) This contract provides for the design, development, or operation of a system of records about individuals from which information about an individual is retrieved by the individual's name or by some other identifying particular assigned to the individual.

(b) The System of Records Notice(s) (SORN(s)) that is applicable to this contract is/are: *09-20-0147, or any successor SORNs of which the NPA Contractor will be notified.*

(c) The System of Records design, development, or operation work the Contractor is to perform is: *See Section 3: "Scope of Work."*

(d) The disposition to be made of the Privacy Act records upon completion of contract performance is as follows: *Records created under contract shall be returned to CDC/NIOSH at the conclusion or termination of this Contract, whichever occurs first. In the event the NPA Contractor is required to maintain a copy of any records created under this Contract, the records shall be destroyed by the NPA Contractor within 7 years upon completion of Contract performance. This timeframe provides support for after-the-fact investigations of security incidents and to meet regulatory and CDC/NIOSH information retention requirements.*

(e) *Subcontract flow down.* The Contractor is required to include this clause in all subcontracts at any tier performing work under the prime contract involving design, development, or operation work involving a System of Records.

(End of clause)

HHSAR 352.224-71 Confidential Information (FEB 2024) (DEVIATION)

(a) Definition. As used in this clause –

Confidential information means information or data of a personal nature about an individual, or proprietary information or data submitted by or pertaining to an institution or organization.

(b) Identification of information. Specific confidential information or categories of information that the Government will furnish to the Contractor, or that the Contractor is expected to generate, is identified in this contract.

(c) Disclosure. The Contractor shall not disclose confidential information or records until written notice is provided to the Contracting Officer at least 45 days in advance of the Contractor's intent to release findings of studies or research, to which an agency response may be appropriate to protect the public interest or that of the agency. The Contractor shall not disseminate or publish such information without the written consent of the Contracting Officer.

(d) Government furnished or provided information. For information provided by or on behalf of the government,

(1) The publication or dissemination of the following types of information are restricted under this contract: Included but not limited to WTC Health Program data and personally identifiable information (PII).

(2) The reason(s) for restricting the types of information identified in subparagraph (d)(1) is/are: Requirements under the Privacy Act, Health Insurance Portability and Accountability Act (HIPAA), and other laws.

(e) The Contractor shall consult with the Contracting Officer when there is uncertainty with regard to the confidentiality of, or a property interest in, information under this contract prior to the release, disclosure, dissemination, or publication of such information.

HHSAR 352.237-75 Key Personnel (DEC 2015).

The key personnel specified in this contract are considered to be essential to work performance. At least 30 days prior to the contractor voluntarily diverting any of the specified individuals to other programs or contracts the Contractor shall notify the Contracting Officer and shall submit a justification for the diversion or replacement and a request to replace the individual. The request must identify the proposed replacement and provide an explanation of how the replacement's skills, experience, and credentials meet or exceed the requirements of the contract (including, when applicable, Human Subjects Testing requirements). If the employee of the contractor is terminated for cause or separates from the contractor voluntarily with less than thirty days notice, the Contractor shall provide the maximum notice practicable under the circumstances. The Contractor shall not divert, replace, or announce any such change to key personnel without the written consent of the Contracting Officer. The contract will be modified to add or delete key personnel as necessary to reflect the agreement of the parties.

(End of clause)

HHSAR 352.239-71 Security Requirements for Information Technology Resources (FEB 2024) (DEVIATION)

(a) *Definitions.* As used in this clause—

Information technology has the same meaning in FAR 2.101.

Information and communication technology (ICT) also means information technology (see FAR 2.101 for definitions).

Information system security plan means a formal document that provides an overview of the security requirements for an information system or an information security program and describes the security controls in place or planned for meeting those requirements.

(b) *Responsibilities.* The Contractor shall be responsible for information technology security for all systems connected to a Department of Health and Human Services (HHS) network or operated by the Contractor for HHS, regardless of location. This clause is applicable to all or any part of the contract that includes information technology resources or services in which the Contractor has physical or other system access to HHS information that directly supports the mission of HHS. Examples of tasks that require security provisions include—

- (1) Hosting of HHS e-Government sites or other information technology operations;
- (2) Acquisition, transmission, or analysis of data owned by HHS with significant replacement cost should the contractor's copy be corrupted; and
- (3) Access to HHS general support systems/major applications at a level beyond that granted the general public, e.g., bypassing a firewall.

(c) *Information system security plan.* The Contractor shall develop, provide, implement, and maintain an Information System Security Plan. HHS information system and platform information technology systems must have a security plan that provides an overview of the security requirements for the system and describes the security controls in place or the plan for meeting those requirements. Generally, this plan shall describe the processes and procedures that the Contractor will follow to ensure appropriate security of information system resources developed, processed, or used under this contract. The security plan should include implementation status, responsible entities, resources, and estimated completion dates. Security plans may also include, but are not limited to, a compiled list of system characteristics or qualities required for system registration, and key security-related documents such as a risk assessment, privacy impact assessment (PIA), system interconnection agreements, contingency plan, security configurations, configuration management plan, incident response plan, and the system privacy plan, as determined by the Contracting Officer's Representative. The plan shall address the specific contract requirements regarding information systems or related support or services included in the contract, to include the PWS or SOW. The Contractor's Information System Security Plan shall comply with applicable Federal Laws that include, but are not

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limited to, 40 U.S.C. 11331, the Federal Information Security Modernization Act (FISMA) of 2014 and the E-Government Act of 2002. The plan shall meet information technology security requirements in accordance with Federal and HHS policies and procedures, and as amended during the term of this contract, and include, but are not limited to the following:

(1) OMB Circular A-130, Managing Information as a Strategic Resource;

(2) National Institute of Standards and Technology (NIST) Guidelines;

(3) Federal Information Processing Standard (FIPS) 200; and

(4) HHS Cybersecurity Program related to HHS information (including HHS sensitive information and information systems security and privacy), as well as those set forth in the contract specifications, statement of work, or performance work statement. These include, but are not limited to, the Cyber Security Checklist and Cyber Security Infographic at <https://www.hhs.gov/hipaa/for-professionals/security/guidance/cybersecurity/index.html>, which provides HHS procedures, responsibilities, and processes for complying with current Federal law, Executive Orders, policies, regulations, standards and guidance for protecting HHS information, information systems (see 302.101, Definitions), information technology, and ICT, security and privacy, and adhering to personnel security requirements when accessing HHS information or information systems.

(d) *Submittal of plan.* Within 60 days after contract award, the Contractor shall submit the Information System Security Plan to the Contracting Officer for review and approval.

(e) *Authority to Operate (ATO).* As required by current HHS policy, the Contractor shall submit written proof of information technology security accreditation with a valid ATO to the Contracting Officer. Such written proof may be furnished either by the Contractor or by a third party. Accreditation shall be in accordance with HHS policy available from the Contracting Officer upon request. The Contractor shall submit for acceptance by the Contracting Officer along with this ATO a final security plan, risk assessment, security test and evaluation, privacy threshold analysis or privacy impact assessment, and a disaster recovery plan/continuity of operations plan.

(f) *Annual validation.* On an annual basis, the Contractor shall verify in writing to the Contracting Officer that the IT Security Plan remains valid.

(g) *Banners.* The Contractor shall ensure that the official HHS banners are displayed on all HHS systems (both public and private) operated by the Contractor that contain Privacy Act or other sensitive information before allowing anyone access to the system. The Office of Information Technology will make official HHS banners available to the Contractor.

(h) *Screening and access.* The Contractor shall screen all personnel requiring privileged access or limited privileged access to systems operated by the Contractor for HHS or interconnected to an HHS network in accordance with HHS policies referenced in paragraph (c).

(i) *Training.* The Contractor shall ensure that its employees performing services under this contract complete HHS security awareness and privacy training on an annual basis. This includes signing an acknowledgment on an annual basis that they have read, understand, and agree to abide by the HHS Rules of Behavior for the Use of HHS Information and IT Resources Policy as required; FAR 39.105, Privacy; clause 352.204-71, Information and Information Systems Security, and this clause.

(j) *Government access.* The Contractor shall provide the Government access to the Contractor's and subcontractors' facilities, installations, operations, documentation, information systems, databases, and personnel used in performance of the contract. The Contractor shall provide access to enable a program of information technology inspection (to include vulnerability testing), investigation and audit (to safeguard against threats and hazards to the integrity, availability and confidentiality of HHS data or to the function of information technology systems operated on behalf of HHS), and to preserve evidence of computer crime.

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(k) *Notification of termination of employees.* The Contractor shall immediately notify the Contracting Officer when an employee who has access to HHS information systems or data terminates employment.

(l) *Subcontract flow down requirement.* The Contractor shall incorporate and flow down the substance of this clause to all subcontracts that meet the conditions in paragraph (a) of this clause.

(End of clause)

HHSAR 352.239-76 Security Requirements for Government-Owned Contractor-Operated and Contractor-Owned Contractor-Operated Resources (FEB 2024) (DEVIATION)

(a) *Federal policies.* The Contractor shall comply with applicable federal laws, regulations, and HHS policies that include, but are not limited to—

- (1) HHS Policy for Information Security and Privacy Protection (IS2P);
- (2) Federal Information Security Modernization Act (FISMA) of 2014, (44 U.S.C. 101);
- (3) National Institute of Standards and Technology (NIST) Special Publication (SP) 800-53, latest revision, Security and Privacy Controls for Information Systems and Organizations;
- (4) Office of Management and Budget (OMB) Circular A-130, Managing Information as a Strategic Resource; and
- (5) Any other applicable federal laws, regulations, NIST guidance, and local HHS policies.

(b) *Assessment and Authorization (A&A).* A valid authority to operate (ATO) certifies that the Contractor's information system meets the contract's requirements to protect the agency data. If the system under this contract does not have a valid ATO, the Contractor shall work with the agency and supply the deliverables required to complete the ATO within the specified timeline(s) of Section E PWS. The Contractor must conduct the A&A requirements in accordance with *HHS IS2P*, NIST SP 800-37, *Guide for Applying the Risk Management Framework to Information Systems: A Security Life Cycle Approach* (latest revision), NIST SP 800-53B, *Control Baselines for Information Systems and Organizations*, and the NIST SP 800-53A (latest revision). HHS acceptance of the ATO does not alleviate the Contractor's responsibility to ensure the system security and privacy controls are implemented and operating effectively.

(1) A&A package deliverables. The Contractor shall provide an A&A package within [to be determined] to the Contracting Officer and/or the Contracting Officer's Representative (COR). The following A&A deliverables are required to complete the A&A package—

(i) [to be determined];

(ii) A System Security Plan (SSP) is due with contractor proposal. The SSP shall comply with the NIST SP 800-18, *Guide for Developing Security Plans for Federal Information Systems*, the Federal Information Processing Standard (FIPS) 200, *Recommended Security Controls for Information Systems*, and NIST SP 800-53, *Security and Privacy Controls for Federal Information Systems and Organizations* applicable baseline requirements, and other applicable NIST guidance as well as HHS policies and other guidance. The SSP must be consistent with and detail the approach to IT security contained in the Contractor's bid or proposal that resulted in the award of this contract. The SSP must provide an overview of the system environment and security requirements to protect the information system (see HHSAR 302.101, Definitions) as well as describe all applicable security controls in place or planned for meeting those requirements. It should provide a structured process for planning adequate, cost-effective security protection for a system. The Contractor shall review and update the SSP at least annually thereafter and if requested, provide a copy of the updated SSP to the COR.

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(iii) A Security Assessment Plan/Report (SAP/SAR) is due *[to be determined]*. The security assessment must be conducted by *[to be determined]* assessor and be consistent with NIST SP 800-53A, NIST SP 800-30, and HHS policies. The assessor will document the assessment results in the SAR. Thereafter, the Contractor, in coordination with the COR shall conduct, or as directed, in the assessment of the security controls *[to be determined]* and update the SAR at least annually. A copy of the updated SAR shall be provided to the COR, if requested.

(iv) All systems shall have a completed privacy threshold analysis (PTA). If the PTA results determines the system contains personally identifiable information, a privacy impact assessment, approved by the HHS SAOP, is required.

(v) System Privacy Plan and Privacy Control Assessment. As required in OMB Circular A-130, a System Privacy Plan and Privacy Control Assessment shall be included in A&A. The plan and assessment may be included in the Security System Plan, or a separate report in the A&A, as determined by the COR.

(vi) An Independent Assessment is due prior to the CDC authorizing official signing off on an authorization to operate. The Contractor shall have an independent third-party validate the security and privacy controls in place for the system(s) commensurate with the risk levels per NIST SP 800-53B. The independent third party shall review and analyze the security authorization package and report on technical, operational, and management level deficiencies as outlined in NIST SP 800-53. The Contractor shall address all "high" deficiencies before submitting the package to the Government for acceptance and document all remaining deficiencies in a system Plan of Actions and Milestones (POA&M).

(vii) The POA&M is due as follows — the CDC Cybersecurity Weakness Management Standard that will be provided upon award.

(A) From the date the weaknesses are formally identified and documented;

(B) Critical-risk weaknesses must be mitigated within *[to be determined]* from the date the weaknesses are formally identified and documented;

(C) High-risk weaknesses must be mitigated within *[to be determined]* from the date the weaknesses are formally identified and documented;

(D) Medium weaknesses must be mitigated within *[to be determined]* from the date the weaknesses are formally identified and documented; and

(E) Low weaknesses must be mitigated within *[to be determined]*, from the date the weaknesses are formally identified and documented.

(2) HHS will determine the risk rating of all vulnerabilities. Identified risks stemming from deficiencies related to the security control baseline implementation, assessment, continuous monitoring, vulnerability scanning, flaws and security defect in a system (that require to create a patch for remediation), and other security reviews and sources, as documented in the SAR, shall be documented and tracked by the Contractor for mitigation in the POA&M document consistent with the HHS Standard for Plan of Action and Milestones policies. Depending on the severity of the risks, HHS may require designated POA&M weaknesses to be remediated before an ATO is issued. Thereafter, continue to remediate weaknesses throughout the contract. The POA&M document shall be updated at least quarterly.

(3) A Contingency Plan and Contingency Plan Test are due based on the CDC Control Catalog - Organizationally Defined Minimum Numbers and Time Periods document that will be provided upon award. The Contingency Plan shall be developed in accordance with NIST SP 800-34, *Contingency Planning Guide for Federal Information Systems*, and be consistent with HHS policies. Upon final acceptance by the System Owner, the Contractor, in coordination with the COR and System Owner, shall test the Contingency Plan and prepare a Contingency Plan Test Report that includes the test results, lessons learned, and any remaining action items to be addressed. The Contractor shall update and test the Contingency Plan at least *annually*.

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(4) An E-Authentication Questionnaire is required. The Contractor shall collaborate with at the COR's direction to ensure that the E-Authentication Guidance requirements are implemented in accordance with OMB 04-04 and NIST SP 800-63 series, latest versions. Based on the level of assurance determined by the E-Authentication, the Contractor shall ensure appropriate authentication to the system, including remote authentication, is in-place in accordance with the assurance level determined by the E-Authentication (when required), in accordance with HHS *Guidance for Selection of e-Authentication Assurance Levels* and any other applicable HHS policies.

(5) *Information security continuous monitoring.* Upon the government issuance of an ATO, the Contractor-owned/operated systems that input, store, process, output, and/or transmit government information shall meet or exceed the information security continuous monitoring (ISCM) requirements in accordance with FISMA and NIST SP 800-137, Information Security Continuous Monitoring (ISCM) for Federal Information Systems and Organizations, HHS ISCM Strategy, and HHS IS2P.

(6) *Annual assessment/penetration (pen) test.* The Contractor shall assess the system security and privacy controls (or ensure an assessment of the controls is conducted) at least annually to determine the implemented security and privacy controls are operating as intended and producing the desired results (this involves penetration testing conducted by the agency or independent third-party. In addition, review all relevant A&A documentation (SSP, POA&M, Contingency Plan, etc.) and provide updates by the specified due date provided by the COR.

(7) *Asset management.* Using any available Security Content Automation Protocol (SCAP)-compliant automated tools for active/passive scans, the Contractor shall provide an inventory of all information technology (IT) assets for hardware and software (computers, servers, routers, databases, operating systems, etc.) that are processing HHS-owned information and/or data. It is anticipated that this inventory information will be required to be produced at least annually. IT asset inventory information shall include—

- (i) IP address;
- (ii) Machine name;
- (iii) Operating system level;
- (iv) Security patch level; and

(v) SCAP-compliant format information. The Contractor shall maintain a capability to provide an inventory of 100% of its IT assets using SCAP-compliant automated tools in accordance with the *HHS Policy for Information Technology Asset Management (ITAM)* and any other applicable HHS policy.

(8) *Configuration management.* The Contractor shall use available SCAP-compliant automated tools as per NIST IR 7511 and *HHS Minimum Security Configurations Standards Guidance* to scan all IT assets, including but not limited to computers, servers, routers, databases, operating systems, application, etc., that store and process government information. The Contractor shall provide scan reports to the COR upon request. The Contractor shall maintain a capability to provide security configuration compliance information for 100% of its IT assets using SCAP-compliant automated tools.

(9) *Vulnerability management.* The Contractor shall actively manage system vulnerabilities using automated tools and technologies where practicable and in accordance with *HHS Policy for Vulnerability Management*. Automated tools must be compliant with NIST-specified SCAP standards for vulnerability identification and management. The contractor shall maintain a capability to provide security vulnerability scanning information for 100% of IT assets using SCAP-compliant automated tools and report to the agency at least monthly.

(10) *Patching and vulnerability remediation.* The Contractor shall install vendor-released security patches and remediate critical and high vulnerabilities in systems processing government information in an expedited manner, within vendor and agency specified timeframes.

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(11) *Secure coding.* The Contractor shall follow the *HHS Policy for Software Development Secure Coding Practices* and secure coding best practice requirements, as directed by United States Computer Emergency Readiness Team specified standards, the Software Engineering Institute (SEI) CERT and the Open Web Application Security Project, that will limit system software vulnerability exploits.

(12) *Boundary protection.* The Contractor shall ensure that government information, other than unrestricted information, being transmitted from federal government entities to external entities is routed through a Trusted Internet Connection.

(13) *Government access for security assessment.*

(i) In addition to the Inspection Clause in the contract, the Contractor shall afford the Government access to the Contractor's facilities, installations, operations, documentation, information systems, and personnel used in performance of this contract to the extent required to carry out a program of security assessment (to include vulnerability testing), investigation, and audit to safeguard against threats and hazards to the confidentiality, integrity, and availability of federal data or to the protection of information systems operated on behalf of HHS. This access includes, but is not limited to—

(1) At any tier handling or accessing information, consent to and allow the Government, or an independent third party working at the Government's direction, without notice at any time during a weekday during regular business hours contractor local time, access to Contractor and subcontractor installations, facilities, infrastructure, data centers, equipment (including but not limited to all servers, computing devices, and portable media), operations, documentation (whether in electronic, paper, or other forms), databases, and personnel which are used in performance of the contract. For the purposes of this paragraph only, *Government* includes, but is not limited to, Contracting Officer, COR, the U.S. Department of Justice, U.S. Government Accountability Office, and the HHS Office of the Inspector General (OIG). The purpose of access is to facilitate performance inspections and reviews, security and compliance audits, and law enforcement investigations. For security audits, the audit may include, but not be limited to, such items as buffer overflows, open ports, unnecessary services, lack of user input filtering, cross site scripting vulnerabilities, Structured Query Language injection vulnerabilities, and any other known vulnerabilities.

(2) At any tier handling or accessing protected information, fully cooperate with all audits, inspections, investigations, forensic analysis, or other reviews or requirements needed to carry out requirements presented in applicable law or policy. Beyond providing access, full cooperation also includes, but is not limited to, disclosure to investigators of information sufficient to identify the nature and extent of any criminal or fraudulent activity and the individuals responsible for that activity. It includes timely and complete production of requested data, metadata, information, and records relevant to any inspection, audit, investigation, or review, and making employees of the contractor available for interview by inspectors, auditors, and investigators upon request. Full cooperation also includes allowing the Government to make reproductions or copies of information and equipment, including, if necessary, collecting a machine or system image capture.

(ii) The Contractor shall segregate Government protected information and metadata on the handling of Government protected information from other information. Commingling of information is prohibited. Government inspectors, auditors, and investigators will not be precluded from having access to the sought information if sought information is commingled with other information.

(iii) The Contractor shall cooperate with inspections, audits, investigations, and reviews.

(c) *End of Life compliance.* The Contractor shall use Commercial off the Shelf (COTS) software or other software that is supported by the manufacturer. In addition, the COTS/other software need to be within one major version of the current version; deviation from this requirement will only be allowed via the HHS waiver process (approved by HHS CISO if it impacts enterprise-wide systems and services, or by the OPDIV CISO if it impacts only the OPDIV). The contractor must retire and/or upgrade all software/systems that have reached end-of-life in accordance with *HHS End of Life Operating Systems, Software and Application Policy*.

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(d) *Desktops, laptops, and other computing devices required for use by the Contractor.* The Contractor shall ensure that all IT equipment (e.g., laptops, desktops, servers, routers, mobile devices, peripheral devices, etc.) used to process information on behalf of HHS are deployed and operated in accordance with approved security configurations and meet the following minimum requirements:

(1) Encrypt equipment and sensitive information stored and/or processed by such equipment in accordance with HHS encryption standard and current FIPS 140 validation certificate from the NIST Cryptographic Module Validation Program.

(2) Configure laptops and desktops in accordance with the latest applicable United States Government Configuration Baseline and HHS Minimum Security Configuration Standards;

(3) Maintain the latest operating system patch release and anti-virus software definitions at least monthly;

(4) Validate the configuration settings after hardware and software installation, operation, maintenance, update, and patching and ensure changes in hardware and software do not alter the approved configuration settings; and

(5) Automate configuration settings and configuration management in accordance with HHS security policies, including but not limited to:

(i) Configuring its systems to allow for periodic HHS vulnerability and security configuration assessment scanning; and

(ii) Using SCAP-validated tools with capabilities to scan its systems at least on a monthly basis and report the results of these scans to the Contracting Officer and/or COR, Project Officer, and any other point of contract designated by the Contracting Officer or COR.

(e) *Rights to data.* Contractors shall specify any data rights asserted and mark such deliverables accordingly in accordance with applicable data rights clauses set forth in the contract.

(f) *Information and Communications Technology (ICT) Cybersecurity Supply Chain Risk Management (C-SCRM) requirements.* The Contractor shall secure their ICT supply chain in compliance with *HHS Policy for Cyber Supply Chain Risk Management* and Public Law 115-232, section 889. At a minimum, the Contractor shall—

(1) Develop rules for suppliers' development methods, techniques, or practices;

(2) Use secondary market components;

(3) Prohibit counterfeit products;

(4) Dispose and/or retain elements such as components, data, or intellectual property securely;

(5) Ensure adequate supply of components;

(6) Require external providers handling federal information or operating systems on behalf of the federal government to meet the same security and privacy requirements as federal agencies;

(7) Require external providers to express security and privacy requirements (including the controls for systems processing, storing, or transmitting federal information) in contracts or other formal agreements;

(8) Establish Service Level Agreements (SLAs), patching vehicles, and disclosure requirements in the case of an incident or new vulnerability being discovered; and

(9) Ensure that the supplier applies same contractual requirements to any sub- contractors/suppliers that they involve in the provision of the product or service to the customer; and

(10) Prohibit the use of covered telecommunications and video surveillance equipment or services.

(g) *Subcontract flow down.* The Contractor shall include the substance of this clause, including this paragraph (g), in subcontracts and third-party agreements, at any tier, of any amount and in which subcontractor employees and third-party servicers/employees, will perform functions or provide products under the scope of this contract, and have access to HHS information (including HHS sensitive information, i.e., protected health information), information systems, information technology (IT) or providing and accessing information technology-related contract services, support services, and related resources (see HHSAR 302.101, Definitions).

(End of clause)

HHSAR 352.239-79 Information and Communication Technology Accessibility (FEB 2024) (DEVIATION)

(a) Pursuant to Section 508 of the Rehabilitation Act of 1973 (29 U.S.C. 794d), as amended by the Workforce Investment Act of 1998, all information and communication technology (ICT) supplies, products, platforms, information, documentation, and services support developed, acquired, maintained or delivered under this contract or order must comply with the Revised 508 Standards, which are located at 36 C.F.R. 1194.1 and Appendices A, B, and C, and are available at <https://www.access-board.gov/ict/>. Information about Section 508 is available at <https://www.hhs.gov/web/section-508/index.html>.

(b) Additional Section 508 accessibility standards applicable to this contract or order are identified as follows:

- 205 WCAG 2.0 Level A & AA Success Criteria
- 302 Functional Performance Criteria
- 502 Inoperability with Assistive Technology
- 504 Authoring Tools
- 602 Support Documentation
- 603 Support Services

If it is determined by the Government that ICT supplies, products, platforms, information, documentation, and services support provided by the Contractor do not conform to the described accessibility standards in the contract, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(c) In the event of a modification(s) to this contract or order, which adds new ICT supplies or services or revises the type of, or specifications for, supplies, products, platforms, information, documentation, or services support, the Contracting Officer shall require that the Contractor submit a completed HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an Accessibility Conformance Report (ACR) (based on the Voluntary Product Accessibility Template (VPAT) see <https://www.itic.org/policy/accessibility/vpat>), and any other additional information necessary to assist the Government in determining that the ICT supplies or services conform to Section 508 accessibility standards. If it is determined by the Government that ICT supplies, products, platforms, information, documentation, and services support provided by the Contractor do not conform to the described accessibility standards in the contract, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(d) If this is an Indefinite-Delivery type contract, a Blanket Purchase Agreement or a Basic Ordering Agreement, the task/delivery order requests that include ICT supplies, products, platforms, information, documentation, or services support will define the specifications and accessibility standards for the order. In those cases, the Contractor shall be required to provide a completed HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an ACR (based on the VPAT see <https://www.itic.org/policy/accessibility/vpat>), and any other additional information necessary to assist the Government in determining that the ICT supplies, products, platforms, information, documentation, or services support conform to Section 508 accessibility standards. If it is determined by the Government that ICT supplies and services provided by the Contractor do not conform to the described accessibility standards in the provided

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documentation, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(e) The contractor shall identify to the Contracting Officer any perceived exception or exemption to Section 508 requirements.

(End of clause)

HHSAR Information System Hosting, Operation, Maintenance, or Use (FEB 2024) (DEVIATION)

(a) *Definitions.* As used in this clause—

Assessment and Authorization (A&A) means the process used to ensure information systems including Major Applications and General Support Systems have effective security safeguards which have been implemented, planned for, and documented in an Information Technology Security Plan. The A&A process per applicable HHS policies and procedures is the mechanism by which HHS provides an Authorization to Operate (ATO), the official management decision given by the HHS to authorize operation of an information system.

Information system security plan means a formal document that provides an overview of the security requirements for an information system or an information security program and describes the security controls in place or planned for meeting those requirements.

(b) *Hosting, operation, maintenance, or use at non-HHS facilities.* For information systems that are hosted, operated, maintained, or used on behalf of HHS at non-HHS facilities, Contractors/subcontractors are fully responsible and accountable for ensuring compliance with all applicable Health Insurance Portability and Accountability (HIPAA) Act of 1996 (HIPAA) regulations, the Privacy Act and other required HHS confidentiality statutes included in HHS mandatory yearly training and privacy policy, Federal Information Security Modernization Act (FISMA), National Institutes of Standards and Technology (NIST), Federal Information Processing Standards (FIPS), and HHS security and privacy policy. This includes conducting compliant risk assessments, routine vulnerability scanning, system patching and change management procedures, and the completion of an acceptable contingency plan for each system. The Contractor's security and privacy control procedures must be equivalent to or exceed those procedures used to secure HHS systems. A Privacy Impact Assessment (PIA) (if the system includes Personally Identifiable Information (PII)) or a Privacy Threshold Analysis (to determine if the system includes PII) must also be provided to the Contracting Officer Representative (COR) and approved by HHS Senior Agency Official for Privacy (SAOP) or designee prior to ATO. All external Internet connections to HHS network involving HHS information must be in accordance with the Trusted Internet Connections (TIC) Reference Architecture and reviewed and approved by HHS prior to implementation. For Cloud Services hosting, the Contractor shall also ensure compliance with the Federal Risk and Authorization Management Program (FedRAMP).

(c) *Collecting, processing, transmitting, and storing of PII.* Adequate security and privacy controls for collecting, processing, transmitting, and storing of PII, as determined by the HHS SAOP or designee, must be in place, tested, and approved by HHS prior to hosting, operation, maintenance, or use of the information system, or systems by or on behalf of HHS. These security and privacy controls are to be assessed and stated within the PIA, Information System Security Plan, Information System Privacy Plan, Security Control Assessment Report, and/or Privacy Control Assessment Report, as agreed upon by the Contractor, COR, and the Operating Division Senior Official for Privacy. If these controls are determined not to be in place, or inadequate, a Plan of Action and Milestones (POA&M) must be submitted and approved prior to the collection of PII.

(d) *Annual FISMA security controls assessment.* The Contractor/subcontractor's system must adhere to all FISMA, FIPS, and NIST standards related to the annual FISMA security controls assessment and review and update the PIA. Any deficiencies noted during this assessment must be provided to the Contracting Officer for entry into HHS Plan of Action & Milestones (POA&M) management process. The Contractor/subcontractor must use HHS POA&M process to document planned remedial actions to address any deficiencies in information security policies, procedures, and practices, and the completion of those activities. Security deficiencies must be corrected within the timeframes specified by the HHS in the performance work statement or statement of work, or in the approved remediation plan through the HHS POA&M process. Contractor/subcontractor procedures are subject to periodic,

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unannounced assessments by HHS officials, including the HHS Office of Inspector General. The physical security aspects associated with Contractor/subcontractor activities must also be subject to such assessments. The results of an annual review or a major change in the cybersecurity posture at any time may indicate the need for reassessment and reauthorization of the system. If major changes to the system occur that may affect the privacy or security of the data or the system, the A&A of the system may need to be reviewed, retested and re-authorized per HHS Rules of Behavior for the Use of HHS Information and IT Resources Policy. Major changes introducing new privacy risks require an updated and reapproved PIA.

(e) *Annual self-assessment.* The Contractor/subcontractor must conduct an annual self-assessment on all systems and outsourced services as required. Both hard copy and electronic copies of the assessment must be provided to the COR. HHS reserves the right to conduct such an assessment using government personnel or another Contractor/subcontractor. The Contractor/subcontractor must take appropriate and timely action, as may be specifically addressed in the contract, to correct or mitigate any weaknesses discovered during such testing, at no additional cost to the Government to correct Contractor/subcontractor systems and outsourced services.

(f) *Prohibition of installation and use of personally-owned or Contractor-owned equipment or software on HHS networks.* HHS prohibits the installation and use of personally-owned or Contractor/subcontractor-owned equipment or software on HHS networks. If non-HHS owned equipment must be used to fulfill the requirements of a contract, it must be stated in the service agreement, performance work statement, statement of work, or contract. All of the security controls required for government furnished equipment must also be utilized in approved other equipment (OE) at the Contractor's expense. All remote systems must be equipped with, and use, an HHS- approved antivirus software and a personal (host-based or enclave based) firewall that is configured with an HHS-approved configuration. Software must be kept current, including all critical updates and patches. Owners of approved OE are responsible for providing and maintaining the anti-viral software and the firewall on the non-HHS owned OE.

(g) *Disposal or return of electronic storage media on non-HHS leased or non-HHS owned IT equipment.* All electronic storage media used on non-HHS leased or non-HHS owned IT equipment that is used to store, process, or access HHS information must be handled in adherence with disposition instructions upon—

(1) Completion or termination of the contract; or

(2) Disposal or return of the IT equipment by the Contractor/subcontractor or any person acting on behalf of the Contractor/subcontractor, whichever is earlier. Media (e.g., hard drives, optical disks, CDs, back-up tapes) used by the Contractors/ subcontractors that contain HHS information must be returned to the HHS for sanitization or destruction or the Contractor/subcontractor must self-certify that the media has been disposed of per disposition instructions. This must be completed within 30 days of termination of the contract.

(h) *Bio-Medical devices and other equipment or systems.* Bio-Medical devices and other equipment or systems containing media (e.g., hard drives, optical disks) with HHS sensitive information will not be returned to the Contractor at the end of lease, for trade-in, or other purposes. For purposes of these devices and protection of HHS sensitive information the devices may be provided back to the Contractor under one of three scenarios—

(1) The Contractor must accept the system without the drive;

(2) A spare drive must be installed in place of the original drive at time of turn- in if HHS initial medical device purchase included a spare drive; or

(3) The Contractor may request reimbursement for the drive at a reasonable open market replacement cost to be separately negotiated by the Contracting Officer and the Contractor at time of contract closeout.

(End of clause)

CDCH.16 Non-Disclosure Agreement for Contractor and Contractor Employees (Jun 2020)

(a) The contractor and contractor employees shall prepare and submit Non-Disclosure Agreements (NDA) to the Contracting Officer prior to access of government information or the commencement of work at CDC.

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- (b) The NDAs, at Exhibit I and II, are required in service contracts where contractor's employees will have access to non-public and procurement-sensitive information while performing functions in support of the Government. The NDA also requires contractor's employees properly identify themselves as employees of a contractor when communicating or interacting with CDC employees, employees of other governmental entities, and members of the public (when communication or interaction relates to the contractor's work with the CDC). The Federal Acquisition Regulation (FAR) 37.114 (c), states "All contractor personnel attending meetings, answering Government telephones, and working in other situations where their contractor status is not obvious to third parties are required to identify themselves as such to avoid creating an impression in the minds of members of the public or Congress that they are Government officials, unless, in the judgment of the agency, no harm can come from failing to identify themselves. They must also ensure that all documents or reports produced by contractors are suitably marked as contractor products or that contractor participation is appropriately disclosed."
- (c) The contractor shall inform contractor employees of the identification requirements by which they must abide and monitor employee compliance with the identification requirements.
- (d) During the contract performance period, the contractor is responsible to ensure that all additional or replacement contractors' employees sign an NDA and it is submitted to the Contracting Officer prior to commencement of their work with the CDC.
- (e) Contractor employees in designated positions or functions that have not signed the appropriate NDA shall not have access to any non-public, procurement sensitive information or participate in government meetings where sensitive information may be discussed.
- (f) The Contractor shall prepare and maintain a current list of employees working under NDAs and submit to the Contracting Officer upon request during the contract period of performance. The list should at a minimum include: contract number, employee's name, position, date of hire and NDA requirement.

EXHIBIT I

**Centers for Disease Control and Prevention (CDC)
Contractor Non-Disclosure Agreement**

I. Non-public Information

[Name of contractor] understands that in order to fulfill the responsibilities pursuant to [contract name and number] between the Centers for Disease Control and Prevention and [Name of CDC contractor] dated [date], employees of [contractor] will have access to non-public information, including confidential and privileged information contained in government-owned information technology systems. For purposes of this agreement, confidential information means government information that is not or will not be generally available to the public. Privileged information means information which cannot be disclosed without the prior written consent of the CDC.

In order to properly safeguard non-public information, [contractor] agrees to ensure that prior to being granted access to government information or the commencement of work for the CDC, whichever is applicable, all contractor employees will sign a Non-Disclosure Agreement (NDA) provided by the CDC prior to beginning work for the CDC. Contractor agrees to submit to the Contracting Officer the original signed copies of NDAs signed by the contractor's employees in accordance with the instructions provided by the Contracting Officer. Failure to provide signed NDAs in accordance with this agreement and instructions provided by the Contracting Officer could delay or prevent the employee from commencing or continuing work at the CDC until such agreement is signed and returned to the Contracting Officer.

Contractor further agrees that it will not cause or encourage any employee to disclose, publish, divulge, release, or make known in any manner or to any extent, to any individual other than an authorized Government employee any non-public information that the employee may obtain in connection with the performance of the employee's responsibilities to the CDC.

II. Procurement-Sensitive Information

Contractor further agrees that it will not cause or encourage any employee to disclose, publish, divulge, release, or make known in any manner or to any extent, to any individual, other than an authorized Government employee, any procurement-sensitive information gained while in connection with fulfilling the employee's responsibilities at the CDC. For purposes of this agreement, procurement-sensitive information includes, but is not limited to, all information in Statements of Work (SOW), Procurement Requests (PR), and Requests for Proposal (RFP); Responses to RFPs, including proposals, questions from potential offerors; non-public information regarding procurements; all documents, conversations, discussions, data, correspondence, electronic mail (e-mail), presentations, or any other written or verbal communications relating to, concerning, or affecting proposed or pending solicitations or awards; procurement data; contract information plans; strategies; source selection information and documentation; offerors' identities; technical and cost data; the identity of government personnel involved in the solicitation; the schedule of key technical and procurement events in the award determination process; and any other information that may provide an unfair competitive advantage to a contractor or potential contractor if improperly disclosed to them, or any of their employees.

Contractor understands and agrees that employee access to any procurement-sensitive information may create a conflict of interest which will preclude contractor from becoming a competitor for any acquisition(s) resulting from this information. Therefore, if an employee participates in any discussions relating to procurement-sensitive information, assists in developing any procurement-sensitive information, or otherwise obtains any procurement-sensitive information while performing duties at the CDC, contractor understands and agrees that contractor may be excluded from competing for any acquisition(s) resulting from this information.

III. Identification of Non-Government Employees

Contractor understands that its employees are not agents of the Government. Therefore, unless otherwise directed in writing by the CDC, contractor agrees to assist and monitor employee compliance with the following identification procedures:

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- A.** At the beginning of interactions with CDC employees, employees of other governmental entities, and members of the public (when such communication or interaction relates to the contractor's work with the CDC), contractors' employees will identify themselves as an employee of a contractor.
- B.** Contractors' employees will include the following disclosures in all written communications, including outgoing electronic mail (e-mail) messages, in connection with contractual duties to the CDC:

Employee's name
Name of contractor
Center or office affiliation
Centers for Disease Control and Prevention

- C.** At the beginning of telephone conversations or conference calls, contractors' employees will identify themselves as an employee of a contractor.
- D.** Contractors' employees should not wear any CDC logo on clothing, except for a CDC issued security badge while carrying out work for CDC or on CDC premises. The only other exception is when a CDC management official has granted permission to use the CDC logo.
- E.** Contractors' employees will program CDC voice mail message to identify themselves as an employee of a contractor.

I understand that federal laws including, 18 U.S.C. 641 and 18 U.S.C. 2071, provide criminal penalties for, among other things, unlawfully removing, destroying or converting to personal use, or use of another, any public records. Contractor acknowledges that contractor has read and fully understands this agreement.

Name of contractor: _____

Signature of Authorized Representative of Contractor: _____

Date: _____

Copies retained by: Contracting Officer and contractor

EXHIBIT II

**Centers for Disease Control and Prevention (CDC)
Contractors' Employee Non-Disclosure Agreement**

I. Non-Public Information

I understand that in order to fulfill my responsibilities as an employee of **[Name of CDC contractor]**, I will have access to non-public information, including confidential and privileged information contained in government-owned information technology systems. For purposes of this agreement, confidential information means government information that is not or will not be generally available to the public. Privileged information means information which cannot be disclosed without the prior written consent of the CDC.

I, **[Name of Employee]**, agree to use non-public information only in performance of my responsibilities to the CDC. I agree further that I will not disclose, publish, divulge, release, or make known in any manner or to any extent, to any individual other than an authorized Government employee, any non-public information that I may obtain in connection with the performance of my responsibilities to the CDC.

II. Procurement-Sensitive Information

I further agree that unless I have prior written permission from the CDC, I will not disclose, publish, divulge, release, or make known in any manner or to any extent, to any individual other than an authorized Government employee, any procurement-sensitive information gained in connection with the performance of my responsibilities to the CDC. I specifically agree not to disclose any non-public, procurement-sensitive information to employees of my company or any other organization unless so authorized in writing by the CDC. For purposes of this agreement, procurement-sensitive information includes, but is not limited to, all information in Statements of Work (SOW), Procurement Requests (PR), and Requests for Proposal (RFP); Responses to RFPs, including proposals, questions from potential offerors; non-public information regarding procurements; all documents, conversations, discussions, data, correspondence, electronic mail (e-mail), presentations, or any other written or verbal communications relating to, concerning, or affecting proposed or pending solicitations or awards; procurement data; contract information plans; strategies; source selection information and documentation; offerors' identities; technical and cost data; the identity of government personnel involved in the acquisition; the schedule of key technical and procurement events in the award determination process; and any other information that may provide an unfair competitive advantage to a contractor or potential contractor if improperly disclosed to them, or any of their employees.

I understand and agree that my access to any procurement-sensitive information may create a conflict of interest which will preclude me, my current employer, or a future employer from becoming a competitor for any resulting government acquisition derived from this information. Therefore, if I participate in any discussions relating to procurement-sensitive information, assist in developing any procurement-sensitive information, or otherwise obtain any procurement-sensitive information while performing my duties at the CDC, I understand and agree that I, my current employer, and any future employer(s) may be excluded from competing for any resulting acquisitions.

III. Special Non-Disclosure Agreement for Contractors with Access to CDC Grants Management and Procurement-Related Information Technology Systems

In addition to complying with the non-disclosure requirements and safeguards stated above, I understand that my authorization to use CDC's grants management and procurement systems is strictly limited to the access and functions necessary for the performance of my responsibilities to the CDC and which have been approved in advance by the CDC. I understand that I am not authorized to enter procurement requests for any requirements pertaining to contracts or subcontracts held by me or my employer.

IV. Identification as a Non-Government Employee

I understand that as an employee of a government contractor, I represent an independent organization and I am not an agent of the Government. Therefore, I agree that unless I have prior written authorization from the CDC, I will, at the beginning of interactions with CDC employees, employees of other governmental entities, members of the

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public (when such communication or interaction relates to the contractor's work with the CDC), identify myself as an employee of a contractor. I further agree to use the following identification procedures in connection with my work at the CDC:

A. I will include the following disclosures in all written communications, including outgoing electronic mail (e-mail) messages:

Employee's name
Name of contractor
Center or office affiliation
Centers for Disease Control and Prevention

B. I will identify myself as an employee of a contractor at the beginning of telephone conversations or conference calls;

C. I will not wear any CDC logo on clothing, except for a CDC issued security badge while carrying out work for CDC or on CDC premises; the only other exception is when a CDC management official has granted permission to use the CDC logo.

D. I will program my CDC voice mail message to identify myself as a contractors' employee.

I understand that federal laws including, 18 U.S.C. 641 and 18 U.S.C. 2071, provide criminal penalties for, among other things, unlawfully removing, destroying or converting to personal use, or use of another, any public records. I acknowledge that I have read and fully understand this agreement.

Name of contractor: _____

Name of Employee: _____

Signature of Employee: _____

Date: _____

Copies retained by: Contracting Officer, contractor, and Contractor Employee

(End of Clause)

CDCG.02 Contracting Officer's Representative (COR) (JUL 2017)

(a) Performance of the work hereunder shall be subject to the technical directions of the designated COR for this contract.

(b) As used herein, technical directions are directions to the Contractor which fill in details, suggests possible lines of inquiry, or otherwise completes the general scope of work set forth herein. These technical directions must be within the general scope of work and may not alter the scope of work or cause changes of such a nature as to justify an adjustment in the stated contract price/cost, or any stated limitation thereof.

(c) In the event that the Contractor believes full implementation of any of these directions may exceed the scope of the contract, he or she shall notify the originator of the technical direction and the Contracting Officer, immediately or as soon as possible, in a letter or e-mail separate of any required report(s). No technical direction, nor its fulfillment, shall alter or abrogate the rights and obligations fixed in this contract.

(d) The Government COR is not authorized to change any of the terms and conditions of this contract. Contract changes shall be made only by the Contracting Officer through properly written modification(s) to the contract.

(e) The Government will provide the Contractor with a copy of the COR delegation memorandum upon request.

CDCG.03 Contracting Officer (JUL 1999)

(a) No person other than the Contracting Officer can make any changes to the terms, conditions, general provisions, or other stipulations of this contract.

(b) No information, other than that contained in an authorized modification to this contract, duly issued by the Contracting Officer, received from any person employed by the United States Government, or otherwise, shall be considered grounds for deviation from any stipulation of this contract.

CDCG.07 Payment by Electronic Funds Transfer (FEB 2018)

(a) The Government shall use electronic funds transfer to the maximum extent possible when making payments under this contract. FAR 52.232-33, Payment by Electronic Funds Transfer–System for Award Management, in Section C, requires the contractor to designate in writing a financial institution for receipt of electronic funds transfer payments.

(b) In the case that EFT information is not within the System of Award Management, FAR 52.232-34 requires mandatory submission of Contractor's EFT information directly to the office designated in this contract to receive that information (hereafter: "designated office"); see below. The contractor shall submit the EFT information within the form titled "ACH Vendor/Miscellaneous Payment Enrollment Form" to the address indicated below. Note: The form is either attached to this contract (see Table 4 of PWS) or may be obtained by contacting the Contracting Officer or the CDC Office of Financial Resources at 678-475-4510.

(c) In cases where the contractor has previously provided such information, i.e., pursuant to a prior contract/order, and been enrolled in the program, the form is not required unless the designated financial institution has changed.

(d) The completed form shall be mailed after award, but no later than 14 calendar days before an invoice is submitted, to the following address:

The Centers for Disease Control and Prevention
Office of Financial Resources (OFR)
P.O. Box 15580
Atlanta, GA 30333
Or – Fax copy to: 404-638-5342

CDCG.08 Applicable Service Contract Labor Standards Wages (Jul 2017)

The referenced Wage Determination, No. 2015-5831* dated July 8, 2025, specifies hourly rates of wages and fringe benefits that shall be paid to covered employees performing work required under this contract. These rates have been determined by the Secretary of Labor in accordance with the provisions of the Service Contract Labor Standards statute. (See FAR 52.222-41, incorporated by reference in Section C.)

*Applicable Service contract Act WD to be determined by awardee locality

CDCH.93 Data Subject to Privacy Act Requirements (JUL 2017)

(a) Notification is hereby given that the Contractor and its employees are subject to criminal penalties for violation of the Privacy Act to the same extent as employees of the Government. The Contractor shall assure that each of its employees knows the prescribed rules of conduct and that each is aware that he or she can be subjected to criminal penalty for violation of the Act.

(b) In accordance with HHSAR Clause 352.224-70, Privacy Act, which has been incorporated into this contract, certain data provided to the Contractor under this contract shall be treated confidentially. The type(s) of data subject to this clause are as follows:

- (1) Care information.
- (2) Health Care Administration Information
- (3) Health Care Delivery Services Information
- (4) Research and Development Information
- (5) Knowledge Dissemination Information
- (6) Inspections and Auditing Information
- (7) Personal Identity and Authentication Information

(c) Following are the requirements for handling this data and the disposition to be made of this data upon completion of contract performance: Data subject to the Privacy Act shall be handled in accordance with all applicable legal, regulatory, and contractual requirements, and disposition is subject to all applicable legal, regulatory, and contractual requirements.

(d) The Contracting Officer's Representative (COR) is hereby designated as the official who is responsible for monitoring contractor compliance with the Privacy Act.

CDCA_H007 Subcontracting Plan (JUL 1999)

The Contractor's subcontracting plan, as negotiated, submitted in response to solicitation no. 75D301-26-R-73374, is hereby incorporated into this contract by reference.

CDCH.08 Smoke Free Working Environment (MAY 2009)

In compliance with Department of Health and Human Services (DHHS) regulations, all contractor personnel performing work within CDC/ATSDR facilities shall observe the CDC/ATSDR smoke-free working environment policy at all times. This policy prohibits smoking in all CDC/ATSDR buildings and in front of buildings which are open to the public. This policy is also applicable to contractor personnel who do not work full-time within CDC/ATSDR facilities but are attending meetings within CDC/ATSDR facilities.

CDCH.14 Records Management Obligations (JUN 2020)

A. Applicability.

The following applies to all Contractors whose employees create, work with, or otherwise handle Federal records, as defined in Section B, regardless of the medium in which the record exists.

B. Definitions.

“Federal record” as defined in 44 U.S.C. § 3301, includes all recorded information, regardless of form or characteristics, made or received by a Federal agency under Federal law or in connection with the transaction of public business and preserved or appropriate for preservation by that agency or its legitimate successor as evidence of the organization, functions, policies, decisions, procedures, operations, or other activities of the United States Government or because of the informational value of data in them.

The term Federal record:

- (1) includes Centers for Disease Control and Prevention (CDC) records.
- (2) does not include personal materials.
- (3) applies to records created, received, or maintained by Contractors pursuant to their CDC contract.
- (4) may include deliverables and documentation associated with deliverables.

C. Requirements.

(1) Contractor shall comply with all applicable records management laws and regulations, as well as National Archives and Records Administration (NARA) records policies, including but not limited to the Federal Records Act (44 U.S.C. chs. 21, 29, 31, 33), NARA regulations at 36 C.F.R. Chapter XII Subchapter B, and those policies associated with the safeguarding of records covered by the Privacy Act of 1974 (5 U.S.C. 552a). These policies include the preservation of all records, regardless of form or characteristics, mode of transmission, or state of completion.

(2) In accordance with 36 C.F.R. 1222.32, all data created for Government use and delivered to, or falling under the legal control of, the Government are Federal records subject to the provisions of 44 U.S.C. chapters 21, 29, 31, and 33, the Freedom of Information Act (FOIA) (5 U.S.C. 552), as amended, and the Privacy Act of 1974 (5 U.S.C. 552a), as amended and must be managed and scheduled for disposition only as permitted by statute or regulation.

(3) In accordance with 36 C.F.R. 1222.32, Contractor shall maintain all records created for Government use or created in the course of performing the contract and/or delivered to, or under the legal control of the Government and must be managed in accordance with Federal law. Electronic records and associated metadata must be accompanied by sufficient technical documentation to permit understanding and use of the records and data.

(4) CDC and its contractors are responsible for preventing the alienation or unauthorized destruction of records, including all forms of mutilation. Records may not be removed from the legal custody of CDC or destroyed except for in accordance with the provisions of the agency records schedules and with the written concurrence of the Head of the Contracting Activity. Willful and unlawful destruction, damage or alienation of Federal records is subject to the fines and penalties imposed by 18 U.S.C. 2701. In the event of any unlawful or accidental removal, defacing, alteration, or destruction of records, Contractor must report to the Contracting Officer and the Contracting Officer's Representative. The agency must report promptly to NARA in accordance with 36 C.F.R. 1230.

(5) The Contractor shall immediately notify the appropriate Contracting Officer upon discovery of any inadvertent or unauthorized disclosures of information, data, documentary materials, records or equipment. Disclosure of non-public information is limited to authorized personnel with a need-to-know as described in the contract. The Contractor shall ensure that the appropriate personnel, administrative, technical, and physical safeguards are established to ensure the security and confidentiality of this information, data, documentary material, records and/or equipment is properly protected. The Contractor shall not remove material from Government facilities or systems, or facilities or systems operated or maintained on the Government's behalf, without the express written permission of the Head of the Contracting Activity. When information, data, documentary material, records and/or equipment is no longer required, it shall be returned to CDC control, or the Contractor must hold it until otherwise directed. Items returned to the Government shall be hand carried, mailed, emailed, or securely electronically transmitted to the Contracting Officer or address prescribed in the contract. Destruction of records is EXPRESSLY PROHIBITED unless in accordance with Paragraph (4).

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(6) The Contractor is required to obtain the Contracting Officer's approval prior to engaging in any contractual relationship (sub-contractor) in support of this contract requiring the disclosure of information, documentary material and/or records generated under, or relating to, contracts. The Contractor (and any sub-contractor) is required to abide by Government and CDC guidance for protecting sensitive, proprietary information, classified, and controlled unclassified information.

(7) The Contractor shall only use Government IT equipment for purposes specifically tied to or authorized by the contract and in accordance with CDC policy.

(8) The Contractor shall not create or maintain any records containing any non-public CDC information that are not specifically tied to or authorized by the contract.

(9) The Contractor shall not retain, use, sell, or disseminate copies of any deliverable that contains information covered by the Privacy Act of 1974 or that which is generally protected from public disclosure by an exemption to the Freedom of Information Act.

(10) Training. All Contractor employees assigned to this contract who create, work with, or otherwise handle records are required to take CDC-provided records management training. The Contractor is responsible for confirming training has been completed according to agency policies, including initial training and any annual or refresher training.

D. Flowdown of requirements to subcontractors.

(1) The Contractor shall incorporate the entire substance of the terms and conditions herein, including this paragraph, in all subcontracts under this contract, and must require written subcontractor acknowledgment of same.

(2) Violation by a subcontractor of any provision set forth herein will be attributed to the Contractor.

CDCH.19 Health Information Technology (JUN 2020)

(a) Executive Order 13410: Promoting Quality and Efficient Health Care in Federal Government Administered or Sponsored Health Care Programs promotes efficient delivery of quality health care through the use of health information technology, transparency regarding health care quality and price, and incentives to promote the widespread adoption of health information technology and quality of care. To support this objective the awardee shall adhere to the following interoperability standards and requirements. For purposes of the information described herein, interoperability is defined as the ability of two or more systems or components to exchange information and to use the information that has been exchanged.

(b) Interoperability of Health Information Technology

This Section applies to contracts that involve the exchange of public health data between public health entities, CDC, and other federal agencies. This supports the development and use of interoperable information systems for public health functions such as biosurveillance, situational awareness, laboratory science, or environmental health.

(1) In addition to complying with HHS Standards for Security Configurations, HHS Standards for Encryption Language, and Federal Laws, Regulations and Standards related to IT Security, the awardee is required to meet recognized interoperability standards as follows:

(2) The awardee shall use nationally adopted Public Health Information Network (PHIN) standards at the time of the system implementation, acquisition, or upgrade, in all relevant information technology systems supported, in whole or in part, through this contract. Where no PHIN standards exist, the awardee shall utilize recognized standards established by the Office of the National Coordinator for Health Information Technology (ONC) as described in their Nationwide Health Information Network (NwHIN) standards.

(3) The awardee shall use PHIN policies, standards, practices, and services in the research and implementation of efficient and effective public health information systems, to facilitate interoperability (the ability

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of two or more systems or components to exchange information and to use the information that has been exchanged) with public health organizations and networks, including NwHIN.

(4) Where the awardee supports or participates in health information or data exchange with disparate entities, the awardee must have an architecture that is compatible with the architecture of PHIN, where appropriate. If PHIN standards do not exist, the awardee shall refer to NwHIN standards.

(c) Public Health Data within Clinical Healthcare

This Section applies to contracts that involve the exchange of public health data within the clinical setting, such as hospitals and providers. This supports the exchange of public health information, including laboratory exchange data, between public health entities and healthcare systems.

(1) The awardee shall use health IT investments, such as electronic health records (EHRs), personalized health records, and network components through which they operate and share information, that are certified by a recognized certification board and/or validated and tested with PHIN or NwHIN certification to ensure a minimum level of interoperability or compatibility of health IT products.

(2) The awardee may use services available from the Agency for Healthcare Research and Quality (AHRQ) at www.healthit.ahrq.gov. This link provides technical assistance, identifies challenges to health IT adoption and use, and identifies solutions and best practices that have the potential to transform clinical practice through the best and most effective use of IT.

(3) Where feasible, the awardee shall collaborate with other healthcare entities, local quality improvement organizations, and/or local medical societies to promote the adoption of certified EHRs in clinical provider settings.

(4) Where feasible, the awardee shall collaborate with other healthcare entities and/or local hospital associations to promote the adoption of certified EHRs in hospital settings.

(5) The awardee shall identify public health entities that have a stake in the award and demonstrate interactions with them to implement and promote the adoption of PHIN and/or NwHIN data exchange standards in support of federal health initiatives (e.g., situational awareness, EHR Meaningful Use).

(6) The awardee shall report on public health information exchange activities that include partnerships with health information exchanges, other public health partners, or any other stakeholders within program reports, progress reports, or as requested by CDC.

CDCH.20 Non-Personal Services (JUN 2020)

(a) Personal services shall not be performed under this contract. Although the Government may provide sporadic or occasional instructions within the scope of the contract, the Contractor is responsible for control and supervision of its employees. If the Contractor (including its employees) believes any Government action or communication has been given that would create a personal services relationship between the Government and any Contractor employee, the Contractor shall promptly notify the Contracting Officer of this communication or action.

(b) The contractor shall comply with, and ensure their employees and subcontractors comply with, CDC Policy titled "Contractor Identification and Safeguarding of Non-Public Information". No Contractor employee shall hold him or herself out to be a Government employee, agent, or representative. No Contractor employee shall state orally or in writing at any time that he or she is acting on behalf of the Government. In all communications with third parties in connection with this contract, Contractor employees shall identify themselves as Contractor employees and specify the name of the company for which they work. The contractor is limited to performing the services identified in the contract statement of work and shall not interpret any communication with anyone as a permissible change in contract scope or as authorization to perform work not described in the contract. All contract changes will be incorporated by a modification signed by the Contracting Officer.

(c) The Contractor shall ensure that all of its employees and subcontractor employees working on this contract are informed of the terms and conditions herein. The Contractor agrees that this is a non-personal services contract;

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and that for all the purposes of the contract, the Contractor is not, nor shall it hold itself out to be an agent or partner of, or joint venture with, the Government. The Contractor shall notify its employees that they shall neither supervise nor accept supervision from Government employees. The substance of the terms herein shall be included in all subcontracts at any tier.

(d) The terms and conditions above do not limit the Government's rights under other terms of the contract, including those related to the Government's right to inspect and accept or reject the services performed under this contract.

CDCH.023 Evaluation of Contractor Performance Utilizing CPARS (AUG 2021)

(a) In accordance with FAR 42.15, Centers for Disease Control and Prevention (CDC) will review and evaluate contract performance. FAR 42.1502 and 42.1503 requires agencies to prepare evaluations of contractor performance and submit them to the Contractor Performance Assessment Reporting System (CPARS). CDC utilizes this web-based system to prepare and report contractor performance evaluations. All information contained in these assessments may be used by the Government, within the limitations of FAR 42.15, for future source selections in accordance with FAR 15.304 where past performance is an evaluation factor.

(b) The CPARS system requires a contractor representative to be assigned so that the contractor has appropriate input into the performance evaluation process. The CPARS contractor representative will be given access to CPARS and will be given the opportunity to concur or not-concur with performance evaluations before the evaluations are complete. The CPARS contractor representative will also have the opportunity to add comments to performance evaluations.

(c) The assessment is not subject to the Disputes clause of the contract, nor is it subject to appeal beyond the review and comment procedures described in the guides on the CPARS website. Refer to: www.cpars.gov for details and additional information related to CPARS, CPARS user access, how contract performance assessments are conducted, and how contractors participate. Access and training for all persons responsible for the preparation and review of performance assessments is also available at the CPARS website.

(d) The contractor must provide the CDC contracting office with the name, e-mail address, and phone number of their designated CPARS representative who will be responsible for logging into CPARS and reviewing and commenting on performance evaluations. The contractor must maintain a current representative to serve as the contractor representative in CPARS. It is the contractor's responsibility to notify the CDC contracting office, in writing (letter or email), when their CPARS representative information needs to be changed or updated. Failure to maintain current CPARS contractor representative information will result in the loss of an opportunity to review and comment on performance evaluations.

CDCH.26 Public Access To CDC Funded Digital Public Health Data (Nov 2018)

(a) Public Health Data

(1) Definition: Public Health data means digitally recorded factual material commonly accepted in the scientific community as a basis for public health findings, conclusions, and implementation

(2) When CDC is funding, in whole or in part, via a contract as defined in FAR 2.101, with respect to public health data, a CDC-approved Data Management Plan (DMP) – a plan for digital data management, sharing, and preservation is required prior to commencing any related services or work. For contracts where public health data collection or generation activities may become necessary during the period of performance (e.g. via contract modification), a DMP will be required to be submitted and evaluated during the period of performance. The DMP is a deliverable and a living document that should be updated throughout the life cycle of data. A final DMP is required at the end of the contract performance that shows where the data are deposited and how they are being made accessible or justification provided for not doing so.

(b) Data Management Plan

(1) A DMP for each collection and/or generation of public health data should include the following information:

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- (i) A description of the public health data to be collected or generated in the contract period of performance;
- (ii) Standards to be used for the collected or generated public health data;
- (iii) Mechanisms for or limitations to providing access to and sharing of the data (include a description of provisions for the protection of privacy, confidentiality, security, intellectual property, or other rights) or justification for why data cannot be made accessible. This Section should address access to identifiable and de-identified data (see below for additional information about access);
- (iv) Statement of the use of data standards that ensure all released data have appropriate documentation that describes the method of collection, what the data represent, and potential limitations for use; and
- (v) Plans for archiving and long-term preservation of the data, or explanation of why long-term preservation and access are not justified. This Section should address archiving and preservation of identifiable and de-identified data (see below for additional information regarding archiving).

(c) Examples of Data Management Plan Templates and Tools: University of California:
<https://www.cdlib.org/services/uc3/dmpt.html>

(d) Access to and Archiving of the Data

(1) To the extent that is feasible, contractors should make public health data accessible. Rights in Data clauses (FAR 52.227-14 Rights in Data – General, 52.227-16, Additional Data Requirements, FAR 52.227-17 Rights in Data – Special Works, or FAR 52.227-18 Rights in Data-Existing Works), may be applicable and incorporated into contracts, depending on the Statement of Work involved. The data rights clauses give the government “unlimited rights” in data first produced (when funded by government solely) in the performance of a contract. “Unlimited rights” is an unlimited license to use, disclose or reproduce the data; it does not give the government ownership of the data. Unlimited rights in data would allow the government to archive and make public non-proprietary data first produced in contract performance.

(2) Contracts that do not include terms for submittal of public health data to CDC, are expected to plan and prepare for providing access to, and archiving/long-term preservation of, collected and/or generated data within the contract period of performance, as set forth below. The final version of a collected and/or generated data set intended for release or sharing should be made available within thirty (30) months after the end of the data collection or generation, except surveillance data, which should be made accessible within a year of the end of a collection cycle. For public use de-identified (removal of sensitive identifiable or potentially identifiable information) datasets, an accompanying data dictionary, and other documentation relevant to use of the data set should be deposited in a sustainable repository to provide access to the data. Data that cannot be de-identified can be provided as restricted data upon request under a data-use agreement or onsite controlled use.

(3) For data underlying a scientific publication, the contractor shall make the data available coincident with publication of the paper, at a minimum a machine-readable version of the data tables shown in the paper, unless the data set is already available via a release or sharing mechanism. In addition, contractors should ensure the quality of data they make accessible and seek to provide the data in a machine readable and nonproprietary format. Contractors who fail to release public health data in a timely fashion may be subject to procedures normally used to address failure to comply with the terms and conditions of the contract and may be grounds for the Contracting Officer to terminate the contract for default. Irrespective of whether the data are made accessible or not, Public health data of value should be preserved long-term.

(4) A final DMP is required at the end of the contract performance. The final DMP will indicate the location of the deposited data and the manner of access granted to the data. There needs to be an adequate justification for not making data accessible and this justification must be documented in the DMP and approved by the Contracting Officer’s Representative.

(e) Additional information is available at <https://www.hhs.gov/open/publicaccess/index.html>.

SECTION D – Contract Documents, Exhibits, or Attachments

SOLICITATION ATTACHMENTS

Name	Description
ATT001_NPA_Glossary	Brief description and context for terms and acronyms used within the solicitation (Section 1 of PWS)
ATT002_NPA_QASP	Program-proposed performance standards and relevant acceptable quality levels to meet after SHCD (Section 4 of PWS)
ATT003_NPA_WrapUpCodes	Minimum wrap-up codes required for Program reporting on call center activities (Section 4.1.3.1 of PWS)
ATT004_Template_RAIDLog	Template for tracking risks, assumptions, issues, and decisions (Section 4.2.1.3 of PWS)
ATT005_TGD003_SemiAnnualReport	Technical guidance containing reporting requirements for the SAR (Section 4.2.2.4 of PWS)
ATT006_NPA_MandatoryTraining	Requirements for onboarding and/or annual training (Section 4.2.3.3 of PWS)
ATT007_TGD020_CommsPlan	Requirements for communications plan and approval of communications materials (Section 4.2.4 of PWS)
ATT008_BrandGuidelines	Program brand guidelines (Section 4.2.4 of PWS)
ATT009_TranslationGuide	Guide for translating key terms into Spanish, Polish, and Chinese (Section 4.2.4 of PWS)
ATT010_TGD028_DataManagement	Technical guidance related to data collection, management, and reporting (Section 4.3.3.4 of PWS)
ATT011_TransferHandbook	Requirements to coordinate clinical transfers (Section 4.4.1.2 of PWS)
ATT012_XP012_DisruptiveMember	External SOP related to disruptive member identification and reporting (Section 4.4.1.3 of PWS)
ATT013_DisruptiveMember_IncidentReport	Reporting form for incidents related to disruptive members (Section 4.4.1.3 of PWS)

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ATT014_NPA_BEASTQuestions	Benefits counseling questions for BEAST administration (Section 4.4.2.2.1 of PWS)
ATT015_TGD011_IHE_MonitoringExams	Technical guidance related to components of IHE, IME, and AME and eligibility requirements (Section 4.4.3.1 of PWS)
ATT016_ReferenceGuide_CMUM	Reference guide for case management and utilization management (Section 4.4.6 of PWS)
ATT017_XP005_PharmacyNetworkDispensing	External SOP regarding network and dispensing rules and limitations (Section 4.4.7.2 of PWS)
ATT018_XP001_PharmacyCAFA	External SOP regarding Cancer Formulary Assignment (CAFA) (Section 4.4.7.3 of PWS)
ATT019_XP007_PharmacyProviderBlock	External SOP regarding how to resolve provider blocks in the PBM Contractor's provider block portal (Section 4.4.7.4 of PWS)
ATT020_Template_BAA	Template for Business Associate Agreement agreed upon prior to award (Section 4.6.1 of PWS)
ATT021_TGD009_MonthlyReport	Technical guidance that provides essential elements that are required for the <i>MPR</i> . (Section 8.3.2.5 of PWS)
ATT022_NPA_SummaryVolumeData	Historical operational data to be used to determine level of effort and to inform NPA Contractor strategy in developing a proposal
ATT023_OAS_LaborCategories_Vol1  ATT023_OAS_Labor Categories_Vol1.docx	To be provided with Vol 1 – Tech Proposal. Labor Categories, Duties, Qualifications, and Task Areas
ATT024_OAS_FullyBurdenedRates_Vol2  ATT024_OAS_FullyB urdenedLaborRates.xlsx	To be provided with Vol 2 – Business Proposal. Labor Categories and rates for all CLINs
ATT025_NPA_QuestionsandAnswersTemplates  ATT025_NPA_Questi onsandAnswersTem	Please Provide solicitation Questions and Inquiries in this format to the Contracting Officer
Member Vital Status (TGD 012)	*To be provided after award

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Review of Medical Records Requirements (TGD 034)	*To be provided after award
Licensure Requirements (TGD 035)	*To be provided after award
Requirements for Contact of Existing Members (TGD 036)	*To be provided after award
CDC Cybersecurity Weakness Management Standards	*To be provided after award
CDC Control Catalog -Organizationally Defined Minimum Numbers and Time Periods	*To be provided after award

ATTACHMENT ATT023_LaborCategories_Vol1

LABOR CATEGORIES, DESCRIPTIONS/DUTIES, AND QUALIFICATIONS/EDUCATION
TO BE INCLUDED AS PART OF
VOLUME I—TECHNICAL PROPOSAL

The following labor categories shall be used for all firm-fixed-price (FFP) AND time-and-materials (T&M) CLINs:

Labor Categories	Description/Duties	Qualifications/Education	Identify which Specific Task Areas in the Basic PWS are Applicable to the Labor Category (e.g. Cross-Walk)

Enter the proposed labor categories and the pertinent descriptions/duties and qualifications/education for each labor category. If more convenient, Offeror may provide a separate attachment for the descriptions/duties and qualification/education columns and insert “See Attachment” in the columns. Offeror also has the option of creating his or her own spreadsheet with the same columns. If creating your own spreadsheet, you are not allowed to substitute or add additional columns. **There is no page limit for this attachment.**

Offeror shall include ALL labor categories that it identifies as necessary to perform the full range of tasks included in the contract, Section E, Performance Work Statement, and identify the specific task area(s) (e.g. cross-walk) applicable to the labor category. **There is no limit to the number of labor categories an Offeror can propose.**

ATT_024 Fully Burdened Labor Rates and Other Direct Costs- To be provided with Vol 2 – Business Proposal. Labor Categories and rates for all CLINs:

ATTACHMENT ATT024

FULLY BURDENED LABOR RATES

TO BE INCLUDED AS PART OF **VOLUME II** – BUSINESS PROPSAL

The following labor categories and labor rates shall be used for all CLINs issued under this contract.

Labor Rates

ESTIMATED HOURS & FIXED HOURLY RATES ARE AS FOLLOWS:

Base Period: Performance Period: 03/18/2026-03/17/2027		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 1 Period: Performance Period: 03/18/2027-03/17/2028		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 2 Period: Performance Period: 03/18/2028-03/17/2029		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 3 Period: Performance Period: 03/18/2029-03/17/2030		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

Option Period 4 Period: Performance Period: 03/18/2030-03/17/2031		
TITLE	ESTIMATED HOURS	FIXED HOURLY RATE
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____
		\$ _____

NOTATION REGARDING LABOR HOUR VARIANCE: Performance under this Time and Materials Contract is in accordance with FAR 52.232-7, "Payments under Time and Materials and Labor Hour Contracts," incorporated by reference in Section C, which requires the vendor to manage to the ceiling price in the contract and the ceiling price of the line items. The number of hours per labor category are estimates. The COR must be notified of any variance from the estimated hours shown in the Level of Effort/Labor Categories chart. If the variance in hours for a specific labor category line in the Level of Effort/Labor Categories chart above is 10% or more, the Contracting Officer must be notified.

SECTION E – Performance Work Statement

Performance Work Statement World Trade Center Health Program National Program Administrator

Period of performance of this Contract: March 18, 2026 to March 17, 2031. This anticipated period of performance consists of a base year and four option years.

Base Period:	March 18, 2026 to March 17, 2027
Option Period One:	March 18, 2027 to March 17, 2028
Option Period Two:	March 18, 2028 to March 17, 2029
Option Period Three:	March 18, 2029 to March 17, 2030
Option Period Four:	March 18, 2030 to March 17, 2031

SECTION 1 – BACKGROUND

On January 2, 2011, the President signed Public Law No. 111-347, the James Zadroga 9/11 Health and Compensation Act of 2010 (Zadroga Act). The Zadroga Act was amended in 2015 by Public Law No. 114-113 to provide authority and funding for the Program to continue through 2090 and further amended in 2019-2024 by Public Law No. 116-59, Public Law No. 117-328, and Public Law No. 118-31. Title I of the Zadroga Act amended the Public Health Service Act to add Title XXXIII establishing the World Trade Center (WTC) Health Program, referred to herein as the “Program,” within the U.S. Department of Health and Human Services (HHS) and is codified at 42 U.S.C. §§300mm-300mm-64.² In addition to the statutory requirements for the Program, the WTC Program Administrator (Administrator) has also promulgated regulations governing the administration of the Program at 42 C.F.R. Part 88.

The Program is a limited health benefit program, meaning that treatment is only available for specific covered health condition(s) for which exposure to airborne toxins, any other hazard, or any other adverse condition resulting from the September 11, 2001, terrorist attacks has been determined to be substantially likely to be a significant factor in aggravating, contributing to, or causing the illness or health condition in an individual member. The Program is administered by the National Institute for Occupational Safety and Health (NIOSH), part of the Centers for Disease Control and Prevention (CDC), a component of HHS, and provides medical monitoring and treatment for Program members. Members comprise Responders to the 9/11 terrorist attacks in New York City (NYC), Responders at the Pentagon and near Shanksville, Pennsylvania, and Survivors in the NYC disaster area. To receive treatment for a specific health condition, members must be both enrolled in the Program and have their health condition certified as WTC-related by the Program.

Only certain health conditions established by statute or regulation qualify for coverage, and an enrolled member must meet certain exposure, latency, and symptom-onset requirements for their health condition to be certified. Program enrollment and certification decisions are carried out by Program enrollment and certification specialists within NIOSH, at the direction of the Administrator. The Zadroga Act establishes responsibilities associated with the administration of enrollment, initial health evaluation, medical monitoring, treatment, and reimbursement for services for covered health conditions. The Program contracts with contractors to carry out specific Program functions and operations for the limited benefit plan.

The Program’s geographic area of responsibility for provision of monitoring and treatment includes all fifty U.S. states, the District of Columbia, Guam, the Commonwealth of Puerto Rico, the Northern Mariana Islands, the Virgin Islands, American Samoa, the Federated States of Micronesia, the Republic of the Marshall Islands, and the Republic of Palau.³ For members residing in the New York Metropolitan Area (NYMA),⁴ monitoring and treatment

² The Zadroga Act, as amended, can be found at:

<https://uscode.house.gov/view.xhtml?path=/prelim@title42/chapter6A/subchapter31&edition=prelim>

³ See 42 USC 201(f).

⁴ Per 42 CFR 88.1, the New York metropolitan area means the combined statistical areas comprising the Bridgeport-Stamford-Norwalk, CT Metropolitan Statistical Area; Kingston, NY Metropolitan Statistical Area; New Haven-Milford, CT Metropolitan Statistical Area; New York-Northern New Jersey-Long Island, NY-NJ-PA Metropolitan Statistical Area; Poughkeepsie-Newburgh-Middletown, NY Metropolitan Statistical Area; Torrington, CT Micropolitan Statistical Area; Trenton-Ewing, NJ Metropolitan Statistical Area, as defined in OMB Bulletin 10-02, December 1, 2009.

are administered by Clinical Centers of Excellence (CCEs) and a NYMA-based External Provider Network (EPN) administered by the Program's Third-Party Administrator (TPA) Contractor. In contrast, monitoring and treatment for members residing outside of the NYMA (but within the Program's geographic area of responsibility) are coordinated through the World Trade Center Health Program National Program Administrator (NPA)—the awardee of this Contract and referred to herein as the "NPA Contractor"—utilizing the Nationwide Provider Network (NPN) administered by the Program's TPA Contractor.

Key terms are described, and acronyms are expanded upon in ATT001_NPA_Glossary referenced in Section D.

Section 1.1 – Membership Categories

WTC Health Program members are categorized into two primary membership categories: Responders and Survivors. To enroll in the Program, Responders and Survivors must meet certain eligibility criteria described in the Zadroga Act.⁵

Responders include workers or volunteers who provided rescue, recovery, debris cleanup, and related support services on or in the aftermath of the September 11, 2001, terrorist attacks at the World Trade Center, the Pentagon, or the aircraft crash site near Shanksville, Pennsylvania. Responders are classified as FDNY, FDNY Family, General, or Pentagon/Shanksville Responders upon enrollment, based on their eligibility group. FDNY Family members are eligible for coverage of mental health conditions only, as described in the Administrative Manual.

Survivors include persons who were present in the New York City (NYC) disaster area in the dust or dust cloud on September 11, 2001, or who worked, resided, or attended school, childcare, or adult daycare in the NYC disaster area for a certain period following the September 11, 2001, terrorist attacks. Survivors are classified as "screening eligible" upon enrollment, indicating their eligibility for an Initial Health Evaluation (IHE). Screening-eligible survivors that become certified for a WTC-related health condition are then classified as "certified-eligible".

The NPA Contractor is responsible for coordinating medical monitoring and treatment for both Responder and Survivor members and ensuring provision of appropriate benefits based on the membership category. For example, Survivor members who have completed their IHE but do not have a certification are not eligible for annual monitoring, so should not be scheduled by the NPA Contractor for annual monitoring.

Section 1.2 – Selected Roles and Business Associates

The following is a non-exclusive list of other WTC Health Program contractors with which the NPA Contractor is expected to engage while carrying out Program services and functions. Unless otherwise specified, references to "other Program contractors" shall mean one or more of the contractors listed below or in Section 1.3.⁶

Clinical Center of Excellence (CCE): A NYMA-based medical facility that, in accordance with the Zadroga Act, operates under an individual contractual relationship with the WTC Health Program to provide medical monitoring, diagnosis, certification, treatment services, case management, and benefits counseling for members in the NYMA. Members residing in the NYMA or otherwise electing care through a CCE are assigned a CCE that is responsible for coordinating their care under the WTC Health Program. The Zadroga Act defines a CCE as "a Center that demonstrates to the satisfaction of the Administrator that the Center—(i) uses an integrated, centralized health care provider approach to create a comprehensive suite of health services under this title that are accessible to enrolled WTC Responders, screening-eligible WTC Survivors, or certified-eligible WTC Survivors; (ii) has experience in caring for WTC Responders and screening-eligible WTC Survivors or includes health care providers who have been trained pursuant to section 3313(c); (iii) employs health care provider staff with expertise that includes, at a minimum, occupational medicine, environmental medicine, trauma-related psychiatry and psychology, and social services counseling; and (iv) meets such other requirements as specified by the Administrator." 42 U.S.C. § 300mm-4(b)(1)(A). There are currently eight CCEs contracted with the Program: two for the Survivor population and six for the Responder population. The NPA Contractor is responsible for working with the CCEs to ensure continuity of care for members who transfer between a CCE and the NPA.

Data Center (DC): A vendor contracted with the Program that supports various functions, including receiving data from the CCEs, NPA, and TPA Contractor; developing monitoring, and treatment protocols; and facilitating WTC

⁵ See www.cdc.gov/wtc/eligiblegroups.html for more information on eligibility criteria.

⁶ The contractor descriptions provided herein are included for ease of understanding this Performance Work Statement; they are not official descriptions of the various contractors and their roles in the Program. Official definitions may be found in the Zadroga Act, as amended, codified at 42 U.S.C. §§ 300mm – 300mm-64, and Program regulations in 42 C.F.R. pt. 88.

Health Program-related research. There are currently three DCs contracted with the Program, serving the FDNY Responder, General Responder, and Survivor populations. The NPA Contractor is responsible for sending data for NPA members to the appropriate Data Center.

Pharmacy Benefits Manager (PBM): A contractor with the Program that provides comprehensive pharmacy services to the WTC Health Program for filling prescriptions at retail and mail-order pharmacies, with controls on formularies, members, and prescribers. Pharmacy benefits are administered through a pharmacy-specific call center, pharmacy claims adjudication, formulary management, pharmacy network management, and clinical reviews. The NPA Contractor is responsible for working with the PBM to ensure pharmacy benefits are provided to NPA members in a timely and appropriate manner.

Section 1.3 – Program Contracts under Transformation

The WTC Health Program is currently transforming its model for fulfilling responsibilities to administer the Program’s limited benefit health plan to members living outside of the NYMA. The transformation will be achieved by award of two new contracts, Third-Party Administrator (TPA) and National Program Administrator (NPA), which will replace current contracts, Health Program Support (HPS) and Nationwide Provider Network (NPN), respectively. The outgoing HPS and NPN contracts will expire in late 2026. The incoming TPA and NPA contracts are expected to be fully operational and ready for Start of Healthcare Delivery (SHCD) by late 2026. Under the incoming TPA and NPA contracts, provider-facing functions will generally fall under the TPA, and member-facing functions will generally fall under the NPA. A high-level summary of responsibilities is as follows:

Section 1.3.1 – Current-State Contractors (Outgoing)

Health Program Support (HPS) Contractor: The contractor who is currently contracted with the Program to perform various Program functions, including processing member enrollment and certification applications, managing mail center services for the Program (processing incoming and outgoing mail), managing the customer service call center for the Program, managing the NYMA External Provider Network (EPN) serving members within the NYMA, maintaining member eligibility data, and processing medical claims for CCE and EPN providers.

Nationwide Provider Network (NPN) Contractor: The contractor who is currently contracted with the Program to facilitate access to WTC-related care for members residing outside of the NYMA and assigned to the NPA. Tasks of the NPN Contractor include managing the Nationwide Provider Network (NPN) serving members outside of the NYMA, providing customer service and case management, processing certification submissions, providing benefits counseling, assisting with care coordination, and processing medical claims for NPN providers.

Section 1.3.2 – Future-State Contractors (Incoming)

Third Party Administrator (TPA) Contractor: A contractor with the Program that performs various Program functions, including processing member enrollment and certification applications, managing mail center services for the Program (processing incoming and outgoing mail), managing the provider services call center for the Program, managing the networks of providers serving Program members (NYMA EPN and NPN)⁷, maintaining member eligibility data, and processing medical claims for CCE, EPN-affiliated, and NPN-affiliated providers.

National Program Administrator (NPA) Contractor: A contractor with the Program that, as defined herein, ensures access to WTC-related care for members residing outside of the NYMA and assigned to the NPA (“NPA members”)⁸. Tasks of the NPA Contractor include providing customer service, case management, benefits counseling, care coordination, and processing certification submissions, for members to utilize medical monitoring and treatment benefits.

Table 1. Task Summary of Contracts Under Transformation

⁷ Under current state, the term “NPN” is used to refer both to the network of providers serving members outside of the NYMA and the contractor who administers that network. Under future state, the term “NPN” will exclusively be used to refer to the network of providers outside the NYMA, as administered by the TPA Contractor.

⁸ Under current state, the term “NPN member” is used to refer to members assigned to the NPN Contractor, who will be assigned to the NPA Contractor with this Contract. To the extent that Program materials reference members assigned to the NPA or “NPN members,” it shall be understood that the reference generally applies to NPA members.

HPS Contractor (Current/Outgoing)	TPA Contractor (Future/Incoming)
Member Enrollment Application Processing	Member Enrollment Application Processing
Member Certification Processing	Member Certification Processing
Managing NYMA External Provider Network (EPN)	Managing NYMA EPN and Nationwide Provider Network (NPN)
Mail Center for the Program, EPN-affiliated Providers	Mail Center for the Program, EPN- and NPN-affiliated Providers
Call Center for the Program	Call Center for the Program, EPN- and NPN-affiliated Providers
Maintaining Member Eligibility Data for CCEs	Maintaining Member Eligibility Data for CCEs and NPA
Processing Medical Claims for CCE and EPN-affiliated Providers	Processing Medical Claims for CCE, EPN-affiliated, and NPN-affiliated Providers
N/A	Concurrent and Retrospective UM for all Members*
NPN Contractor (Current/Outgoing)	NPA Contractor (Future/Incoming)
Call Center for NPN Members	Call Center for NPA Members
Mail Center for NPN Members and NPN-affiliated Providers	Mail Center for NPA Members
Case Management for NPN Members	Case Management for NPA Members
Submitting Certification Requests for NPN Members	Submitting Certifications for NPA Members
Benefits Counseling for NPN Members	Benefits Counseling for NPA Members
Care Coordination for NPN Members	Care Coordination for NPA Members
Utilization Management for NPN Members	Prior Authorization Review for NPA Members ^{9*}
Managing Nationwide Provider Network (NPN)	N/A
Maintaining Medical Eligibility Data for NPN Members	N/A
Processing Medical Claims for NPN-affiliated Providers	N/A

*Utilization Management activities for NPA members are divided between the TPA Contractor and the NPA Contractor. *See* Section 4.4.6.2.

SECTION 2 – PURPOSE/OBJECTIVE

The purpose of this Contract is to secure the services of an NPA Contractor. The NPA Contractor, including all subcontractors, will provide member services and medical care coordination for WTC Health Program Responders and Survivors living outside of the NYMA and assigned to the NPA. The NPA Contractor is responsible for ensuring that assigned members have timely, appropriate access to WTC Health Program benefits.

⁹ Although the Program refers to all authorizations as “prior” authorizations (PA2 or PA3); note that in limited circumstances, PA2s and PA3s may be submitted concurrently or retrospectively. The NPA Contractor is responsible for reviewing prior authorizations submitted prospectively and retrospectively. The TPA Contractor will review prior authorizations submitted concurrently to dates of service as part of concurrent UM activities.

SECTION 3 – SCOPE OF WORK

The NPA Contractor shall provide general assistance to coordinate all monitoring and treatment services that are covered by the Program's limited benefit health plan for Responder and Survivor members assigned to the NPA, as described in Section 1.1. The NPA Contractor shall facilitate appropriate provision of benefits based on the membership category. For example, Survivor members who have completed their initial health evaluation but do not have a certification are not eligible for annual monitoring, so they should not be scheduled by the NPA Contractor for annual monitoring.

The NPA Contractor shall perform all tasks, maintain all systems, and otherwise satisfy all responsibilities and requirements herein in full compliance with all terms of this Contract and applicable federal, state, and other relevant laws, regulations, policies, procedures, and WTC Health Program requirements.¹⁰

The NPA Contractor shall manage all tasks in accordance with Program guidelines and best business practices, ensuring continuous improvement and adherence to Program medical benefit guidelines and utilization and case management standards for Program members. The NPA Contractor shall provide services with the goals of improving member health and providing superior and clinically excellent member services, while also minimizing costs to the WTC Health Program.

The Program Administrative Manual, available at <https://www.cdc.gov/wtc/ppm.html>, is referenced throughout this Performance Work Statement (PWS). The Program makes every effort to ensure the Administrative Manual is current and reflective of all applicable Program rules and requirements and Program contractor roles and responsibilities. In the event this PWS is inconsistent with the Administrative Manual or other Program materials as they relate to NPA Contractor responsibilities, the following order of precedence applies: The Performance Work Statement, followed by, in order of most recent publication date, the Technical Guidance Documents, Program Administrative Manual, other attachments in Section D, and all other Program Materials.

Technical Guidance Documents (TGDs) are referenced throughout this PWS. The TGD is a tool used to communicate official direction from the Contracting Officer (CO) or the Contracting Officer Representative (COR) and is used by the Program to advise Program contractors how to implement or interpret certain contractual obligations. All TGDs will be provided after award. The NPA Contractor is required to comply with all applicable and established TGDs and any TGD revisions or new TGDs implemented during the Contract period. Contents of all TGDs are subject to change at the discretion of the Program.

SECTION 4 – TASKS TO BE PERFORMED

This Contract includes tasks to be performed during contract phases referenced as Transition-In, Sustainment, and Transition-Out, as well as Regulatory Compliance requirements.

Transition-In begins upon the award effective date and ends 12 months after the award effective date. The first eight months of Transition-In include development, testing, and validation of the NPA Contractor's systems and operations as defined under Sections 4.1.1 through 4.1.4 of the PWS. Eight months after the award effective date, the NPA Contractor shall achieve SHCD and begin Sustainment.

Handover commences immediately following SHCD, encompassing the final four months of the Transition-In phase and runs concurrently with the first four months of Sustainment. During Handover, the NPA Contractor will facilitate seamless operations between the Transition-In and Sustainment phases.

¹⁰ Applicable requirements include but are not limited to those found in Title XXXIII of the Public Health Service Act (codifying the Zadroga Act, as amended, at 42 U.S.C. §§ 300mm to 300mm-64), 42 C.F.R. Part 88, the Privacy Act (5 U.S.C. § 552a), the Health Insurance Portability and Accountability Act of 1996 (HIPAA) (Pub. L. 104–191; 42 U.S.C. § 1320d *et seq.*, as modified, and the corresponding implementing HIPAA regulations at 45 C.F.R. pts. 160, 162, and 164), the Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164), all other applicable state and Federal laws and regulations, and WTC Health Program policies and procedures, standard operating procedures, guidance documents, and the WTC Health Program Administrative Manual (available here: <https://www.cdc.gov/wtc/ppm.html>).

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Sustainment is defined as the operational time between SHCD and Transition-Out, where the NPA Contractor is solely responsible for performing all healthcare delivery tasks defined in Sections 4.2 through 4.4 of the PWS.

Transition-Out is defined as the period during which the NPA Contractor is responsible for transferring all duties and responsibilities to a Program-appointed designee, whether a government entity, another contractor, or a combination of both. Transition-Out Tasks are defined in Section 4.5 of the PWS.

Regulatory Compliance requirements specific to this Contract are defined in Section 4.6 of the PWS. This Section outlines requirements applicable to all sections of the PWS.

Throughout the performance of this Contract, the NPA Contractor shall perform all tasks at or above the expected levels defined by the PWS and the Quality Assurance Surveillance Plan (QASP) (ATT002_NPA_QASP referenced in Section D) The NPA Contractor shall coordinate with the Program and regularly communicate and interact with other Program contractors to conduct all functions outlined herein. All communications between the NPA Contractor and other Program contractors shall include the Contracting Officer's Representative (COR) or their designee.

Section 4.1 – Transition-In

The NPA Contractor shall have a Transition-In period of 12 months from the award effective date. During the first eight months of Transition-In, the NPA Contractor shall employ the necessary resources to ensure that all services and systems are fully operational at SHCD (Sections 4.1.1 through 4.1.4). The NPA Contractor shall establish SHCD and move to Sustainment eight months after the award effective date. The final four months of Transition-In (following SHCD) runs concurrently with the first four months of Sustainment and is called "Handover" (Section 4.1.5).

Section 4.1.1 – Initial Meetings

The NPA Contractor shall have an introductory call with the COR within seven calendar days¹¹ after the award effective date to establish dates for a virtual NPA Launch Meeting and an in-person Kickoff Meeting.

The NPA Launch Meeting shall occur as soon as possible but no later than 15 calendar days after the award effective date. The purpose of this meeting is to review the NPA Contractor's proposed solution(s) at a high level, establish transition-in logistics (project management tools and processes, meeting cadence), and review requirements and strategy for establishing connectivity with Program and other Program contractor systems. During this meeting, the Program will provide Program-specific documents, information, data, and instruction to further inform the NPA Contractor's approach. The Program will address Program expectations against the NPA Contractor's approach and identify areas of risk. The NPA Contractor shall produce and distribute *NPA Launch Meeting Minutes* within three calendar days after the NPA Launch Meeting.

The Kickoff Meeting shall occur after the NPA Launch Meeting and no later than 45 calendar days after the award effective date at the Program's offices in Washington, DC. The NPA Contractor shall publish a *Draft Kickoff Meeting Agenda* no later than seven calendar days prior to the Kickoff Meeting. At the Kickoff Meeting, the NPA Contractor shall present a detailed plan to meet all contractual requirements. The *Kickoff Meeting Agenda* and draft *Kickoff Presentation* shall be published no later than three calendar days prior to the Kickoff Meeting. The NPA Contractor shall create and deliver at the Kickoff Meeting a *Kickoff Presentation* that summarizes the NPA Contractor's technical solutions and how all Contract functions shall be fulfilled and operationalized within Contract parameters. The NPA Contractor shall produce and distribute *Kickoff Meeting Minutes* within three calendar days after the Kickoff Meeting.

Section 4.1.2 – Transition-In Meetings, Plans and Reporting Requirements

The NPA Contractor shall deliver detailed written documentation of their approach to Transition-In in the *Transition-In Plan (TIP)*. See Section 8.3.1.1. The *TIP* is due within 14 calendar days after the Launch Meeting and

¹¹ "Calendar day", as used in this Performance Work Statement unless otherwise specified, includes weekends and holidays. For deliverable due dates that fall on a weekend or a federal holiday, the due date is the next business day (Monday through Friday, excluding federal holidays). Federal holidays are defined in 5 U.S.C. § 6103.

will serve as the basis for executing Transition-In. The NPA Contractor shall ensure that all Program-requested changes and revisions to the Transition-In approach presented in the Launch Meeting are addressed in the *TIP*.

The NPA Contractor shall also deliver a *Transition-In Master Schedule (TIMS)* within 14 calendar days after the Launch Meeting. See Section 8.3.1.2. Throughout Transition-In, the NPA Contractor shall track Transition-In progress against the *TIMS* and maintain the *TIMS* to reflect any agreed-upon changes to the timeline.

Starting no more than seven calendar days after the Launch Meeting, and recurring weekly¹² until SHCD (unless otherwise agreed upon in writing with the Program), the NPA Contractor shall establish Transition Meetings with the Program; ad hoc meetings shall be scheduled as necessary at the request of the COR. Every two weeks during the Transition Meeting, the NPA Contractor shall deliver and present a *Transition-In Progress Report (TIPR)*. See Section 8.3.1.3. The NPA Contractor shall deliver alongside each *TIPR* an up-to-date copy of the *TIMS*; *Risks, Assumptions, Issues and Decisions (RAID) Log*; and, beginning 90 calendar days after the Contract award effective date and if updated since last submission, *Data Dictionary*. The NPA Contractor shall be able to provide an up-to-date copy of the *TIMS* and *RAID Log* at any time, within one business day after request.

Within 30 calendar days after the Launch Meeting, the NPA Contractor shall submit an *Operations Plan* consistent with the requirements in Section 8.3.2.1. The NPA Contractor shall ensure that all Program-requested changes and revisions to Sustainment approaches presented in the Launch Meeting are addressed in the *Operations Plan*. The *Operations Plan* shall be approved by the Program prior to implementation.

No later than 60 calendar days prior to SHCD, the NPA Contractor shall submit an *Operations Manual* consistent with the requirements in Section 8.3.2.2.

Section 4.1.3 – Preparation for Sustainment

The NPA Contractor shall coordinate with the Program and other Program contractors to ensure that the NPA Contractor has implemented the resources, systems, data, and other information necessary to perform the full scope of work defined in this Contract by SHCD. The NPA Contractor shall minimize the quantity and severity of risks associated with services that have been identified but not fully resolved by the outgoing NPN Contractor, including member certification requests, clinic transfers, and open member issues. The Transition-In shall cause minimal disruption to customer service and other benefits/services provided to NPA members. The NPA Contractor shall ensure that its activities during Transition-In have a minimal negative impact to the member experience and result in minimal changes to other Program contractor systems and processes, unless agreed upon in writing with the Program.

During Transition-In, the NPA Contractor shall perform the following tasks to prepare for SHCD:

1. Contract Management – Track status of transition activities, tasks, and outcomes to ensure alignment with the *TIP*, *TIMS*, and contract management section of the *Operations Plan* during Transition-In, and establish a contract management structure to ensure ongoing, effective contract management upon SHCD and during Sustainment consistent with requirements set forth in Section 4.2.
2. Communications – Execute *Transition-In Communications Plan* and prepare for implementation of the *Communications Plan* immediately upon SHCD, as described in Section 4.1.3.4 and Section 4.2.4.
3. Member Services, Infrastructure – Establish sufficient infrastructure, resources, and mechanisms to meet all requirements set forth in Section 4.3.
4. Member Services, Areas of Assistance – Ensure education, training, and structure of NPA Contractor workforce is such to enable delivery of meaningful and effective member assistance spanning all areas of assistance identified in Section 4.4.
5. Establish Readiness for SHCD – Sufficiently demonstrate Readiness for SHCD as defined in Section 4.1.4.

¹² As used in this contract, “weekly” shall mean occurring at least once every Monday through Friday period, excluding federal holidays.

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Section 4.1.3.1 – System Configuration and Authorization

The NPA Contractor shall configure its IT system(s) to be immediately able to collect and manage data elements and deliver functionalities required to meet the requirements of this PWS, as summarized in Table 2 in Section 4.3.2.1.

Within 90 calendar days after the award effective date, the NPA Contractor shall submit a *Data Dictionary*. See Section 8.3.2.3.

With regards to the Call Center (Section 4.3.1.1), the NPA Contractor shall establish incoming call routing logic and document such logic within the Operations Plan. The NPA Contractor shall configure its call center platform to capture and report on, at a minimum, the call wrap-up codes (labels that capture the reason for the engagement) required by the Program, as defined in ATT003_NPA_WrapUpCodes referenced in Section D.

The NPA Contractor shall submit an initial *Authorization to Operate (ATO) package* within 45 calendar days after the award effective date.

Section 4.1.3.2 – Initial Data Migration and Exchanges

The NPA Contractor shall establish connections to exchange data and files with the Program and other Program contractors, governed by Data Use Agreements (DUAs) and Interconnection Security Agreements (ISAs). These connections shall be secure and meet all security requirements described in Appendix A.

The NPA Contractor shall collaborate with the Program and the outgoing NPN Contractor to ensure that historical data and files are extracted, transformed, and loaded appropriately into the NPA Contractor's systems and are usable to inform NPA Contractor activities during Sustainment.

The NPA Contractor shall also collaborate with the Program and other Program contractors to establish and test bidirectional data exchanges to facilitate ongoing exchanges throughout Sustainment and Transition-Out, as required in Section 4.3.3. Incoming and outgoing data exchanges may occur daily, weekly, monthly, and/or on an ad hoc basis, and shall leverage standardized data models and queries, where possible.

Section 4.1.3.3 – Access to Other Systems

To meet all requirements of this PWS, the NPA Contractor will not only need its own systems, but will also need access to other Program systems, as described in Section 4.2.3.5.

During Transition-In, the NPA Contractor shall collaborate with the Program and applicable other Program contractors to establish and implement processes for requesting access, modifying access, and removing access for NPA Contractor staff. Management of access to other Program systems shall be incorporated into the NPA Contractor's onboarding and offboarding processes documented in the *Operations Manual*.

Section 4.1.3.4 – Communications During Transition-In

The NPA Contractor shall deliver a *Transition-in Communications Plan* within 14 calendar days after the Launch Meeting. See Section 8.3.1.4. This document will focus on the NPA Contractor's communications strategy during the first eight months of Transition-In through SHCD. Upon Program approval, the NPA Contractor shall conduct all communications activities described in the *Transition-In Communications Plan*. An updated *Transition-In Communications Plan* must be submitted at least 10 business days prior to the date a proposed change (e.g., event, new or modified *Communications Product*), would be scheduled to take effect.

The NPA Contractor shall deliver a separate *Communications Plan* within 90 calendar days after the award effective date. See Section 8.3.2.4. This document will focus on the NPA Contractor's communications strategy throughout Sustainment, with an emphasis on communications upon SHCD and through the first four months of Sustainment (Handover phase). The *Communications Plan* will serve as a living document throughout Contract performance. An updated *Communications Plan* must be submitted at least 10 business days prior to the date a proposed change (e.g., event, new or modified *Communications Product*), would be scheduled to take effect.

The *Transition-In Communications Plan* and *Communications Plan* shall ensure accurate, high-quality, and timely messaging tailored to target audiences. The NPA Contractor's communications strategy shall be multi-modal, to include mailed letters, emails, and text messages to be disseminated directly by the NPA Contractor. The NPA

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Contractor may implement a text message strategy that includes protected health information (PHI) or excludes PHI. Text messages containing protected health information (PHI), as this term is defined in 45 C.F.R. § 160.103 (e.g., 911#, member's certified WTC-related health condition), whether embedded or linked in the message, must be transmitted on a HIPAA-compliant text platform and comply applicable federal laws and regulations, including the HIPAA Security and Privacy Rules (45 C.F.R. Parts 160 and 164), as well as the Telephone Consumer Protection Act (47 U.S.C. § 227). The NPA Contractor shall not include social media or a public website in their communications strategy.

During Transition-In, the NPA Contractor shall demonstrate the ability to successfully distribute timely and accurate member communications through multiple communication methods (e.g., by phone, by mail). During Transition-In, the NPA Contractor shall also ensure that all communications products to be distributed within the first four months of Sustainment (during the Handover phase) are approved by the Program and ready for timely and accurate distribution.

Section 4.1.3.5 – Clinical Coordination and Case Transfer

The NPA Contractor shall collaborate with the outgoing NPN Contractor to transfer members in case management at the time of transition from an NPN Contractor staff member to an NPA Contractor staff member. This case transfer shall be well-coordinated to ensure services are not interrupted and shall occur on an agreed-upon timeline, starting approximately 30 calendar days before SHCD.

Section 4.1.4 – Readiness for SHCD

The NPA Contractor shall execute the *TIP* to establish full readiness for SHCD. To be deemed “ready” by Program, all services and systems shall be fully operational in such a manner that the performance requirements and standards established in this PWS and QASP will be achievable immediately by SHCD. See ATT0002_NPA_QASP, which is incorporated by reference herein as if fully set forth herein.

Section 4.1.4.1 – Testing and Validation of NPA Contractor Systems

The NPA Contractor shall test all required functions to demonstrate that operations are ready for SHCD. Testing methodology shall be documented in the *Transition-In Plan* and approved by the Program prior to commencing testing activities. The NPA Contractor has discretion over internal testing and validation review activities, methodologies, accuracy/timeliness thresholds, and procedures. The NPA Contractor's validation process shall be structured to demonstrate their ability to meet or exceed Program performance requirements and standards as outlined in this PWS and the QASP (ATT002 in Section D). The Government reserves the right to request review and validation, including roundtable exercises and other scenario-based testing, until system and process functionalities are demonstrated to the Program's satisfaction.

The NPA Contractor shall include in its validation process system compatibility tests with other Program contractors to ensure historical Program data is migrated accurately and verified to be usable, and to ensure key data exchanges are functioning appropriately and in accordance with Program rules. The NPA Contractor shall clearly communicate and coordinate testing and validation needs with the Program and other Program contractors. Tests shall assess and confirm adequate accessibility to records, accuracy of data, and the recipient's ability to use the data to correctly apply Program requirements.

Testing includes, but is not limited to, ensuring that the Member Profile can be maintained and updated, data can be collected, reported, and exchanged timely and accurately, and all other required functions can be performed by the NPA Contractor in order to comply with all aspects of the PWS. The Government prefers a mix of automated and manual testing with industry-standard volume and variety in test case scenarios to ensure readability and usability of information.

The NPA Contractor shall submit *System Testing Results* to the Program incrementally throughout the Transition-In Period in accordance with the *TIP*. The NPA Contractor shall deliver comprehensive *System Testing Results* to the Program no later than 90 calendar days prior to SHCD. This Deliverable shall provide a summary of all testing completed since the award effective date, including test scenarios used, the intended results prior to testing, and the outcomes.

Test scenarios shall include all manner of scenarios, including best-case as well as complex scenarios, to ensure errors and edge cases are also handled appropriately. The NPA Contractor shall ensure that each business and

technical requirement is tested. Individual tests deemed unsuccessful by the NPA Contractor or the Program, in accordance with the success criteria contained in the *Transition-In Plan*, shall be remediated and re-run within seven calendar days of identification. If the Program identifies unsuccessful testing that affects multiple, interrelated testing scenarios, the NPA Contractor shall, within 14 calendar days of notification by the Program, submit a detailed *Mitigation and Revised Testing Plan*.

Section 4.1.4.2 – Testing and Validation of Other Program Contractor Systems

The NPA Contractor shall participate in system testing and validation sessions facilitated by the Program or other Program contractors to validate that key data exchanges are functioning appropriately and in accordance with Program rules and that the NPA Contractor is able to complete all its tasks in the Program/other Program contractor systems. The NPA Contractor shall confirm adequate accessibility to records and its ability to use incoming data to correctly apply Program requirements. The NPA Contractor shall escalate issues identified that are known or expected to affect the NPA Contractor's ability to meet performance requirements and standards as outlined in this PWS and in the QASP.

Section 4.1.4.3 – Demonstration of Readiness for SHCD

Readiness for SHCD shall be assessed via integrated systems and process testing and document reviews to be conducted prior to SHCD for each area of responsibility. Readiness shall be demonstrated by the following:

1. **Staffing** – The NPA Contractor has established the requisite staffing, resources, and functionalities to perform all requirements under the PWS. The NPA Contractor has established a well-trained, knowledgeable, and qualified workforce capable of efficiently meeting all requirements of the Contract. The NPA Contractor has selected Key Personnel to fill leadership roles in support of Program operations and staff. The NPA Contractor has sufficient resources, including any relevant subcontractors, to meet Contract requirements in the required timeframes and to the expected performance levels outlined in the PWS and the QASP. The NPA Contractor has established that all staff have received the required and appropriate training specific to their role, can demonstrate the technical expertise to meet Contract requirements, and has the requisite access to NPA Contractor, Program, and other Program contractor systems to complete all tasks. The NPA Contractor has established an ongoing training schedule to reinforce requirements and address changes to training materials.
2. **Member Engagement and Communications** – The *Transition-In Communications Plan* has been executed, and the NPA Contractor has demonstrated the ability to successfully distribute timely and accurate member communications through multiple communication methods (e.g., by phone, by mail). The *Communications Plan* has been approved, and all *Communications Products* to be implemented within the first four months of SHCD (Handover phase) are approved by the Program and ready for timely and accurate distribution immediately upon SHCD.
3. **Member Services** – The NPA Contractor has established the requisite infrastructure, resources, and functionalities to operate a Member Service Center compliant with all performance requirements and standards set forth herein. The NPA Contractor has developed and implemented solutions, and has successfully completed end-to-end integration testing, to confirm its ability provide member services assistance in the full scope of member service areas defined herein. The NPA Contractor has attested and demonstrated the capability to deliver meaningful and effective assistance across all areas of responsibility identified in Section 4.4.
4. **Data and Management and Access** – The NPA Contractor has developed and implemented a comprehensive approach to manage all data and business information as identified and in accordance with requirements of this PWS, primarily identified in Sections 4.3.2, 4.3.3, and 4.3.4, and in accordance with regulatory compliance and security requirements in Section 4.6. The NPA Contractor has obtained an ATO and can demonstrate that all applicable Program data has been loaded and is usable in the NPA Contractor's systems. The NPA Contractor has attested and demonstrated that bidirectional data exchanges have been established and are operational and that its systems are configured and compliant to support Program operations. The NPA Contractor can successfully access the Program and other Program contractor systems identified in Section 4.2.3.5 and has the necessary permissions to complete Contract tasks.
5. **Regulatory Compliance and Security Requirements** – The NPA Contractor shall attest to and demonstrate capabilities to identify and notify the Program of HIPAA privacy and security incidents,

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including potential breaches, and potential fraud, waste, and abuse, as described in Sections 4.6.1 and 4.6.2. The NPA Contractor shall demonstrate compliance with all records management, privacy, and security requirements, as described in Sections 4.6.1, 4.6.3, and 4.6.4.

Section 4.1.5 – Handover

SHCD shall occur eight months after the Contract award effective date. The last four months of Transition-In is referred to as Handover and runs concurrently with the first four months of Sustainment. During Handover, the NPA Contractor's Transition-In team shall support the Contract to ensure a seamless transition from Transition-In to Sustainment.

Within 60 calendar days after SHCD, the NPA Contractor shall submit an interim *Transition-In Summary Report* documenting the actual Transition-In activities. Within 120 calendar days after SHCD, but prior to the end of the base period of the contract, the NPA Contractor shall submit a final *Transition-In Summary Report*. The *Transition-In Summary Report* shall summarize lessons learned during the implementation of each of the NPA Contractor's areas of responsibility; analyze issues that were realized, their root cause, and mitigation; and highlight accomplishments where risks were avoided or significantly mitigated.

Section 4.2 – Contract Management During Sustainment

At SHCD, the NPA Contractor shall move into Sustainment, the last four months concurrent with the Transition-In during the Handover period. Throughout Sustainment, the NPA Contractor shall effectively manage the Contract in accordance with the NPA Contractor's proposal and the contract management section of the *Operations Plan*, and in compliance with processes and workflows documented in the *Operations Manual*.

Section 4.2.1 – Project Management

The NPA Contractor shall engage with the COR and Program personnel in alignment with standard project management principles (i.e., Project Management Body of Knowledge) throughout performance of this Contract. The NPA Contractor shall coordinate and schedule project activities across various areas of responsibility through the life of the Contract. The NPA Contractor shall notify the CO and the COR within one business day of any change to the project schedule that alters any scheduled milestone; the notification shall be delivered to the CO and the COR in writing and shall address how the NPA Contractor plans to manage the change to complete the delayed task and mitigate impact on dependent tasks. Any Contract changes shall be made only by the Contracting Officer through properly written modification(s) to the contract. These risks and associated mitigation strategies shall be incorporated in the *RAID Log*.

Section 4.2.1.1 – Cost Tracking and Invoicing

The NPA Contractor shall develop and manage budgets for all work to be performed. Budget management includes budget development, tracking and analysis of cost, and reporting budget-related issues to the COR and the Contracting Officer (CO).

The NPA Contractor shall develop, review, and submit invoices on a monthly basis which reflect adherence to budget/funding requirements within 60 calendar days of the performance period in accordance with Health and Human Services Acquisition Regulation (HHSAR) 352.232-71 Electronic Submission of Payment Requests (Feb 2022). The invoice must be submitted using a Program-provided format (to be provided after award) and include standardized forms (SCF-1034 & SF-1035), a budget snapshot for the Option Period showing the funding received, prior period expenses, current period expenses, cumulative expenses, and remaining balance for the Option Period. Labor invoices must include names, labor category, hourly rate, hours worked, labor cost, fringe, and total monthly cost.

The NPA Contractor shall provide detailed line items and supporting documentation for all other direct costs (ODCs), and if expenses are apportioned to other programs, an explanation of the calculations used to apportion the expenses to the WTC Health Program. All travel expenses require prior written approval from the WTC Health Program and must include all expense documentation.

All invoice documentation must be submitted to the assigned COR and/or their designee by email as well as the invoicing inbox (invoice_deliverable@cdc.gov) prior to submitting the invoice in the Invoice Processing Platform (IPP) system.

Section 4.2.1.2 – Change Control Process

The NPA Contractor shall manage this Contract to ensure that required services are being performed and required Contract deliverables are being submitted in accordance with Contract-established timelines. The NPA Contractor shall maintain a change control process, documented in the *Operations Manual*, to identify, record, assess, and secure approval for changes to the project (e.g., scope, budget, schedule, milestones, or level of effort) that do not align with the submitted contract proposal. The NPA Contractor shall clearly define the purpose and impact of each change and provide relevant supporting documentation to justify the request; this includes changes requested by the Program. Any Contract changes shall be made only after the Contracting Officer's approval and only through properly written modification(s) to the contract.

To initiate a CR, the NPA Contractor shall submit the CR to the CO and the COR via email. The Program will review the request and respond within seven calendar days of receipt with either an approval or denial of the CR or a request for additional information. The NPA Contractor must receive written approval from the Contracting Officer before implementing any requested changes. The NPA Contractor shall maintain a *Change Request Log* that summarizes every change request, the business justification/impact, decision, and implementation date, if approved. The *Change Request Log* shall be submitted with the *Monthly Progress Report (MPR)* within one business day upon request.

Any Contract changes shall require a contract modification signed by the CO.

Section 4.2.1.3 – Risk and Issue Management

The NPA Contractor shall, as part of risk and issue management, coordinate with the Program to establish priority, scope, and resources for managing Program risks and issues, as well as assessing and executing mitigation strategies and contingency plans. The NPA Contractor shall implement an approach to risk and issue management that incorporates identification, tracking, mitigation, and escalation of risks and issues identified across all areas of responsibility. The NPA Contractor shall anticipate and prepare mitigation strategies for possible surge activities from members where volumes (e.g., certification requests, phone calls) exceed normal operating capacity.

The NPA Contractor shall document and report risks, assumptions, issues, and decisions to the Program via a *RAID Log* (template provided in ATT004_Template_RAIDLog) that catalogs all identified RAID items associated with the performance of the responsibilities in this Contract by the NPA Contractor or their subcontractors throughout the period(s) of performance. The NPA Contractor shall also monitor its data for indicators of risks and issues affecting the Program (such as an influx of certification requests for a condition not covered by the Program) and escalate to the Program and other Program contractors, as appropriate. The *RAID Log* shall detail any milestones, progress updates, mitigation strategies, and other relevant information associated with identified RAID items.

The *RAID Log* shall be submitted monthly with the *MPR* during Sustainment and Transition-Out. The NPA Contractor shall keep in communication with the Program by participating in ad hoc meetings to establish priority, scope, and resources for managing RAID items and report on mitigation strategies and contingency plans. Any issues that arise shall be transparent to both parties to minimize escalation and mitigate impact. The NPA Contractor shall notify the Program immediately of any urgent or significant risks and issues. The NPA Contractor shall make the *RAID Log* available to the Program within one business day upon request.

Section 4.2.2 – Sustainment Meetings, Plans, and Reporting Requirements

Section 4.2.2.1 – Meeting Requirements

Throughout Sustainment, the NPA Contractor shall conduct weekly meetings with appropriate WTC Health Program staff to provide status updates. The NPA Contractor shall provide *Meeting Agendas* at least one business day before each scheduled meeting and *Meeting Minutes* within three business days after. Minutes shall capture, at a minimum, all decision points, takeaways, and action items (with associated assignee and estimated completion date) resulting from the meeting and shall be in a consistent format.

The NPA Contractor shall also participate in both recurring and ad hoc meetings with the WTC Health Program, other Program contractors, and/or Program stakeholders that are necessary for operational success. This will include meetings to reconcile data integrity issues, coordinate communication strategy and materials, resolve member issues, discuss emerging issues and potential resolutions, and more. During these meetings, the NPA Contractor may be asked to provide subject matter expertise, collaborate with the Program and other attendees, and/or report on NPA activities. Where the NPA Contractor is designated by the Program to be the “owner” of the meeting, the NPA

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Contractor shall provide *Meeting Agendas* at least one business day before the scheduled meeting and *Meeting Minutes* within three business days after. The NPA Contractor may propose or attend additional collaborative meetings with other Program contractors, such as those to share best practices or identify potential efficiencies, upon approval from the Program. All NPA Contractor meetings with other Program contractors shall have a Program representative in attendance.

1. Medical Forum (bi-weekly).
2. Contracting Officer's Representative Meeting (monthly).
3. Case Management Forum (monthly).
4. Transfer Workgroup (quarterly).
5. Pharmacy Office Hours (monthly).
6. Pharmacy and Therapeutics Forum (quarterly).

For all of the meetings listed above, unless otherwise established by the standing meeting's charter, the Contractor shall select no more than 2 individuals (Key Personnel or their designees) who possess the authority and subject matter expertise to attend. The Contractor may request additional attendees, if needed.

The Responder and Survivor Steering Committee Meetings serve as a platform for gathering input from affected stakeholders and coordinating services for eligible individuals. These meetings are organized and managed by external stakeholders, and not by the Program itself.

Meetings are conducted in-person twice a year in New York, NY, with additional meetings held virtually. The NPA Contractor may attend these meetings upon request and with prior approval from the Program's Contracting Officer Representative (COR). This includes approval for agenda items, discussion topics, staff participation, and any necessary travel arrangements.

The NPA Contractor shall also participate in other meetings upon request. The NPA Contractor shall ensure staff is available to travel, when applicable, for in-person meetings, as described in Section 7.

Section 4.2.2.2 – Monthly Reporting

Upon SHCD, the NPA Contractor shall deliver *MPRs* to track and report on the NPA Contractor's activities, as described in Section 8.3.2.5. The *MPR* is due no later than 15 calendar days after the end of each calendar month, starting in the first month after SHCD. With each *MPR*, the NPA Contractor shall submit an up-to-date copy of the *RAID Log* and *Change Request Log*.

Section 4.2.2.3 – Quarterly Reporting

The NPA Contractor shall provide a quarterly reconciled *Staffing and Wages Report* (template to be provided after award) no later than 15 calendar days after the end of each quarter that aligns with the accepted proposal and any approved change requests to staffing levels. The report shall contain an up-to-date staff list that includes the hiring date, salary for all staff performing work on behalf of the NPA Contractor, new positions for hire, unfilled positions, and describe changes to staffing since the last report (including hiring date and/or termination date).

Section 4.2.2.4 – Semi-Annual Reporting

The NPA Contractor shall deliver a *Semi-Annual Report (SAR)* every six months, in May and November. The report shall include the elements and format set forth within TGD 003 (ATT05 in Section D). The NPA Contractor shall incorporate any feedback provided by the Program into the next scheduled report or respond in a timely manner to any comments from the Program that require a more immediate response, as directed.

The NPA Contractor shall perform semi-annual pharmacy claims reviews, using data available in the PBM Contractor's system(s), in accordance with the instructions provided with the Outlier Semi-Annual Report (OSAR) request provided by the Program. The NPA Contractor shall review outlier data (data identified to deviate from

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standard prescribing or medication-receiving patterns) to determine cost savings opportunities, inappropriate prescribing trends, compliance with clinical best practice trends, compliance with benchmarks set by the Program, and provide relevant conclusions on population health status and outcomes. On a date agreed upon by the parties, no later than 60 days after the OSAR is provided to the NPA Contractor, the NPA Contractor shall deliver an *OSAR Response* capturing their findings and, where appropriate, implement corrective actions to resolve identified risks and issues. Corrective actions resulting from *OSAR Response* findings may include implementing a point-of-sale restriction, working with prescribers to taper medications, providing member education, and more.

Section 4.2.2.5 – Annual Reporting

If exercised, within 60 calendar days after the start of each Option Period, the NPA Contractor shall submit up-to-date versions of the *Operations Plan* and *Operations Manual*.

If exercised, within 60 calendar days of the start of each Option Period, the NPA Contractor shall submit and have approved its updated *Communications Plan*. An updated *Communications Plan* must also be submitted at least 10 business days prior to the date a proposed change (e.g., event, new or modified *Communications Product*), would be scheduled to take effect.

The NPA Contractor shall review the annual Pharmacy Report Card—a report provided by the Program that summarizes prescribing and medication-receiving trends relevant to the NPA—and assess areas of opportunity to improve pharmacy utilization and oversight for NPA members. Where appropriate, the NPA Contractor shall implement corrective actions to resolve identified risks and issues.

Section 4.2.2.6 – Ad Hoc Reporting

The NPA Contractor shall provide responsive information to the Program to support timely Program response to congressional inquiries, controlled correspondence, or other external reporting requests. Where an ad hoc report is requested, it shall be delivered to the Program in a mutually agreed upon format and timeline, with mutually agreed upon contents.

As part of performance oversight, the Program may request access to WTC Health Program data and files maintained by the NPA Contractor beyond the standard reporting outlined in this PWS. Such requests will be made in writing. The NPA Contractor shall fulfill these requests on a timeline determined by the Program based on the complexity of the request.

The Program has the right to request and receive any information related to this Contract necessary to audit or formally review any of the NPA Contractor's activities regarding this Contract, regardless of whether the task has a corresponding QASP metric. Audits and formal reviews may be performed by the Program and/or their designee, without restriction. The NPA Contractor shall maintain and make available to the government (or designee) for audit, materials including call logs, call records, communications, notes, documentation, quality controls, and reporting. The NPA Contractor shall also grant government (or designee) access to the NPA Contractor's system, analytical platform, and/or data repository where necessary (e.g., security audit).

If performance issues are identified, the CO or COR will notify the NPA Contractor. The NPA Contractor shall submit an acknowledgement or rebuttal within 14 calendar days of notification and subsequently, correct any finding of noncompliance with any law, regulation, or requirement related to Program services and/or Contract terms within 30 calendar days of notification. The NPA Contractor shall deliver a corrective action plan to the COR and CO that addresses the deficiencies and describes how the NPA Contractor will prevent future recurrences. The NPA Contractor shall also submit payment or credit to the WTC Health Program for all monies attributed to the agreed-upon findings.

Additionally, the Program may provide the NPA Contractor with data and information on NPA-related activities obtained by the Program or other Program contractors and request NPA Contractor feedback and analysis. Where appropriate, the NPA Contractor shall implement corrective actions to resolve identified risks and issues.

Section 4.2.3 – Staffing Requirements

Throughout performance of the Contract, the NPA Contractor shall maintain a well-trained, knowledgeable, and qualified workforce capable of efficiently meeting all Contract requirements in the required timeframes and to the expected performance levels outlined in this PWS and the QASP. The NPA Contractor shall utilize effective hiring

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and retention strategies and manage staff in a way that limits degradation or disruption of services due to staffing changes.

Section 4.2.3.1 – Key Personnel

The NPA Contractor shall identify senior-level team member(s) who possess the authority and capability to make management decisions for the NPA Contractor and obligate NPA Contractor resources within the scope of the Contract. Specific Key Personnel and other leadership roles are listed below. This leadership team shall serve as primary liaison(s) for contract management and shall be prepared to address urgent concerns, regardless of the source of the concern, through the life of the Contract in coordination with Program staff and other Program contractors. The NPA Contractor shall ensure that qualified individuals are available to assume these leadership roles when designated leads are on leave or otherwise unable to perform their duties. These alternate leads shall also possess the authority and capability to make management decisions for the NPA Contractor.

Fully dedicated Key Personnel, individuals assigned exclusively and full-time to the Program, shall be appointed to serve in the following roles:

1. **Contract Administrator:** responsible for overseeing all operations and requirements and accountable for overall success of NPA Contractor performance. Serves as the primary point of contact between the NPA Contractor and the Program.
2. **Project Manager:** responsible for global project oversight; guides project execution and control through formal documentation of project goals, objectives, scope, risks, and timelines (as captured in *Transition-In Plan*, *Operations Plan*, *TIMS*, *RAID Log*, etc.)
3. **Clinical Director:** responsible for clinical and administrative oversight for all monitoring, diagnostics, screening and treatment services (including medications) and their adherence to medical policy, monitoring protocols, and medical necessity of rendered treatment; responsible for providing recommendations for the WTC Health Program and member population based on clinical expertise. The individual assigned to the role of Clinical Director shall be a physician (MD or DO) with an active medical license.

Each Key Personnel role must be filled by a separate person. Changes to Key Personnel shall require written consent by the CO and Contract modification, per HHSAR 352.237-75, *Key Personnel (Dec 2015)*.

Section 4.2.3.2 – Additional Leadership Roles

The NPA Contractor shall identify personnel in additional leadership roles to support the three Key Personnel in liaising with the Program. These individuals are not considered Key Personnel and therefore do not need to be fully allocated to each role nor fully allocated to the Program. The individuals filling these roles can be changed at the NPA Contractor's discretion with notice to the COR 14 calendar days prior to any planned vacancy or change:

1. **Transition-In/Transition-Out Lead:** responsible for overall Transition-In/Transition-Out.
2. **Communications Lead:** coordinates member outreach, communications, education, and engagement to ensure plain language communications and consistency in messaging, per Section 4.2.4.
3. **Quality Assurance Lead:** oversees quality monitoring, management, and improvement initiatives, per Section 4.2.5.
4. **Member Services Lead:** oversees member engagement and general member services, as described in Section 4.4.1 and overall Member Service Center operations, as described in Section 4.3.1.
5. **Medical Services Lead:** oversees care coordination; monitoring, diagnostics, and screening services; treatment eligibility determinations; treatment services; case management; and utilization management activities, as described in Sections 4.4.2 through 4.4.6.
6. **Pharmacy Services Lead:** oversees prescription management for members; troubleshoots prescription processing issues and conducts pharmacy benefits quality monitoring and reporting, per Section 4.4.7.
7. **Data Management and Exchange Lead:** oversees completion of data use and interconnection security agreements, system development and maintenance, data engineering and governance, data exchanges and integration, and continuity of operations, per Sections 4.3.2 through 4.3.4. Ensures data management aligns with records management requirements in Section 4.6.3.

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8. **HIPAA Compliance Officer:** oversees development of HIPAA privacy and security policies and procedures, to ensure compliance with HIPAA statutory and regulatory requirements, per Section 4.6.1 and the *Business Associate Agreement (BAA)*.
9. **Fraud Officer:** oversees Fraud, Waste, and Abuse (FWA) prevention, detection, reporting, and mitigation activities, per Section 4.6.2.
10. **System Security Officer:** oversees compliance and security of NPA Contractor information technology systems; ensures appropriate safeguarding of Personally Identifiable Information (PII) and PHI, per Section 4.6.4.

Section 4.2.3.3 – NPA Contractor Staff Training

The NPA Contractor shall ensure that all NPA Contractor staff have received the required and appropriate training specific to their role and can demonstrate the technical expertise to meet Contract requirements. The NPA Contractor shall conduct training on an ongoing basis sufficient to reinforce requirements and address changes to regulatory, CDC, and Program requirements and NPA Contractor-specific policies and procedures. The NPA Contractor shall ensure its workforce is familiar with the Zadroga Act and other relevant federal health laws and regulations, and maintains the knowledge and skills needed to administer a limited benefit federal healthcare program.

Prior to the delivery of training, the NPA Contractor shall ensure that *Staff Training Materials* are reviewed and approved by the Program. The NPA Contractor shall maintain records of training in a manner that can be reported, including written documentation from NPA Contractor staff attesting to completion of training. To the extent applicable to specific roles, NPA Contractor Key Personnel and clinical staff shall remain up to date on research findings funded by the Program and best practices that are established from those findings and other sources available to NPA Contractor staff.

The NPA Contractor shall provide its workforce with role-based training on NPA Contractor, Program, and other Program contractor processes and systems to support operational processes described throughout this PWS; technical training to support data and information management and continuity of operations (Section 4.3); and regulatory compliance training on HIPAA, FWA, and Records Management (Section 4.6). Trainings related to the NPA Contractor's systems and associated processes are to occur at no charge to the Government.

The NPA Contractor shall attend Program-facilitated administrative and technical trainings as well as trainings facilitated by other Program contractors aligned to the NPA Contractor's roles and responsibilities (e.g., training on PBM Contractor Portal). NPA Contractor workforce members shall also complete onboarding trainings required by the Program and/or CDC prior to starting work. Similarly, any mandatory annual trainings shall be completed by the required due dates. A list of the current mandatory training can be found in ATT006_NPA_MandatoryTraining referenced in Section D.

Section 4.2.3.4 – Other Staffing Requirements

The NPA Contractor's staffing model and organizational structure shall consist of personnel with relevant expertise and clearly defined roles and responsibilities. The NPA Contractor shall provide an efficient multi- and inter-disciplinary team of personnel at all operational levels with advanced technical expertise, health service experience, and other appropriate skills to perform the required services to meet the requirements in this PWS. The NPA Contractor's multi- and inter-disciplinary team shall include, at a minimum, mid- to high-level medical (physical and mental) and nursing professionals, licensed case managers and social workers, and other staff necessary to support program management and operations such as quality assurance and analytics. The NPA Contractor shall provide a well-defined and cost-effective mix of specialists who are knowledgeable in policies and procedures related to federal healthcare delivery.

NPA Contractor staff performing tasks that require licensure shall ensure licensure requirements are met, as described in TGD 035 (provided after award). Tasks that require active licensure include, but are not limited to:

1. Physician to attest to a positive determination on a certification request, as described in Section 4.4.4.3.
2. Social Worker to review BEAST results, as described in Section 4.4.2.2.1.
3. Social Worker or Registered Nurse to perform case management assessments and manage higher-acuity

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- members, as described in Section 4.4.6.1.
- 4. Registered Nurse to provide first level medical necessity review, as described in Section 4.4.6.2.
- 5. Physician to provide second-level medical necessity review, as described in Section 4.4.6.2.

NPA Contractor staff shall maintain working hours in the Eastern Time Zone (ET), when most work is performed for the Contract, unless otherwise approved in writing by the Government. The NPA Contractor shall maintain standard working hours and be available to attend meetings (in person or virtually) with the Program and other Program contractors between the hours of 9:00 AM and 5:00 PM ET, Monday through Friday, and extended hours as needed.

Changes to proposed staffing plans must be submitted in writing to the Program in a CR, detailing the impact to budget and justification for the change. Written approval by the Program is required prior to implementation.

Section 4.2.3.5 – NPA Contractor Staff System Access

The NPA Contractor shall ensure its staff obtain the appropriate role-based access to not only the NPA Contractor's internal systems, but also to systems hosted by the Program and/or other Program contractors, as appropriate to complete all tasks under this PWS. Though the responsibility for provisioning and maintaining appropriate access lies with the entity hosting the system, the NPA Contractor shall notify the Program of any issues or delays in accessing Program and other Program contractor system functionalities that are anticipated to impact performance of associated NPA Contractor tasks as summarized below.

WTC Health Program CARE Portal

The Centralized, Accessible, Real-Time Enterprise (CARE) Portal supports consistency in process and access to information across Program contractors. For example, the CARE Portal Library serves as a document repository to access currently published versions of controlled Program documentation. New features are anticipated throughout the Contract.

The NPA Contractor shall use the CARE Portal, where appropriate, to facilitate collaboration and coordination with the Program and other Program contractors. The NPA Contractor shall use CARE Portal features, including but not limited to the following:

- 1. The Message Center to securely send and receive PII/PHI with the Program and other Program contractors
- 2. The Transfer Module to submit or decide upon member transfer requests to and from a CCE.

TPA Contractor Portal

The TPA Contractor's Portal provides the NPA Contractor with real-time access to TPA Contractor data and files relevant to NPA members (e.g., certification requests, medical records received for medical necessity review, provider directory, prior authorization decisions and supporting documentation, claims and appeal decision statuses). The TPA Contractor's Portal also includes a reference library of Program medical policies, medical coverage guidelines, TPA Contractor system training materials, and other reference materials relevant to users of the TPA Contractor's system.

The NPA Contractor shall use the TPA Contractor's Portal to:

- 1. Inform responses to member questions related to utilization management and medical claims processing.
- 2. Identify WTC Health Program providers and prescribers to schedule NPA members for appointments, per Section 4.4.2.2.1.
- 3. Approve and deny medical prior authorization requests submitted by rendering providers, per Section 4.4.6.2.
- 4. Manage member pharmacy formulary assignment, per Section 4.4.7.3.

PBM Contractor Portal

The PBM Contractor's Portal provides the NPA Contractor with real-time access to PBM Contractor data relevant to NPA members (e.g., pharmacy claims status). The PBM Contractor Portal also includes a reference library of Program pharmacy policies, pharmacy formulary and coverage rules, PBM Contractor system training materials, and other reference materials relevant to users of the PBM Contractor's systems.

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The NPA Contractor shall use the PBM Contractor's Portal to:

1. Inform responses to member questions related to pharmacy benefits and pharmacy claims processing.
2. Review claims in Provider Block Portal to make decisions to override or uphold claim block, per Section 4.4.7.4.
3. Submit Cancer Formulary Assignment (CAFA) request for Program review, per Section 4.4.7.3.

Section 4.2.4 – Communications

The NPA Contractor shall deliver an updated *Communications Plan* within 60 days after the start of each Option Period, if exercised. See Section 8.3.2.4. Throughout Sustainment, the NPA Contractor shall conduct Communications activities in accordance with the approved *Communications Plan* and TGD 020 (Att007 in Section D). The NPA Contractor shall also ensure adherence to TGD 008, which provides direction on appropriate communication with external stakeholders (e.g., Congressional offices, members of the media, law firms).

Section 4.2.4.1 – Communications Products: General Requirements

The WTC Health Program defines a Communications Product as any written, visual, or any other material produced or utilized under this Contract for the purpose of distribution to members, providers, or any other individual or entity outside of the Program and other Program contractors. Communications Products include newsletters, announcements, presentations, fact sheets, forms, templated letters, pamphlets, and other materials created in connection with this Contract and the tasks required herein.

The WTC Health Program Communications and External Affairs Team (CEAT) will make pre-approved Communications Products available to the NPA Contractor after award and throughout the period of performance, for inclusion in the NPA Contractor's communication strategy and to promote consistency in Program-related messaging. Any edits to the Program-provided Communications Products recommended by the NPA Contractor shall be approved in writing by the WTC Health Program prior to implementation.

The NPA Contractor shall develop additional *Communications Products* where pre-existing, approved Communications Products are not available. The NPA Contractor shall ensure that all *Communications Products* developed under this Contract are:

1. Developed by staff with communications, marketing, or similar experience.
2. Tailored in content and dissemination method to target the intended audience(s).
3. Consistent with Program's communication tone and style, utilizing shared vocabulary found in existing communications products (e.g., Program website, Member Handbook).
4. Aligned with the Program's brand guidelines (ATT008 in Section D), with all proposed usage of the WTC Health Program logo to be approved by the Program.
5. Translated into Spanish, Chinese, and Polish, with translation of key terms aligning with the Program's Translation Guide (ATT0009 in Section D) and any Federal requirements.¹³
6. Reviewed and approved in writing by the CEAT prior to initial use and subsequent modification.
 - a. Concepts for new Communications Products and communications strategies may be submitted to the CEAT for consideration prior to submitting the *Communications Product* itself. This will promote more efficient review of *Communication Products* submitted for review and approval.
 - b. All modifications to a *Communications Product* require an updated approval, including using existing materials during different time periods or for purposes other than originally intended.

1) Where the intent of the *Communications Product* is to enforce Program requirements, the NPA Contractor shall ensure the content of the *Communications Product* aligns with the relevant technical documents (e.g., TGDs, Administrative Manual, Policy & Procedures, Standard Operating Procedures).

All *Communication Products* submitted by the NPA Contractor to the CEAT for review shall be publication-ready and free from editing marks, tracked changes, and grammatical or typographical errors. The NPA Contractor shall also provide, with each submission of a *Communications Product* for review, a version of the *Communication*

¹³ Under certain circumstances, to ensure urgent communication is provided timely, the Program may waive the requirement for full translation of the *Communications Product* and provide an alternative solution.

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Product that includes tracked changes, as well as context on the *Communications Product* to support an effective Program review; contextual information shall include information on the target audience, reason for the communication material, event (if any) that triggered the need for the communication material, any additional communications or touchpoints planned to address the triggering event, and other details as requested by the Program.

Upon written approval by the CEAT, the NPA Contractor shall ensure a copy of each *Communications Product* in its final, published form is provided to the Program.

The NPA Contractor shall monitor the impact of communications, such as changes in call volume or content. Where indicated, the NPA Contractor shall coordinate changes in communication strategy and products with the CEAT, with all changes documented in the *Communications Plan*.

The NPA Contractor shall perform an annual review of all *Communications Products* for continued relevancy and accuracy and deliver to the Program a complete inventory of all current *Communications Products* in use. The NPA Contractor shall also assess the overall communication strategy for communication gaps and areas of opportunity to continue to standardize outgoing communications. Proposed changes to the communication strategy, including any recommendations to retire *Communication Products* that are no longer necessary, shall be documented and submitted in an updated *Communications Plan*.

Section 4.2.4.2 – Required Communications Product Types

The NPA Contractor is responsible for five types of *Communications Products*, though additional product types may be proposed by the NPA Contractor in the *Communications Plan*.

1. NPA Member Education and Engagement

The NPA Contractor shall ensure members assigned to the NPA have a general understanding of the Program, its benefits, and the NPA's role in supporting members in navigating Program benefits. The NPA Contractor's Member Education and Engagement communication strategy shall align with its overall member engagement responsibilities detailed in Section 4.4.1.1.

The NPA Contractor shall maintain a regular cadence of communication with all NPA members, including non-participating members, to keep members informed. This shall occur at least once quarterly and include a combination of print and digital materials (e.g., physical mailings, email blast, newsletter).

2. Member Services Administration

The NPA Contractor shall ensure members assigned to the NPA understand the WTC Health Program benefits they are personally eligible for and how to utilize such benefits. The NPA Contractor's Member Services Administration communication strategy shall, as much as possible, standardize outgoing communications to balance consistency and efficiency of communications with specificity of messaging tailored to the member and their situation.

The NPA Contractor shall communicate with members upon the following events:

- a. Enrollment/Transfer-In:** The purpose of this communication is to establish the relationship between the member and the NPA Contractor.
- b. Pre-Appointment:** The purpose of this communication is to confirm details of appointments that the NPA Contractor scheduled on behalf of the member and notify members of any action items to prepare for the appointment.
- c. Post-Monitoring Appointment:** The purpose of this communication is to summarize monitoring results and notify the member of any follow-up action items.
- d. Receipt of Medical Records for Possible Certification:** The purpose of this communication is to notify the member of status and outcome of medical records review, and any action items required of the member, in accordance with Section 4.4.4.2.
- e. Post-Certification:** The purpose of this communication is to notify the member of the treatment benefits and how to initiate treatment services.

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Multiple touch points and/or multi-modal approaches (e.g., phone call *and* physical mailing) may be necessary at each event. Additionally, the above list of events is not exhaustive; additional events may be appropriate and proposed in the *Communications Plan* and implemented in coordination between the NPA Contractor and the CEAT.

3. Care Coordination with Providers

The NPA Contractor shall ensure WTC Health Program providers with whom the NPA Contractor has scheduled appointments are sent the necessary information about the member and the appointment details to facilitate a smooth appointment experience for both parties.

Though general provider education will be performed by the TPA Contractor, the NPA Contractor is responsible for communicating NPA-specific processes to providers. For example, all monitoring providers will be educated by the TPA Contractor of the requirement to send medical records received or generated during the monitoring visit to the Program; however, the NPA Contractor shall provide instruction to providers on how to submit those records directly to the NPA.

Communications Products of this type shall be coordinated with both the CEAT and the TPA Contractor to ensure consistent provider experience, as described in Section 4.4.2.1.

4. Distribution of Research Consent Materials

The NPA Contractor shall ensure WTC Health Program members are aware of the research opportunities available through the Program and shall facilitate member participation in research activities conducted by the Program and other Program contractors.

The NPA Contractor shall send research consent forms and explanatory letters to members who have neither provided research consent nor refused to provide such consent (i.e., new members, members with no response to previous outreach). The research consent materials will be provided to the NPA Contractor after award and the content and cadence of outreach will be dependent on the specific process approved by the relevant Institutional Review Board (IRB) at the respective Data Center.

The NPA Contractor shall adhere to the communication requirements set forth by the IRB. Generally, this involves sending out research consent materials to NPA members upon enrollment, after completion of each monitoring exam, and annually until the member has either provided research consent or refusal to provide such consent.

The NPA Contractor shall collaborate with each Data Center to develop a process to exchange member consent status, as described in Section 4.3.3.3, to communicate which members have responded to previous recruitment attempts to ensure that ongoing outreach attempts are limited to members who have not yet responded.

5. Ad Hoc Communications

The NPA Contractor shall be prepared to develop ad hoc communications throughout the period of performance. Ad hoc communication may be developed proactively in anticipation of known external or internal changes that may affect Program operations (e.g., new regulation, new policy), developed in response to external activities and events (e.g., spike in incoming member call volume, natural disasters), or developed as part of a change in the Program's communications strategy.

Ad hoc communication needs may be identified by either the NPA Contractor or the Program and shall be implemented in coordination between the NPA Contractor and the CEAT. As with all other *Communications Products*, ad hoc communication products need to be reviewed and approved in writing by the CEAT prior to dissemination.

Section 4.2.5 – Quality Assurance

The NPA Contractor shall perform all tasks required in this Contract in an accurate, consistent, and timely manner. The Program will evaluate the NPA Contractor on the results attained from the NPA Contractor's performance; however, in cases where limited outcome-related data are available or unpredictable situations arise, the Program will evaluate the NPA Contractor's processes or methodology. The Program's standards for minimum acceptable quality levels (both qualitative and quantitative) for performance standards are detailed in the QASP (ATT002 in

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Section D). The QASP provides a summary of how the Program intends to evaluate this performance-based Contract, including:

1. What will be monitored.
2. How monitoring will take place.
3. What Program methodology will be used for evaluating performance.
4. How monitoring efforts and results will be documented.
5. When (cadence) QASP elements are to be reported, as applicable.

The NPA Contractor shall recognize that the QASP is not intended to be comprehensive; tasks that are not detailed in the QASP are still required to be completed in accordance with this Contract. The NPA Contractor is responsible for ensuring that all performance standards are met throughout the period of performance. The Program may propose to change elements of the QASP throughout the lifetime of this Contract. Any such changes would be communicated by the Program to the NPA Contractor. The NPA Contractor would evaluate proposed changes to the QASP through the change control process described in Section 4.2.1.2 and/or if necessary, through formal modification of the Contract.

The NPA Contractor shall implement mechanisms to identify discrepancies and anomalies that may indicate potential fraud, waste, or abuse to inform FWA activities described in Section 4.6.2. For example, if the NPA Contractor discovers that exposure hours reported during the exposure assessment are inconsistent with hours reported during the enrollment process, the NPA Contractor should assess whether the discrepancies require additional investigation and justification.

Section 4.2.5.1 – Continuous Improvement

The NPA Contractor shall perform regular assessments to monitor both NPA Contractor compliance and performance in addition to assessing their activities and contributions to inform overall Program success. These assessments shall examine:

1. NPA Contractor level of effort and compliance with Contract requirements, Program performance standards, and acceptable quality levels
2. Administrative and clinical trends that may indicate changes in the WTC Health Program members' health, continuity of care, and/or stakeholder experience
3. Quality indicators related to safety, effectiveness, efficiency, accuracy, timeliness, and cost-effectiveness of monitoring, screening, diagnostics, and treatment services
4. Standards and best practices established by the WTC Health Program, the healthcare industry, and other federal payors

Assessments shall be actionable and result in continuous, proactive improvement of overall Program performance in alignment with the Program's strategic goals and priorities. Risks and issues identified shall be reported and addressed in accordance with Section 4.2.1.3. Continuous improvement initiatives shall be evaluated for effectiveness and impact.

The NPA Contractor shall implement mechanisms to obtain feedback from NPA members on the member experience and quality of services and incorporate such feedback into the NPA Contractor's continuous improvement assessments and initiatives.

Section 4.2.5.2 – Call Center Performance

The NPA Contractor shall coordinate quarterly calls with the Program to perform an audit of calls where calls are reviewed for quality, accuracy, and courtesy. The NPA Contractor shall work with the Program to identify opportunities for enhancing training, improving language, and/or updating processes to improve overall call center performance.

The NPA Contractor shall also review call center metrics, escalations, and wrap-up codes to identify trends and opportunities to streamline responses and add to the knowledge index used by the general member service team, as described in Section 4.3.1.5.

Section 4.3 – Infrastructure, Resources, and Functionalities

The NPA Contractor shall establish and maintain a Member Service Center with operational features, infrastructure, resources, and capabilities sufficient to meet minimum standards described herein.

Section 4.3.1 – Member Service Center - Public-Facing Components

The NPA Contractor shall establish and maintain a Member Service Center with sufficient infrastructure and mechanisms to establish and maintain member contact through, at a minimum, phone calls, mailed correspondence, and secure emails in members' preferred language, and provide timely and accurate information and inquiry responses during established hours of operation. Live chat and/or secure text message functionalities may also be implemented and, where implemented, shall be in compliance with minimum performance standards set forth in the QASP and with applicable federal laws and regulations (e.g., HIPAA Security and Privacy Rules, Telephone Consumer Protection Act (47 U.S.C. § 227)).

The NPA Contractor shall ensure Member Service Center staff have the resources necessary to provide knowledgeable and meaningful assistance to guide members assigned to the NPA through the continuum of care under the Program. Member Service Center staff shall be able to identify Survivor and Responder members and tailor assistance according to membership category and associated allowable benefits and requirements. Through the Member Service Center, the NPA Contractor shall provide general member assistance, as well as assistance with functional service areas identified, consistent with performance metrics set forth in the QASP (ATT002 in Section D).

Section 4.3.1.1 – Call Center

The Member Service Center shall include a Call Center that handles incoming and outgoing calls, serving as the primary touchpoint for members assigned to the NPA. Through the Call Center, the NPA Contractor shall: 1) answer routine questions about the Program, member eligibility, and benefits, 2) triage member issues to appropriate care coordination team(s) for further assistance and troubleshooting, and 3) triage members to the appropriate external party, including but not limited to the PBM Contractor, TPA Contractor, a CCE Contractor, or rendering NPN provider.

The Call Center shall be equipped with a dedicated toll-free number exclusive to WTC Health Program for incoming service calls, including telephone technology for the hearing impaired and multi-lingual support; Interactive Voice Response (IVR) system messaging that is exclusive to NPA member services; and warm transfer call routing capability for members so they do not have to disconnect from the original call to be connected to separate services. The NPA Contractor shall meet or exceed performance standards in the QASP (ATT002 in Section D) for hold times, average speed to answer, and call abandonment rate.

The NPA Contractor shall save all call recordings for a minimum of 90 calendar days and store the recordings in an environment secured to the level of their system and data types being saved. Upon request, and within two business days of such request, the NPA Contractor shall provide digital recordings of phone calls to the Program.

Section 4.3.1.2 – Mail/Document Center

Incoming Mail

The NPA Contractor shall ensure all documents received electronically and by fax, mail, or other means are date-stamped, catalogued, organized, and processed appropriately. All paper documents received shall be digitized in accordance with the National Archives and Records Administration's (NARA) digitization policies.¹⁴ The NPA Contractor shall organize and "split" documents received such that each digital file consists of a single document type, even if multiple document types are received within a single mailing/submission.¹⁵ Additionally, the NPA Contractor shall ensure that each digital file containing member-related information pertains to one specific individual only, with extraneous information removed or redacted, to reduce the risk of inappropriate disclosures of

¹⁴ See Digitization of Federal Records, at <https://www.archives.gov/records-mgmt/policy/digitization>

¹⁵ In this context, the term "document type" refers to the business function of a record and is used to categorize Program records. A list of document types will be provided after award.

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PII/PHI. Documents received shall be appropriately digitized as described above and added to the appropriate member's Member Profile within seven calendar days of document receipt.

The NPA Contractor shall manage and escalate incoming mail/documents to the appropriate processing team within seven calendar days of receipt, unless otherwise specified. This includes escalating incoming mail to the Program or other Program contractors, as appropriate based on the document type. For example, certification or treatment appeals received should be escalated to the Program's Appeal Coordinator. A list of document types and their appropriate processing team will be provided after award.

Outgoing Mail

The NPA Contractor shall ensure all outgoing documents are correctly addressed to the intended recipient(s) and are free from grammatical and typographical errors. The NPA Contractor shall ensure that members who have opted out of receiving mail from the Program (on the "Do Not Mail" list) or are believed to have inaccurate mailing information (as described in Section 4.4.1.1) are excluded from mailings, unless otherwise authorized by the Program. The NPA Contractor shall implement quality assurance checks to ensure that outgoing mail is reviewed for accuracy and applicability to targeted members.

The NPA Contractor shall leverage letter generation automation tools to the maximum extent possible, to allow for consistent customization of letters with minimal manual effort.

Electronic copies of all outgoing mail related to members and their care through the Program shall be captured in the NPA Contractor's systems and added to the appropriate member's Member Profile within seven calendar days of document creation.

Returned Mail

The NPA Contractor shall be able to identify WTC Health Program-related mail returned to the NPA Contractor and document information related to the returned mail, including information about the intended recipient, document type (welcome letter, newsletter, etc.), date sent, and date received/returned. The NPA Contractor shall flag the address in the Member Profile to prevent further mailings until such time a corrected address is identified. The NPA Contractor shall perform outreach to the member to obtain accurate member mailing address information. Corrections shall be made in the Member Profile and returned materials (and any other materials "held" due to lack of current address information) re-sent to the member once a corrected address has been identified.

Section 4.3.1.3 – Non-Telephonic Communication Channels

The Member Service Center shall be equipped to receive and respond to non-telephonic inquiries and to communicate health care-related information to members through the phone and/or in writing via email, mail, and facsimile, as appropriate and upon request by the Program and in compliance with HIPAA, records management, and security requirements in Section 4.6. The NPA Contractor shall respond to written inquiries within ten calendar days using the same medium in which the inquiry was received, unless otherwise requested by the member or provider or otherwise necessary for successful care coordination. If the inquiry cannot be fully resolved at the time of the NPA Contractor's response, the response should provide a timeline for resolution of the inquiry and any steps that may be required to address the issue. The NPA Contractor shall use best efforts to resolve the inquiry within 30 calendar days of inquiry receipt. A substantive response directly addresses the issue raised by the inquiry, providing relevant information and the most comprehensive response possible at the time. Simply confirming receipt of the inquiry or generally indicating that further investigation or action is needed, without further detail, is not a substantive response.

Transactional, one-off, communications are not considered Communications Products, and do not require formal approval prior to distribution; however, the NPA Contractor shall provide copies of transactional letters to the Program upon request and include copies within the member's record. Formal communications related to the addition or removal of benefits, shall be provided to the member in writing.

Section 4.3.1.4 – Accessibility of Member Service Center

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The Member Service Center shall be accessible between 8:00am and 7:00pm, Monday through Friday in the Eastern Time Zone (ET) excluding federal holidays, with the capability of shifting or extending operating hours based on Program need. This will ensure coverage during business hours for all geographic time zones in the U.S. mainland including Eastern Time (ET), Central Time (CT), Mountain Time (MT), and Pacific Time (PT). The NPA Contractor shall identify when members reside in the U.S. territories and accommodate partial business hours (e.g., two hours per week) for Alaskan Time (AKT), Hawaii-Aleutian Time (HT), Samoa Time (UTC-11) and Chamorro Time (UTC+10).

The Member Service Center shall be able to receive and make international calls and otherwise communicate with members living outside of the United States and its territories, as needed.

The NPA Contractor shall provide translational and interpretive services for members who are not proficient in spoken and/or written English, have vision loss, and/or have hearing loss. Translational/interpretive services shall be available based on the members' primary language/dialect, during all operational hours.¹⁶ Members shall have access to a translator/interpreter for all interactions with the NPA Contractor—whether electronic, face-to-face, or written.

Section 4.3.1.5 – Inquiry Response Structure

The Member Service Center shall implement a tiered response structure to effectively manage member issues of varying complexity. Incoming calls/inquiries shall generally be routed first through a general member service team able to handle general, standard member inquiries by application of a knowledge index.

When appropriate or required for effective and meaningful response, inquiries shall be escalated to a second tier of individuals, grouped into specialized Care Coordination Team(s), capable of handling more complex and/or time-consuming tasks. This includes tasks such as locating a new provider, making appointments, performing benefits counseling and/or case management, and assisting with certification processing and/or medical reviews. This also includes troubleshooting complex inquiries such as issues related to claims, transfers in and out from CCEs, coordination of benefits, and treatment authorization. The NPA Contractor shall ensure Care Coordination Team(s) have specialized training appropriate for tasks being handled.

The Care Coordination Team(s) shall work directly with the Program or other Program contractors to address inquiries, when necessary. For example, if an NPN provider notifies an NPA member or the NPA Contractor that they are unwilling/unable to provide services to new and/or current members, the NPA Contractor shall notify the TPA Contractor and work with the TPA Contractor to identify an alternative provider.

The Care Coordination Team(s) shall be able to receive incoming and make outgoing calls. The NPA Contractor shall assign a primary point of contact for each issue/case escalated to the Care Coordination Team(s) and set up processes to allow for direct follow-up communications between the member and care coordinator until the issue is fully resolved.

The NPA Contractor shall implement a process to track all inquiries, including those escalated to the Care Coordination Team(s), from receipt through resolution. All inquiries shall be documented in the NPA Contractor's systems, per Section 4.3.2.1, and accessible through the Member Profile and/or through reporting. The NPA Contractor shall be able to report on individual inquiry resolution details (e.g., date received, dates of outreach attempts, call notes, date resolved) and aggregate inquiry outcomes (e.g., topics, time to resolve).

The NPA Contractor shall ensure that, in situations where a caller needs to be transferred from one team to another (including from the NPA Contractor to the Program or other Program contractor), that there is a warm transfer to provide context to the receiving individual about the caller and their needs.

Section 4.3.2 – Systems, Data Collection, Management, and Reporting

The NPA Contractor shall provide a comprehensive approach to managing member data, administrative and health records, and all other data and information that comprise WTC Health Program data. WTC Health Program data is defined as all administrative and medical records related to WTC Health Program members. These records include

¹⁶ At a minimum, the NPA Contractor shall be able to support Chinese (Mandarin and Cantonese), Spanish, and Polish speakers, though a phone interpretation service that allows for a broader range of languages is preferred. Similarly, the NPA Contractor shall be able to support Chinese (Simplified and Traditional), Spanish, and Polish written translations at a minimum, though a translation service that allows for a broader range of languages is preferred.

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administrative and medical information created, collected, received/transmitted, stored/maintained, and/or otherwise managed as part of the administration of the WTC Health Program. Data can consist of documents, media, or machine-readable materials, regardless of physical form or characteristics, that are created or obtained by the WTC Health Program (whether by the federal government or through Program business vendors/contractors) conducting official WTC Health Program business. The NPA Contractor shall collect and submit such information to the Program and/or a location designated by the Program, per Sections 3304 and 3313 of the Zadroga Act.

A *Data Dictionary* shall be submitted at the end of each Option Period through Sustainment and no more than 10 business days following an addition, modification, or removal of a data element.

Section 4.3.2.1 – Required Contractor System Components

The NPA Contractor shall provide an integrated system tailored to provide the necessary functionalities to support all areas of responsibility under this Contract, as delineated in Table 2 below. The system shall accept data from and send data and files to the Program and other Program contractor systems to ensure accurate, up-to-date member information.

Table 2. List of Required NPA System Components and Functionalities

System Component	Required Functionalities
Call Center	Track inbound and outbound calls to capture qualitative (e.g., wrap-up code) and quantitative data (e.g., speed of answer, hold time, call duration) to support call center operations
Member Profile Management	Centralizes member demographic data and member documents (e.g., medical records, outgoing mail) created, collected, or received by the NPA Contractor, as described in Section 4.3.2.2
Member Eligibility Management	Tracks member health conditions, status of review, eligibility determination, and other associated information, as described in Section 4.4.4.2
Care Coordination and Case Management	Tracks incoming inquiries and ensures identified issue items are addressed Captures care coordination activities for all members (scheduled appointments, benefits counseling requests) Facilitates care planning and progress tracking for members in case management
Records Management	Stores Program documents with appropriate metadata per the requirements in Section 4.6.3
Secure Communication Pathway	Allows members to send and receive secure messages from the NPA Contractor (real-time and/or asynchronous) Allows members to submit documents securely for NPA Contractor review

The NPA Contractor's system shall consolidate all Program data and files collected through administration of this Contract, from incumbent contractors, and/or through data exchanges with the Program and/or other Program contractors into a single repository. The NPA Contractor shall be able to retrieve data and records from the repository in a standard format (e.g., .csv file) as well as access and extract metadata and develop queries to locate data and information for reporting.

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The NPA Contractor shall maintain its system such that it runs optimally and in compliance with all applicable federal laws, regulations, policies, and directives governing the security and privacy of information systems and data. This obligation includes, but is not limited to, compliance with the following:

1. Department of Health and Human Services (HHS) Information Systems Security and Privacy Policy (IS2P).
2. Centers for Disease Control and Prevention (CDC) Protection of Information Resources Policy.
3. Federal Information Security Modernization Act (FISMA) of 2014 (44 U.S.C. § 101).
4. National Institute of Standards and Technology (NIST) Special Publication (SP) 800-53, Security and Privacy Controls for Federal Information Systems and Organizations.
5. Office of Management and Budget (OMB) Circular A-130, Managing Information as a Strategic Resource.
6. Health Insurance Portability and Accountability Act of 1996 (HIPAA) (Pub. L. 104–191; 42 U.S.C. § 1320d *et seq.*).
7. Health Information Technology for Economic and Clinical Health (HITECH) Act (Pub. L. 111–5; 42 U.S.C. §§ 300jj *et seq.*).
8. HIPAA Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164).
9. Applicable HHS HIPAA policies.
10. Any other applicable federal laws, regulations, executive orders, NIST guidance, and departmental policies in effect during the term of this contract.

The NPA Contractor shall ensure that all personnel, systems, and processes involved in the performance of this contract are in full compliance with the above requirements. The NPA Contractor is responsible for monitoring updates to these authorities and implementing any changes as required.

Section 4.3.2.2 – NPA Member Profile

The NPA Contractor shall establish and maintain a member-specific and auditable system to maintain NPA Member Profiles to centralize information pertaining to members assigned to the NPA. The Member Profile shall capture critical data elements and files required to support Program operations, including the member's vital status information; demographics; contact information; Designated Representative information; Other Health Insurance (OHI) information; certifications; scheduled appointments; service authorization and utilization history; any acquired medical and pharmacy records; exposure questionnaires, medical history questionnaires, benefits counseling history, communications through the Member Service Center; and other relevant information about the member.

Where appropriate, the NPA Contractor shall leverage member engagement opportunities, as described in Section 4.4.1.1, to solicit updated Member Profile information. The NPA Contractor's system shall be able to track and report on when each data point was last verified/updated and be able to add temporary flags to indicate information that requires validation/correction.

The NPA Contractor's Member Profile system shall be accessible to all Member Service Center staff in a manner sufficient to deliver effective assistance across all areas of responsibility, including access to member records and consent forms (e.g., HIPAA authorizations, Designated Representative forms, etc.). The NPA Member Profile System shall include a records handling and storage solution sufficient to ensure that WTC Health Program records (including medical records) are maintained in a manner such that the information is secure and readily accessible to authorized parties.

Section 4.3.2.3 – Data Integrity and Governance

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The NPA Contractor shall implement data governance processes and structures to ensure the availability, consistency, integrity, and security of all the Program records in its custody. The NPA Contractor shall manage the integrity and structure of information and resulting records so that it conforms to the agreed-upon content and quality specifications for each field. The NPA Contractor shall implement prospective and retrospective data validation methodologies to efficiently identify and correct data anomalies that do not reflect Program business rules and policies. The NPA Contractor shall ensure that records are secure, and that information is consistent across sources. The NPA Contractor is encouraged to leverage existing tools/systems to ensure data integrity and governance.

The NPA Contractor shall exchange data and files with the Program and other Program contractors to ensure accuracy and consistency of member demographic information across sources and systems, as described in Section 4.3.3. The NPA Contractor shall review and reconcile updates sent by the Program and/or other Program contractors to resolve conflicts and ensure an up-to-date Member Profile, performing outreach to members as necessary to verify or update information.

The Program and other Program contractors may identify data integrity issues and notify the NPA Contractor. Data integrity issues affecting member access to care shall be corrected within two business days of notification. Information that cannot be consumed due to non-compliance with required specifications shall be corrected in a timeframe that is mutually agreed upon between the Program and NPA Contractor. Any other data integrity issue identified by the Program shall be corrected within 30 calendar days of notification. The NPA Contractor shall identify the root cause of each data integrity issue and develop a plan to prevent future reoccurrences, where appropriate.

Section 4.3.3 – Data Exchange/Integration with Program and other Program Contractors

The NPA Contractor is one of many Program contractors responsible for collecting, using, and maintaining integrity of Program data. Though the NPA Contractor is the primary entity responsible for NPA members, the Program and other Program contractors also interact with NPA members and their data. The NPA Contractor shall establish and maintain bidirectional exchange of information with the Program and other Program contractors as needed to fulfill the requirements of this Contract and ensure all relevant entities and their systems have accurate and consistent information to properly support member benefit administration and inquiry resolution.

Incoming and outgoing data and/or record/document exchanges may occur daily, weekly, monthly, or on an ad hoc basis. All data exchanges shall be made through a secure connection with an appropriate DUA and ISA, and shall leverage standardized data models and queries, where possible. Throughout the period of performance, the content, cadence, and number of data exchanges may shift depending on the Program's data storage and governance modernization efforts. The NPA Contractor shall coordinate with the Program to adapt information exchange and integration policies and capabilities accordingly.

The NPA Contractor shall implement an approach for data exchanges that includes data crosswalking, technical requirements documentation, and validations to integrate the NPA Contractor's systems with the Program and/or other Program contractors. Testing to confirm successful, accurate, and HIPAA-compliant data and record/document transmission amongst all parties shall play a critical role in assessing NPA Contractor readiness in advance of SHCD, per Section 4.1.4.

Section 4.3.3.1 – Exchange of Member Profile Elements

The NPA Contractor shall establish and automate exchanges of Member Profile elements (as described in Section 4.3.2.2) with the Program and/or their designee. NPA Contractor-collected member information shall be transmitted at least once every seven days in a mutually agreed-upon format to the Program (or designee). Similarly, the NPA Contractor shall ingest Program (or designee)-collected member information into the NPA Contractor's systems. The NPA Contractor shall review and reconcile any identified conflicts, in accordance with Section 4.3.2.3.

Data exchanges shall include, but are not limited to, exchanges of member demographic and contact information (bidirectional); OHI Information (bidirectional); and member benefit eligibility information (from Program or TPA Contractor).

Section 4.3.3.2 – List of Authorized Prescribers

The TPA Contractor is responsible for managing the Program’s provider networks (NYMA EPN and NPN). Being in the Program’s provider network, however, does not automatically mean that the provider is an authorized prescriber. The TPA Contractor will communicate the list of authorized prescribers to the PBM Contractor so that the PBM Contractor can automatically deny any pharmacy claims from providers who are not authorized prescribers. The Program will only pay pharmacy claims for medications prescribed by an authorized prescriber.

The NPA Contractor is responsible for maintaining the list of authorized prescribers for NPA members. The NPA Contractor shall determine which Program providers meet Program criteria to prescribe medication and communicate such determinations to the TPA Contractor. This list shall be provided to the Program (and/or the TPA Contractor) on a cadence and in a format mutually agreed-upon by the NPA Contractor and the Program.

The TPA Contractor will display this information within the Provider Directory to facilitate care coordination and will send Program provider and associated authorized prescriber network information to the PBM Contractor weekly to facilitate pharmacy claims adjudication.

Prescriptions processed for a medication that was not prescribed by an authorized NPA prescriber will be blocked for the NPA Contractor’s review, as described in Section 4.4.7.4.

Section 4.3.3.3 – Data and Files to Facilitate Research

While there are no research activities required by the NPA Contractor under this Contract, the NPA Contractor is responsible for providing data and files to the Program and/or their designee to facilitate the Program’s evaluation and research goals.

The NPA Contractor shall enter and/or transmit member data—such as that collected during IHEs, IMEs, AMEs, cancer screening, and other relevant member interactions—to the appropriate Data Center. Data shall be provided using Program-provided questions and data validations in a Program-provided format.

The NPA Contractor shall receive member consent status data from the appropriate Data Center, as described in Section 4.2.4.2. These data shall be integrated into the NPA Contractor’s systems and utilized to ensure that member outreach letters are only sent to members who have not yet responded.

The NPA Contractor shall support the Data Centers in additional surveillance activities, as requested by the Program, to include reviewing or provisioning NPA member medical records to the Data Center to determine if members meet established case definitions. The NPA Contractor may also need to obtain additional medical records from the member, where necessary for case confirmation.

Section 4.3.3.4 – Comprehensive Data Extract

The NPA Contractor shall provide all Program data and information, as defined in TGD 028 (ATT010 in Section D), to the Program at the end of Handover (end of Transition-In month 12) and each Option Year, at the termination of the Contract, and at any other time as requested by the Program. Program data and information include:

1. All medical and administrative records (including paper records) pertaining to Program members in the NPA Contractor’s systems.
2. Up-to-date *Data Dictionary*.
3. Complete data schema.
4. Data flow diagrams of all data exchanges with external entities that the NPA Contractor participates, describing the flow of data, the data being transferred, and directionality.
5. WTC-specific data, configurations, reference tables, process documents, and specifications.
6. Documentation relating to software and interfaces.
7. Reports and all scripts, templates, and data objects used to create the reports; business needs statement showing who requested the report and who signed off on final format and delivery.
8. Copies of all outgoing communication generated on behalf of the Program (templates are acceptable) and any scripts or automation needed to generate the results of the template. This could also include database objects.

The NPA Contractor shall extract specific data sets and documentation (e.g., case management data) on a more frequent basis, as requested.

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For purposes of this requirement, “documentation” means all operations, technical, and user manuals used in conjunction with the software, services, and deliverables, in whole or in part, that the Program determines are necessary to view and extract application data in a usable format. The NPA Contractor shall make available the documentation and all Program data in non-proprietary formats, and such file specifications and documentation as may be necessary for interpretation of these files.

This Program data shall be provided in a HIPAA-compliant manner, according to the schedule approved by the Program, and in a media and format approved by the Program. All relevant data and information must be received and verified by the Program.

Section 4.3.4 – Continuity of Operations

The NPA Contractor shall make best efforts to ensure continuity of operations and minimize disruptions to care throughout the period of performance, including during and following local, state, national, or global public health emergencies and natural or man-made disasters. During a disaster or public health emergency, the Program may administer temporary changes and waivers to medical and pharmacy benefits to ensure continuity of care. The NPA Contractor shall comply with all changes and waivers that may be administered by the Program during these times.

The NPA Contractor shall implement data security safeguards while maintaining operation of all systems, data availability and exchange, and organizational support of the Program. The NPA Contractor shall monitor systems and provide timely notification to the Program of any issues affecting delivery of services (individuals or groups); access to data and files (e.g., medical records), particularly if access issues result in disruption to care; or system uptime and accessibility. Where service delivery is impacted, the NPA Contractor shall collaborate with the Program to implement alternative solutions and communicate associated direction to members to continue care through the Program.

The NPA Contractor shall be prepared to manage temporary system unavailability and establish a communication method that shall be used to ensure minimal process disruption if/when the Continuity of Operations Plan (documented in the *Operations Plan*) is activated.

Section 4.3.4.1 – Notification of System Maintenance

The NPA Contractor shall provide a *System Maintenance Notification* to the Program at least two weeks in advance of any scheduled system maintenance. The notification shall include the system(s) affected, changes that will occur, and the date/time changes will be in effect. To the maximum extent practicable, the NPA Contractor shall schedule system maintenance outside standard business hours (e.g., weekends) to minimize disruption of services. In cases where an issue is detected, the Program recognizes that the NPA Contractor may need to perform unscheduled, responsive system maintenance. In such cases, the NPA Contractor shall notify the Program via email within one hour of the issue being detected.

Section 4.3.4.2 – Notification of System Outages

The NPA Contractor shall notify the Program within one hour of identification of any unplanned system outage that impacts Member Service Center functions and within one hour of system restoration.

When any system experiences unscheduled downtime for more than one cumulative hour in any given 24-hour period, the Program may request that the NPA Contractor conduct a *Root Cause Analysis*. The NPA Contractor shall complete such analysis and provide its findings and planned corrective actions to the COR within 10 business days of the request. The NPA Contractor shall provide the COR with a schedule to resolve any identified issues and/or implement safeguards to prevent future issues within two business days of completion of the *Root Cause Analysis*.

Section 4.4 – Areas of Assistance

Section 4.4.1 – General Member Services

The NPA Contractor shall establish and maintain customer service capabilities to support members assigned to the NPA in understanding and receiving all eligible benefits available to them through the WTC Health Program. The NPA Contractor shall support NPA members in overcoming barriers to care by providing accurate information, clearly communicating action items for the members, and clearly describing the next steps the NPA Contractor will

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take to resolve the issue. Though the NPA Contractor will not be able to independently resolve all NPA member issues, the NPA Contractor is responsible for tracking member issues through resolution and coordinating with the Program and other Program contractors to resolve issues affecting NPA members.

Section 4.4.1.1 – Member Engagement

The NPA Contractor shall implement a member engagement strategy to maintain regular contact with members and encourage participation in the Program. Member engagement activities are performed to maintain accurate information on enrolled members; educate members on their rights, responsibilities, and the monitoring, treatment, and pharmacy benefits available to them; and provide clear direction on how to access such care. Member engagement activities shall be documented in the *Communications Plan*, as described in Section 4.2.4.

The NPA Contractor's member engagement strategy shall include regional "Town Hall" events, either virtually or in-person, targeting geographic areas with high volumes of NPA members. The purpose of these events is to educate members on Program requirements and benefits, discuss Program and/or NPA changes, and foster stronger relationships between members and the NPA. The NPA Contractor shall host at least two Town Hall events each year in coordination with the Program.

New Enrollees and Transfers-In

The WTC Health Program is responsible for receiving all applications and making all enrollment decisions. On at least a weekly basis, the NPA Contractor will receive a list of newly enrolled members who selected or were automatically assigned to the NPA. The NPA Contractor shall attempt to contact all new members within three business days of receiving the list of newly enrolled members and begin assisting members with navigating the continuum of care.

The NPA Contractor will also receive member requests to transfer their Program-related care from a NYMA-based CCE to the NPA (transfer-in). In accordance with Section 4.4.1.2, the NPA Contractor shall accept or deny transfer requests. The NPA Contractor shall attempt to contact all new transfers-in within three business days of the effective date of transfer and formally begin assisting members with navigating the continuum of care.

Existing Members

The NPA Contractor shall attempt to contact all enrolled members assigned to the NPA at least once annually. Additional contact attempts may be required, depending on each member's health status and their historical utilization of services, as described in TGD 036 (provided after award).

The NPA Contractor shall ensure that staff performing these engagement activities are trained to identify indicators of potential treatment or case management needs and can refer members for formal assessment, as described in Section 4.4.6.1. The NPA Contractor shall also ensure that members are offered services available to them, including benefits counseling, cancer screening, and IHEs/IMEs/AMEs.

Maintenance of Member Profile Information

The NPA Contractor shall ensure that Member Profile information (as described in Section 4.3.2.2) is complete, accurate, and up to date at every interaction with the member. The NPA Contractor shall also be responsive to member attempts to update their information.

There are multiple ways that the NPA Contractor may be notified that a member's contact information has changed, including returned mail, phone calls, via data exchanges with other Program contractors, or other sources. In such cases, the NPA Contractor shall flag the phone number/address in the Member Profile to prevent further outreach until updated contact information is obtained. The NPA Contractor shall perform outreach to the member to obtain accurate contact information. Corrections shall be made in the Member Profile and the flag removed once corrected contact information is obtained. Members who have no valid phone numbers, email addresses, nor mailing address shall be categorized as "Lost to Follow-Up" in accordance with the process described in TGD 003 (ATT005 in Section D).

The NPA Contractor may also be notified of a member's death. In such cases, the NPA Contractor shall verify the member's vital status, if needed, and follow the process described in TGD 012 (provided after award).

Section 4.4.1.2 – Member Transfer Facilitation

Generally, WTC Health Program members are allowed to change their assigned clinic up to once a year. This is called a “transfer” and requires extensive coordination between the originating CCE/NPA (from where the member is transferring) and the destination CCE/NPA (to where the member is transferring). In some cases—such as when a transfer exception is needed, a transfer denial is being contemplated, or there are complex needs—Program involvement and/or approval may be necessary.

The NPA Contractor shall designate a specific Care Coordination Team responsible for coordinating all incoming and outgoing transfers between the NPA Contractor and WTC Health Program CCEs. The NPA Contractor’s transfer team shall coordinate each transfer in accordance with the WTC Health Program Member Transfer Handbook (ATT011 in Section D), in a manner that ensures continuity of care for the transferring member. For each incoming transfer, the NPA Contractor shall review the member’s monitoring exam history and maintain the member’s monitoring exam cycle as consistent with the previous CCE (applicable to Responders and certified-eligible Survivors only). The NPA Contractor shall also understand the member’s treatment needs, identify appropriate NPN providers to meet member treatment needs, and evaluate the member for potential case management needs prior to accepting the transfer. The NPA Contractor shall create a Member Profile for the incoming member with an effective NPA assignment date that matches the date the NPA Contractor accepted the member as an incoming transfer.

For outgoing transfers, the NPA Contractor shall provide all relevant medical and exposure records and clearly communicate to the destination CCE the members’ monitoring and treatment history and known future treatment needs. The NPA Contractor shall update the Member Profile upon the effective date of the transfer to reflect that the NPA Contractor is no longer coordinating care for the member as of the date the destination CCE accepts the member as an incoming transfer.

Section 4.4.1.3 – Disruptive Members and Members in Crisis

The NPA Contractor shall ensure their staff is trained to handle situations that may include, but are not limited to, members in crisis (i.e., threatening to hurt themselves or others, having a medical emergency) and members who exhibit disruptive behavior (e.g., inappropriate, conflict-causing, escalating, or other difficult behavior, to include chronically missing appointments). Training shall include how to identify, communicate, manage, and de-escalate crises and disruptive behavior by Program members. The NPA Contractor shall provide education to their staff upon hire and annually and ensure all employees are aware of the established protocols and procedures for addressing and managing disruptive members and members in crisis.

The NPA Contractor shall establish a process, including actionable steps, to be implemented if a member becomes disruptive; this process shall align with the Program’s Disruptive Member XP (ATT012 in Section D), which includes protocols for escalation to the Program for situational awareness, assistance, and intervention. When disruptive behavior occurs, the NPA Contractor shall submit a Disruptive Member Incident Report (ATT013 in Section D) summarizing the situation within three business days of the incident. All required fields on the form must be completed, with any relevant documentation attached to the form upon submission via the CARE Portal. The NPA Contractor shall also establish criteria and protocols to engage emergency services for medical and behavioral health crises, if such a situation is reported during communication with a member or representative.

The NPA Contractor shall track effectiveness of implemented interventions and update intervention strategy, as appropriate. If an incident requires the NPA Contractor to consider restricting care or discharge, the NPA Contractor shall indicate this on the Disruptive Member Incident Report and collaborate closely with the Program to determine next steps.

Section 4.4.1.4 – Member Access to WTC Health Program-Acquired Medical Records

The NPA Contractor shall allow members and their authorized representatives to access available medical records maintained by the NPA Contractor, to include medical records received by or created by the NPA Contractor or its

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predecessor contractors. Such records requests submitted by the member shall be fulfilled at no cost to the member, whether provided through secure electronic access or via paper. The NPA Contractor shall fulfill all records requests within 30 calendar days of request receipt, per HIPAA requirements. If a member submits a request for access to their records under the Privacy Act of 1974 (5 U.S.C. § 552a), the NPA Contractor must fulfill such requests within 10 calendar days of receipt.

The NPA Contractor shall provide member education about the limited nature of the medical records maintained by the NPA Contractor and that if the member wants a more complete medical record set, they need to request such records from the rendering provider. Where appropriate, the NPA Contractor shall assist the member in identifying historical rendering providers for WTC-related health conditions based on WTC Health Program claims history. The NPA Contractor is not responsible for facilitating the solicitation of medical records from the member's healthcare providers.

The NPA Contractor shall also facilitate Program access to member records maintained by the NPA Contractor.

Section 4.4.1.5 – Communicating Member Right to Appeal

The NPA Contractor shall ensure members understand their right to appeal certain Program decisions, including disenrollment, certification denial, decertification, or treatment denial. The NPA Contractor shall ensure members understand when the appeal right is applicable and when it is not, and the process of how to file an appeal.

Section 4.4.2 – General Care Coordination

The NPA Contractor shall implement care coordination services to encourage and support member utilization of services. The Program currently uses Benefit Plans to group and define the types of medical procedures or treatment that Program members are eligible to receive; however, this is subject to change.¹⁷ Individual members may have access to one or more Benefit Plans simultaneously. The TPA Contractor will communicate member eligibility details to the NPA Contractor.

Section 4.4.2.1 – Appointment Scheduling

The NPA Contractor is responsible for ensuring that all NPA members are given the opportunity to receive the monitoring, diagnostic, screening, and treatment benefits they are eligible for and are prompted to engage with the Program at least once annually, per Section 4.4.1.1.

NPA members generally receive their medical benefits through the Nationwide Provider Network (NPN). In some limited cases, the Program may authorize an exception for an NPA member to utilize a provider in the Program's NYMA- External Provider Network (EPN); however, for simplicity throughout the rest of this Contract, the group of WTC Health Program providers that NPA members are authorized to see will be referred to as "NPN providers." The NPA Contractor shall utilize the TPA Contractor's Provider Directory (accessible via the TPA portal) to identify NPN providers and shall coordinate members' care such that all NPA members receive Program health benefits from, and are exclusively referred to, NPN providers.

The NPA Contractor shall ensure NPN providers with whom the NPA Contractor has scheduled appointments are sent the necessary pre-appointment information and are willing and capable of performing the requested services to facilitate a smooth appointment experience for members, as described in Section 4.2.4.2.

All Program benefits coordinated by the NPA Contractor must be medically necessary to manage, ameliorate, or cure a certified WTC-related health condition or medically associated condition, and member access to medical monitoring and treatment services shall reflect allowable benefits and must comply with Program requirements.

Section 4.4.2.2 – Benefits Counseling

The NPA Contractor shall provide comprehensive benefits counseling to all members as required in Section 3305 (a)(1)(C) and (D) of the Zadroga Act.

¹⁷ See WTC Health Program Administrative Manual, Chapter 4, Section 2, at https://www.cdc.gov/wtc/ppm.html#medical_benefitPlans

Benefits counseling activities shall include providing information on WTC Health Program benefits and eligibility; determining whether members meet the requirements for Minimum Essential Coverage (MEC) as required by the Zadroga Act and, where applicable, assisting members with obtaining MEC. Benefits counseling may also include assessing needs and eligibility for external benefits, such as workers' compensation, public health insurance (e.g., Medicare or Medicaid), private health insurance (e.g., employer-based plans), and social services (e.g., housing, transportation, food, or medical services outside of those covered by the WTC Health Program); and assisting members with applying for external benefits (e.g., Supplemental Security Income (SSI)/Social Security Disability Insurance (SSDI), Public Safety Officers' Benefit, Line of Duty Injury benefits).

The NPA Contractor shall ensure benefits counseling services are provided by individuals who are trained and qualified to perform benefits counseling. The NPA Contractor shall offer benefits counseling in conjunction with the IHE, IME, and each AME and when requested by the member or as appropriate based on changes to member status that may affect benefit eligibility (e.g., certification, MEC status).

Section 4.4.2.2.1 – Benefits Eligibility Assessment and Screening

The NPA Contractor shall use the electronic Benefits Eligibility Assessment and Screening Tool (BEAST) to identify potential benefit eligibility and gaps. BEAST questions are provided in Attachment ATT014_NPA_BEASTQuestions in Section D. The NPA Contractor shall perform a benefits eligibility assessment with each monitoring exam or at least once annually. Upon completion, a licensed social worker shall review the BEAST results with the member and where indicated, create a customized action plan to address the member's needs. Where appropriate, the NPA Contractor shall refer the member to case management to promote adherence to the action plan.

BEAST assessment results, benefits counseling action plans, and benefits counseling outcomes shall be documented as part of the member's record.

Section 4.4.2.2.2 – Counseling Regarding Minimum Essential Coverage

The NPA Contractor shall document each member's health insurance status and determine if the member meets MEC requirements. If a member does not meet the requirements for MEC, the NPA Contractor shall notify the member in writing of the requirement and the potential impact to Program benefits if they do not obtain appropriate coverage. The NPA Contractor shall conduct benefits counseling to determine if the member is eligible for MEC and document when a member is offered benefits counseling and other support for obtaining MEC and declines.

If it is determined that a member does not meet the requirements for MEC, the NPA Contractor shall follow up with the member at least annually to determine if they have acquired MEC or if their eligibility for MEC has changed. If it is determined that the member may be eligible for a publicly funded health insurance benefit program, the NPA Contractor shall assist the member in submitting an enrollment application to the appropriate agency.

For any member that is in the process of obtaining MEC, the NPA Contractor shall perform periodic follow-up throughout the process to document progress and eventual outcome.

Section 4.4.2.2.3 – Counseling Regarding Workers' Compensation and Healthcare Benefits

The NPA Contractor shall assist members with applying for Workers' Compensation benefits and shall assist providers with completing Workers' Compensation paperwork. Similarly, if other sources of payment are available, the NPA Contractor shall assist members in accessing those benefits.

Section 4.4.3 – Monitoring, Diagnostic, and Screening Services

Section 4.4.3.1 – Monitoring Services

Upon enrollment, WTC Survivors are categorized as screening-eligible Survivors and are eligible for an Initial Health Evaluation (IHE). Upon enrollment, WTC Responders are eligible for an Initial Monitoring Exam (IME). The IHE (for Survivors) and IME (for Responders) is a baseline exam that assesses the member's physical and mental health. These initial exams include completion of a Medical History Questionnaire (MHQ); an exposure

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assessment; vital signs assessment; benefits eligibility assessment (as described in Section 4.4.2.2.1); physical examination; spirometry; blood and urine analyses; and an EKG and/or a chest x-ray (both, as needed).

Upon certification, WTC Survivors are categorized as certified-eligible Survivors. Certified-eligible Survivors and all Responders are eligible for ongoing Annual Monitoring Exams (AMEs). Ongoing AMEs include the same components as the IHE and IME, except for the exposure assessment. Screening-eligible Survivors who completed their IHE but were not certified for any WTC-related conditions are not eligible for an AME.

The purpose of the IHE, IME, and ongoing AMEs is to identify acute and latent health effects related to a member's exposure to airborne toxins, other hazards, or adverse conditions resulting from the September 11, 2001, terrorist attacks and to serve as an avenue for clinical data collection and analysis. More information on the IHE, IME, and ongoing AMEs can be found in TGD 011 (ATT015_TGD011_IHE_MonitoringExams in Section D).

The NPA Contractor is responsible for directly conducting the MHQ, exposure assessment, and benefits eligibility assessment; for scheduling all other components of the monitoring exam with an NPN provider; and for notifying the member of their monitoring exam results and next steps after the monitoring exam is completed in accordance with TGD 003 (ATT005_TGD003_SemiAnnualReport in Section D). The NPA Contractor shall ensure that all services provided adhere to appropriate protocols. No determination regarding certification of a health condition may be made until all components of an IHE or IME are completed.

Section 4.4.3.1.1 – Exposure Assessment

The exposure assessment is an interviewer-administered survey used to collect detailed information about a member's exposure to airborne toxins, any other hazard, or any other adverse condition resulting from the September 11, 2001, terrorist attacks. The exposure assessment considers various aspects of exposure, to include location(s), duration, timeframe, activities performed, and dust and smoke exposure. The exposure assessment also considers exposures to toxins unrelated to 9/11/2001 (e.g., exposures before 9/11/2001). NPA Contractor shall use survey instruments provided by the Program to administer the exposure assessment and shall ensure that the correct instrument is used based on the member type (Responder or Survivor).

Section 4.4.3.1.2 – Medical History Questionnaire (MHQ)

The MHQ is an interviewer-administered survey that consists of an occupational history, clinical history, and review of symptoms. The NPA Contractor shall use survey instruments provided by the Program to administer the MHQ and shall ensure that the correct instrument is used based on the member type (Responder or Survivor). The NPA Contractor shall ensure that its MHQ data collection tool can implement branch logic and pre-populate data from previous MHQ responses to minimize question repetition. The Program intends to provide the NPA Contractor with a Responder MHQ data collection tool, but the NPA Contractor is responsible for developing the Survivor MHQ data collection tool to align with the provided instrument.

Section 4.4.3.1.3 – Member Outreach to Schedule IHEs, IMEs, and AMEs

The NPA Contractor shall make at least one attempt to schedule the IHE or IME, as applicable, within the first four weeks of a member's assignment to the NPA. The NPA Contractor shall make at least three attempts to schedule the IHE or IME within the first three months of a member's assignment to the NPA.

The NPA Contractor shall make best efforts to schedule all components of the IHE, IME, and AME such that all elements are completed in a similar timeframe (e.g., two-week period).

The NPA Contractor shall make sufficient attempts to schedule each monitoring exam at a frequency to meet retention requirements outlined in TGD 003 (ATT005_TGD003_SemiAnnualReport in Section D). The NPA Contractor shall review and summarize the member's relevant medical records to ensure the monitoring exam provider has a summary report prior to the scheduled exam appointment.

Section 4.4.3.1.4 – Monitoring Exam Results

The NPA Contractor shall establish and implement a process to obtain exam results and records that are generated or obtained as part of an IHE, IME, or AME. The process shall be clearly communicated with NPN providers when appointments are scheduled, as described in Section 4.2.4.2.

The NPA Contractor shall implement tracking mechanisms to ensure records are received no later than 30 days after the date of service and notify NPN providers when records are overdue. The NPA Contractor shall also notify the Program and the TPA Contractor when an NPN provider repeatedly fails to follow the established record submission process and timelines to allow for targeted provider re-education by the TPA Contractor.

The NPA Contractor shall make a good faith attempt to obtain medical records and test results from a monitoring exam provider on a timeline that facilitates members receiving their monitoring exam results within 15 business days of the monitoring exam, as described in TGD 003 (ATT005_TGD003_SemiAnnualReport in Section D). The NPA Contractor shall notify the member of monitoring exam results within 14 calendar days of the results being made available to the NPA Contractor.

The NPA Contractor shall review monitoring exam results to determine certification eligibility, as described in Section 4.4.4.2.2.

Section 4.4.3.1.5 – Additional Initial Health Evaluations

Survivor members who have completed their IHE but did not receive any certifications shall be counseled on how to notify the NPA Contractor of health conditions that may develop in the future, as described in Section 4.4.4.5. Such members shall also be counseled on their ability to self-pay for additional health evaluations, if necessary. The NPA Contractor shall assist the member in scheduling the additional health evaluations to facilitate member access to each of the components of the IHE, where possible.

Section 4.4.3.2 – Diagnostic Services

The WTC Health Program covers diagnostic services when evaluating a member's symptoms to determine whether the member has a WTC-related health condition. Screening-eligible Survivors are eligible for diagnostic services for a limited time during and after the IHE, per TGD 011. Responders and certified-eligible Survivors are eligible for diagnostic services with no time limitation.

The NPA Contractor shall coordinate diagnostic services for members when monitoring exam results suggest a need to continue testing for a conclusive diagnosis that may result in a WTC-related certification. Appropriate and necessary diagnostic testing should continue until there is sufficient information for the NPA Contractor to either submit a certification request or determine the health condition is not related to the member's 9/11 exposure. The NPA Contractor shall not coordinate diagnostic services after it is determined that the member will not be certified for a WTC-related health condition.

Diagnostic services may include specialty consultation and/or temporary diagnostic treatment to enable differential determination of a covered health condition, or to determine the health condition is not WTC-related or medically associated with a WTC-related health condition.

Section 4.4.3.3 – Cancer Screening Services

The WTC Health Program's coverage of cancer screening services is based upon guidelines recommended by the U.S. Preventive Services Task Force (USPSTF). The Program only covers cancer screening services with a USPSTF Grade A or B recommendation. Screening-eligible Survivors, Responders, and certified-eligible Survivors are eligible for cancer screenings as outlined in Chapter 4 of the Administrative Manual. The NPA Contractor shall provide outreach to members and assistance scheduling cancer screenings for which they are eligible.

Section 4.4.4 – Treatment Eligibility Determinations (Certifications)

The NPA Contractor is responsible for reviewing NPA members' available medical records, tracking their health conditions, and determining whether members' health conditions meet WTC Health Program criteria for certification.

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The NPA Contractor shall make best efforts to identify all member health conditions that meet WTC Health Program criteria for certification and submit timely requests for certification to the WTC Health Program. Once their health condition is certified by the Administrator of the WTC Health Program, members become eligible for treatment of their certified condition under the Program.

Section 4.4.4.1 – Medical Records Review for Certification Eligibility

To submit a certification request, the NPA Contractor must first obtain and review member medical records. Generally, member medical records are submitted to the NPA Contractor in one of three ways:

1. **After Initial Health Evaluation (IHE)/Annual Monitoring Exam (AME).** Upon completion of the IHE/AME, the NPA Contractor shall obtain a copy of IHE/AME results as well as copies of any historical medical records brought by the member to the IHE/AME visit.
2. **After Diagnostics.** Upon completion of any diagnostic services coordinated by the NPA Contractor, the NPA Contractor shall obtain a copy of the diagnostic results.
3. **Direct Submission from Member.** If additional medical records are needed to determine certification eligibility, the NPA Contractor shall provide instructions to the member on specific medical records needed and how to submit the records directly to the NPA Contractor.

In all cases, the NPA Contractor shall attach the medical records received to the correct Member Profile, and review NPA member-submitted medical records to assess if the following WTC Health Program requirements for a valid certification request are met:¹⁸

- 2) The member's health condition is a covered condition on the List of WTC-Related Health Conditions;¹⁹
- 3) The member's health condition and 9/11-related exposures meet all relevant WTC Health Program policies and criteria for certification, such as maximum time intervals, cancer latency periods, and any applicable requirements;²⁰ **AND**
- 4) The member's exposure to airborne toxins, any other hazard, or any other adverse condition resulting from the September 11, 2001, terrorist attacks, is substantially likely to be a significant factor in aggravating, contributing to, or causing the illness or health condition or mental health condition.

The third criterion requires a physician to assess the individual's 9/11-related exposures, the type of symptoms experienced by the individual, and the temporal sequence thereof to make a physician determination that the member's 9/11-related exposures are sufficient to establish the required positive association with the health condition (i.e., that the exposures are substantially likely to be a significant factor in aggravating, contributing to, or causing the illness or health condition).

Section 4.4.4.2 – Identifying Health Conditions and Tracking Certification Eligibility

The NPA Contractor shall track member health conditions identified through medical records received from IHE/AME providers, diagnostics providers, and directly from the member. The NPA Contractor shall document, within its systems, the conditions that have been identified for the member, origin of the condition identified (e.g., IHE/AME, diagnostics, member), status of certification review (e.g., in progress, awaiting additional information, submitted to NIOSH, NIOSH approved, NIOSH denied), and NPA determination on whether certification criteria has been met (i.e., invalid certification, valid certification).

The NPA Contractor shall, upon receipt and review of medical records for possible certification, send letters to members to notify them of review status and next steps, per Section 4.2.4.2.

If records received are insufficient to make a determination of whether the health condition meets certification criteria, the NPA Contractor shall notify the member of the additional information that would be needed.

¹⁸ See generally WTC Health Program Administrative Manual, Chapter 3, Section 5, at https://www.cdc.gov/wtc/ppm.html#certification_physician

¹⁹ See List of WTC-Related Health Conditions eligible for coverage by the Program at 42 C.F.R. § 88.15.

²⁰ Additional requirements may be described in WTC Health Program documents, including *Policies and Procedures*, Medical Coverage Determinations, and the WTC Health Program Codebook.

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If a condition is determined to not meet Program requirements and therefore no certification request can be submitted to the Program, the NPA Contractor shall:

1. Document the member's health condition in the NPA Contractor's system(s).
2. Direct the member to their primary care provider for further evaluation and treatment.
3. Inform the member in plain language of the reasons why a request for certification of their condition cannot be submitted—citing Program policy where appropriate—and that the member does not have an appeal right with the WTC Health Program at this stage.
4. Clearly detail, in writing, potential next steps (e.g., secondary review)

Section 4.4.4.2.1 – Member Requests for Review of Certification Eligibility

NPA members may contact the NPA Contractor seeking certification for WTC-related health condition(s) previously diagnosed. To certify these conditions, the NPA Contractor must receive sufficient medical records to confirm the members' health condition(s)—within the context of their exposures to toxins—meet WTC Health Program certification criteria. The NPA Contractor shall establish processes for managing incoming inquiries from NPA members requesting the review of medical records for potential certifications of health condition in accordance with TGD 034 (provided after award).

The NPA Contractor shall ensure the Call Center is able to answer questions about the overall certification process and the records that must be submitted and escalate more complex questions to the applicable Care Coordination Team, as described in Section 4.3.1.5. The NPA Contractor shall follow up with written guidance describing the medical records or other information needed to certify the member's health condition.

The NPA Contractor shall provide general member education on how to request medical records from their providers and how to submit medical records to the NPA Contractor to initiate a medical records review for certification eligibility. The NPA Contractor is not responsible for ensuring members submit medical records following an incoming inquiry about the certification process. The NPA Contractor is also not responsible for facilitating the solicitation of medical records from the member's healthcare providers for services that were not provided through the WTC Health Program.

Section 4.4.4.2.2 – Review of Test Results and Medical Records from Monitoring Exams

NPN providers are responsible for sending the NPA Contractor the exam results and records generated or obtained as part of an IHE, AME, or diagnostics appointment, per Section 4.4.3.1.4.

The NPA Contractor shall establish and implement a process to review such results to determine certification eligibility and request additional records from members as needed, per Section 4.4.4.2.

Section 4.4.4.3 – Conditions that Meet Certification Criteria

The NPA Contractor shall submit valid certification requests, defined as requests that meet all criteria listed in Section 4.4.4.1, to the Program using the Program's Certification Request (WTC-3) Form (provided after award).

The NPA Contractor shall ensure the WTC-3 form is filled out comprehensively and that all supporting clinical and exposure documentation is submitted with the certification request. The NPA Contractor shall ensure certification requests meet all clinical and administrative Program requirements prior to submission to the Program, including those set forth in TGD 005 (provided after award), the Administrative Manual, and Program Policies and Procedures. The NPA Contractor shall establish a quality assurance process to ensure the certification requests submitted to the Program meet all the Program certification requirements.

All certification requests must include a signed affirmative statement by an NPA physician attesting to a positive determination that the member's exposure to airborne toxins, any other hazard, or any other adverse condition resulting from the September 11, 2001, terrorist attacks, is substantially likely to be a significant factor in aggravating, contributing to, or causing the illness or health condition or mental health condition.

The NPA Contractor shall not submit certification requests known by the NPA Contractor to be invalid. Certification requests received that are invalid, illegible, or missing requiring information will be deemed

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“Unprocessable” and sent back to the NPA Contractor for resubmission. The NPA Contractor shall review certification requests returned as “Unprocessable” and where applicable, resubmit them as soon as possible.

The Program will review all valid certification requests submitted by the NPA Contractor and will either approve or deny the certification request. Occasionally, the Program will request a “Redo.” The NPA Contractor shall review the “Redo” request rationale and resubmit the certification request with additional justification, context, or medical records, as requested; this shall be done as soon as possible, but no later than 60 calendar days after the “Redo” is requested. If the NPA Contractor does not respond to the “Redo” request within 60 calendar days, the Program will administratively close the certification request.

Once a condition is certified by the Program, the NPA Contractor shall perform outreach to the member to determine if treatment services are needed and initiate treatment services in accordance with Section 4.4.5.

Section 4.4.4.4 – Certification Secondary Reviews

When an NPA physician is unable to submit a request for certification of the member’s health condition due to finding that exposure to airborne toxins, any other hazard, or any other adverse condition resulting from the September 11, 2001, terrorist attacks, is *not* substantially likely to be a significant factor in aggravating, contributing to, or causing the health condition or mental health condition, it is called a Negative Physician Determination.

Members may request a secondary review of an NPA physician’s Negative Physician Determination by writing the NPA Clinical Director or their designee.

The NPA Contractor shall establish a process to utilize a qualified physician (other than the physician who rendered the initial Negative Physician Determination) to perform the secondary review.²¹ The secondary review must be completed, and the member notified of the reviewer’s findings, within 30 calendar days of receiving the member’s request for secondary review. The member is eligible to receive further diagnostic testing and diagnostic care if deemed necessary by the secondary reviewer (and documented in the Member Profile) during this 30-day time frame.²²

Section 4.4.4.5 – Survivor Members Without Certifications

If the NPA Contractor determines that a screening-eligible Survivor does not have any certifiable conditions, the NPA Contractor shall notify the member in writing of their monitoring exam results and their benefits status. The NPA Contractor shall also counsel the member regarding how to access Program benefits if conditions develop in the future, explain self-pay requirements for subsequent exams, and notify the member that all services, aside from appropriate cancer screening (as described in Section 4.4.3.3), are discontinued until such time that the member develops certifiable WTC-related health conditions.

Section 4.4.5 – Treatment Services

Generally, Program members are eligible for medically necessary treatment for their certified WTC-related health conditions, health conditions medically associated with a certified WTC-related health condition, and ancillary conditions.

Once a health condition is certified by the Program, the NPA Contractor shall perform prompt outreach to the member to determine if the member requires treatment and initiate treatment services. The NPA Contractor shall make best efforts to ensure timely access to and continuity of care. Considerations shall include treatment urgency, wait times for the provider, travel distance/time for the member, and provider qualifications.

The NPA Contractor shall ensure that members are educated on the limitations of the Program’s coverage and shall ensure that members are only referred to providers who are in the Program’s provider networks. The NPA Contractor shall ensure that all treatment coordinated by the NPA Contractor aligns with Program guidance and is reasonable and appropriate based on medical necessity principles, acuity level, evidence-based practice, scientific

²¹ If the NPA Contractor does not have multiple physicians on staff, the Program can assist in identifying qualified physicians (likely via a CCE Contractor) to support certification secondary reviews.

²² Additional detail and requirements pertaining to secondary review can be found in Chapter 3, Section 5 of the Administrative Manual at https://www.cdc.gov/wtc/ppm.html#certification_physician

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evidence, professional standards of care, or other relevant information. The authorized services, service limitations, and services that require prior authorization for reimbursement are found in Program documents, including the Administrative Manual, Policies and Procedures, Medical Coverage Determinations, Reference Guides, External Standard Operating Procedures, Clinical Guidance Documents, and Technical Guidance Documents.

Section 4.4.5.1 – Coordination of Benefits for Survivor Members

The WTC Health Program is required to perform coordination of benefits for Survivor treatment services and prescriptions.²³ The NPA Contractor shall facilitate compliance with this requirement by ensuring the following requirements are met when scheduling Survivor members for treatment:

1. Collect and verify current and comprehensive health benefit coverage information to include both medical and pharmacy insurance in a Program-required format (provided after award). Document same in the Member Profile described in Section 4.3.2.2.
2. Ensure rendering provider is in-network with both the member's personal health benefit coverage and the NPN and understands requirements for coordination of benefits.

Sometimes, a referral to a provider in both the member's OHI network and the NPN network is not possible. In these cases, the NPA Contractor shall notify the Program and collaborate with the Program and the TPA Contractor to determine next steps.

Section 4.4.5.2 – Treatment Pending Certification

In limited cases where the NPA Contractor determines that a member requires urgent treatment for a health condition for which certification has been requested but which has not yet been certified, the NPA Contractor may request "Treatment Pending Certification" to provide timely institution of therapy or continuity of care while a certification request is being reviewed by the Program. Authorization of Treatment Pending Certification is permitted pursuant to Section 3312(b)(5) of the Zadroga Act and 42 C.F.R. § 88.20(c).

Treatment Pending Certification requests may only be submitted with a completed WTC-3 form, after the NPA Clinical Director has determined that the member meets all certification criteria. Upon submission of the WTC-3 form with a request for Treatment Pending Certification, the NPA Contractor may begin scheduling the member for treatment services. Services provided under the Treatment Pending Certification authority shall meet the same Program standards and limitations as other treatment services.

Section 4.4.5.3 – Non-Emergency Transport

The Program may cover expenses for non-emergency transportation services for NPA members who need to travel over 250 miles roundtrip from their place of residence to secure medically necessary treatment for a certified condition from a Program-affiliated provider.²⁴

In such cases, the NPA Contractor shall submit a prior authorization request to the Program for approval using the TPA Contractor's system, as described in Section 4.4.6.2. Upon approval and prior to scheduled travel, the NPA Contractor shall educate the member on allowable expenses and how to submit expenses for reimbursement.

After completion of travel and receipt of the member's submitted expenses, the NPA Contractor shall review submitted documentation (e.g., invoice, receipts) to ensure compliance with TGD 025 (provided after award). After verifying that documentation is accurate and complete, the NPA Contractor shall submit the documentation to the TPA Contractor for processing and reimbursement.

²³ See <https://www.cdc.gov/wtc/policies.html#payer-guidance> for more information on payer order for treatment services.

²⁴ See <https://www.cdc.gov/wtc/policies.html#transportation> for more information on non-emergency transportation requirements.

Section 4.4.5.4 – VCF Disability Evaluations

With requisite approval from the Department of Justice’s September 11th Victim Compensation Fund (VCF), and only after requested by the Program, the NPA Contractor shall coordinate disability evaluations for members on behalf of the VCF. The goal of the evaluation is to assist the VCF in determining whether a member who reports that they are partially or fully disabled due to a certified WTC-related health condition—but does not have a disability finding from a third party, such as workers’ compensation or the Social Security Administration—has sufficient evidence to support their claim. A VCF-approved form to be completed by the evaluating provider will be provided to the NPA Contractor after award.

When requested by the Program, the NPA Contractor shall identify and coordinate an evaluation with an NPN provider with the necessary skills to conduct such an evaluation by providing the NPN provider with the VCF-approved form and instructions on how to submit the claim. The NPA Contractor shall inform the evaluating provider that, in evaluating the member, the expectation is that no additional testing or imaging is required unless medically necessary to assess the extent of the member’s disability due to WTC Health Program-certified physical conditions. Any medically necessary testing or imaging performed will be submitted to the Program through a medical claim, as with any other diagnostic testing. Following the evaluation, the NPA Contractor shall provide the completed VCF-approved form to the member; the member is responsible for providing it to the VCF. A copy of the evaluation is to be maintained by the NPA Contractor within the member’s record.

Section 4.4.5.5 – Workers’ Compensation

Program members who experienced an injury or illness resulting from their 9/11 exposures may have access to other sources of health benefits such as workers’ compensation and other benefit programs for work-related injuries or illnesses, disability insurance, or other benefit plans. Because there can be an extended period of time between the time a claim is filed and when benefits are awarded, the NPA Contractor’s provision of services must not be based on or hindered by expected Workers’ Compensation benefits.

Upon request, the NPA Contractor shall work with the Program and other Program contractors to determine if a claim should be recouped against workers' compensation, including acquiring more information from an NPA member or NPN provider to assist in the determination.

Section 4.4.6 – Managed Care Operations

The NPA Contractor shall know and understand the clinical and administrative needs of its member population and how they relate to the Program’s limited health benefit model. The NPA Contractor shall provide a comprehensive managed care program that includes clinical and non-clinical care services, facilitates effective resource coordination throughout the continuum of care, and enhances medical management of certified conditions to address the unique healthcare needs of individual members. The NPA Contractor’s case management and utilization management activities shall align with the Case Management and Utilization Management Reference Guide (ATT016_ReferenceGuide_CMUM in Section D).

Section 4.4.6.1 – Case Management

Per Section 4.4.1.1, the NPA Contractor shall ensure that staff performing member engagement activities are trained to identify members with case management needs and refer such members for formal assessment with a licensed registered nurse or social worker.

Members assessed to require case management services shall be assigned an acuity level; acuity assignments shall align with the NIOSH-defined criteria documented in the Case Management and Utilization Management Reference Guide (ATT016_ReferenceGuide_CMUM in Section D) and be documented in the Case Management System. Depending on the member’s specific clinical needs and their acuity level, members may be assigned to a care coordinator, social worker, and/or registered nurse. Members identified as high acuity are considered to be in Intensive Case Management (ICM) and shall be assigned to a nurse case manager.

Though formal plans of care are only required for members in ICM, the NPA Contractor shall ensure that all members in case management have established goals in place and that member progress towards goals is monitored

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at each interaction to determine success of case management interventions. Assigned acuity levels shall be updated as case management needs change.

Generally, the NPA Contractor shall conduct case management with the goal of gradually reducing intensity of support and ultimately, discharging members from the case management program once the member has demonstrated sufficient self-management to no longer require case management services. The NPA Contractor shall develop clear strategies for discharging members from case management, when appropriate. For members in ICM with complex needs, the NPA Contractor shall provide consistent, continuous support for as long as the needs persist.

The NPA Contractor shall employ sufficient levels of nurse case managers, social workers, and care coordinators to meet performance standards in the QASP.

Inpatient Services

The TPA Contractor is responsible for conducting concurrent reviews for continuation of stay for all inpatient and institutional services in accordance with Program guidance and service limitations. Where the NPA Contractor becomes aware of an unplanned member hospitalization, the NPA Contractor shall notify the TPA Contractor as soon as possible to support the TPA Contractor in providing timely concurrent review and assist the TPA in concurrent review, if necessary.

While the member is hospitalized for either planned or unplanned hospitalizations, the NPA Contractor shall participate in discharge planning activities as requested by the facility and/or TPA Contractor.

When it is determined that the member is ready for discharge, the TPA Contractor is responsible for notifying the NPA Contractor. The NPA Contractor shall conduct a post-discharge outreach call to all hospitalized members within 48 hours of discharge or as soon as possible thereafter in the case the NPA Contractor receives notification of hospitalization after discharge.

During the post-discharge outreach call, the NPA Contractor shall determine if the member exhibits transitional case management needs and, where indicated, assign the member a case manager to ensure a smooth transition and reduce the likelihood of readmission.

Section 4.4.6.2 – Utilization Management

The NPA Contractor shall be responsible for reviewing prior authorization requests to determine if the services meet the Program's authorization criteria.²⁵

The Program currently has two levels of medical authorization:²⁶

1. Level 2 (PA2): These are sent to the NPA Contractor for clinical review and approval.
2. Level 3 (PA3): These are sent to the NPA Contractor for initial review and approval and then routed to the Program for clinical review and final approval. The NPA Contractor shall be available to assist the TPA Contractor or Program in ensuring all necessary information to render a decision has been obtained from the requesting provider.

All prior authorization requests submitted to the Program by rendering provider(s) will be sent to the TPA Contractor. The TPA Contractor Portal will serve as a web interface through which the NPA Contractor shall review incoming PA2 and PA3 requests. The NPA Contractor shall ensure that submitted requests meet Program criteria.

For PA2s, the NPA Contractor makes the approval determination. PA3s approved by the NPA Contractor will be automatically routed to the Program for an additional review and final approval. Authorization criteria, Program guidelines, and service and coverage limitation information will be provided to the NPA Contractor after award.

²⁵ See WTC Health Program Administrative Manual, Chapter 4, Section 3, at https://www.cdc.gov/wtc/ppm.html#medical_authorization

²⁶ Note that although the Program refers to all authorizations as "prior" authorizations (PA2 or PA3); in limited circumstances PA2s and PA3s may be submitted concurrently or retrospectively.

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The NPA Contractor shall also be responsible for submitting certain PA3 requests that require coordination by the NPA Clinical Director (e.g., non-emergency general transportation request described in Section 4.4.5.3) to the Program for clinical review and final approval, using the TPA Contractor Portal.

In addition to following Program prior authorization requirements, the NPA Contractor shall also adhere to authorization requirements of members' primary insurance. The NPA Contractor shall coordinate with the member's primary insurance and submit authorization requests to the member's primary insurance when necessary to obtain coverage and coordinate benefits.

The NPA Contractor shall collaborate with the Program or their designee to optimize member care and ensure programmatic, legal, and fiscal oversight as appropriate for utilization management (UM) facilitation. For detailed information on the Program's UM requirements, refer to the Case Management/Utilization Management (CM/UM) Reference Guide and applicable TGDs attached and/or to be provided after award.

The NPA Contractor shall maintain all prior authorization, case management, care coordination, and UM records, incorporating them into members' medical records, where appropriate.

Treatment Secondary Reviews

Where the NPA Contractor denies a PA2 or PA3 due to determination that the treatment does not meet Program criteria for coverage, members have the right to request a secondary review.

The NPA Contractor shall establish a process to perform and communicate outcomes of a secondary review. The secondary review shall be performed by a physician other than the one who made the initial denial decision. The NPA Contractor shall review the request and respond in writing to the member of the final decision and provide the rationale. A copy of the secondary review request, response, and rationale must be included in the Member Profile. The NPA Contractor may also request that a treatment service be considered for addition to the authorized services, using the Program's Medical Code Request Form (provided after award).

Treatment Appeals

Where the Program denies a PA3, members have the right to request an appeal. In this situation, the NPA Contractor shall instruct the member to send a letter to the Program in accordance with the treatment appeal process found on the Program's website.

Section 4.4.7 – Pharmacy Benefit Support

The WTC Health Program contracts with a Pharmacy Benefits Manager (PBM) to provide comprehensive pharmacy benefits to all Program members, as described in Section 1.2. However, the PBM Contractor relies on the Program and other Program contractors (including the NPA Contractor) to ensure medical necessity of all pharmaceutical treatment.

The NPA Contractor shall be knowledgeable about the Program's pharmacy benefits, UM rules, as well as the roles and responsibilities of the Program and Program contractors (including itself) in addressing pharmacy coverage issues or questions. The NPA Contractor shall provide oversight of the pharmacy benefit for NPA members.

Section 4.4.7.1 – Pharmacy Member Services

Though the PBM Contractor maintains call centers to support members, providers, and pharmacies, NPA members may contact the NPA Contractor for pharmacy benefits support. The NPA Contractor shall proactively educate members about their pharmacy benefits and how to access them, in alignment with Section 4.4.1.1.

When issues arise, the NPA Contractor shall follow general member services issue resolution and communication processes required in Section 4.4.1. The NPA Contractor shall utilize PBM Contractor Portal tools, as described in Section 4.2.3.5, to research the root cause of a member's pharmacy issue, clearly and accurately communicate member action items necessary to resolve the issue, and the next steps the NPA Contractor will take to support resolution.

The NPA Contractor shall maintain a general understanding of how NPA member pharmacy issues are to be resolved, even if they are not able to resolve the issue themselves. The NPA Contractor shall assist the Program and PBM Contractor in resolving member issues by collecting relevant member information (e.g., OHI information) and

provider information as part of the issue escalation process. The NPA Contractor shall limit escalations to the Program and/or the PBM Contractor to those issues the NPA Contractor is unable to directly resolve. Escalations to the Program and/or the PBM Contractor shall be performed using the CARE Portal. The NPA Contractor shall keep the member updated on the status of escalated issues and confirm the member's successful receipt of prescriptions prior to considering an issue resolved. The NPA Contractor shall ensure timely resolution of pharmacy issues, in accordance with the QASP.

Section 4.4.7.2 – Pharmacy Care Coordination

The NPA Contractor shall ensure that pre-appointment information sent to NPN providers includes information regarding pharmacy benefits and limitations, as relevant to the provider, their status as a Program prescriber, and the anticipated need for prescriptions resulting from the appointment. Where prescriptions are anticipated, the NPA Contractor shall remind the provider of Program pharmacy benefit requirements, including:

1. “Generic First” policy
2. Pharmacy prior authorization requirements and how to obtain such authorization
3. Requirement to coordinate benefits for Survivor pharmacy claims, in accordance with Section 4.4.5.1.
4. Applicable network and dispensing rules and limitations, as described in the Pharmacy Network and Dispensing Rules XP (ATT017_XP005_PharmacyNetworkDispensing in Section D)
5. Distinction between Program provider and prescriber criteria, per Section 4.4.7.4, and how to enroll as a prescriber

Section 4.4.7.3 – Pharmacy Formulary Assignment

The Program uses multiple formularies to group and define the types of medications that Program members are eligible to receive.²⁷ Each member can only be assigned to a single formulary at any given time, or, alternatively, to the “No Coverage Group” if the member is not eligible for any medications. Member formulary assignment are distinct from member Benefit Plans mentioned in Section 4.4.2 and shall be assigned per the following Program rules:

1. **No Coverage Group:** Automatically assigned by the TPA Contractor upon enrollment.
2. **Diagnostic Formulary:** Manually assigned by the NPA Contractor using a TPA Contractor system interface for this purpose. Automatically placed back into the “No Coverage Group” by the TPA Contractor 180 calendar days after the initial assignment by the NPA Contractor, unless a decision is made to extend.
3. **Standard Formulary:** Automatically assigned by the TPA Contractor upon certification for most members, other than FDNY Family members.
4. **FDNY Family Mental Health Formulary:** Automatically assigned by the TPA Contractor upon certification of an FDNY Family member.
5. **Cancer Formulary:** Manually assigned by the Program using a TPA Contractor system interface for this purpose. Manually removed by the Program using a TPA Contractor system interface for this purpose.

The NPA Contractor shall assign NPA members to the Diagnostic Formulary where clinically indicated, in accordance with Program criteria. Where indicated, the NPA Contractor may request an extension via the CARE Portal. If approved, the Program will notify the TPA of the extension and limits of same.

The NPA Contractor shall assess members with cancer certifications to determine if they meet criteria for the Cancer Formulary and request member assignment to the Cancer Formulary. Similarly, the NPA Contractor shall assess members assigned to the Cancer Formulary to determine if they no longer meet eligibility criteria and request assignment back to the Standard Formulary. These requests shall be submitted to the PBM Contractor through the PBM Contractor system, in accordance with the Cancer Formulary Assignment (CAFA) XP (ATT018_XP001_PharmacyCAFA in Section D). The PBM Contractor routes CAFA requests to the Program for review. If approved, the Program will update the member's formulary assignment as indicated.

²⁷ See WTC Health Program Administrative Manual, Chapter 12, Section 3, at https://www.cdc.gov/wtc/ppm.html#pharmacy_formulary

Section 4.4.7.4 – Prescriber Network Management

The NPA Contractor shall maintain the list of EPN- and NPN-affiliated providers authorized to prescribe to NPA members, as described in Section 4.3.3.2.

If a prescription is processed for a medication that was not prescribed by an authorized NPA prescriber, the prescription will block at the point-of-sale. The NPA Contractor shall utilize the PBM Contractor's Provider Block Portal to review blocked prescription claims and make decisions to override or uphold the claim block in accordance with Program criteria. Decisions made in the Provider Block Portal are specific to that member, drug, strength, and provider. The NPA Contractor shall ensure timely decision-making in the Provider Block Portal in accordance with the QASP.

More information on the NPA Contractor's role in prescriber network management can be found in the Provider Blocks and the Provider Block Portal XP (ATT019_XP007_PharmacyProviderBlock in Section D).

Section 4.4.7.5 – Pharmacy Benefit Monitoring

The NPA Contractor shall monitor reported pharmacy issues for trends and notify the Program of any perceived gaps in Program coverage allowances and of any perceived systemic issues in implementation of the Program's pharmacy benefits plan in the PBM Contractor system (i.e., formulary build design appears non-compliant with benefit rules).

The NPA Contractor shall review pharmacy data (e.g., asthma medication ratio or other quality assurance data) and provide assistance (e.g., counseling, follow-up telehealth visits) to members who may require additional support in meeting monitoring and treatment goals.

The NPA Contractor shall review outcomes from clinical reviews and/or audits, as required by the Program under Section 4.2.2 and take action where indicated (e.g., implement point-of-sale restriction) to prevent inappropriate utilization of pharmacy benefits.

Section 4.5 – Transition-Out

If and once the Transition-Out CLIN is exercised, the NPA Contractor shall move from Sustainment to Transition-Out. The Government may exercise Transition-Out at any time, provided that the Government gives the NPA Contractor a preliminary written notice of its intent to do so no less than 60 days prior to the start of Transition-Out. During the 12-month Transition-Out, which will run concurrently with the successor contractor's Transition-In, the NPA Contractor shall transfer duties and responsibilities to a Program-appointed designee, whether a government entity, another contractor, or a combination of both.

The NPA Contractor shall collaborate with the Program, other Program contractors, and the successor contractor(s), if applicable, to ensure a successful Transition-Out is completed in the required timeframe while maintaining performance standards as defined by the PWS and QASP. The NPA Contractor shall cooperate fully with the Program and the successor contractor during the Transition-Out period, to facilitate an orderly transfer of Program functions and operations to the successor contractor. The NPA Contractor shall ensure continuity of services during this period of transition, while prioritizing minimal disruption of services to members receiving care and providers delivering care.

The NPA Contractor shall submit a *Transition-Out Plan*, *Transition-Out Master Schedule (TOMS)*, and *Transition-Out Communications Plan* within 30 calendar days after the Transition-Out CLIN period of performance begins, detailing all activities necessary for the NPA Contractor to provide Program data and information to the successor contractor, maintain operational standards through the end of the Contract, and complete all Transition-Out tasks successfully, within a risk management framework. The NPA Contractor shall implement the plan and make revisions as needed throughout the Transition-Out period to document changes and lessons learned. The NPA Contractor shall incorporate additional feedback provided by the Program and collaborate with the successor contractor to ensure alignment of the NPA Contractor's *Transition-Out Plan* with the successor's Transition-In Plan.

During the Transition-Out period, the NPA Contractor shall:

1. Maintain continuity of operations.
2. Provide current and accurate program and system management documentation (e.g., interface library, data dictionaries, data schema), no later than 14 calendar days after the Transition-Out Option is exercised.

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3. Provide a current, validated inventory of all Government-owned assets used by the NPA Contractor over the life of the Contract along with full support in the reconciliation of the inventory as needed.
4. Proactively manage transition activities and milestones with the *Transition-Out Master Schedule (TOMS)* and ensure alignment with the *Transition-Out Plan* and the successor's Transition-In Plan (if applicable) and convene regular meetings to provide regular updates to the Program.
5. Attend and participate in meetings to transfer information, processes, and data; mitigate transition risks and issues; and report status of transition tasks.
6. Execute the approved *Transition-Out Communications Plan* in coordination with the Program and the successor to ensure consistent messaging.
7. Adapt to reductions in operational volume as appropriate.
8. Upon completion of the transfer of all Program data to the Program and/or successor contractor(s), execute all required "system decommissioning" tasks, including destruction of all non-public or other protected data from NPA Contractor-owned systems, and certify execution.

The NPA Contractor shall coordinate with the Program and any successor(s) to minimize the quantity and severity of risks associated with services that have been initiated but have not been fully resolved, including member certification requests, clinic transfers, and open member issues. The NPA Contractor shall ensure that these services are tracked and communicated to relevant stakeholders and facilitate the handoff of these services to the successor contractor, in accordance with the *TOMS*. The NPA Contractor shall ensure all handoffs are completed by SHCD of the successor contractor, unless otherwise agreed upon between the Program, the NPA Contractor, and the successor contractor.

The NPA Contractor shall also work with the WTC Health Program Information Security Officer (ISO) and COR to transfer all Program records, data, and other information (including process and policy guidance materials) to the Program and/or successor contractor(s) within 14 calendar days of the Transition-Out CLIN start, with updates provided in accordance with the approved schedule. The data transfer shall be performed in a manner and format that enables the Program and/or the successor contractor(s) to map all data from the NPA Contractor's system(s) to the replacement system(s) of the Government or a successor contractor. This includes all data and data-related documentation as listed in Section 4.3.3.4, as well as:

1. Functional business process flows, including operational manuals, documentation of all processes to accomplish work on behalf of the Program, and sample data files.
2. Operational information including correspondence, documentation of ongoing or outstanding issues, operations support documentation, and operational information regarding subcontractors to the successor contractor.

Section 4.6 – Regulatory Compliance and Security Requirements

Section 4.6.1 – HIPAA Compliance and Business Associate Agreement

As further explained below, the NPA Contractor is a Business Associate of the WTC Health Program; the WTC Health Program is a healthcare component of a hybrid entity, HHS, under HIPAA, and functions as a Covered Entity. To the extent that the Business Associate performs functions or activities on behalf of, or provides certain services to, the Covered Entity where the Business Associate creates, receives, maintains, or transmits "protected health information" (PHI), the following HIPAA provisions apply: the Health Insurance Portability and Accountability Act of 1996 (HIPAA) (Pub. L. 104–191; 42 U.S.C. § 1320d *et seq.*); the Health Information Technology for Economic and Clinical Health (HITECH) Act²⁸ (Pub. L. 111-5; 42 U.S.C. § 300jj *et seq.*); the HIPAA Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164); and HHS HIPAA policies.

²⁸ The HITECH Act was passed as Title XIII of Division A and Title IV of Division B of the American Recovery and Reinvestment Act of 2009 (ARRA).

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For purposes of this Contract, “Business Associate” shall mean the NPA Contractor; “Covered Entity”²⁹ shall mean the WTC Health Program and any other NIOSH, CDC, or HHS components to the extent that they assist in administering the WTC Health Program and where PHI is involved. These terms are used as defined in 45 C.F.R. § 160.103.

For any potential HIPAA incident that should occur involving the Business Associate, the NPA Contractor shall submit an initial notification to the WTC Health Program’s HIPAA Privacy Officer within one hour of discovery, followed by a *HIPAA Incident Report* within three business days, utilizing the Incident Report Form provided after award. As the Covered Entity, the WTC Health Program is responsible for determining the risk level of each incident and for determining in which cases a breach notification shall occur. Notification to the Program’s HIPAA Privacy Officer shall be provided prior to the WTC Health Program learning of the incident from outside sources (HHS Office for Civil Rights, Organizational Press Release, Letter sent to WTC Health Program member, etc.). Notification to the Program shall be transmitted securely and shall include the date and nature of the incident (including the identification of each WTC Health Program member whose information was implicated in the incident and the extent of the information breached) and actions being taken by the NPA Contractor as a result of the breach.

The *Business Associate Agreement (BAA)* provides the detailed legal obligations and requirements of the Business Associate and the Covered Entity and is incorporated into this Contract. The template can be found in Attachment ATT020_Template_BAA.

At all times throughout the duration of this Contract and until the Business Associate fulfills its obligations under this Contract and the *BAA*, the Business Associate is subject to and shall comply with all applicable HIPAA provisions regarding business associates, as well as any updates to those provisions.

The parties acknowledge that the Business Associate is directly liable under HIPAA for the following violations:³⁰

1. Impermissible uses and disclosures of PHI.
2. Failure to provide breach notification to the Covered Entity.
3. Failure to provide access to a copy of electronic PHI to either the Covered Entity, the individual, or the individual’s designee, as specified in the *BAA*.
4. Failure to disclose PHI where required by the Secretary of HHS to investigate or determine the Business Associate’s compliance with HIPAA.
5. Failure to provide an accounting of disclosures.
6. Failure to comply with the requirements of the Security Rule.

Furthermore, the NPA Contractor shall provide the detailed process of how it will meet the obligations of HIPAA in the *Operations Manual*.

Section 4.6.2 – Fraud, Waste, and Abuse (FWA) Prevention, Detection, Reporting, and Mitigation

The NPA Contractor shall implement and maintain a solution to detect, prevent, and report potential FWA related to any of the services covered by this Contract to ensure program integrity. The NPA Contractor shall provide role-based training for NPA Contractor staff on identifying and reporting potential FWA.

²⁹ The term “Covered Entity” is used in this section for ease of understanding. However, a more precise description of the application of HIPAA to the WTC Health Program is as follows: HHS is a hybrid entity under HIPAA, meaning HHS is a covered entity that conducts business activities, including both covered and non-covered functions, and designates “health care components” in accordance with 45 C.F.R. § 164.105(a)(2)(iii)(D). 45 C.F.R. § 164.103. As a hybrid entity, HHS must designate any component that would “meet the definition of a covered entity or business associate if it were a separate legal entity” as a health care component; a health care component also may include a component only to the extent that it performs covered functions. 45 C.F.R. § 164.105(a)(2)(iii)(D). Accordingly, the WTC Health Program is a “health care component” of the covered entity, HHS; as are any other NIOSH, CDC, or HHS components if they would meet the definition of a covered entity or business associate if they were separate legal entities and only to the extent that they perform covered functions.

³⁰ See Modifications to the HIPAA Privacy, Security, Enforcement, and Breach Notification Rules Under the Health Information Technology for Economic and Clinical Health Act and the Genetic Information Nondiscrimination Act; Other Modifications to the HIPAA Rules [Omnibus Rule], 78 Fed. Reg. 5566, 5598-99 (Jan. 25, 2013).

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If the NPA Contractor identifies a potential FWA incident, they must notify the Program in writing within one business day of discovery. The NPA Contractor shall then provide a written *FWA Incident Report* and share relevant data with the Program within five business days of the potential FWA incident. The report shall be reviewed and signed by a Key Personnel of the NPA Contractor, and include the following information, at a minimum:

1. The name of the individual or entity suspected to be involved in FWA.
2. The source that identified the FWA.
3. The type of provider, entity or organization that committed the FWA.
4. A description of the FWA.
5. The approximate dollar amount (known or anticipated) of the FWA.
6. The legal and administrative disposition of the case including actions taken by law enforcement officials to whom the case has been referred (if applicable).
7. Other data/information as prescribed by the WTC Health Program.

The NPA Contractor shall coordinate with the Program and other Program contractors to manage member communications and care in a manner that supports, responds to, and mitigates identified FWA risks. For example, the NPA Contractor shall notify members of identified FWA events that impact their care and shall implement care management practices that ensure appropriate oversight.

Section 4.6.3 – Records Management

The NPA Contractor shall develop, implement, and administer a comprehensive records management program that complies with all applicable records management laws and regulations, including the Federal Records Act, NARA records policies, CDC and NIOSH agency-specific records policies, and those policies associated with the safeguarding of records covered by HIPAA and the Privacy Act. To the extent that any records are not scheduled by the above, the NPA Contractor shall consider the records permanent and retain the records for the full period of performance. All records shall be returned to the Program at the conclusion or termination of the Contract, whichever occurs first. Records returned to the Program shall be hand carried, mailed, emailed, or securely transmitted electronically (such as through a security-compliant USB device, file transfer process, or other agreed-upon medium).

The NPA Contractor shall maintain all records created for Program use, created in the course of performing the Contract, and/or delivered to or under the legal control of the Program in accordance with federal law. The NPA Contractor shall digitize temporary and permanent paper records, designate the digitized version as the recordkeeping copy, and destroy the original source record after digitizing the records and validating digitization activities against NARA-established provisions, as listed in 36 C.F.R. pts. 1224, 1225, and 1236. Digitization resources can be found at <https://www.archives.gov/records-mgmt/policy/digitization>. Electronic records and associated metadata must be accompanied by sufficient technical documentation to permit understanding and use of the records and data.

The NPA Contractor shall prevent alienation or unauthorized destruction of records, including all forms of mutilation. Willful and unlawful destruction, damage, or alienation of federal records is subject to the fines and penalties imposed by 18 U.S.C. § 2071. The NPA Contractor must immediately report any unlawful or accidental removal, defacing, alteration, or destruction of records.

The NPA Contractor shall obtain the CO's written approval prior to engaging in any contractual relationship (subcontractor) in support of this Contract requiring the disclosure of information, documentary material, and/or records generated under or relating to contracts. The NPA Contractor (and any subcontractor) shall abide by federal, CDC, NIOSH, and Program guidance for protecting sensitive, proprietary information, classified, and controlled unclassified information.

A draft Records Management Plan shall be submitted as part of the *Operations Plan*. Once approved, the NPA Contractor shall perform all required services in accordance with their established Records Management Plan.

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Deliverable Title/Description	Due Date
Roster	By effective date of this Contract/Quarterly/Upon request from government
Contractor Employee Non- Disclosure Agreement (NDA)	Prior to performing any work on behalf of HHS
Assist in the completion of a PTA/PIA form	Within specified date after Contract award
Copy of training records for all mandatory training	In conjunction with Contract award and annually thereafter or upon request
Signed ROB for all employees	Initiation of Contract and at least annually thereafter
Incident Report (as incidents or breaches occur)	As soon as possible and without reasonable delay and no later than one hour of discovery
Incident and Breach Response Plan	Prior to performing any work on behalf of HHS/Annually/Upon request from government
List of Personnel with defined roles and responsibilities	Prior to performing any work on behalf of HHS/Annually/Upon request from government
Off-boarding documentation, equipment and badge when leaving Contract	Within seven days of contractor departure and at Contract expiration
Onboarding documentation when beginning Contract	Prior to performing any work on behalf of HHS
Form or deliverables required by CDC/NIOSH	At Contract expiration
If the procurement involves a system or cloud service, Disposition/Decommission Plan	At Contract expiration
Contract initiation and expiration	Due after Contract award
SA&A Package including BSI, SSP, RAR, POA&M, CP and CPT Report, E-Auth, PTA/PIA, SSN Usage Approval Request (if applicable), ROB (if applicable), Interconnection/Data Use Agreements (if applicable), Authorization Letter, Configuration Management Plan (if applicable), Configuration Baseline	Due in accordance with the timeline proscribed in the <i>CDC SA&A SOP</i>
Retirement Checklist, all deliverables and data/records retained in accordance with Records Management and other requirements, Device sanitization documentation, Destruction documentation for all other CDC data/information (soft/hard copy)	Prior to system retirement/decommissioning
POA&M updates; Revised security documentation/Agreements	Monthly/as requested by CDC
Incident reports (as needed) Incident Response Plan	As requested by CDC
Computer software, including the source code	Prior to performing any work on behalf of HHS
Other source code	Prior to performing any work on behalf of HHS

Baseline Security Requirements

Applicability. The requirements herein apply whether the entire Contract or order (hereafter “Contract”), or portion thereof, includes either or both of the following:

Access (Physical or Logical) to Government Information: A Contractor (and/or any subcontractor) employee will have or will be given the ability to have, routine physical (entry) or logical (electronic) access to government information.

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Operate a Federal System Containing Information: A Contractor (and/or any subcontractor) employee will operate a federal system and information technology containing data that supports the HHS mission. In addition to the Federal Acquisition Regulation (FAR) Subpart 2.1 definition of “information technology” (IT), the term as used in this section includes computers, ancillary equipment (including imaging peripherals, input, output, and storage devices necessary for security and surveillance), peripheral equipment designed to be controlled by the central processing unit of a computer, software, firmware and similar procedures, services (including support services), and related resources.

5) **Safeguarding Information and Information Systems.** All government information and information systems must be protected in accordance with HHS/CDC/NIOSH policies and level of risk. At a minimum, the NPA Contractor (and/or any subcontractor) must:

In accordance with the Federal Information Processing Standards Publication (FIPS) 199, *Standards for Security Categorization of Federal Information and Information Systems*, the NPA Contractor (and/or any subcontractor) shall:

Protect the:

- **Confidentiality**, which means preserving authorized restrictions on access and disclosure, based on the security terms found in this Contract, including means for protecting personal privacy and proprietary information;
- **Integrity**, which means guarding against improper information modification or destruction, and ensuring information non-repudiation and authenticity; and
- **Availability**, which means ensuring timely and reliable access to and use of information.

Provide security for any NPA Contractor systems, and information contained therein, connected to an HHS network or operated by the NPA Contractor on behalf of HHS regardless of location. In addition, if new or unanticipated threats or hazards are discovered by either the agency or the NPA Contractor, or if existing safeguards have ceased to function, the discoverer shall immediately, **within one hour or less**, bring the situation to the attention of the other party.

Adopt and implement the policies, procedures, controls, and standards required by the HHS Information Security Program to ensure the confidentiality, integrity, and availability of government information and government information systems for which the NPA Contractor is responsible under this Contract or to which the NPA Contractor may otherwise have access under this contract. Obtain the HHS Information Systems Security Program security requirements, outlined in the HHS Information Security and Privacy Policy (IS2P), by contacting the CO/COR or emailing fisma@hhs.gov.

Comply with the Privacy Act requirements and tailor FAR clauses as needed.

Categorize all information owned and/or collected/managed on behalf of HHS/CDC/NIOSH and information systems that store, process, and/or transmit HHS information in accordance with FIPS 199, Standards for Security Categorization of Federal Information and Information Systems and National Institute of Standards and Technology (NIST) Special Publication (SP) 800-60, Volume II: Appendices to Guide for Mapping Types of Information and Information Systems to Security Categories, and based on information provided by the ISSO, CISO, or other security representative, the risk level for each Security Objective and the Overall Risk Level, which is the highest watermark of the three factors (Confidentiality, Integrity, and Availability) of the information or information system are the following:

Confidentiality:	☐ Low ☒ Moderate ☐ High
Integrity:	☐ Low ☒ Moderate ☐ High
Availability:	☐ Low ☒ Moderate ☐ High
Overall Risk Level:	☐ Low ☒ Moderate ☐ High

Based on information provided by the SSPO, Privacy Office, system/data owner, or other security or privacy representative, it has been determined that this solicitation/contract involves:

[] No PII [X] Yes PII

Based on the agreed-upon level of impact, implement the necessary safeguards to protect all information systems and information collected and/or managed on behalf of HHS/CDC/NIOSH regardless of location or purpose.

Report any discovered or unanticipated threats or hazards by either the agency or the NPA Contractor, or if existing safeguards have ceased to function immediately after discovery, within one (1) hour or less, to the government representative(s).

Adopt and implement all applicable policies, procedures, controls, and standards required by the HHS/CDC/NIOSH Information Security Program to ensure the confidentiality, integrity, and availability of government information and government information systems for which the NPA Contractor is responsible under this Contract or to which the NPA Contractor may otherwise have access under this Contract. Obtain all applicable security and privacy policies by contacting the CO/COR or HHS/CDC/NIOSH security and/or privacy officials.

6) **Privacy Act.**

7) It has been determined that this Contract is subject to the Privacy Act of 1974, because this contract provides for the design, development, or operation of a system of records on individuals.

8) The System of Records Notice (SORN) that is applicable to this contract is: 09-90-2001 Records Used for Surveillance and Study of Epidemics, Preventable Diseases and Problems 09-20-0136 Epidemiologic Studies

9) and Surveillance of Disease Problems; 09-20-0147 Occupational Health Epidemiological Studies and EEOICPA Program Records SORN history: 76 FR 34706 (6/14/11), *[83 FR 6591](#) (2/14/18)

10) The design, development, or operation work the NPA Contractor is to perform is: As outlined in Section 3 – Scope of Work and Section 4.4.1.4 – Member Access to WTC Health Program-Acquired Medical Records.

11) The disposition to be made of the Privacy Act records upon completion of Contract performance is: Records are maintained in accordance with General Records Schedule (GRS) and comply with CDC Records Control Schedule (RCS). Input/output records are disposed of when no longer needed: GRS 5.2, item 020. Disposal methods include erasing computer tapes, burning or shredding paper materials or transferring records to the Federal Records Center when no longer needed for evaluation and analysis.

12) **Privacy Compliance.** Comply with the E-Government Act of 2002, NIST SP 800-53, and applicable HHS/CDC/NIOSH privacy policies, and complete all the requirements below:

Per the Office of Management and Budget (OMB) Circular A-130, Personally Identifiable Information (PII), is "information that can be used to distinguish or trace an individual's identity, either alone or when combined with other information that is linked or linkable to a specific individual." Examples of PII include, but are not limited to the following: Social Security number, date and place of birth, mother's maiden name, biometric records, etc.

PII Confidentiality Impact Level has been determined to be: [] No PII [] Low [X] Moderate [] High

To ensure that the public's personal information is protected in a manner commensurate with the privacy risks, HHS uses a privacy analysis process to assess the risks associated with HHS's collection and maintenance of PII and to ensure information is handled in accordance with applicable legal, regulatory, and policy requirements. PTAs analyze how information is handled in IT systems and electronic information collections and determines if the IT system or electronic information collection collects, disseminates, maintains, or disposes of PII. PIAs are used to assess the privacy risks of IT systems and electronic information collections that collect, disseminate, maintain, or dispose of PII about members of the public. PIAs also provide transparency into how HHS collects, disseminates, maintains, or disposes of the public's PII.

The NPA Contractor must support the agency with conducting a Privacy Threshold Analysis (PTA) for the information system and/or information handled under this Contract to determine whether or not PII is collected, disseminated, maintained, or disposed as part of the Contract. The PTA will determine if a full Privacy Impact Assessment (PIA) needs to be completed.

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1. If the results of the PTA show that a full PIA is needed, the NPA Contractor must support the agency with completing a PIA for the system or information within 30 calendar days after completion of the PTA and in accordance with HHS policy and OMB M-03-22, Guidance for Implementing the Privacy Provisions of the E-Government Act of 2002.
2. The NPA Contractor must support the agency in reviewing the PIA at least every three years throughout the system development lifecycle (SDLC)/information lifecycle, or when determined by the agency that a review is required based on a major change to the system, or when new types of PII are collected that introduces new or increased privacy risks, whichever comes first.

13) **Controlled Unclassified Information (CUI).** CUI is defined as “information that laws, regulations, or Government-wide policies require to have safeguarding or dissemination controls, excluding classified information.” The NPA Contractor (and/or any subcontractor) must comply with *Executive Order 13556, Controlled Unclassified Information, (implemented at 32 C.F.R., part 2002)* when handling CUI. 32 C.F.R. § 2002.4(aa) As implemented the term “handling” refers to “...any use of CUI, including but not limited to marking, safeguarding, transporting, disseminating, re-using, and disposing of the information.” 81 Fed. Reg. 63323. All sensitive information that has been identified as CUI by a regulation or statute, handled by this solicitation/Contract, shall be:

marked appropriately;

disclosed to authorized personnel on a Need-To-Know basis;

protected in accordance with NIST SP 800-53, *Security and Privacy Controls for Federal Information Systems and Organizations* applicable baseline if handled by an NPA Contractor system operated on behalf of the agency; and

returned to HHS control, destroyed when no longer needed, or held until otherwise directed. Destruction of information and/or data shall be accomplished in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*.

14) **Protection of Sensitive Information.** For security purposes, information is *or* may be sensitive because it requires security to protect its confidentiality, integrity, and/or availability. The NPA Contractor (and/or any subcontractor) shall protect all government information that is or may be sensitive by securing it with a solution that is validated with current FIPS 140, *Security Requirements for Cryptographic Modules* validation certificate from the NIST Cryptographic Module Validation Program (CMVP).

15) **Confidentiality and Nondisclosure of Information.** Any information provided to the NPA Contractor (and/or any subcontractor) by HHS or collected by the NPA Contractor on behalf of HHS shall be used only for the purpose of carrying out the provisions of this Contract and shall not be disclosed or made known in any manner to any persons except as may be necessary in the performance of the Contract. The NPA Contractor assumes responsibility for protection of the confidentiality of Government records and shall ensure that all work performed by its employees and subcontractors shall be under the supervision of the NPA Contractor. Each NPA Contractor employee or any of its subcontractors to whom any HHS records may be made available or disclosed shall be notified in writing by the NPA Contractor that information disclosed to such employee or subcontractor can be used only for that purpose and to the extent authorized herein.

The confidentiality, integrity, and availability of such information shall be protected in accordance with HHS and CDC policies. Unauthorized disclosure of information will be subject to the HHS/CDC sanction policies and/or governed by the following laws and regulations:

18 U.S.C. § 641 (Criminal Code: Public Money, Property or Records);

18 U.S.C. § 1905 (Criminal Code: Disclosure of Confidential Information); and

44 U.S.C. Chapter 35, Subchapter I (Paperwork Reduction Act).

16) **Internet Protocol Version 6 (IPv6).** All procurements using Internet Protocol shall comply with OMB Memorandum M-05-22, *Transition Planning for Internet Protocol Version 6 (IPv6)*.

17) **Government Websites.** All new and existing public-facing government websites must be securely configured with Hypertext Transfer Protocol Secure (HTTPS) using the most recent version of Transport Layer Security (TLS). In addition, HTTPS shall enable HTTP Strict Transport Security (HSTS) to instruct compliant

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browsers to assume HTTPS at all times to reduce the number of insecure redirects and protect against attacks that attempt to downgrade connections to plain HTTP. For internal-facing websites, the HTTPS is not required, but it is highly recommended.

18) **Contract Documentation.** The NPA Contractor shall use provided templates, policies, forms and other agency documents to comply with Contract deliverables as appropriate.

19) **Standard for Encryption.** The NPA Contractor (and/or any subcontractor) shall:

Comply with the HHS Standard for Encryption of Computing Devices and Information to prevent unauthorized access to government information.

Encrypt all sensitive federal data and information (i.e., PII, protected health information [PHI], proprietary information, etc.) in transit (i.e., email, network connections, etc.) and at rest (i.e., servers, storage devices, mobile devices, backup media, etc.) with FIPS 140 validated encryption solution.

Secure all devices (i.e.: desktops, laptops, mobile devices, etc.) that store and process government information and ensure devices meet HHS and CDC-specific encryption standard requirements. Maintain a complete and current inventory of all laptop computers, desktop computers, and other mobile devices and portable media that store or process sensitive government information (including PII).

Verify that the encryption solutions in use have been validated under the Cryptographic Module Validation Program to confirm compliance with current FIPS 140 validation certificate from the NIST CMVP. The NPA Contractor shall provide a written copy of the validation documentation to the COR.

Use the Key Management system on the HHS personal identification verification (PIV) card or establish and use a key recovery mechanism to ensure the ability for authorized personnel to encrypt/decrypt information and recover encryption keys. Encryption keys shall be provided to CDC Cybersecurity Program Office (CSPO) inside the Office of the Chief Information Officer (OCIO).

20) **Contractor Non-Disclosure Agreement (NDA).** Each NPA Contractor (and/or any subcontractor) employee having access to non-public government information under this Contract shall complete the CDC non-disclosure agreement, as applicable. NPA Contractor (and/or subcontractors) must provide a copy of each signed and witnessed NDA to the Contracting Officer (CO) and/or CO Representative (COR) prior to performing any work under this acquisition.

21) **Privacy Threshold Analysis (PTA)/Privacy Impact Assessment (PIA)** – The NPA Contractor shall assist the CDC Senior Official for Privacy (SOP) or designee with conducting a PTA for the information system and/or information handled under this Contract in accordance with HHS policy and OMB M-03-22, Guidance for Implementing the Privacy Provisions of the E-Government Act of 2002.

The NPA Contractor shall assist the CDC SOP or designee in reviewing the PIA at least every three years throughout the system development lifecycle (SDLC)/information lifecycle, or when determined by the CDC SOP that a review is required based on a major change to the system (e.g., new uses of information collected, changes to the way information is shared or disclosed and for what purpose, or when new types of PII are collected that could introduce new or increased privacy risks), whichever comes first.

Training

22) **Mandatory Training for All NPA Contractor Staff.** All NPA Contractor (and/or any subcontractor) employees assigned to work on this Contract shall complete the applicable HHS/CDC Contractor Information Security Awareness, Privacy, and Records Management training (provided upon contract award) before performing any work under this Contract. Thereafter, the employees shall complete *CDC Security Awareness Training (SAT)*, *Privacy*, and Records Management training at least **annually**, during the life of this Contract. All provided training shall be compliant with HHS training policies.

23) **Role-based Training.** All NPA Contractor (and/or any subcontractor) employees with significant security responsibilities (as determined by the program manager) must complete role-based training (RBT) **within 60 days** of assuming their new responsibilities. Thereafter, they shall complete RBT at least **annually** commensurate with their role and responsibilities in accordance with HHS policy and the *HHS Role-Based Training (RBT) of Personnel with Significant Security Responsibilities (SSR) Memorandum*.

24) All HHS employees and contractors with SSR who **have not** completed the required training within the

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mandated timeframes shall have their user accounts disabled until they have met their RBT requirement.

25) **Training Records.** The NPA Contractor (and/or any subcontractor) shall maintain training records for all its employees working under this Contract in accordance with HHS policy. A copy of the training records shall be provided to the CO and/or COR within **30 days** after Contract award and **annually** thereafter or upon request.

Rules of Behavior

26) The NPA Contractor (and/or any subcontractor) shall ensure that all employees performing on the Contract comply with the *HHS Information Technology General Rules of Behavior*, *HHS Rules of Behavior for Privileged Users*.

27) All NPA Contractor employees performing on the Contract must read and adhere to the Rules of Behavior (ROB) before accessing Department data or other information, systems, and/or networks that store/process government information, initially at the beginning of the Contract and at least **annually** thereafter, which may be done as part of annual *CDC Security Awareness Training*. If the training is provided by the NPA Contractor, the signed ROB must be provided as a separate deliverable to the CO and/or COR per defined timelines above.

Incident Response

Federal Information Security Modernization Act (FISMA) defines an incident as “an occurrence that (1) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information or an information system; or (2) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable use policies.” The *HHS Policy for IT Security and Privacy Incident Reporting and Response* further defines incidents as events involving cybersecurity and privacy threats, such as viruses, malicious user activity, loss of, unauthorized disclosure or destruction of data, and so on.

A privacy breach is a type of incident and is defined by FISMA as the loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where (1) a person other than an authorized user accesses or potentially accesses personally identifiable information or (2) an authorized user accesses or potentially accesses personally identifiable information for an other than authorized purpose.

OMB Memorandum M-17-12, “Preparing for and Responding to a Breach of Personally Identifiable Information” (03 January 2017) states:

Definition of an Incident:

An occurrence that (1) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information or an information system; or (2) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable use policies.

Definition of a Breach:

The loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where (1) a person other than an authorized user accesses or potentially accesses personally identifiable information or (2) an authorized user accesses or potentially accesses personally identifiable information for an other than authorized purpose.

It further adds:

A breach is not limited to an occurrence where a person other than an authorized user potentially accesses PII by means of a network intrusion, a targeted attack that exploits website vulnerabilities, or an attack executed through an email message or attachment. A breach may also include the loss or theft of physical documents that include PII and portable electronic storage media that store PII, the inadvertent disclosure of PII on a public website, or an oral disclosure of PII to a person who is not authorized to receive that information. It may also include an authorized user accessing PII for an other than authorized purpose.

The *HHS Policy for IT Security and Privacy Incident Reporting and Response* further defines a breach as “a suspected or confirmed incident involving PII”.

Contracts with entities that collect, maintain, use, or operate Federal information or information systems on behalf of CDC shall include the following requirements:

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- 1) The NPA Contractor shall cooperate with and exchange information with CDC officials, as deemed necessary by the CDC Breach Response Team, to report and manage a suspected or confirmed breach.
- 2) The NPA Contractor (and/or any subcontractor) must respond to all alerts/Indicators of Compromise (IOCs) provided by HHS Computer Security Incident Response Center (CSIRC)/CDC IRT teams within 24 hours, whether the response is positive or negative. In accordance with FISMA and OMB M-17-12, Preparing for and Responding to a Breach of Personally Identifiable Information (PII), an incident is "an occurrence that (1) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information or an information system; or (2) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable use policies" and a privacy breach is "the loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where (1) a person other than an authorized user accesses or potentially accesses personally identifiable information or (2) an authorized user accesses or potentially accesses personally identifiable information for an other than authorized purpose." For additional information on the HHS breach response process, please see the HHS Policy and Plan for Preparing for and Responding to a Breach of Personally Identifiable Information (PII).

All contractors and subcontractors shall participate in regular training on how to identify and report a breach.

All contractors and subcontractors shall report a suspected or confirmed breach in any medium or form, including paper, oral, or electronic, as soon as possible and without unreasonable delay as soon as possible and no later than one hour of discovery, consistent with applicable CDC IT acquisitions guidance, HHS/CDC, incident management policy, and United States Computer Emergency Readiness Team (US-CERT) notification guidelines. To this end, the NPA Contractor (and/or any subcontractor) shall respond to all alerts/Indicators of Compromise (IOCs) provided by HHS Computer Security Incident Response Center (CSIRC) or CDC Computer Incident Response Team (CSIRT) within 24 hours via email at csirt@cdc.gov or telephone at 866-655-2245, whether the response is positive or negative. The types of information required in an incident report must include at a minimum: company and point of contact information, contact information, impact classifications/threat vector, and the type of information compromised. In addition, the NPA Contractor must:

1. Cooperate and exchange any information, as determined by the Agency, necessary to effectively manage or mitigate a suspected or confirmed breach;
2. Not include any sensitive information in the subject or body of any reporting e-mail; and
3. Encrypt sensitive information in attachments to email, media, etc.

Comply with OMB M-17-12, Preparing for and Responding to a Breach of Personally Identifiable Information, and HHS/CDC privacy breach response policies when handling PII breaches.

Provide full access and cooperate on all activities as determined by the Government to ensure an effective incident response, including providing all requested images, log files, and event information to facilitate rapid resolution of sensitive information incidents. This may involve disconnecting the system processing, storing, or transmitting the sensitive information from the Internet or other networks or applying additional security controls. This may also involve physical access to contractor facilities during a breach/incident investigation.

All contractors and subcontractors shall be able to determine what Federal information was or could have been accessed and by whom, construct a timeline of user activity, determine methods and techniques used to access Federal information, and identify the initial attack vector.

All contractors and subcontractors shall allow for an inspection, investigation, forensic analysis, and any other action necessary to ensure compliance with HHS/CDC Policy and the HHS/CDC Breach Response Plan and to assist with responding to a breach.

Cloud service providers shall use guidance provided in the FedRAMP Incident Communications Procedures when deciding when to report directly to US-CERT first or notify CDC first.

Identify roles and responsibilities, in accordance with HHS/CDC Breach Response Policy and the HHS/CDC Breach Response Plan. To this end, the NPA Contractor shall NOT notify affected individuals unless and until so instructed by the CO or designated representative. If so instructed by the CO or COR, all notifications must be pre-approved by the appropriate CDC officials, consistent with HHS/CDC Breach Response Plan, and the NPA Contractor shall then

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send CDC- approved notifications to affected individuals; and,

Acknowledge that CDC will not interpret report of a breach, by itself, as conclusive evidence that the NPA Contractor or its subcontractor failed to provide adequate safeguards for PII.

Position Sensitivity Designations

All NPA Contractor (and/or any subcontractor) employees must obtain a background investigation commensurate with their position sensitivity designation that complies with 5 C.F.R. pts. 1400 and 731.

All NPA Contractor (and/or any subcontractor) employees with routine access to PII requires both a valid need to know and requires a Tier 2S background investigation to ensure the individual being provided access is suitable. These requirements extend to developers of information systems containing PII and PHI.

☐ Tier 1

☒ Tier 2s

Homeland Security Presidential Directive (HSPD)-12

The NPA Contractor (and/or any subcontractor) and its employees shall comply with Homeland Security Presidential Directive (HSPD)-12, *Policy for a Common Identification Standard for Federal Employees and Contractors*; OMB M-05-24; FIPS 201, *Personal Identity Verification (PIV) of Federal Employees and Contractors*; HHS HSPD-12 policy; and *Executive Order 13467, Part 1 §1.2*.

Roster

Roster. The NPA Contractor (and/or any subcontractor) shall submit a roster by name, position, e-mail address, phone number and responsibility of all staff working under this acquisition where the NPA Contractor will develop, have the ability to access, or host and/or maintain a government information system(s). The roster shall be submitted to the COR and/or CO by the effective date of this Contract. Any revisions to the roster as a result of staffing changes shall be submitted immediately upon change. The COR will notify the NPA Contractor of the appropriate level of investigation required for each staff member.

If the employee is filling a new position, the NPA Contractor shall provide a position description and the Government will determine the appropriate suitability level.

Contract Initiation and Expiration

General Security Requirements. NPA Contractor (and/or any subcontractor) shall comply with information security and privacy requirements, Enterprise Performance Life Cycle (EPLC) processes, HHS Enterprise Architecture requirements to ensure information is appropriately protected from initiation to expiration of the Contract. All information systems development or enhancement tasks supported by the contractor shall follow the HHS EPLC framework and methodology and in accordance with the HHS Contract Closeout Guide (2012).

System Documentation. The NPA Contractor (and/or any subcontractors) must follow and adhere to *Security Considerations in the System Development Life Cycle*, at a minimum, for system development and provide system documentation at designated intervals (specifically, at the expiration of the Contract) within the EPLC that require artifact review and approval.

Sanitization of Government Files and Information. As part of contract closeout and at expiration of the Contract, the NPA Contractor (and/or any subcontractor) shall provide all required documentation to the CO and/or COR to certify that, at the Government's direction, all electronic and paper records are appropriately disposed of and all devices and media are sanitized in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*.

Notification. The NPA Contractor (and/or any subcontractor) shall notify the CO and/or COR and system ISSO before an employee stops working under this Contract.

Contractor Responsibilities Upon Physical Completion of the Contract. The NPA Contractor (and/or any subcontractors) shall return all government information and IT resources (i.e., government information in non-government-owned systems, media, and backup systems) acquired during the term of this Contract to the CO and/or COR. Additionally, the NPA Contractor shall provide a certification that all government information has been properly sanitized and purged from NPA Contractor-owned systems, including backup systems and media used during Contract performance, in accordance with HHS and/or CDC policies.

The NPA Contractor (and/or any subcontractor) shall perform and document the actions identified in the CDC Out-

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Processing Checklist (http://intranet.cdc.gov/od/hcrmo/pdfs/hr/Out_Processing_Checklist.pdf) when an employee terminates work under this Contract. All documentation shall be made available to the CO and/or COR upon request.

Records Management and Retention

The NPA Contractor (and/or any subcontractor) shall maintain all information in accordance with Executive Order 13556 -- Controlled Unclassified Information, National Archives and Records Administration (NARA) records retention policies and schedules and HHS policies and shall not dispose of any records unless authorized by HHS.

In the event that NPA Contractor (and/or any subcontractor) accidentally disposes of or destroys a record without proper authorization, it shall be documented and reported as an incident in accordance with HHS policies.

Hardware

1. **Card Readers.** The Contractor (and/or any subcontractor) shall include Federal Information Processing Standard (FIPS) 201-compliant smart card readers (referred to as LACS Transparent Readers) with the purchase of servers, printers, desktops, and laptops.
2. **Mobile Devices.** The contractor shall follow NIST 800-124, Rev. 1, *Guidelines for Managing the Security of Mobile Devices in the Enterprise* when using mobile devices that process or store HHS data.
3. Contracts for information systems, components, or services must meet the privacy requirements.
4. The Contractor (and/or any subcontractor) shall Include privacy controls in the security testing and evaluation and minimize the use of live data in development and testing.
5. The Contractor shall produce a design specification and security architecture that accurately and completely describes the privacy requirements, and the allocation of security and privacy controls among physical and logical components for any developed information system, system component, or information system service.

Information Technology Application Design, Development, or Support

6. Computer programs that comprise a series of instructions, rules, routines, or statements, regardless of the media in which recorded, that allow or cause a computer to perform a specific operation or series of operations; and
7. Recorded information comprising source code listings, design details, algorithms, processes, flow charts, formulas, and related material that would enable the computer program to be produced, created, or compiled.

“Computer software” does not include computer databases or computer software documentation.
8. The Contractor (and/or any subcontractor) shall ensure IT applications designed and developed for CDC/NIOSH must not require elevated or administrative privileges running in any context.
9. The Contractor (and/or any subcontractor) shall ensure non-interactive applications and the associated accounts running on servers (e.g., server applications, application servers, system applications, middleware, databases, containers, etc.) do not run with or require full administrative or elevated privileges.
10. The Contractor (and/or any subcontractor) shall follow secure coding best practice requirements, as directed by United States Computer Emergency Readiness Team (US-CERT) specified standards and the Open Web Application Security Project (OWASP), that will limit system software vulnerability exploits.
11. The Contractor (and/or any subcontractor) shall ensure that computer software developed on behalf of HHS or tailored from an open-source product, is fully functional and operates correctly on systems configured in accordance with government policy and federal configuration standards. The contractor shall test applicable products and versions with all relevant and current updates and patches updated prior to installing in the HHS environment. No sensitive data shall be used during software testing.
12. The Contractor (and/or any subcontractor) shall protect information that is deemed sensitive from unauthorized disclosure to persons, organizations or subcontractors who do not have a need to know the information. Information which, either alone or when compared with other reasonably-available

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information, is deemed sensitive or proprietary by HHS shall be protected as instructed in accordance with the magnitude of the loss or harm that could result from inadvertent or deliberate disclosure, alteration, or destruction of the data. This language also applies to all subcontractors that are performing under this contract.

13. The Contractor (and/or any subcontractor) shall remediate all risks or vulnerabilities immediately after the risks or vulnerabilities have been identified in the software.
14. When transferring information between different security domains, the Contractor (and/or any subcontractor) shall examine for the presence of unsanctioned information for malicious code and prohibits the transfer of such information. Unsanctioned information (information not authorized to be stored, processed or transmitted by the system) is any information not owned by the Institute nor covered by an existing DUA, DSA, MOU or other authorization. When trust relationships are established with external information systems (systems that are outside of the authorization boundary to access, process, store, or transmit PII), the Contractor (and/or any subcontractor) shall limit access to PII from external information systems with a binding agreement to terms and conditions of the privacy requirements.
15. The Contractor (and/or any subcontractor) shall review of user privileges to ensure privileges are revoked for those who no longer require access to PII or PHI and limit the following:
 - a. Limit maximum number of concurrent sessions for system accounts globally, by account type, by account, or a combination thereof
 - b. Limit access to PII from mobile devices, if applicable

Non-Commercial and Open Source Computer Software Procurements

The Contractor (and/or any subcontractor) shall follow secure coding best practice requirements, as directed by the United States Computer Emergency Readiness Team (US-CERT) specified standards and the Open Web Application Security Project (OWASP) that will limit system software vulnerability exploits.

Security Requirements for GOCO and COCO Resources

1. **Federal Policies.** The Contractor (and/or any subcontractor) shall comply with applicable federal directives that include, but are not limited to, the *HHS Information Security and Privacy Policy (IS2P)*, the *CDC Protection of Information Resources* policy; *Federal Information Security Modernization Act (FISMA) of 2014, (44 U.S.C. 101)*; National Institute of Standards and Technology (NIST) Special Publication (SP) 800-53, *Security and Privacy Controls for Federal Information Systems and Organizations*; Office of Management and Budget (OMB) Circular A-130, *Managing Information as a Strategic Resource*; and other applicable federal laws, regulations, NIST guidance, and Departmental policies.
2. **Security Assessment and Authorization (SA&A).** A valid authority to operate (ATO) certifies that the Contractor's information system meets the contract's requirements to protect the agency data. If the system under this contract does not have a valid ATO, the Contractor (and/or any subcontractor) shall work with the agency and supply the deliverables required to complete the ATO prior to any use of the system in a production capacity, i.e., its intended users able to collect, store, process or transmit data to fulfill the system's function. The Contractor shall conduct the SA&A requirements in accordance with *HHS IS2P/ CDC Protection of Information Resources*; the *CDC IT Security Program Implementation Standards*; the *CDC Security Assessment and Authorization (SA&A) Standard Operating Procedure*; and NIST SP 800-37, *Guide for Applying the Risk Management Framework to Federal Information Systems: A Security Life Cycle Approach* (latest revision).

CDC acceptance of the ATO does not alleviate the Contractor's responsibility to ensure the system security and privacy controls are implemented and operating effectively.

SA&A Package Deliverables - The Contractor (and/or any subcontractor) shall provide an SA&A package to the C/I/O Information System Security Officer (ISSO) in accordance with the timeline, process and formats proscribed for a Full system authorization in the CDC Security Assessment and Authorization Standard Operating Procedure (CDC SA&A SOP). The following SA&A deliverables are required to complete the SA&A package:

- **Baseline System Information (BSI)** – The Contractor will document a system overview, in accordance with the timeline, process and formats described in the *CDC SA&A SOP*. The BSI

will include information concerning: system identification and ownership; system data, information types, impact levels and system categorization; system functional description / general purpose; system authorization boundary and environment; system user descriptions; and system interconnections and dependencies. The Contractor shall update the BSI at least **annually** thereafter.

- **Privacy Threshold Analysis / Privacy Impact Analysis** – The Contractor (and/or any subcontractor) shall provide a PTA/PIA (as appropriate), in accordance with the timeline, process and formats described in the *CDC SA&A SOP*, if applicable. Also see the sections of this contract concerning “Privacy Threshold Analysis (PTA)/Privacy Impact Assessment (PIA)” and “Requirements for Procurements Involving Privacy Act Records.” If social security numbers (SSN) are expected to be handled by the system, the program and Contractor must include a *SSN Elimination or Usage Approval Request* along with the PTA/PIA. That request will be processed in accordance with the *OCISO Standard for Limiting the Use of Social Security Numbers in CDC Information Systems*.
- **System Security Plan (SSP)** – The SSP must be provided in a digital format supporting copy or export of all content into the HHS/CDC automated SA&A tool. The SSP shall comply with the NIST SP 800-18, *Guide for Developing Security Plans for Federal Information Systems*, the Federal Information Processing Standard (FIPS) 200, *Recommended Security Controls for Federal Information Systems*, and NIST SP 800-53, *Security and Privacy Controls for Federal Information Systems and Organizations* applicable baseline requirements, and other applicable NIST guidance as well as HHS and CDC policies and other guidance. The SSP shall be consistent with and detail the approach to IT security contained in the Contractor’s bid or proposal that resulted in the award of this contract. The SSP shall provide an overview of the system environment (including an inventory of all devices and software contained within the system boundary) and security requirements to protect the information system as well as describe all applicable security controls in place or planned for meeting those requirements. It should provide a structured process for planning adequate, cost-effective security protection for a system. The Contractor shall update the SSP at least **annually** thereafter.
- **Risk Assessment Report (RAR)** The initial security assessment shall be conducted by the Contractor in conjunction with the program’s Information System Security Officer, consistent with NIST SP 800-53A, NIST SP 800-30, and HHS and CDC policies. The assessor will document and submit the assessment results in the RAR, in accordance with the process and formats described in the *CDC SA&A SOP*. The Contractor shall address all “High” deficiencies before submitting the package to the Government for acceptance. All remaining deficiencies must be documented in a system Plan of Actions and Milestones (POA&M) for CDC OCISO approval in accordance with the *CDC SA&A SOP*. Thereafter, the Contractor, in coordination with CDC shall conduct an assessment of the security controls and update the RAR within 365 days.
- **POA&M** –The POA&M shall be documented consistent with the HHS Standard for Plan of Action and Milestones and CDC policies. Identified risks stemming from deficiencies related to the security control baseline implementation, assessment, continuous monitoring, vulnerability scanning, and other security reviews and sources, as documented in the Security Assessment Report (SAR), shall be documented and tracked by the Contractor for mitigation in the POA&M document. Depending on the severity of the risks, CDC may require designated POAM weaknesses to be remediated before an ATO is issued. Thereafter, the POA&M shall be updated at least quarterly.
- **Contingency Plan and Contingency Plan Test** –The Contingency Plan must be developed in accordance with NIST SP 800-34, *Contingency Planning Guide for Federal Information Systems*, and be consistent with HHS and CDC policies. Upon acceptance by the System Owner, the Contractor, in coordination with the System Owner, shall test the Contingency Plan and prepare a Contingency Plan Test Report that includes the test results, lessons learned and any action items that need to be addressed. Thereafter, the Contractor shall update and

test the Contingency Plan at least **annually**.

- **E-Authentication Assessment** – The contractor (and/or any subcontractor) shall collaborate with government personnel to ensure that an E-Authentication Threshold Analysis (E-auth TA) is completed to determine if a full E-Authentication Risk Assessment (E-auth RA) is necessary. System documentation developed for a system using E-auth TA/E-auth RA methods shall follow OMB 04-04; NIST SP 800-63, *Digital Identity Guidelines*; the *OCISO Standard for Electronic Authentication (E-Authentication)*; and the *CDC SA&A SOP*.

Based on the level of assurance determined by the E-Auth, the Contractor (and/or subcontractor) must ensure appropriate authentication to the system, including remote authentication, is in-place in accordance with the assurance level determined by the E-Auth (when required) in accordance with HHS policies.

Information Security Continuous Monitoring - Upon the government issuance of an Authority to Operate (ATO), the Contractor (and/or subcontractor)-owned/operated systems that input, store, process, output, and/or transmit government information, shall meet or exceed the information security continuous monitoring (ISCM) requirements in accordance with FISMA and NIST SP 800-137, *Information Security Continuous Monitoring (ISCM) for Federal Information Systems and Organizations*, and HHS IS2P. The following are the minimum requirements for ISCM:

- **Annual Assessment/Pen Test** - Assess the system security and privacy controls (or ensure an assessment of the controls is conducted) at least annually to determine the implemented security and privacy controls are operating as intended and producing the desired results (this may involve penetration testing conducted by the agency or independent third-party). In addition, review all relevant SA&A documentation (SSP, POA&M, Contingency Plan, etc.) and provide updates by specified due date.
- **Asset Management** - Using any available Security Content Automation Protocol (SCAP)-compliant automated tools for active/passive scans, provide an inventory of all information technology (IT) assets for hardware and software, (computers, servers, routers, databases, operating systems, etc.) that are processing HHS-owned information/data. It is anticipated that this inventory information will be required to be produced at least annually. IT asset inventory information shall include IP address, machine name, operating system level, security patch level, and SCAP-compliant format information. The contractor shall maintain a capability to provide an inventory of 100% of its IT assets using SCAP-compliant automated tools.
- **Account Management** – Account management is a critical function for developing and implementing an access control framework that is appropriate for the information contained in systems and applications. When implemented effectively, the access control framework provides the necessary constructs for controlling access to PII, limiting disclosure of records about individuals to only those system and application users that have a need for the information to perform their job functions. Define circumstances and/or usage conditions to be enforced for information system accounts. Usage conditions help to enforce the principle of least privilege, increase user accountability, and enable more effective account monitoring. Such monitoring includes, for example, alerts generated if the account is used outside of specified parameters. Monitor for atypical usage includes accessing systems at certain times of the day and from locations that are not consistent with the normal usage patterns of individuals working in organizations. Disabling accounts for high-risk individuals is a minimum requirement for the organization's rules of behavior as a consequence for abusing access privileges to access PII.
- **Audit and Accountability**
 - Off-loading preserve the confidentiality and integrity of audit records by moving the records from the primary system to a secondary or alternate system.
 - Storing audit information in a repository separate from the audited system or system component helps to ensure that a compromise of the system being audited does not also result in a compromise of the audit records. It may also enable management of

audit records as an organization-wide activity.

- Implement cryptographic mechanisms to protect audit log integrity and the confidentiality of the information in the logs, including information related to privacy incidents and breaches.
- Retain audit records for 7 Years. Audit records must be returned to NIOSH at the end of contract.
- Ensure long-term retrieval systems adequately address necessary safeguards to protect audit information contains PII.
- Implements the capability for authorized users to select a user session to capture/record and log content. Session auditing activities are developed, integrated, and used in consultation with legal counsel in accordance with applicable laws, Executive Orders, directives, policies, regulations, standard, and guidelines.
- The information system provides the capability for authorized users to capture/record and log content related to a user session

- **Data Monitoring**

- Implementation of security policy filters reduce the potential for unauthorized transfer of PII by providing a form of continuous monitoring for compliance with privacy laws and regulations.
- Use data type identifier to automatically detect data types and usage when being transferred from one security domain to another to protect the confidentiality of PHI
- Monitor and prevent transfer of PII across different security domains, a system needs to have mechanisms to automatically detect, and where appropriate, prohibit the unauthorized transfer of PII across different security domains.
- identify source and destination points for PII flow within information systems is necessary for attribution and compliance with need to know requirements.
- To ensure the protection of PII throughout its information flow, this control should be used to protect PII as it travels within and among information systems and information system components, such as database servers, application servers, shared storage environments, document repositories, and file folders.
- Authorize network access to organization-defined privileged commands and document the rationale for such access in the security plan for the system.
- Review of user privileges is necessary in order to ensure privileges are revoked for those who no longer require access to PII or PHI.
- Implement security attributes to support privacy protections by assigning security attributes that characterize information, devices, or processes (object) as containing PII or associate an organization's mandatory security/privacy training requirement with a user (subject) of an information system containing PII.
- Implement automated systems controls for PII and PHI objects, such as intrusion detection and key-word detection tools, maintaining the association and integrity of security attributes to subjects and objects can be used as the basis of automated policy actions
- Limit access to PII from mobile devices
- Implement either full-device or virtual container encryption to reduce the vulnerability of PII contained on mobile devices.
- Implement data mining prevention and detection techniques to protect organizational information from data mining while such information resides in organizational data

stores.

- If located in the US and storing PII about non-US Citizens, then the contractor must address the data transfer requirements of the country whose citizens PII is collected or maintained and must ensure that country's privacy/data protection legal requirements are met.

- **Data Management**

- Employ automated tools for information location includes identifying where PII and reside in system components; and how PII is being processed so that information flow can be understood and adequate protection and to ensure that the required protection measures are in place.
- Encrypt backups at the storage location.
- Employ multi-factor authentication to privileged accounts and non-privileged accounts with access to PII. This requires the use of two- factor authentication with one factor being separate from the computer itself.
- Must track, document, and verify media sanitization and disposal actions for media that contains PII.
- In any situation where PII is present, PII shall be stored on a logical or physical partition separate from the applications and software partition.

- **Configuration Management** - Use available SCAP-compliant automated tools, per NIST IR 7511, for authenticated scans to provide visibility into the security configuration compliance status of all IT assets, (computers, servers, routers, databases, operating systems, application, etc.) that store and process government information. Compliance will be measured using IT assets and standard HHS and government configuration baselines at least annually. The contractor shall maintain a capability to provide security configuration compliance information for 100% of its IT assets using SCAP-compliant automated tools. NIOSH Security and Privacy specified the requirements as follows:

- limit maximum number of concurrent sessions for system accounts globally, by account type, by account, or a combination thereof.
- Implement security compliance checks that include an assessment, prior to initial connection, of specific components, e.g., printers, based on sensitivity of PII processed by that component. Any change to the components' security posture would require a re-verification of the configuration settings.
- Employ automated mechanisms to centrally manage, apply, and verify configuration settings to detect and react to unauthorized changes related security and privacy incidents.
- Responses to unauthorized changes to the system configuration baselines, log files, and critical system files including sensitive system and application executables, libraries, and configurations.

- **Business Continuity** - NIOSH Security and Privacy specified the requirements as follows:

- Implement binding agreement to terms and conditions in agreements that cover relationships in which external information systems are used to access, process, store, or transmit and manage PII
- Implement boundary protection devices protect systems containing PII from unauthorized access by individuals outside the organization.

- **Vulnerability Management** - Use SCAP-compliant automated tools for authenticated scans to scan information system(s) and detect any security vulnerabilities in all assets (computers,

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servers, routers, Web applications, databases, operating systems, etc.) that store and process government information. Contractors shall actively manage system vulnerabilities using automated tools and technologies where practicable and in accordance with HHS policy. Automated tools shall be compliant with NIST-specified SCAP standards for vulnerability identification and management. The contractor shall maintain a capability to provide security vulnerability scanning information for 100% of IT assets using SCAP-compliant automated tools and report to the agency at least annually.

- **Patching and Vulnerability Remediation** - Install vendor released security patches and remediate critical and high vulnerabilities in systems processing government information in an expedited manner, within vendor and agency specified timeline per OCISO Vulnerability Remediation Framework Standard.
- **Secure Coding** - Follow secure coding best practice requirements, as directed by United States Computer Emergency Readiness Team (US-CERT) specified standards and the Open Web Application Security Project (OWASP), that will limit system software vulnerability exploits. If PII is used in the test environment, then the same controls required for systems containing PII must be applied to the test environment. Simulated PII information should be used to the maximum extent practicable when testing system functionality.
- **Boundary Protection** - The contractor shall ensure that government information, other than unrestricted information, being transmitted from federal government entities to external entities is routed through a Trusted Internet Connection (TIC).

3. **Government Access for Security Assessment.** In addition to the Inspection Clause in the contract, the Contractor (and/or any subcontractor) shall afford the Government access to the Contractor's facilities, installations, operations, documentation, information systems, and personnel used in performance of this contract to the extent required to carry out a program of security assessment (to include vulnerability testing), investigation, and audit to safeguard against threats and hazards to the confidentiality, integrity, and availability of federal data or to the protection of information systems operated on behalf of HHS, including but are not limited to:

At any tier handling or accessing information, consent to and allow the Government, or an independent third party working at the Government's direction, without notice at any time during a weekday during regular business hours contractor local time, to access contractor and subcontractor installations, facilities, infrastructure, data centers, equipment (including but not limited to all servers, computing devices, and portable media), operations, documentation (whether in electronic, paper, or other forms), databases, and personnel which are used in performance of the contract

The Government includes but is not limited to the U.S. Department of Justice, U.S. Government Accountability Office, and the HHS Office of the Inspector General (OIG). The purpose of the access is to facilitate performance inspections and reviews, security and compliance audits, and law enforcement investigations. For security audits, the audit may include but not be limited to such items as buffer overflows, open ports, unnecessary services, lack of user input filtering, cross-site scripting vulnerabilities, SQL injection vulnerabilities, and any other known vulnerabilities.

At any tier handling or accessing protected information, fully cooperate with all audits, inspections, investigations, forensic analysis, or other reviews or requirements needed to carry out requirements presented in applicable law or policy. Beyond providing access, full cooperation also includes, but is not limited to, disclosure to investigators of information sufficient to identify the nature and extent of any criminal or fraudulent activity and the individuals responsible for that activity. It includes timely and complete production of requested data, metadata, information, and records relevant to any inspection, audit, investigation, or review, and making employees of the contractor available for interview by inspectors, auditors, and investigators upon request. Full cooperation also includes allowing the Government to make reproductions or copies of information and equipment, including, if necessary, collecting a machine or system image capture.

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Segregate Government protected information and metadata on the handling of Government protected information from other information. Commingling of information is prohibited. Inspectors, auditors, and investigators will not be precluded from having access to the sought information if sought information is commingled with other information.

Cooperate with inspections, audits, investigations, and reviews.

4. **End of Life Compliance.** The Contractor (and/or any subcontractor) must use Commercial off the Shelf (COTS) software or other software that is supported by the manufacturer. In addition, the COTS/other software need to be within one major version of the current version; deviation from this requirement will only be allowed via the HHS waiver process (approved by HHS CISO). The contractor shall retire and/or upgrade all software/systems that have reached end-of-life in accordance with HHS *End-of-Life Operating Systems, Software, and Applications Policy*.
5. **Desktops, Laptops, and Other Computing Devices Required for Use by the Contractor.** The Contractor (and/or any subcontractor) shall ensure that all IT equipment (e.g., laptops, desktops, servers, routers, mobile devices, peripheral devices, etc.) used to process information on behalf of HHS are deployed and operated in accordance with approved security configurations and meet the following minimum requirements:
 - Encrypt information categorized as moderate or high impact as required by OMB Memorandum A-130, *Managing Information as Strategic Resource*, in accordance with the HHS *Standard for Encryption of Computing Devices and Information* and FIPS 140-2.
 - Configure laptops and desktops in accordance with the latest applicable United States Government Configuration Baseline (USGCB) and HHS Minimum Security Configuration Standards;
 - Maintain the latest operating system patch release and anti-virus software definitions;
 - Validate the configuration settings after hardware and software installation, operation, maintenance, update, and patching and ensure changes in hardware and software do not alter the approved configuration settings; and
 - Automate configuration settings and configuration management in accordance with HHS security policies, including but not limited to:
 - Configuring its systems to allow for periodic HHS vulnerability and security configuration assessment scanning; and
 - Using Security Content Automation Protocol (SCAP)-validated tools with USGCB Scanner capabilities to scan its systems at least on a monthly basis and report the results of these scans to the CO and/or COR, Project Officer, and any other applicable designated POC.
6. **Change Management.** Once a system is authorized, all changes must be approved by CDC in accordance with NIST SP 800-53, *Security and Privacy Controls for Federal Information Systems and Organizations*; the *HHS IS2P*; and the timeline, process and formats proscribed in the CDC *OCISO Change Management Standard Operating Procedure*.
7. **Retirement / Decommissioning.** When the CDC program and Contractor determine the system is no longer required, it must be decommissioned in accordance NIST SP 800-88, *Guidelines for Media Sanitization*; the *HHS IS2P*; and the timeline, process and formats proscribed in the CDC *OCISO System Retirement Standard Operating Procedure*.
8. **Identifier management** - The Contractor (and/or any subcontractor) shall ensure that SSNs, and parts of SSNs, must not be used as system identifiers. Identifier management must ensure that any access to, or action involving, PII is attributable to a unique individual.

HHS FedRAMP Privacy and Security Requirements Involving Cloud Services

The Contractor (and/or any subcontractor) shall be responsible for the following privacy and security requirements:

1. **FedRAMP Compliant ATO.** Comply with FedRAMP Security Assessment and Authorization (SA&A) requirements and ensure the information system/service under this contract has a valid FedRAMP

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compliant (approved) authority to operate (ATO) in accordance with Federal Information Processing Standard (FIPS) Publication 199 defined security categorization. If a FedRAMP compliant ATO has not been granted, the Contractor shall submit a plan to obtain a FedRAMP compliant ATO.

Implement applicable FedRAMP baseline controls commensurate with the agency-defined security categorization and the applicable FedRAMP security control baseline (www.FedRAMP.gov). The HHS Information Security and Privacy Policy (IS2P) and HHS Cloud Computing and Federal Risk and Authorization Management Program (FedRAMP) Guidance further define the baseline policies as well as roles and responsibilities. The Contractor shall also implement a set of additional controls identified by the agency when applicable.

A security control assessment must be conducted by a FedRAMP third-party assessment organization (3PAO) for the initial ATO and annually thereafter or whenever there is a significant change to the system's security posture in accordance with the FedRAMP Continuous Monitoring Plan.

2. **Data Jurisdiction.** The contractor shall store all information within the security authorization boundary, data at rest or data backup, within the continental United States (CONUS) if so required.
3. **Service Level Agreements.** The Contractor shall understand the terms of the service agreements that define the legal relationships between cloud customers and cloud providers and work with CDC to develop and maintain an SLA.
4. **Interconnection Agreements/Memorandum of Agreements.** The Contractor shall establish and maintain Interconnection Agreements and or Memorandum of Agreements/Understanding in accordance with HHS/CDC policies.

Protection of Information in a Cloud Environment

1. If contractor (and/or any subcontractor) personnel must remove any information from the primary work area, they shall protect it to the same extent they would the proprietary data and/or company trade secrets and in accordance with HHS/CDC policies <http://www.hhs.gov/ocio/policy/>.
2. HHS will retain unrestricted rights to federal data handled under this contract. Specifically, HHS retains ownership of any user created/loaded data and applications collected, maintained, used, or operated on behalf of HHS and hosted on contractor's infrastructure, as well as maintains the right to request full copies of these at any time. If requested, data must be available to HHS within **one (1) business day** from request date or within the timeframe specified otherwise. In addition, the data shall be provided at no additional cost to HHS.
3. The Contractor (and/or any subcontractor) shall ensure that the facilities that house the network infrastructure are physically and logically secure in accordance with FedRAMP requirements and HHS policies.
4. The contractor shall support a system of records in accordance with NARA-approved records schedule(s) and protection requirements for federal agencies to manage their electronic records in accordance with 36 CFR § 1236.20 & 1236.22 (ref. a), including but not limited to the following:
 - Maintenance of links between records and metadata, and
 - Categorization of records to manage retention and disposal, either through transfer of permanent records to NARA or deletion of temporary records in accordance with NARA-approved retention schedules.
5. The disposition of all HHS data shall be at the written direction of HHS/CDC. This may include documents returned to HHS control; destroyed; or held as specified until otherwise directed. Items returned to the Government shall be hand carried or sent by certified mail to the COR.
6. If the system involves the design, development, or operation of a system of records on individuals, the Contractor shall comply with the contract language herein related to "Requirements for Procurements Involving Privacy Act Records".

Security Assessment and Authorization (SA&A) Process Involving Cloud Services

1. The Contractor (and/or any subcontractor) shall comply with HHS and FedRAMP requirements as

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mandated by federal laws, regulations, and HHS policies, including making available any documentation, physical access, and logical access needed to support the SA&A requirement. The level of effort for the SA&A is based on the system's FIPS 199 security categorization and HHS/CDC security policies and in accordance with the contract language herein related to "Procurements Involving Government Information Processed on GOCO or COCO Systems".

In addition to the FedRAMP compliant ATO, the contractor shall complete and maintain an agency SA&A package to obtain agency ATO prior to system deployment/service implementation in accordance with the contract language herein related to "Procurements Involving Government Information Processed on GOCO or COCO Systems". The agency ATO must be approved by the CDC Authorizing Official (AO) prior to implementation of system and/or service being acquired.

CSP systems must leverage a FedRAMP accredited third-party assessment organization (3PAO).

For all acquired cloud services, the SA&A package must contain documentation in accordance with the contract language herein related to "Procurements Involving Government Information Processed on GOCO or COCO Systems". Following the initial ATO, the Contractor must review and maintain the ATO in accordance with HHS/CDC policies.

2. HHS reserves the right to perform penetration testing (pen testing) on all systems operated on behalf of agency. If HHS exercises this right, the Contractor (and/or any subcontractor) shall allow HHS employees (and/or designated third parties) to conduct Security Assessment activities to include control reviews in accordance with HHS requirements. Review activities include, but are not limited to, scanning operating systems, web applications, wireless scanning; network device scanning to include routers, switches, and firewall, and IDS/IPS; databases and other applicable systems, including general support structure, that support the processing, transportation, storage, or security of Government information for vulnerabilities.
3. The Contractor must identify any gaps between required FedRAMP Security Control Baseline/Continuous Monitoring controls and the contractor's implementation status as documented in the Security Assessment Report and related Continuous Monitoring artifacts. In addition, all gaps shall be documented and tracked by the contractor for mitigation in a Plan of Action and Milestones (POA&M) document. Depending on the severity of the risks, HHS may require remediation at the contractor's expense, before HHS issues an ATO.
4. The Contractor (and/or any subcontractor) shall mitigate security risks for which they are responsible, including those identified during SA&A and continuous monitoring activities. All vulnerabilities and other risk findings shall be remediated by the prescribed timelines from discovery: (1) critical vulnerabilities no later than **thirty (30) days** and (2) high, medium and low vulnerabilities no later than **sixty (60) days**. In the event a vulnerability or other risk finding cannot be mitigated within the prescribed timelines above, they shall be added to the designated POA&M and mitigated within the newly designated timelines. For all system-level weaknesses, the following are specified mitigation timelines from weakness creation date in the POA&M:

30 days for critical weaknesses;

60 days for high weaknesses;

180 days for medium weaknesses; and

365 days for low weakness.

HHS will determine the risk rating of vulnerabilities using FedRAMP baselines.

5. **Revocation of a Cloud Service.** HHS/CDC/OCIO have the right to take action in response to the CSP's lack of compliance and/or increased level of risk. In the event the CSP fails to meet HHS and FedRAMP security and privacy requirements and/or there is an incident involving sensitive information, HHS and/or CDC may suspend or revoke an existing agency ATO (either in part or in whole) and/or cease operations. If an ATO is suspended or revoked in accordance with this provision, the CO and/or COR may direct the CSP to take additional security measures to secure sensitive information. These measures may include restricting access to sensitive information on the Contractor information system under this contract. Restricting access may include disconnecting the system processing, storing, or transmitting the sensitive information from the Internet or other networks or applying additional security controls.

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Reporting and Continuous Monitoring Involving Cloud Services

1. Following the initial ATOs, the Contractor (and/or any subcontractor) must perform the minimum ongoing continuous monitoring activities specified below, submit required deliverables by the specified due dates, and meet with the system/service owner and other relevant stakeholders to discuss the ongoing continuous monitoring activities, findings, and other relevant matters. The CSP will work with the agency to schedule ongoing continuous monitoring activities.
2. At a minimum, the Contractor must provide the following artifacts/deliverables on a **monthly** basis:

Vendor/Contractor that owns infrastructure where the system resides:

- a. Perform periodic Authenticated Vulnerability Scans and Application Scans (if applicable) according to CSPO ISCM guidance
- b. Perform weekly scans (at a minimum) and provide results to C/I/O/ISSO and CSPO ISCM for systems with a FIPS 199 impact level of High, HVA, or if the system contains PII, and ensure scan results are submitted in either CSV or PDF format
- c. Remediate vulnerabilities in accordance with CSPO Vulnerability Remediation Framework Policy
- d. Advise the C/I/O/ISSO for any instance when critical/high vulnerabilities cannot be remediated as in accordance with the CSPO Vulnerability Framework Standard
- e. Submit monthly Authenticated Vulnerability scans and Application scans (if applicable) to CDC//NIOSH (business owner) and C/I/O/ISSO

Business Stewards (such as System Owner):

- f. Confirm Vendor/Contractor is performing Authenticated Vulnerability Scans and Application Scans (if applicable) according to CSPO ISCM guidance
- g. Review monthly Authenticated Vulnerability Scans and Application Scans (if applicable); Develop POA&Ms as needed
- h. Submit monthly Authenticated Vulnerability Scans and Application Scans (if applicable) to CSPO ISCM
- i. Submit written waiver requests to the CISO when systems cannot comply with the provisions of this standard
- j. Track remediation/mitigation of security gaps to closure

Operating system, database, Web application, and network vulnerability scan results;

Updated POA&Ms;

Any updated authorization package documentation as required by the annual attestation/assessment/review or as requested by the System Owner or AO; and

Any configuration changes to the system and/or system components or CSP's cloud environment, that may impact HHS/CDC's security posture. Changes to the configuration of the system, its components, or environment that may impact the security posture of the system under this contract must be approved by the agency.

Configuration Baseline

1. The contractor shall certify that applications are fully functional and operate correctly as intended on systems using the US Government Configuration Baseline (USGCB), DISA Security Technical Implementation Guides (STIGs), Center for Information Security (CIS) Security Benchmarks or any other HHS-identified configuration baseline. The standard installation, operation, maintenance, updates, and/or patching of software shall not alter the configuration settings from the approved HHS/CDC configuration baseline.
2. The contractor shall use Security Content Automation Protocol (SCAP) validated tools with configuration baseline scanner capability to certify their products operate correctly with HHS and NIST defined

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configurations and do not alter these settings.

Incident Reporting

1. The Contractor (and/or any subcontractor) shall provide an Incident and Breach Response Plan (IRP) in accordance with HHS CDC, OMB, and US-CERT requirements and obtain approval from the CDC. In addition, the Contractor must follow the incident response and US-CERT reporting guidance contained in the FedRAMP Incident Communications.
2. The Contractor (and/or any subcontractor) must implement a program of inspection to safeguard against threats and hazards to the security, confidentiality, integrity, and availability of federal data, afford HHS access to its facilities, installations, technical capabilities, operations, documentation, records, and databases within **72 hours** of notification. The program of inspection shall include, but is not limited to:

Conduct authenticated and unauthenticated operating system/network/database/Web application vulnerability scans. Automated scans can be performed by HHS/CDC personnel, or agents acting on behalf of HHS/CDC, using agency-operated equipment and/or specified tools. The Contractor may choose to run its own automated scans or audits, provided the scanning tools and configuration settings are compliant with NIST Security Content Automation Protocol (SCAP) standards and have been approved by the agency. The agency may request the Contractor's scanning results and, at the agency discretion, accept those in lieu of agency performed vulnerability scans.

In the event an incident involving sensitive information occurs, cooperate on all required activities determined by the agency to ensure an effective incident or breach response and provide all requested images, log files, and event information to facilitate rapid resolution of sensitive information incidents. In addition, the Contractor must follow the agency reporting procedures and document the steps it takes to contain and eradicate the incident, recover from the incident, and provide a post-incident report that includes at a minimum the following:

- Company and point of contact name;
- Contract information;
- Impact classifications/threat vector;
- Type of information compromised;
- A summary of lessons learned; and
- Explanation of the mitigation steps of exploited vulnerabilities to prevent similar incidents in the future.

Media Transport Involving Cloud Services

1. The Contractor and its employees shall be accountable and document all activities associated with the transport of government information, devices, and media transported outside controlled areas and/or facilities. These include information stored on digital and non-digital media (e.g., CD-ROM, tapes, etc.), mobile/portable devices (e.g., USB flash drives, external hard drives, and SD cards)
2. All information, devices and media must be encrypted with HHS-approved encryption mechanisms to protect the confidentiality, integrity, and availability of all government information transported outside of controlled facilities.

Boundary Protection: Trusted Internet Connections (TIC)

- 1) The contractor shall ensure that government information, other than unrestricted information, being transmitted from federal government entities to external entities using cloud services is inspected by Trusted Internet Connection (TIC) processes.
- 2) The contractor shall route all external connections through a TIC.
- 3) **Non-Repudiation.** The contractor shall provide a system that implements FIPS 140-2 validated encryption that provides for origin authentication, data integrity, and signer non-repudiation.

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Secure Data Transfer Language

Files Transmission

There are two options for electronic submission of documents/deliverables; CDC SFTP Site or Secure Access Management Services (SAMS) are the two approved methods of transmission.

1. Secure File Transfer Protocol (SFTP) site is used for downloading and uploading files from or to the CDC network.
2. SAMS is the CDC's electronic authentication (E-Auth) provider to support access for external partners. E-Auth systems protect CDC by authenticating a user and authorizing that person to use specific IT resources.

SECTION 5 – GOVERNMENT FURNISHED PROPERTY

No Government Furnished Property, as defined in Federal Acquisition Regulation (FAR) Section 45.101, is anticipated to be provided under this Contract.

SECTION 6 – PLACE OF PERFORMANCE AND CONTRACT SUPPORT HOURS

SUBSECTION A – PLACE OF PERFORMANCE

This Contract shall be performed entirely off site (NPA Contractor's facility or other) within the United States.

SUBSECTION B – CONTRACT SUPPORT HOURS

The NPA Contractor personnel shall maintain working hours in the Eastern Time Zone (ET) when most work is performed for this Contract, unless otherwise approved in writing by the Government, thereby enhancing collaboration with the Program. NPA Contractor staff shall maintain standard working hours and be available to attend meetings (in person or virtually) with the Program and other Program contractors between the hours of 9:00am and 5:00pm ET, Monday through Friday excluding federal holidays, and extended hours as needed.

SECTION 7 – TRAVEL

During Transition-In, the NPA Contractor is expected to travel for:

- **Kickoff Meeting** at the Program's Office in Washington, DC. This meeting is expected to last two days.
- **Transition-In Coordination Meetings:** These meetings are expected to last two days and shall occur approximately five times during the 12-month Transition-In period in New York, NY; Washington, DC; Cincinnati, OH; or Atlanta, GA. Meetings may also occur at the NPA Contractor's office(s), as agreed.

During each year of Sustainment, the NPA Contractor is expected to travel for the following contractually required meetings and events:

- **Sustainment Coordination Meetings:** These meetings are expected to last two days and shall occur approximately three times each calendar year in New York, NY; Washington, DC; Cincinnati, OH; or Atlanta, GA. Meetings may also occur at the NPA Contractor's office(s), as agreed.
- **Member Engagement Town Halls:** These meetings are expected to last one day, shall occur two times per year (in-person or virtual) and, where in-person, shall be held at the NPA Contractor's location of choice pursuant to Section 4.4.1.1.

The NPA Contractor shall also be prepared for **Ad Hoc Travel** during both Transition-In and each year of Sustainment. These meetings are expected to last two days and anticipated to occur two times each calendar year in New York, NY.

Travel shall be in accordance with, and not to exceed, the General Services Administration Federal Travel Regulation per diem rates. *See* 41 C.F.R. Subtitle F and FAR Part 31. The NPA Contractor shall obtain written approval from the COR prior to undertaking any travel.

Travel does not include conferences. If the need for conference travel occurs, the Contract will be modified to separately add funding.

SECTION 8 – DELIVERABLES

All deliverables required under this Contract shall be packaged, marked, and sent in accordance with the guidance found in this Section. Deliverables are to be delivered to invoice_deliverable@cdc.gov and the COR and/or their designee(s) by the due date listed in the deliverables table; due dates that fall on a weekend or federal holiday are due the following business day. The NPA Contractor may submit deliverables in advance of the due date if desired. The CO may extend due dates or modify report requirements or formats upon a written request by the NPA Contractor to the CO, where the NPA Contractor has demonstrated a good and compelling reason for the extension or modification. The determination to grant a modification or extension of time will be made by the CO and communicated to the NPA Contractor in writing.

The NPA Contractor shall guarantee that all deliverables are sent in immediately usable and acceptable condition, free of errors. Deliverables submitted under the Contract shall reference and cite the contract number and identify CDC as the sponsoring agency.

All NPA Contractor deliverables shall be submitted in an agreed-upon format. All plans and approaches shall be tailored to the WTC Health Program and align with Program requirements. All files containing PII/PHI must be securely transmitted via the CARE Portal or SFTP, with the delivery mechanism referenced in the deliverable email. The Program may update reporting requirements and templates throughout the period of performance; the NPA Contractor shall implement such changes within a timeframe agreed-upon between the Program and the NPA Contractor.

Deliverables fall into three primary document types:

1. **Plans** shall detail the NPA Contractor's intent and the process of how the NPA Contractor will meet, monitor, and ideally exceed minimum performance standards. Plans shall be updated as needed to reflect changes in NPA Contractor or Program strategy for the future resulting from new information learned and decisions made in the present.
2. **Manuals** shall collate the NPA Contractor's processes, procedures, and reference materials that operationalize the NPA Contractor's plans. Manuals shall detail staff roles and responsibilities, methods, applied industry standards, and any tools or systems utilized.
3. **Reports** shall detail the NPA Contractor's activities and outcomes and analyze how the outcome was achieved and how the outcome can be improved, where applicable. Reports are static snapshots of current performance. Reports shall provide summaries which highlight significant trends and include planned improvements and/or mitigation strategies. All reports shall include narrative text in the document that summarizes and interprets the report data and explains any unusual occurrences or trends.

Section 8.1 – Deliverable Review and Acceptance

Satisfactory reporting performance under the Contract shall be deemed to occur upon delivery and written acceptance by the CO, or their duly authorized representative, of the items listed in Section 8.2.

The Program will review and either accept or reject each deliverable. If a deliverable is rejected, the NPA Contractor shall resubmit a revised deliverable within 14 calendar days of receiving the Program's feedback, unless otherwise specified. This cycle shall continue until the Program accepts the deliverable. All Transition-In deliverables due prior to SHCD must be accepted at least 30 calendar days before SHCD.

The NPA Contractor shall review all plans, manuals, and communications products—except for those marked as being exclusively transition-related—at least annually and submit any updates for Program approval 30 calendar days prior to the end of each contract year unless otherwise specified. Additionally, the NPA Contractor shall be prepared to submit updated deliverables within 60 calendar days of any substantive changes and upon request by the Program.

Deliverables provided pursuant to this Contract will be accepted based in part on satisfaction of the identified requirements in Section 508 of the Rehabilitation Act of 1973, as amended, for accessibility and shall include final test results demonstrating Section 508 compliance. Deliverables shall meet applicable accessibility requirements and

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should not adversely affect accessibility features of existing Electronic Information (EIT) technologies. The Government reserves the right to independently test for Section 508 compliance before delivery. The NPA Contractor shall be able to demonstrate Section 508 compliance upon delivery.

Section 8.2 – Deliverables Summary Table

All Deliverables shall be delivered to the COR and COR designee, in MS Word, MS Excel, or other appropriate and agreed upon format.

4.1 – TRANSITION-IN		
Deliverable	Due Date	Section Reference
NPA Launch Meeting Minutes	Within three calendar days after the NPA Launch Meeting	Section 4.1.1
Draft Kickoff Meeting Agenda	No later than seven calendar days prior to the Kickoff Meeting	Section 4.1.1
Kickoff Meeting Agenda and Draft Kickoff Presentation	No later than three calendar days prior to the Kickoff Meeting	Section 4.1.1
Kickoff Meeting Minutes	Within three calendar days after the Kickoff Meeting	Section 4.1.1
Transition-In Plan (TIP)	Within 14 calendar days after the Launch Meeting	Section 4.1.2 Section 8.3.1.1
Transition-In Master Schedule (TIMS)	Initial submission within 14 calendar days after the Launch Meeting Thereafter submitted with each <i>TIPR</i> and within one business day after request	Section 4.1.2 Section 8.3.1.2
Transition-In Progress Report (TIPR)	Every two weeks during Transition Meetings, starting no more than seven calendar days after the Launch Meeting through SHCD	Section 4.1.2 Section 8.3.1.3
Risk, Assumptions, Issues, and Decisions (RAID) Log	Submitted with each <i>TIPR</i> and within one business day after request	Section 4.1.2
Operations Plan	Within 30 calendar days after the Kickoff Meeting	Section 4.1.2 Section 8.3.2.1
Operations Manual	No later than 60 calendar days prior to SHCD	Section 4.1.2 Section 8.3.2.2
Data Dictionary	Initial submission within 90 calendar days after the award effective date Thereafter submitted with the <i>TIPR</i> , if updated since last submission	Section 4.1.3.1 Section 8.3.2.3
Authorization to Operate (ATO) Package	Within 45 calendar days after the award effective date	Section 4.1.3.1
Transition-in Communications Plan	Within 14 calendar days after the Kickoff Meeting and at least 10 business days prior to date a proposed change would be scheduled to take effect	Section 4.1.3.4 Section 8.3.1.4
Communications Plan	Within 90 calendar days after the award effective date and at least 10 business days prior to date a proposed change would be scheduled to take effect	Section 4.1.3.4 Section 8.3.2.4

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System Testing Results	Incrementally consistent with <i>TIP</i> , with all results delivered no later than 90 calendar days prior to SHCD	Section 4.1.4
Mitigation and Revised Testing Plan	Within 14 calendar days of notification of unsuccessful testing by the Program	Section 4.1.4.1
Transition-In Summary Report	Interim report due within 60 calendar days after SHCD Final report due within 120 calendar days after SHCD	Section 4.1.5
4.2 – CONTRACT MANAGEMENT		
Change Request Log	Submitted with the <i>MPR</i> and within one business day upon request	Section 4.2.1.2
RAID Log	Submitted with the <i>MPR</i> and within one business day upon request	Section 4.2.1.3
Meeting Agenda	At least one business day prior to each scheduled meeting with the Program and each scheduled meeting for which the NPA Contractor is “Owner”	Section 4.2.2.1
Meeting Minutes	Within three business days after each scheduled meeting with the Program and each scheduled meeting for which the NPA Contractor is “Owner”	Section 4.2.2.1
Monthly Progress Report (MPR)	No later than 15 calendar days after the end of each calendar month, starting in the first month after SHCD	Section 4.2.2.2 Section 8.3.2.5
Staffing and Wages Report	No later than 15 calendar days after the end of each quarter	Section 4.2.2.3
Semi-Annual Report (SAR)	Semi-annually every six months, in May and November	Section 4.2.2.4
Outlier Semi-Annual Report (OSAR) Response	No later than 60 days after the OSAR is provided to the NPA Contractor	Section 4.2.2.4
(Updated) Operations Plan	Within 60 calendar days after the start of each Option Period	Section 4.2.2.5 Section 8.3.2.1
(Updated) Operations Manual	Within 60 calendar days after the start of each Option Period	Section 4.2.2.5 Section 8.3.2.2
(Updated) Communications Plan	Within 60 calendar days after the start of each Option Period At least 10 business days prior to the date a proposed change would be scheduled to take effect	Section 4.2.2.5 Section 8.3.2.4
Staff Training Materials	Prior to training delivery	Section 4.2.3.3
Communications Products	Prior to publication (clean and tracked changes) and post-publication (final, published form) Annually, a copy of each Communication Product in use (complete inventory)	Section 4.2.4.1
4.3 – INFRASTRUCTURE, RESOURCES, AND FUNCTIONALITIES		

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Data Dictionary	At the end of each Option Period and no more than 10 business days following an addition, modification, or removal of a data element Submitted with each <i>Comprehensive Data Extract</i>	Section 4.3.2 Section 4.3.3.4 Section 8.3.2.3
Comprehensive Data Extract	At the end of Handover (end of Transition-In Month 12) and each Option Year, at the termination of the Contract, and at any other time as requested by the Program	Section 4.3.3.4
System Maintenance Notification	At least two weeks in advance of any scheduled system maintenance or, in the case of unscheduled, responsive maintenance, within one hour of the issue being detected	Section 4.3.4.1 Section 4.3.4.2
Root Cause Analysis	Within 10 business days of request	Section 4.3.4.2
4.4 – AREAS OF ASSISTANCE		
Disruptive Member Incident Report	Submitted within 3 business days of a disruptive incident	Section 4.4.1.3
4.5 – TRANSITION-OUT		
Transition-Out Plan	Within 30 calendar days after the Transition-Out CLIN period of performance begins	Section 4.5 Section 8.3.3.1
Transition-Out Master Schedule (TOMS)	Within 30 calendar days after the Transition-Out CLIN period of performance begins	Section 4.5 Section 8.3.3.2
Transition-Out Communications Plan	Within 30 calendar days after the Transition-Out CLIN period of performance begins	Section 4.5 Section 8.3.3.3
4.6 - REGULATORY COMPLIANCE AND SECURITY REQUIREMENTS		
Business Associate Agreement (BAA)	Executed upon award, prior to performing any work on behalf of HHS	Section 4.6.1
HIPAA Incident Report	Submitted within three business days after the discovery of a potential HIPAA incident	Section 4.6.1
FWA Incident Report	Five business days after the discovery of a potential FWA incident	Section 4.6.2
Security Deliverables	See Section 4.6.4	Section 4.6.4

Section 8.3 – Components of Select Deliverables

Section 8.3.1 – Transition-In Deliverables

Section 8.3.1.1 – Transition-In Plan (TIP)

The *Transition-In Plan* shall detail all activities that the NPA Contractor will perform during the first eight months of Transition-In to ensure a smooth transition from the current state (NPN contract) to future state (Sustainment phase of this Contract, to include Handover). This includes system development and testing; process development and implementation; hiring and training staff; coordination with the outgoing contractor, the Program, and other Program contractors; and all other Transition-In tasks, as described in Section 4.1. The *Transition-In Plan* shall cover the following topics as they relate to Transition-In:

1. Contract Management
 - a. Role Responsible, Accountable, Consulted, and Informed (RACI) for each area of

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responsibility/task

- b. Plan to systematically identify, analyze, track, mitigate, and escalate project risks and issues, to include internal change control protocols
- c. Points of Contact, activities, and cadence of interaction between the NPA Contractor, the Program, and other Program contractors through system development, testing, and validation

2. Staffing

- a. Hiring plan and timeline to complete Transition-In Tasks and prepare for Sustainment
- b. Training plan for each area of responsibility/task, to include purpose, topics, timeline, cadence, audience, and delivery style/methods for each role, as well as mechanism(s) to evaluate training quality and effectiveness to ensure learning objectives are met

3. Operational Readiness

- a. System testing strategy, to include testing cycles and duration, success criteria, and validation procedures
- b. Comprehensive and complete testing scenarios (including why the scenario was chosen and what associated business rule is being tested)
- c. Process validation strategy, to include success criteria and validation procedures
- d. Metrics and documentation that the NPA Contractor plans to use to track progress towards readiness and demonstrate readiness prior to SHCD

Section 8.3.1.2 – Transition-In Master Schedule (TIMS)

The *TIMS* shall detail all project tasks in a clear work breakdown structure to include anticipated start and finish dates, constraints for each task (including predecessor and successors), and responsible parties. Tasks on the lowest, most granular level shall not have durations any longer than 80 hours (two weeks). The *TIMS* shall list NPA Contractor tasks and, where appropriate, chronological relation to Program and other Program contractor milestones, to demonstrate readiness for a successful SHCD. Throughout Transition-In, the NPA Contractor shall track Transition-In progress against the *TIMS* and maintain the *TIMS* to reflect any agreed-upon changes to the timeline.

Section 8.3.1.3 – Transition-In Progress Report (TIPR)

The *TIPR* shall summarize the status of Transition-In activities, with emphasis on the changes made to the *TIMS* since the previous *TIPR* and emphasis on items that may require Program input or support to prevent impacts on transition cost, project timeline, or key milestones. The NPA Contractor shall track success rate of Transition-In *Communications Products* and report on trends in the *TIPR*.

Section 8.3.1.4 – Transition-In Communications Plan

The *Transition-In Communications Plan* shall describe the NPA Contractor's strategy for conducting outreach, establishing a relationship with existing NPA members, ensuring members are aware of the upcoming transition, and preparing members for any lapse in service or major changes to their member experience at or following SHCD. The NPA Contractor shall clearly describe the *Communications Products* it plans to disseminate throughout the first eight months of Transition-In, as well as the internal processes it plans to implement to ensure consistent and high-quality communications products that meet Program standards.

The *Transition-In Communications Plan* shall include all elements described in TGD 020 (ATT007_TGD020_CommsPlan in Section D).

Section 8.3.2 – Sustainment and Cross-Contract Deliverables

Section 8.3.2.1 – Operations Plan

The *Operations Plan* shall describe the NPA Contractor's proposed approach to meeting all Sustainment requirements in the PWS. The *Operations Plan* shall include end-to-end process workflows for each task, highlighting roles and responsibilities, system(s) involved, and clear indications of whether tasks are intended to be automated or manual.

Section 8.3.2.2 – Operations Manual

The *Operations Manual* shall contain a complete set of written standardized operational procedures, highlighting approximate task durations for manual tasks and cadence and validations for automated tasks. The *Operations Manual* shall be comprehensive and detail how the NPA Contractor will execute Sustainment requirements in compliance with Program requirements and performance standards.

Section 8.3.2.3 – Data Dictionary

The *Data Dictionary* shall comprehensively describe the structure and content of the NPA Contractor's data management approach. This includes descriptions of the names, relationships, validations, and business definitions of all data fields in the NPA Contractor's database(s), as well as indicate the unique identifier or primary key per entry. For any calculated fields in the *Data Dictionary*, any other fields used or referenced in the calculations should be listed. Wherever applicable, the *Data Dictionary* should include a value range for numeric fields, a list of possible values for categorical fields, and sample responses for other types of fields. The *Data Dictionary* should also indicate the source of data for each field (i.e., XYZ form, system, etc.). Key elements of the Data Dictionary include data mapping crosswalks between the NPA Contractor's database(s) internally and between the NPA Contractor's database(s) and other external databases (e.g., Data Center databases) with which the NPA Contractor exchanges data, per Section 4.3.3.

Section 8.3.2.4 – Communications Plan

The *Communications Plan* shall describe the NPA Contractor's overarching strategy for communicating NPA-related information to members and NPN providers to ensure that individuals involved in or affected by the NPA Contractor's activities understand the NPA's role and have a seamless Program experience. The *Communications Plan* shall clearly describe all *Communications Products* the NPA Contractor plans to disseminate throughout each Sustainment period of performance, as well as the internal processes it plans to implement to ensure consistent and high-quality communications products that meet Program standards. The *Communications Plan* will serve as a living document throughout Contract performance. An updated *Communications Plan* must be submitted at least 10 business days prior to the date a proposed change (e.g., event, new or modified *Communications Product*), would be scheduled to take effect. An updated *Communications Plan* shall also be submitted upon SHCD, and at the start of each Option Period, if exercised, as detailed in Section 4.2.4.

Communications Plan content shall be consistent with TGD 020 (ATT007_TGD20_CommsPlan in Section D).

Section 8.3.2.5 – Monthly Progress Report (MPR)

The *Monthly Progress Report (MPR)* shall detail the NPA Contractor's activities and accomplishments during the previous calendar month, planned work during the upcoming month, and identify any potential risks or significant issues requiring resolution, along with proposed corrective actions and the corresponding timeframe for mitigation. The *MPR* should be treated like an operational snapshot and shall provide sufficient technical detail to disclose all work started and results achieved during the reporting period. The intent of the *MPR* is to provide meaningful insight into the NPA Contractor's performance and its members' experiences and health outcomes, as well as allow the Program to track all significant activities and progress across each of the NPA Contractor's areas of responsibility.

The *MPR* shall contain summary-level data as well as narrative description and analysis of findings, trends and emerging patterns, unusual occurrences (such as outliers, errors, and limitations), and all other requirements in TGD 009 (ATT021_TGD009_MonthlyReport in Section D). The NPA Contractor may also report on additional elements of interest identified by the NPA Contractor.

Section 8.3.3 – Transition-Out Deliverables

Section 8.3.3.1 – Transition-Out Plan

The Transition-Out Plan shall detail all activities that the NPA Contractor will perform during the 12 months of Transition-Out to ensure a smooth transfer of duties and responsibilities from the NPA Contractor to a Program-appointed designee, whether a government entity, another contractor, or a combination of both. The Transition-Out Plan shall detail NPA Contractor activities to transfer all Program data and information to the Program and/or successor contractor(s); maintain operational standards; and complete all Transition-Out tasks (as described in Section 4.5) successfully. The Transition-Out Plan shall also outline coordination activities/communication touchpoints with the Program and successor(s) to ensure an efficient and seamless transition and continuity of

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services for NPA members. The Transition-Out Plan shall be maintained in alignment with the successor(s)' Transition-In Plan.

Section 8.3.3.2 – Transition-Out Master Schedule (TOMS)

The *TOMS* shall detail all project tasks in a clear work breakdown structure to include anticipated start and finish dates, constraints for each task (including predecessor and successors), and responsible parties. Tasks on the lowest, most granular level shall not have durations any longer than 80 hours (two weeks). The *TOMS* shall list NPA Contractor tasks and, where appropriate, chronological relation to Program and successor contractor milestones, to ensure all NPA Contractor deliverables are delivered in accordance (or prior to) agreed-upon deadlines necessary for the successor contractor's readiness for a successful SHCD. Throughout Transition-Out, the NPA Contractor shall track Transition-Out progress against the *TOMS* and maintain the *TOMS* to reflect any agreed-upon changes to the timeline.

Section 8.3.3.3 – Transition-Out Communications Plan

The *Transition-Out Communications Plan* shall describe the NPA Contractor's strategy for preparing NPA members for the upcoming transition to a successor contractor and the changes to the member experience with the NPA Contractor. This shall include communicating final dates of usage of current systems and processes and contingency plans for members who continue to reach out to the NPA Contractor after SHCD of the successor contractor (e.g., phone recording to redirect callers).

The *Transition-Out Communications Plan* shall include all elements described in TGD 020 (ATT007_TGD020_CommsPlan in Section D).

SECTION 9 – REFERENCE MATERIALS

Table 3. Materials Available Online

Name	PWS Reference
WTC Health Program Administrative Manual (https://www.cdc.gov/wtc/ppm.html)	Various
WTC Health Program Policies and Procedures (https://www.cdc.gov/wtc/policies.html)	Various
WTC Health Program Medical Coverage Determinations (https://www.cdc.gov/wtc/ppm.html#medical_guidelines)	Various
Title XXXIII of the Public Health Service Act (codifying Title I of the Zadroga Act, as amended)	Section 1
WTC Health Program Regulations (42 C.F.R. Part 88)	Section 1
Program At A Glance (https://www.cdc.gov/wtc/ataglace.html)	Section 1

Section 508 and Accessibility Requirements

Electronic, Information, and Communication Technology Accessibility.

(a) Pursuant to Section 508 of the Rehabilitation Act of 1973 (29 U.S.C. § 794d), as amended by the Workforce Investment Act of 1998, all information and communication technology (ICT) supplies, products, platforms, information, documentation, and services, or support developed, acquired, maintained or delivered under this contract or order must comply with the Revised 508 Standards, which are located at 36 C.F.R. pt. 1194.1 and Appendices A, B, and C, and are available at <https://www.access-board.gov/ict/>. Information about Section 508 is available at <https://www.hhs.gov/web/section-508/index.html>.

(b) Additional Section 508 accessibility standards applicable to this contract or order are identified as follows:

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205 WCAG 2.0 Level A & AA Success Criteria

302 Functional Performance Criteria

502 Inoperability with Assistive Technology

504 Authoring Tools

602 Support Documentation

603 Support Services

If it is determined by the Government that ICT supplies, products, platforms, information, documentation, and services support provided by the Contractor do not conform to the described accessibility standards in the contract, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(c) In the event of a modification(s) to this contract or order, which adds new ICT supplies or services or revises the type of, or specifications for, supplies, products, platforms, information, documentation, or services support, the Contracting Officer shall require that the Contractor submit a completed HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an Accessibility Conformance Report (ACR) (based on the Voluntary Product Accessibility Template (VPAT) see <https://www.itic.org/policy/accessibility/vpat>), and any other additional information necessary to assist the Government in determining that the ICT supplies or services conform to Section 508 accessibility standards. If it is determined by the Government that ICT supplies, products, platforms, information, documentation, and services support provided by the Contractor do not conform to the described accessibility standards in the contract, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(d) If this is an Indefinite-Delivery type contract, a Blanket Purchase Agreement or a Basic Ordering Agreement, the task/delivery order requests that include ICT supplies, products, platforms, information, documentation, or services support will define the specifications and accessibility standards for the order. In those cases, the Contractor shall be required to provide a completed HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an ACR (based on the VPAT see <https://www.itic.org/policy/accessibility/vpat>), and any other additional information necessary to assist the Government in determining that the ICT supplies, products, platforms, information, documentation, or services support conform to Section 508 accessibility standards. If it is determined by the Government that ICT supplies and services provided by the Contractor do not conform to the described accessibility standards in the provided documentation, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(e) The contractor shall identify to the Contracting Officer any perceived exception or exemption to Section 508 requirements for review by the agency Section 508 Program.

SECTION F - SOLICITATION PROVISIONS

FAR 52.252-1 Solicitation Provisions Incorporated by Reference (Feb 1998)

This solicitation incorporates one or more solicitation provisions by reference, with the same force and effect as if they were given in full text. Upon request, the Contracting Officer will make their full text available. The Offeror is cautioned that the listed provisions may include blocks that must be completed by the Offeror and submitted with its quotation or offer. In lieu of submitting the full text of those provisions, the Offeror may identify the provision by paragraph identifier and provide the appropriate information with its quotation or offer. Also, the full text of a solicitation provision may be accessed electronically at this/these address(es):

<https://www.acquisition.gov/browse/index/far>

(End of provision)

FAR Reference	Title and Date
52.215-1	Instructions to Offerors—Competitive Acquisition (DEVIATION) (RFO AUG 2025)
52.215-1 Alt 1	Instructions to Offerors- Competitive Acquisition Alternate I (DEVIATION) (RFO AUG 2025)
52.232-28 Alt 1	Invitation to Propose Performance-Based Payments Alt I (MAR 2000)

52.212-1 Instructions to Offerors—Commercial Products and Commercial Services.

(a) *Submission of offers.* Submit signed and dated offers to the office specified in this solicitation at or before the exact time specified in this solicitation. As a minimum, offers shall include—

- (1) The solicitation number;
- (2) The name, address, telephone number of the Offeror;
- (3) The Offeror's Unique Entity Identifier (UEI) and, if applicable, Electronic Funds Transfer (EFT) indicator;
- (4) Information necessary to evaluate the factors contained in the provision at 52.212-2 or as described in the solicitation;
- (5) Responses to provisions that require Offeror completion of information, representations, and certifications (other than those collected via the System for Award Management (SAM)); and
- (6) A statement specifying the extent of agreement with all terms, conditions, and provisions included in the solicitation and any solicitation amendments.

(b) *Period for acceptance of offers.* The Offeror agrees to hold the prices in its offer firm for 60 calendar days from the date specified for receipt of offers, unless another time period is specified in an addendum to the solicitation.

(c) *Late submissions, modifications, revisions, and withdrawals of offers.*

- (1) Offerors are responsible for submitting offers and any modifications or revisions to the Government office designated in the solicitation by the time specified in the solicitation.

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(2) Any offer, modification, or revision received after the time specified for receipt of offers is “late” and will not be considered unless it is received before award is made and the Contracting Officer determines that accepting the late offer would not unduly delay the acquisition. However, a late modification of an otherwise successful offer that makes its terms more favorable to the Government will be considered at any time it is received and may be accepted.

(3) If an emergency or unanticipated event interrupts normal Government processes so that offers cannot be received at the Government office designated for receipt of offers by the exact time specified in the solicitation, and urgent Government requirements preclude amendment of the solicitation or other notice of an extension of the closing date, the time specified for receipt of offers will be deemed to be extended to the same time of day specified in the solicitation on the first work day on which normal Government processes resume.

(4) Offerors may withdraw their offers by written notice to the Government received at any time before award.

(d) *Contract award (not applicable to Invitation for Bids)*. The Government intends to evaluate offers and award a contract without discussions with Offerors. Therefore, the Offeror’s initial offer should contain the Offeror’s best terms. However, the Government reserves the right to conduct discussions, if necessary. The Government may reject any or all offers if such action is in the public interest, accept other than the lowest offer, and waive informalities and minor irregularities in offers received.

(e) *Debriefings*. If a postaward debriefing is given to requesting Offerors, the Government will disclose the following information, if applicable:

(1) The agency’s evaluation of the significant weak or deficient factors in the debriefed Offeror’s offer.

(2) The overall evaluated cost or price and technical rating of the successful Offeror and the debriefed Offeror and past performance information on the debriefed Offeror.

(3) The overall ranking of all Offerors when any ranking was developed by the agency during source selection.

(4) A summary of the rationale for award.

(5) For acquisitions of commercial products, the make and model of the product to be delivered by the successful Offeror.

(6) Reasonable responses to relevant questions posed by the debriefed Offeror as to whether the agency followed source-selection procedures set forth in the solicitation, applicable regulations, and other applicable authorities.

(End of provision)

52.212-2 Evaluation—Commercial Products and Commercial Services (Nov 2021)

(a) The Government will award a contract resulting from this solicitation to the responsible offeror whose offer conforming to the solicitation will be most advantageous to the Government, price and other factors considered. The following factors shall be used to evaluate offers:

Technical
Past Performance
Cost

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The evaluation factors in descending order of importance are: Technical, Past Performance, and Cost. Technical and past performance are more important than cost. Award will be made to the responsible Offeror determined to be the most advantageous and of the best value to the Government as evaluated under the evaluation criteria. The Government may select for award the Offeror whose price is not the lowest, but whose Technical and Past Performance ratings are sufficiently more advantageous to justify the payment of a higher price. Conversely, the Government may select for award the Offeror whose price is lower, when the other Offerors' Technical and Past Performance ratings are not sufficiently more advantageous to justify the payment of a higher price.

The Government reserves the right and intends to award without negotiations.

(b) *Options*. The Government will evaluate offers for award purposes by adding the total price for all options to the total price for the basic requirement. The Government may determine that an offer is unacceptable if the option prices are significantly unbalanced. Evaluation of options shall not obligate the Government to exercise the option(s).

(c) A written notice of award or acceptance of an offer, mailed or otherwise furnished to the successful offeror within the time for acceptance specified in the offer, shall result in a binding contract without further action by either party. Before the offer's specified expiration time, the Government may accept an offer (or part of an offer), whether or not there are negotiations after its receipt, unless a written notice of withdrawal is received before award.

HHSAR 352.239-73 Electronic and Information Technology Accessibility Notice (December 18, 2015)

[(Deviation)]

(a) Any offeror responding to this solicitation must comply with established HHS Information and Communication Technology (ICT) accessibility standards. Information about Section 508 is available at <https://www.hhs.gov/web/section-508/index.html>.

(b) The Section 508 accessibility standards applicable to this solicitation are stated in the clause at 352.239-79 Information and Communication Technology Accessibility. In order to facilitate the Government's determination whether proposed ICT supplies, products, platforms, information, and documentation meet applicable Section 508 accessibility standards, offerors must submit an appropriate HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an Accessibility Conformance Report (ACR) (based on the Voluntary Product Accessibility Template (VPAT) see <https://www.itic.org/policy/accessibility/vpat>), in accordance with the completion instructions. The purpose of the checklists and conformance reports are to assist HHS acquisition and program officials in determining whether proposed ICT supplies, products, platforms, information, and documentation conform to applicable Section 508 accessibility standards. Checklists and ACRs evaluate—in detail—whether the ICT conforms to specific Section 508 accessibility standards and identifies remediation efforts needed to address conformance issues.

(c) If an offeror claims its supplies or services meet applicable Section 508 accessibility standards, and it is later determined by the Government, *i.e.*, after award of a contract or order, that supplies, products, platforms, information, documentation, or services support delivered do not conform to the described accessibility standards, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its expense.

(d) In order to facilitate the Government's determination whether proposed ICT supplies meet applicable Section 508 accessibility standards, offerors must submit an Accessibility Conformance Report, in accordance with its completion instructions and tailored to the requirements in the solicitation. The purpose of the Report is to assist HHS acquisition and program officials in determining whether proposed ICT supplies conform to applicable Section 508 accessibility standards. The template allows offerors or developers to self-evaluate their supplies and document, in detail, whether they conform to a specific Section 508 accessibility standard, and any underway remediation efforts addressing conformance issues. Instructions for preparing the Template are available at <https://www.section508.gov/>.

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(e) Additionally, offerors must provide enough information to assist the Government in determining that the ICT services conform to Section 508 accessibility standards, including any underway remediation efforts addressing conformance issues.

(f) Respondents to this solicitation must identify any inability to conform to Section 508 requirements. If an offeror claims its supplies or services meet applicable Section 508 accessibility standards, and it is later determined by the Government, i.e., after award of a contract or order, that supplies or services delivered do not conform to the described accessibility standards, remediation of the supplies or services to the level of conformance specified in the contract will be the responsibility of the Contractor at its expense.

(g) Items delivered as electronic content must be accessible to HHS acceptance criteria. Checklists for various formats are available at <http://508.hhs.gov/>. Materials, other than items incidental to contract management, that are final items for delivery should be accompanied by the appropriate checklist, except upon approval of the Contracting Officer or Contracting Officer's Representative.

SECTION G – REPRESENTATIONS AND CERTIFICATIONS

52.203-11 Certification and Disclosure Regarding Payments to Influence Certain Federal Transactions (Sep 2024)

- (a) *Definitions.* As used in this provision—"Lobbying contact" has the meaning provided at [2 U.S.C. 1602\(8\)](#). The terms "agency," "influencing or attempting to influence," "officer or employee of an agency," "person," "reasonable compensation," and "regularly employed" are defined in the FAR clause of this solicitation entitled "Limitation on Payments to Influence Certain Federal Transactions" (52.203-12).
- (b) *Prohibition.* The prohibition and exceptions contained in the FAR clause of this solicitation entitled "Limitation on Payments to Influence Certain Federal Transactions" (52.203-12) are hereby incorporated by reference in this provision.
- (c) *Certification.* The offeror, by signing its offer, hereby certifies to the best of its knowledge and belief that no Federal appropriated funds have been paid or will be paid to any person for influencing or attempting to influence an officer or employee of any agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress on its behalf in connection with the awarding of this contract.
- (d) *Disclosure.* If any registrants under the Lobbying Disclosure Act of 1995 have made a lobbying contact on behalf of the offeror with respect to this contract, the offeror shall complete and submit, with its offer, OMB Standard Form LLL, Disclosure of Lobbying Activities, to provide the name of the registrants. The offeror need not report regularly employed officers or employees of the offeror to whom payments of reasonable compensation were made.
- (e) *Penalties.* Submission of this certification and disclosure is a prerequisite for making or entering into this contract imposed by [31 U.S.C. 1352](#). Any person who makes an expenditure prohibited under this provision or who fails to file or amend the disclosure required to be filed or amended by this provision, shall be subject to civil penalties as provided in [31 U.S.C. 1352](#). An imposition of a civil penalty does not prevent the Government from seeking any other remedy that may be applicable.

(End of provision)

52.204-7 System for Award Management—Registration. (DEVIATION) (RFO AUG 2025)

The Offeror shall have an active Federal Government contracts registration in the System for Award Management (SAM) when submitting an offer or quotation in response to this solicitation and at the time of award. As part of the SAM registration process, the Government collects information, as described in paragraphs (b) through (d) of this provision, that is necessary to identify the Offeror and for the Offeror to be awarded Federal Government contracts. To register in SAM, go to <https://www.sam.gov>. Allow for processing time when registering in SAM. If the Offeror is not registered in SAM, it should register immediately after receiving this solicitation.

- (a) *Definitions.* As used in this provision—
Commercial and Government Entity (CAGE) code has the meaning provided in the clause at the Federal Acquisition Regulation (FAR) 52.204-13, System for Award Management—Maintenance, of this solicitation.

Electronic Funds Transfer (EFT) indicator means a bank account identifier to establish additional System for Award Management records for identifying alternative EFT accounts (see part 32) for the same entity.

Highest-level owner means the entity that owns or controls an immediate owner of the offeror, or that owns or controls one or more entities that control an immediate owner of the offeror. No entity owns or exercises control of the highest-level owner.

Immediate owner means an entity, other than the offeror, that has direct control of the offeror. Indicators of control include, but are not limited to, one or more of the following: ownership or interlocking management, identity of interests among family members, shared facilities and equipment, and the common use of employees. There may be more than one immediate owner (e.g., joint ventures).

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Predecessor means an entity whose assets were acquired by the offeror or another entity (most often through merger or acquisition) and whose affairs are now carried out by the offeror or the other entity under a new name.

Taxpayer identification number means the number required by the Internal Revenue Service (IRS) to be used by the offeror to report income tax and other returns. It may be either a Social Security Number or an Employer Identification Number.

Unique entity identifier (UEI) has the meaning provided in the clause at FAR 52.204-13, System for Award Management—Maintenance, of this solicitation.

(b) *Identifiers*. The Offeror shall obtain and provide the following identifying information:

(1) *Unique entity identifier (UEI)*.

- (i) The Offeror shall obtain a UEI to register in SAM. The Government will independently validate the existence and uniqueness of the Offeror before assigning a UEI to the Offeror. Go to <https://www.sam.gov> for instructions on obtaining a UEI.
- (ii) The Offeror shall enter, in the block with its name and address on the cover page of its offer, the annotation “Unique Entity Identifier” followed by the UEI that identifies the Offeror’s name and address exactly as stated in the offer. The Offeror shall also enter its EFT indicator, if applicable.
- (iii) The Contracting Officer will use the UEI to verify that the Offeror has an active Federal Government contracts registration in SAM.

(2) *Taxpayer identification number (TIN)*.

- (i) The Offeror shall provide its TIN or related information to comply with debt collection requirements of 31 U.S.C. 7701(c) and 3325(d); reporting requirements of 26 U.S.C. 6041, 6041A, and 6050M; and implementing regulations issued by the IRS. The Offeror shall consent for TIN validation; and

(3) *Commercial and Government Entity (CAGE) code*.

- (i) The Offeror shall provide a CAGE code and legal business name (Do not use a “doing business as” name) for—

- (A) Itself;
- (B) Its immediate owner(s), if any;
- (C) Its highest-level owner, if any; and
- (D) Any predecessor(s), or predecessor of an Offeror’s predecessor, that held a Federal contract or grant within the last three years.

(ii) If the Offeror is in the United States or its outlying areas and does not already have a CAGE code assigned, the DLA CAGE Branch will assign a CAGE code to the Offeror as a part of the SAM registration process. For information on obtaining a CAGE code go to <https://cage.dla.mil/>.

(iii) The Offeror shall get from any immediate and/or highest-level owner(s) their respective CAGE code(s) to provide the code(s) as part of the registration (FAR 52.204-7(b)(3)(i)).

(iv) If the Offeror is located outside of the United States or its outlying areas, and does not already have a CAGE code assigned, the Offeror may obtain a CAGE code as indicated in the following table.

If the Offeror is...

Then...

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Located in a country that is a member of the North Atlantic Treaty Organization (NATO) or a sponsored nation

Contact the appropriate National Codification Bureau (<https://www.nato.int/structur/ac/135/about/contacts>)

Located in a country that is not a member of NATO or a sponsored nation

Contact the NATO Support and Procurement Agency (NSPA) (<https://eportal.nspa.nato.int/AC135Public/scage/CageList.aspx>)

(c) Representations and certifications.

(1) The following FAR solicitation provisions contain entity-level representations and certifications that the Offeror shall submit as part of their Federal Government contracts registration in SAM:

Provision	Title	Date
52.204-5	Women-Owned Business (Other Than Small Business)	Oct 2014
52.209-2	Prohibition on Contracting with Inverted Domestic Corporations—Representation	Nov 2015
52.209-5	Certification Regarding Responsibility Matters	Aug 2020
52.209-11	Representation by Corporations Regarding Delinquent Tax Liability or a Felony Conviction under any Federal Law	Feb 2016
52.219-1	Small Business Program Representations	Feb 2024
52.219-1 Alt I	Small Business Program Representations, with its Alternate I	Feb 2024

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52.219-1 Alt II	Small Business Program Representations, with its Alternate II	Mar 2023
52.226-2	Historically Black College or University and Minority Institution Representation	Oct 2014

(2) By submitting its offer, the Offeror verifies that, as of the date of its offer, its representations and certifications posted electronically in SAM for the provisions listed in paragraph (c)(1) of this provision are current, accurate, and complete. The Offeror's representations and certifications in SAM are hereby incorporated by reference into its offer.

- (d) *Other information.* The Offeror shall provide more information on its business operations and type that is necessary to be considered for award of certain contracts and financial information necessary to receive payment under contracts.

(End of provision)

FAR 52.219-28 Postaward Small Business Program Rerepresentation. (DEVIATION) (RFO AUG 2025)

Postaward Small Business Program Rerepresentation (Deviation Date)

(a) *Definitions.* As used in this clause—

Long-term contract means a contract of more than five years in duration, including options. However, the term does not include contracts that exceed five years in duration because the period of performance has been extended for a cumulative period not to exceed six months under the clause at 52.217-8, Option to Extend Services, or other appropriate authority.

Small business concern—

(1) Means a concern, including its affiliates, that is independently owned and operated, not dominant in its field of operation, and qualified as a small business under the criteria in 13 CFR part 121 and the size standard in paragraph (c) of this clause.

(2) *Affiliates*, as used in this definition, means business concerns, one of whom directly or indirectly controls or has the power to control the others, or a third party or parties control or have the power to control the others. In determining whether affiliation exists, consideration is given to all appropriate factors including common ownership, common management, and contractual relationships. SBA determines affiliation based on the factors set forth at 13 CFR 121.103.

(b) If the Contractor represented that it was a small business concern, a small disadvantaged business concern, or a joint venture that was any of the small business concerns identified in 19.000(a)(3) prior to award of this contract, the Contractor shall rerepresent its size and socioeconomic status according to paragraph (e) of this clause or, if applicable, paragraph (g) of this clause, upon occurrence of any of the following:

(1) Within 30 days after execution of a novation agreement or within 30 days after modification of the contract to include this clause, if the novation agreement was executed prior to inclusion of this clause in the contract.

(2) Within 30 days after a merger or acquisition that does not require a novation or within 30 days after modification of the contract to include this clause, if the merger or acquisition occurred prior to inclusion of this clause in the contract.

(3) For long-term contracts-

(i) Within 60 to 120 days prior to the end of the fifth year of the contract; and

(ii) Within 60 to 120 days prior to the date specified in the contract for exercising any option thereafter.

(c) The Contractor shall rerepresent its size status in accordance with the size standard in effect at the time of this rerepresentation that corresponds to the North American Industry Classification System (NAICS) code(s) assigned to this contract. The small business size standard corresponding to this NAICS code(s) can be found at <https://www.sba.gov/document/support--table-size-standards>.

(d) The small business size standard for a Contractor providing an end item that it does not manufacture, process, or produce itself, for a contract other than a construction or service contract, is 500 employees, or 150 employees for information technology value-added resellers under NAICS code 541519, if the acquisition—

(1) Was set aside for small business and has a value above the simplified acquisition threshold;

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(2) Used the HUBZone price evaluation preference regardless of dollar value, unless the Contractor waived the price evaluation preference; or

(3) Was an 8(a), HUBZone, service-disabled veteran-owned, economically disadvantaged women-owned, or women-owned small business set-aside or sole-source award regardless of dollar value.

(e) Except as provided in paragraph (g) of this clause, the Contractor shall make the representation(s) required by paragraph (b) of this clause by validating or updating all its representations in the Representations and Certifications section of the System for Award Management (SAM) and its other data in SAM, as necessary, to ensure that they reflect the Contractor's current status. The Contractor shall notify the contracting officer in writing within the timeframes specified in paragraph (b) of this clause, that the data have been validated or updated, and provide the date of the validation or update.

(f) If the Contractor represented that it was other than a small business concern prior to award of this contract, the Contractor may, but is not required to, take the actions required by paragraphs (e) or (g) of this clause.

(g) If the Contractor does not have representations and certifications in SAM, or does not have a representation in SAM for the NAICS code applicable to this contract, the Contractor is required to complete the following rerepresentation and submit it to the contracting office, along with the contract number and the date on which the rerepresentation was completed:

(1) The Contractor represents that it ☐ is, ☐ is not a small business concern under NAICS Code _____ assigned to contract number _____.

(2) [Complete only if the Contractor represented itself as a small business concern in paragraph (g)(1) of this clause.] The Contractor represents that it ☐ is, ☐ is not, a small disadvantaged business concern as defined in 13 CFR 124.1001.

(3) *Women-owned small business (WOSB) joint venture eligible under the WOSB Program.* The Contractor represents that it ☐ is, ☐ is not a joint venture that complies with the requirements of 13 CFR 127.506(a) through (c). [*The Contractor shall enter the name and unique entity identifier of each party to the joint venture: ____.*]

(4) *Economically disadvantaged women-owned small business (EDWOSB) joint venture.* The Contractor represents that it ☐ is, ☐ is not a joint venture that complies with the requirements of 13 CFR 127.506(a) through (c). [*The Contractor shall enter the name and unique entity identifier of each party to the joint venture: ____.*]

(5) *Service-disabled veteran-owned small business (SDVOSB) joint venture eligible under the SDVOSB Program.* The Contractor represents that it ☐ is, ☐ is not an SDVOSB joint venture eligible under the SDVOSB Program that complies with the requirements of 13 CFR 128.402. [*The Contractor shall enter the name and unique entity identifier of each party to the joint venture: ____.*]

(6) *HUBZone joint venture eligible under the HUBZone Program.* [*Complete only if the offeror is a HUBZone small business concern.*] The offeror represents, as part of its offer, that It ☐ is, ☐ is not a HUBZone joint venture that complies with the requirements of 13 CFR 126.616(a) through (c). [*The Contractor shall enter the name and unique entity identifier of each party to the joint venture: ____.*] Each HUBZone small business concern participating in the HUBZone joint venture must be certified as a HUBZone concern. [*Contractor to sign and date and insert authorized signer's name and title. _____*]

(End of clause)

52.229-11 Tax on Certain Foreign Procurements—Notice and Representation.

Tax on Certain Foreign Procurements—Notice and Representation (Date)

(a) *Definitions.* As used in this provision—

Foreign person means any person other than a United States person.

Specified Federal procurement payment means any payment made pursuant to a contract with a foreign contracting party that is for goods, manufactured or produced, or services provided in a foreign country that is not a party to an international procurement agreement with the United States. For purposes of the prior sentence, a foreign country does not include an outlying area of the United States.

United States person as defined in [26 U.S.C. 7701](#)(a)(30) means

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- (1) A citizen or resident of the United States;
- (2) A domestic partnership;
- (3) A domestic corporation;
- (4) Any estate (other than a foreign estate, within the meaning of [26 U.S.C. 701\(a\)\(31\)](#)); and
- (5) Any trust if—
 - (i) A court within the United States is able to exercise primary supervision over the administration of the trust; and
 - (ii) One or more United States persons have the authority to control all substantial decisions of the trust.
- (b) Unless exempted, there is a 2 percent tax of the amount of a specified Federal procurement payment on any foreign person receiving such payment. See [26 U.S.C. 5000C](#) and its implementing regulations at 26 CFR 1.5000C-1 through 1.5000C-7.
- (c) Exemptions from withholding under this provision are described at 26 CFR 1.5000C-1(d)(5) through (7). The Offeror may claim an exemption from the withholding by using the Department of the Treasury Internal Revenue Service (IRS) Form W-14, Certificate of Foreign Contracting Party Receiving Federal Procurement Payments, available at www.irs.gov/w14. Any exemption claimed and self-certified on the IRS Form W-14 is subject to audit by the IRS. Any disputes regarding the imposition and collection of the [26 U.S.C. 5000C](#) tax are adjudicated by the IRS as the [26 U.S.C. 5000C](#) tax is a tax matter, not a contract issue. The IRS Form W-14 is provided to the acquiring agency rather than to the IRS.
- (d) For purposes of withholding under [26 U.S.C. 5000C](#), the Offeror represents that
 - (1) It ☐ is ☐ is not a foreign person; and
 - (2) If the Offeror indicates "is" in paragraph (d)(1) of this provision, then the Offeror represents that—I am claiming on the IRS Form W-14 ☐ a full exemption, or ☐ partial or no exemption [*Offeror must select one*] from the excise tax.
- (e) If the Offeror represents it is a foreign person in paragraph (d)(1) of this provision, then—
 - (1) The clause at FAR 52.229-12, Tax on Certain Foreign Procurements, will be included in any resulting contract; and
 - (2) The Offeror shall submit with its offer the IRS Form W-14. If the IRS Form W-14 is not submitted with the offer, exemptions will not be applied to any resulting contract and the Government will withhold a full 2 percent of each payment.
- (f) If the Offeror selects "is" in paragraph (d)(1) and "partial or no exemption" in paragraph (d)(2) of this provision, the Offeror will be subject to withholding in accordance with the clause at FAR 52.229-12, Tax on Certain Foreign Procurements, in any resulting contract.
- (g) A taxpayer may, for a fee, seek advice from the IRS as to the proper tax treatment of a transaction. This is called a private letter ruling. Also, the IRS may publish a revenue ruling, which is an official interpretation by the IRS of the Internal Revenue Code, related statutes, tax treaties, and regulations. A revenue ruling is the conclusion of the IRS on how the law is applied to a specific set of facts. For questions relating to the interpretation of the IRS regulations go to <https://www.irs.gov/help/tax-law-questions>.

(End of provision)

CDCH.04 Key Personnel (JAN 2021)

The key personnel cited below are considered essential to the work performed under the contract. Pursuant to the terms of HHSAR Clause 352.237-75, Key Personnel, the Contractor shall not replace or divert any of these individuals without the written consent of the Contracting Officer.

Personnel	Title
[To be proposed]	Contract Administrator
[To be proposed]	Project Manager
[To be proposed]	Clinical Director

CDCL.09 Contractor Performance Assessment Reporting System (CPARS) Requirements (April 2013)

In accordance with FAR 42.15, the Centers for Disease Control and Prevention (CDC) will review and evaluate contract performance. FAR 42.1502 and 42.1503 requires agencies to prepare evaluations of contractor performance and submit them to the Past Performance Information Retrieval System (PPIRS). The CDC utilizes the Department of Defense (DOD) web-based Contractor Performance Assessment Reporting System (CPARS) to prepare and report these contractor performance evaluations. All information contained in these assessments may be used by the Government, within the limitations of FAR 42.15, for future source selections in accordance with FAR 15.304 where past performance is an evaluation factor.

The CPARS system requires a contractor representative to be assigned so that the contractor has appropriate input into the performance evaluation process. The CPARS contractor representative will be given access to CPARS and will be given the opportunity to concur or not-concur with performance evaluations before the evaluations are complete. The CPARS contractor representative will also have the opportunity to add comments to performance evaluations.

The assessment is not subject to the Disputes clause of the contract, nor is it subject to appeal beyond the review and comment procedures described in the guides on the CPARS website. Refer to: www.cpars.gov for details and additional information related to CPARS, CPARS user access, how contract performance assessments are conducted, and how Contractors participate. Access and training for all persons responsible for the preparation and review of performance assessments is also available at the CPARS website.

The contractor must provide the CDC contracting office with the name, e-mail address, and phone number of their designated CPARS representative who will be responsible for logging into CPARS and reviewing and commenting on performance evaluations. The contractor must maintain a current representative to serve as the contractor representative in CPARS. It is the contractor's responsibility to notify the CDC contracting office, in writing (letter or email), when their CPARS representative information needs to be changed or updated. Failure to maintain current CPARS contractor representative information will result in the loss of an opportunity to review and comment on performance evaluations.

Provide the current CPARS representative information below.

PRINT OR TYPE NAME

EMAIL ADDRESS AND PHONE NUMBER

SECTION H – INSTRUCTIONS TO OFFERORS

H.1 FAR 52.216-1 TYPE OF CONTRACT (APR 1984)

The Government contemplates awarding a **time-and-materials** contract with **firm-fixed-price** CLINs. The expected Line Item structure can be found in Section B. However, the Contracting Officer reserves the right to award any contract type/pricing arrangement.

(End of Provision)

H.2 FAR 52.233-2 SERVICE OF PROTEST (SEP 2006)

(a) Protests, as defined in section 33.101 of the Federal Acquisition Regulation, that are filed directly with an agency, and copies of any protests that are filed with the Government Accountability Office (GAO), shall be served on the Contracting Officer (addressed as follows) by obtaining written and dated acknowledgment of receipt from:

Centers for Disease Control & Prevention
Office of Acquisition Services (OAS)
ATTN: Megan Kelly, Contracting Officer
Solicitation # 75D301-26-R-73374

Email: xvp9@cdc.gov

(b) The copy of any protest shall be received in the office designated above within one day of filing a protest with the GAO.

(End of provision)

H.3 CDCL.02 Alternate Protest Procedure (Sep 2024)

- (a) In accordance with FAR 33.103(d)(4), interested parties may request an independent review of their agency protest at a level above the contracting officer. This independent review is available as an alternative to consideration by the contracting officer of a protest or is available as an appeal of a contracting officer decision on a protest. A protest must clearly identify any request regarding the above.
- (b) Protests or appeals described in (a) shall be submitted to the Associate Director for Acquisition Services, Office of Acquisition Services, Office of Financial Resources, Centers for Disease Control and Prevention by email at ftg4@cdc.gov.

Protests will not be considered if the interested party has a protest on the same or similar issue(s) pending with the Contracting Officer.

H.4 GENERAL INSTRUCTIONS

Prior to submission of proposals, Offerors are expected to reach an understanding of the requirements as stated herein. If such a review establishes the need for correction or clarification, such information should immediately be brought to the attention of the Contracting Officer so that the matter can be resolved and, if necessary, official dissemination of such information can be made to all Offerors.

The submission must be signed by **an official, authorized to bind the Offeror** and include a statement that the offer is firm for a period of at least **60 calendar** days from the due date of submission. Submissions shall set forth full, accurate, and complete information as required by this RFP. The penalty for making false statements is prescribed in 18 U.S. Code § 1001. Failure to furnish full and complete information requested may cause an offer to be determined unacceptable and removed from consideration.

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Offerors shall submit, under a separate tab, within each of the respective volumes, all (if any) assumptions, conditions, or exceptions with any of the terms and conditions of this RFP. If not noted within the respective volume, the Government will assume that the Offeror proposes no assumptions, conditions, or exceptions for award and agrees to comply with all the terms and conditions as set forth in the solicitation. It is not the responsibility of the Government to seek out and identify any assumptions, conditions, or exceptions buried within the Offeror's submissions.

Any additional clauses required by public law, executive order, or acquisition regulation, in effect at the time of execution of this award, will be included. The Offeror's submission shall become the property of the Government and will not be returned. It is understood that the Offeror's submission will become part of the official contract file.

The Government reserves the right to request additional information as may be necessary to determine the Offeror's qualifications for award or to clarify any aspects of the technical and/or price submissions. Such information shall be furnished promptly upon the Contracting Officer's request, unless otherwise stated.

This RFP does not commit the Government to pay any cost for the preparation and submission of quotes. In addition, the Contracting Officer is the only individual who can legally commit the Government to the obligation of public funds in connection with this proposed acquisition.

The Government will evaluate submissions in accordance with the evaluation criteria as set forth in Sections H and I of this RFP.

H.5 SUBMISSION DUE DATE AND TIME

Proposal submission is due no later than **10:00AM ET on January 30, 2026**

Proposals must be submitted via email on or before the due date and time specified herein, in Word or PDF compatible read-only format, unless otherwise noted.

Please note it is the Offeror's responsibility to ensure/verify that the Government receives its submission on or before the date and time specified. If the submission is not received by the Government on or before the date and time specified, the Offeror's submission may be considered late.

The Contracting Officer for this effort is Megan Kelly and the email address designated for receipt of submissions is xvp9@cdc.gov.

Late submissions, modifications, revisions, and withdrawals of offers will be handled in accordance with 52.212-1(c).

H.6 INQUIRIES

Offerors are instructed to contact only the Contracting Officer regarding any aspect of this RFP. Prospective Offerors are cautioned against contacting other Government personnel regarding this RFP prior to the award of this procurement. If such contact occurs and is found to be prejudicial to competing Offerors, the Offeror making such contact may be excluded from further consideration for award.

Where possible, inquiries shall be submitted in writing, email, or as otherwise instructed herein.

Questions should be worded to avoid disclosing any potential proposed strategies or proprietary solutions.

All questions for this RFP must be submitted in writing to the issuing office no later than January 5, 2026 at 10AM ET and shall be submitted via electronic mail (email) to Megan Kelly at xvp9@cdc.gov AND Liubov Kriel at vyh1@cdc.gov

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Inquiries shall identify the pertinent RFP Section (e.g., PWS, Evaluation/Quote Preparation Instructions etc.), page number, and paragraph number. To ensure a prompt response, identify: RFP 75D301-26-R-73374 in the email subject line. Please utilize the ATT025_NPA_QuestionsandAnswersTemplate Attachment found in Section D.

Should any request for clarification be received after the date and time stated above, the Government reserves the right not to provide an answer. If, however, the Government determines the request for clarification addresses an issue of significant importance, the Government may provide a written response to all Offerors. Please note, questions and/or comments will not be protected by the Government as proprietary.

Additionally, if Offerors determine or believe that the RFP package contains errors or omissions or is otherwise unsound, the Offeror shall immediately notify the Contracting Officer in writing of such errors, omissions, or other issues in accordance with the instructions contained herein. The Offeror shall provide details and supporting rationale.

H.7 PROPOSAL PREPARATION INSTRUCTIONS

Proposal Formatting:

1. Proposal shall be formatted on 8.5 inch x 11 inch pages and submitted as a standalone file in both Adobe “.pdf” and Microsoft Word editable “.docx” formats.
2. The body of the document shall be set with a minimum of 10-point font size, including any charts, tables, or figures.
3. Normal character spacing and a minimum of single line spacing shall be used.
4. Title and heading document elements may use a different font size.
5. Each page shall have a minimum of one-inch margins on all sides of the page.
6. Header and footer information, which does not include any information to be evaluated, may be included within the one-inch margin space and may use a font size other than 10 points.
7. Cover pages, table of contents, and acronym lists are excluded from page count once per Volume; subsection dividers or covers are included in the page count unless otherwise specified.
8. Page limits for each sub-section are independent. Unused pages in one sub-section may not be reallocated to another.
9. Each Volume and Section of the proposal shall not exceed applicable page limits (see applicable page count for each Volume and Section below).
10. Page formatting can alter for illustrations and tables but must be easily readable. Text included in any graphics, illustrations or diagrams may be no less than 10 pt . Fold-outs will be counted as the appropriate number of pages based upon an 8 ½ inch by 11 inch sheet of paper.
11. All electronic files in Excel, i.e. “.xls”, format as applicable shall be unprotected. Any formulas used in deriving cost/price proposal shall be visible when cursor is positioned in cell (i.e., do not convert formulas to values).

The Government will count pages of the document in order from the first page (omitting from the count any pages specifically exempted by these instructions). Once the maximum page limit is reached, the Government will **not** read or evaluate any additional pages in the document. In the event of a discrepancy in the page count or formatting between the Adobe “.pdf” and Microsoft Word editable “.docx” versions of the file, the Adobe “.pdf” version will control.

In order to ensure all pages of your proposal are included in the evaluation we request that you put a footer in your proposal that includes your company name; RFP number, and Page # of # Pages. In the event that the offeror creates an ambiguity in the numbering of pages, the Government will exercise its own discretion in the counting of pages. Pages in excess of the below limitation will not be evaluated.

Offerors submitting any restricted data shall mark it as such. The Government assumes no liability for disclosure of unmarked data and may use/disclose the unmarked data for any purpose. *Unless restricted, information submitted in response to this request may become subject to disclosure to the public pursuant to the provisions of the Freedom of Information Act (5 U.S.C 552).*

H.8 PROPOSAL CONTENT:

Your proposal must be prepared in two separate parts as instructed herein. (Part I, Technical Proposal, and Part II, Business Proposal (Cost).) The technical proposal must not contain reference to cost; however, resource information, such as data concerning labor hours and categories, material, subcontracts, etc., must be contained in the technical proposal so that the Offeror's understanding of the Performance Work Statement can be evaluated. The technical proposal must disclose your approach in sufficient detail to provide a clear and concise presentation that addresses, but is not limited to, the requirements of the technical proposal instructions.

1. **PART I – Technical Proposal** – Page limit: **135 pages**
2. **PART II – Business Proposal** – Page limit: There will be no page limit.

Each part shall be separate and complete in itself so evaluation of one may be accomplished independently of the other.

PART I – Technical Proposal:**1. Technical Proposal Instructions**

The Technical Proposal includes five Volumes. Volumes 1 and 4 are divided into Sub-Volume Sections. The technical proposal should address the following (See Section I for further details):

Volume 1: Contract Management Approach	60 pages, divided:
Volume 1, Section A: Transition-in	(15 pages)
Volume 1, Section B: Contract Management During Sustainment	(15 pages)
Volume 1, Section C: Transition-out	(15 pages)
Volume 1, Section D: Regulatory Compliance and Security Requirements	(15 pages)
Volume 2: Member Service Center, Infrastructure and Operations	30 pages
Volume 3: Staffing Approach	30 pages
Volume 4: Similar Experience	15 pages, divided:
Volume 4, Reference 1	(5 pages)
Volume 4, Reference 2	(5 pages)
Volume 4, Reference 3	(5 pages)
Volume 5: Data Management Plan	No Page Limit

Volume 1: Contract Management Approach

This Volume 1 shall address the Offeror's technical approach to accomplish tasks under Transition-in (Section 4.1), Contract Management During Sustainment (Section 4.2), Transition-Out (Section 4.5), and Regulatory Compliance and Security Requirements (Section 4.6). Volume 1 is limited to a maximum of 60 pages total and shall be divided into the four distinct Sub-Sections listed in this paragraph, with each Sub-Section limited to a maximum of 15 pages.

Volume 1, Section A, Transition-in: Offerors shall address their technical approach to ensure successful completion of all Transition-in tasks under Section 4.1, including preparation for and demonstration of readiness for Start of Healthcare Delivery (SHCD) and execution of successful Handover. Offerors shall address how systems and processes will be developed and tested, with particular consideration given to connectivity and information exchanges with other Program contractors, as well as timely completion of security authorization processes to ensure systems are operable and an Authorization to Operate (ATO) is received by SHCD. Offerors shall address their Transition-in communications approach, with particular

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attention toward preparation and delivery of high-quality and timely messaging to NPA members to ensure a seamless member transition to the NPA. Offerors shall address their management approach to transition-in, including quality assurance and performance tracking, project management tools and techniques, and the resources proposed to accomplish the work. Offerors shall include a robust discussion of transition-in risks and how these risks will be avoided, managed, mitigated, or resolved.

Volume 1, Section B, Contract Management During Sustainment: Offerors shall address their technical approach to accomplish all tasks under Section 4.2 excluding tasks specific to Section 4.2.3 (Staffing Requirements), which shall be detailed in Volume 3. Offeror shall describe its approach to management of the overall contract and the integration of PWS areas of responsibility, coordinating and collaborating with the Program, contract-wide risk and issue management, member and provider engagement and communications, overseeing subcontractor performance, and ensuring quality assurance. Offerors shall identify methods for quality assurance and performance tracking, project management tools and techniques, and other resources proposed to accomplish management of the contract.

Volume 1, Section C, Transition-out: Offerors shall address their technical approach to accomplish all tasks under Section 4.5, including the phasing of tasks and sequencing of activities to prepare a successor contractor, or other Program-appointed designee, to be ready for SHCD, and transition-out the Offeror from the requirement. Offerors shall address how Program records and data will be transferred to an incoming contractor; how the Offeror will plan for and conduct ramp-down operations, to include the decommissioning of systems and continuity of operations during the transition-out. Offerors shall address their management approach to transition-out, including how performance during transition-out will be tracked and managed. Offerors shall include a discussion of transition-out risks and how these risks will be avoided, managed, mitigated, or resolved.

Volume 1, Section D, Regulatory Compliance and Security Requirements: Offerors shall address their technical approach to accomplish all tasks under Section 4.6, including offeror's approach to HIPAA compliance, fraud, waste, and abuse (FWA) prevention and detection, records management, and continuous compliance with all applicable security requirements. Offerors shall address their management approach to regulatory compliance, including risk management, project management tools and techniques, and the resources proposed to accomplish the work.

Volume 2: Member Service Center, Infrastructure and Operations

This volume shall address the Offeror's technical approach to accomplish tasks under Infrastructure, Resources, and Functionalities (Section 4.3). Volume 2 is limited to a maximum of 30 pages total.

Offerors shall detail their proposed systems, tools, techniques, and approaches to establish and maintain all public-facing infrastructure and functions of the Member Service Center as identified in Section 4.3.1. Offerors shall identify readily available systems for deployment to ensure full operation of the Member Service Center by SHCD, compliant with performance standards as set forth in the QASP. Offerors shall describe their strategy and end-to-end process workflows to implement a tiered inquiry response structure to ensure delivery of effective and meaningful member assistance as required under Section 4.3.1.5. Offerors shall address their approach to establishing and maintaining required systems and performing all data collection and management requirements under Section 4.3.2, with particular consideration given to establishment, management, and accessibility of the NPA Member Profile, as well as establishment of all required system components as described in 4.3.2.1. Offeror shall detail its strategy to maintain bidirectional exchange of information with the Program and other Program contractors as detailed in Section 4.3.3 to support Program-wide member benefit administration and inquiry resolution. Offeror shall describe its plan for continuity of operations under Section 4.3.4, including during periods of planned and unplanned outages, to minimize disruptions to member care throughout contract performance.

Volume 3: Staffing Approach

This Volume shall address the Offeror's contract-wide staffing approach to accomplish all PWS task areas through the proposed technical approach. Volume 3 is limited to a maximum of 30 pages.

Offerors shall describe their contract-wide staffing approach. Offeror shall describe its overall contract management staffing architecture with clearly defined roles, responsibilities, and lines of reporting. Offeror shall clearly define the roles of Key Personnel and non-key staff, and shall identify the labor categories that are to be used, with clear delineation of staffing resources allocated to each task area under the PWS. Offerors shall detail their approach for NPA Contractor Staff Training (Section 4.2.3.3) to ensure contractor staff have sufficient knowledge and resources to complete assigned tasks. Offerors shall describe their approach to maintaining adequate staffing throughout performance of the contract, and to ensuring that contractor staff continually have access to required systems and information.

Offerors shall attach to Volume 3, Section A, Key Personnel Resumes for staff proposed to fill Key Personnel roles identified in Section 4.2.3.1. Each resume is limited to five pages (size 10 font) and is not included in the total Volume page limit. If any individual proposed as Key Personnel is not currently an employee of the Offeror, the Offeror shall submit with this part a letter of commitment signed by the individual(s) stating that the individual makes an unequivocal commitment to perform in the Key Personnel role proposed in the event the Offeror receives this Contract award from the Government.

Offerors shall describe their staffing approach specific to the Member Service Center staff tasked with delivering assistance to NPA members spanning all Areas of Assistance set forth in Section 4.4. Offeror shall clearly describe its Member Service Center staffing architecture, with particular detail toward offeror's tiered inquiry response structure (PWS Section 4.3.1.5 and Volume 2, Section A). Offeror shall explain how the Member Service Center staffing plan correlates with Offeror's overall contract staffing plan, including lines of reporting and positions of authority. Offerors shall describe their approach to establishing and maintaining sufficient staffing resources, particularly where specialized and/or licensed staffing resources are required to deliver assistance. Offerors shall include their proposed case management staff to case ratio and planned allocation to support members of various acuities. Offerors shall detail their NPA Contractor Staff Training plan (Section 4.2.3.3) for Member Service Center staff to ensure staff have sufficient knowledge and resources to provide effective and meaningful member assistance spanning all areas of assistance identified in Section 4.4.

Offerors shall provide Attachment ATT023_OAS_LaborCategories_VolI in Section D to Volume 3, Staffing Approach, a table of their proposed labor categories and estimated labor hours to perform each task area under the contract.

Volume 4, Similar Experience

Offerors shall provide three (3) references, each describing a contract performed within the past five years that clearly demonstrates the Offeror's experience in performing projects of similar scope, size, and complexity to the requirements described in the solicitation. References may include contracts entered into with the Federal Government, agencies of state and local governments, and/or commercial customers. The following information shall be provided for each reference:

- a. Contract number, customer/agency name, and contract title;
- b. brief narrative description of the work performed for each of those contracts, including a discussion of any problems encountered/corrective actions and significant accomplishments, and a discussion of how that experience contributes to Offeror's ability to perform the NPA requirement;
- c. dollar value, contract type, period of performance, place of performance, and the number and types of personnel used in the performance of the contract; and

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- d. name, address and phone number of at least two (2) customer contacts (Contracting Officer and Project Officer, if applicable) for each of the identified contracts.

Volume 4 is limited to a maximum of 15 pages, and each reference is limited to a maximum of 5 pages.

Volume 5, DATA MANAGEMENT PLAN

CDCL.10 Data Management Plan (Instructions) (Nov 2018)

CDC requires awardees of contracts that involve the collection or generation of public health data with federal funds to develop, submit, and comply with the requirement of a Data Management Plan (DMP) for each collection or generation of public health data undertaken. Consistent with the terms of and activities expected under the performance work statement, the Offeror shall develop and submit a DMP as part of its proposal in response to this solicitation, and if awarded a contract, update the DMP throughout the life cycle of the public health data collected or generated during the performance period. The DMP shall describe, to the extent appropriate, the data to be collected or generated; plans for making the data accessible that state what the data represent and potential limitations for use, provisions for the protection of privacy, confidentiality, security, intellectual property, or other rights (where these protections preclude making data accessible, justification must be provided); and plans for archiving and long-term preservation of the data. Costs, if any, associated with developing and implementing a DMP, including costs of sharing, archiving and long-term preservation, are allowable costs and shall therefore be reflected in an Offeror's Business Proposal. A DMP must be developed in accordance with the below guidelines.

A proposal received without a DMP will be deemed "Unacceptable".

A DMP for each collection and/or generation of public health data should include the following information:

- A description of the public health data to be collected or generated in the contract period of performance;
- Standards to be used for the collected or generated public health data;
- Mechanisms for or limitations to providing access to and sharing of the data (include a description of provisions for the protection of privacy, confidentiality, security, intellectual property, or other rights) or justification for why data cannot be made accessible This section should address access to identifiable and de-identified data (see below for additional information about access);
- Statement of the use of data standards that ensure all released data have appropriate documentation that describes the method of collection, what the data represents, and potential limitations for use; and;
- Plans for archiving and long-term preservation of the data, or explanation of why long-term preservation and access are not justified. This section should address archiving and preservation of identifiable and de-identified data (see Public Access to CDC Funded Digital Public Health Data) for additional information regarding archiving).

Examples of Data Management Plan Templates and Tools:

University of California: <https://www.cdlib.org/services/uc3/dmpt.html>

PART II – Business Proposal:

Business (Cost/Price) Proposal shall include the following:

1. Contract Form and Representation and Certifications.

- a) The contract form found in Section A of this solicitation;
- b) The Offeror is required to include a complete Section B (Supplies or Services and Prices/Costs) of the solicitation, Supplies or Services and Prices/Cost, showing proposed prices. Prices proposed shall be inclusive of all direct costs, indirect costs and profit associated with performing the services and providing the required materials; Section be should include the proposed milestone payment schedule for all firm-fixed-price CLINs and the level of effort table with loaded rates;
- c) Completed attachment **ATT024_OAS_FullyBurdenedRates and ODCs_Vol2**. NOTE: Offerors are only allowed to propose labor categories and the fully burdened labor rates and ODCs that are a part of Attachment ATT024
- d) Representations and Certifications contained in Section G, (if applicable).
- e) Other Administrative Data:
 - i. Your proposal must stipulate that it is predicated upon all the terms and conditions of this RFP. In addition, it must contain a statement to the effect that it is firm for a period of at least 60 days from the date of receipt by the Government.
 - ii. Your proposal must list any current commitments with the Government relating to the work or services and indicate whether these commitments will or will not interfere with the completion of work and services as contemplated under this proposal.
 - iii. Your proposal must identify any former HHS employee to be utilized on this project by providing the individual's name when employed by HHS, where employed, and the capacity in which employed.
 - iv. Your proposal must indicate whether you have the necessary financial capacity, working capital, and other resources to perform the contract without assistance from any outside source. If not, indicate the amount required and the anticipated source.
 - v. It is an HHS policy that contractors provide all equipment and facilities necessary for performance of contracts, unless otherwise specified. If equipment must be acquired, you must include in your proposal a description and the estimated cost of each item, and state whether you propose to furnish the item with your own funds.
 - vi. HHS EIT Accessibility Standards (HHSAR Provision 352.239-74(9) – found in PWS) – the Offeror, prior to being awarded a contract, must submit an HHS 508 Product Assessment Template, showing proposed EIT supplies and/or services meet applicable Section 508 accessibility standards and/or any underway remediation efforts addressing conformance issues.
 - vii. The Offeror shall submit its approved individual subcontracting plan in conjunction with proposal submission. See Instructions for submission below in **H.9 CDCL.11 Offeror Subcontracting Plan Submission – Small Business Customer Experience (SBCX) System (Feb 2023)**. CDC fiscal year 2025 Small Business Goals are provided for informational purposes:

Business Type	CDC 2025 Goals
Small Business	11%
Small Disadvantaged Business	3.22%
Woman-Owned Small Business	3.00%
Service-Disabled Veteran-Owned Small Business	1.61%
HUBZone	1.00%

All the above must be signed by an official authorized to bind the Offeror.

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2. **Contract Proposal Cover Sheet.** The cover sheet of your Business Proposal should contain the following information (as applicable):
 - a) Solicitation or modification/amendment number;
 - b) Name and address of Offeror;
 - c) Name and telephone number of point of contact;
 - d) Date of submission;
 - e) Name, title, and signature of authorized representative.
3. **Detailed Pricing/Cost Information.** You must submit, as a minimum, a business proposal supported by **detailed cost and pricing data in accordance with the Federal Acquisition Regulations.**

The itemized cost and the rationale for significant categories should be furnished as follows:

(1) **Direct Labor:** At a minimum, the staffing approach included in the technical proposal shall be included in the business proposal, along with the proposed estimated number of hours and fixed hourly rates and totals for each proposed labor category by year and in MS Excel format (unprotected document).

(2) **Other Direct Costs (ODC):**

In addition to the proposed labor costs for the base period and each of the four (4) option periods, the offeror shall provide a detailed breakdown of proposed Other Direct Costs (ODCs) for each CLIN, along with the basis for all proposed costs. Offerors shall use the MS Excel format.

ODCs include all non-labor costs required to perform the Contract, such as materials; subcontracts for supplies or incidental services not priced using labor categories (e.g., organizational computer use charges); and estimated travel costs (e.g., airfare, transportation, lodging, meals, and incidental expenses).

Additional ODC requirements include:

1. **Materials and Services:** Provide a consolidated, priced summary of all individual material quantities anticipated for contract performance, along with the basis for pricing (e.g., vendor quotes, historical invoice prices).
2. **Subcontracted Items (if applicable):** Conduct cost analyses of all subcontractor proposals, including cost analyses for all cost-reimbursement subcontracts. Include these analyses in your cost submission for subcontracted work (see FAR Subpart 44.2 regarding information required to support a request for subcontract consent).
3. **Travel:** Provide the total proposed travel costs, including destination, duration, purpose, per diem, transportation, and the basis for these proposed costs. A copy of the offeror's corporate travel policy must be included with the proposal.

H.9 CDCL.11 Offeror Subcontracting Plan Submission – Small Business Customer Experience (SBCX) System (Feb 2023)

In accordance with FAR 19.704 and FAR Clause 52.219-9, the submission of a subcontracting plan by other than small business Offeror(s) is a requirement as a part of the proposal submission process and is to be submitted separately from the technical and cost proposals. An Offeror's subcontracting plan must be determined to be acceptable, by the Contracting Officer, prior to the contract award. An offeror is to submit their respective subcontracting plan electronically using the U.S. Department of Health and Human Services (HHS) Small Business Customer Experience (SBCX) system at <https://osdbu.hhs.gov>. The Offeror shall follow the instructions outlined in the SBCX Industry Guide (<https://sbcm-acqtools.s3.amazonaws.com/SBCX+Industry+Subcontracting+Guide.pdf>) to successfully submit their subcontracting plan by the proposal submission deadline.

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The official point of receipt for determining timely submission of an Offeror's subcontracting plan is the SBCX system and/or email notification. Once the subcontracting plan is successfully submitted in the SBCX system the Offeror should receive an email notification and confirmation message of completion upon submission.

If an Offeror's subcontracting plan is not confirmed as received within the SBCX system by the proposal submission date specified in the solicitation, it will be considered late in accordance with subparagraph (c)(3) of FAR Clause 52.215-1, Instructions to Offeror—Competition Acquisition. Disposition of late submittals of a subcontracting plan by an Offeror via the SBCX system is at the discretion of the Contracting Officer.

Any technical questions regarding the use of the SBCX system may be submitted via email message to the SBCX help desk at client.support@apexlogic.com. The client support hours of operation are Monday – Friday, 6:00 A.M. – 8:00 P.M. Eastern Time (ET). Note: help desk tickets can be submitted 24 hours a day / 7 days a week and a representative will respond within the presented client support hours of operation for assistance.

**The link to provide Small Business Subcontracting Plan for this solicitation is:
<https://osdbu.hhs.gov/subcontracting/feb98c7e-6e45-4692-954c-3773a04ee536>**

SECTION I – EVALUATION FACTORS

I.1 Evaluation Factors

FACTOR 1: TECHNICAL EVALUATION

Each subfactor (listed below) will be evaluated according to the extent that it reflects a clear understanding of the subject areas to be accomplished and on the soundness, practicality, and feasibility of the contractor's technical approach for providing the services required for this solicitation.

Technical Factor 1: Contract Management Approach

The Government will evaluate this factor using Volume 1: Contract Management Approach. The Government will evaluate its confidence in the Offeror's approach to transitioning-in to the role as the Program NPA, performing contract management, ensuring regulatory compliance, and conducting a transition-out (as per sub-sections A, B, C and D above). The Government's confidence will be increased by:

1. clear, detailed, and realistic transition-in approaches that will ensure that the Offeror is ready for the SHCD.
2. Offerors that leverage existing, mature, modern systems to standup operations.
3. approaches that include early, thorough, and realistic timelines for submission of security authorization requests to ensure systems are operable and an Authorization to Operate (ATO) is received by SHCD.
4. approaches that have accounted for robust testing and validation, particularly for connectivity and information exchanges with other Program contractors.
5. approaches that minimize disruption to other Program contractors and current processes
6. approaches that account for effective communication with NPA members to ensure minimal disruption to their overall Program experience.
7. approaches that outline a clear, comprehensive, and mature contract management approach, including the integration of PWS areas of responsibility.
8. management approaches that leverage realistic and effective project management tools and techniques, adequate resources to accomplish the work, and a well-developed risk management plan.
9. approaches that emphasize early risk identification and plans for how to avoid, manage, mitigate, or resolve risks.
10. a robust quality assurance approach and a modern, mature information security approach that is compliant with the requirements of this Contract.
11. For transition-out, approaches that have a clear, detailed, and realistic plan to transition the NPA role to a successor contractor.
12. approaches that simultaneously ramp-down operations of the Offeror, facilitate the transition-in of the successor, and ensure continuity of operations in support of Program members in a seamless manner.

Technical Factor 2: Member Service Center, Infrastructure and Operations

The Government will evaluate this factor using Volume 2: Member Service Center, Infrastructure and Operations. The Government will evaluate its confidence in the Offeror's approach to fulfilling all requirements for infrastructure, systems, and tools as set forth in the Contract.

The Government's confidence will be increased by:

1. approaches that deploy readily available infrastructure, systems, and tools to ensure the Member Service Center is fully operational and compliant with QASP standards throughout Sustainment.
2. the use of modern technology stacks and architecture for establishing all required systems, components, and functionalities.
3. approaches that leverage automation to achieve efficient and streamlined operations.

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4. approaches that incorporate robust, end-to-end process workflows and a tiered inquiry response structure to ensure effective and meaningful member assistance.
5. approaches that leverage modern technology and mature processes to reduce wait time, increase the effectiveness of customer support, and achieve high customer satisfaction, while providing a means for detailed and accurate quality assurance tracking.
6. approaches that provide a strategy for transparent data and business information management and exchange with the Program and other Program contractors, as well approaches that provide for data integrity across Program systems.
7. approaches to data and information exchange that facilitates the Offeror adapting to the existing data setup with other Program contractors, so as to limit the impact to other Program contractors as the Offeror onboards, while also modernizing data and information exchange over the life of this Contract to take advantage of modern methods and industry best practices.
8. approaches that incorporate an accessible design to the NPA Member Profile System to ensure easy access to and audit of centralized information
9. approaches that are robust in providing for continuity of operations, to ensure the continuation of critical services in the event of disaster or other contingency.

Technical Factor 3: Staffing Approach

The Government will evaluate this factor using Volume 3: Staffing Approach. The Government will also assess the extent to which Offeror's staffing approach correlates with and is congruent to Offeror's technical approach. The Government will evaluate its confidence in the Offeror's approach to staffing across the entire contract.

The Government's confidence will be increased by:

1. staffing approaches that leverage well-qualified personnel, with an appropriate mix of labor categories and sufficient estimated labor hours to ensure successful completion of all contract tasks.
2. approaches that include clear staffing architecture, with clearly defined roles, responsibilities, and lines of reporting.
3. key personnel proposed, including their suitability to perform this requirement and relevant experience and qualifications.
4. key personnel who exhibit significant relevant experience leading contracts and related projects similar in size and scope to the NPA Contract.
5. the effectiveness of the Offeror's NPA Contractor Staff Training Plan.
6. approaches that ensure staff continually have sufficient knowledge and resources to fulfill assigned responsibilities.
7. the Offeror's approach to staffing the Member Service Center to ensure delivery of effective and meaningful assistance to NPA Members.
8. by approaches that include a clear Member Service Center staffing architecture, with tiered inquiry response structure and clear lines of reporting, responsibilities, and positions of authority as they relate to successful delivery of member assistance and effective resolution of member issues.
9. staffing approaches that place members at the center of the Offeror's approach by reducing wait time, increasing effectiveness of customer support, and achieving high customer satisfaction.
10. approaches that include robust case management strategies to ensure appropriate staff to case ratio and allocation of various acuties across staff members.

Technical Factor 4: Similar Experience

The Government will evaluate this factor using Volume 4: Similar Experience. The Government will evaluate its confidence in the Offeror's ability to perform this Contract based on the Offeror's experience performing contracts or projects of similar size, scope, and complexity to the NPA.

The Government's confidence will be increased by:

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1. Offerors that exhibit deep and wide experience in the areas of responsibility in the PWS.
2. references that are similar in technical complexity, population served, and geographical area served under the NPA PWS.

Technical Factor 5: Data Management Plan – CDCM.03 Evaluation of the Data Management Plan (DMP) (Nov 2018) - Pass/Fail

The Data Management Plan submitted as a part of the Offeror's proposal will be evaluated during the pre- award process for quality and completeness and will potentially require later revision as the DMP will be updated during the contract period of performance. The DMP will be rated on a "Pass/Fail" basis considering the clearness, appropriateness, and inclusiveness of the elements described under Data Management Plan (Instructions). The Government's evaluation will consider the level or magnitude of the standards to be used for the collected or generated public health data as well as the types of mechanisms for or limitations in providing access to and sharing of the data. Evaluation will also consider how well the DMP clearly outlines plans for archiving and long-term preservation of the data.

For statements of work that involve defined projects which include public health data collection or generation of data, proposals submitted without the required DMP will be deemed unacceptable. For contracts where data collection or generation of data activities may later become known and necessary (e.g. via contract modification), DMPs will be required to be submitted for evaluation and acceptance during the period of performance and must be developed in accordance with Public Access to CDC Digital Public Health Data.

FACTOR 2: PAST PERFORMANCE EVALUATION

CDC will evaluate the past performance of the offerors using CPARS and other sources.

The Government will evaluate the Offeror's past performance information for recency, relevancy, quality, and general trends in Offeror's performance.

- 1) Recent examples are defined as contracts either on-going or that were completed within the past five years as of the due date for written proposals. Recent past performance examples are considered a stronger predictor of future success and will be given significantly more weight than non-recent examples.
- 2) Relevant examples are defined by the similarity of the nature of the work and tasks, technical complexity, dollar value, geographical area served, and contract type of the past contract compared to this solicitation. More relevant past performance examples are considered a stronger predictor of future success and will be given significantly more weight than less relevant examples.
- 3) The overall quality of the offeror's past performance will be evaluated based on the ratings received and information available on the performance of previous contracts .

In the case of an offeror without a record of relevant past performance or for whom information on past performance is not available, the offeror may not be evaluated favorably or unfavorably on past performance.

FACTOR 3: COST/PRICE EVALUATION

The Government will evaluate the Offeror's cost/price in relation to the Offeror's proposed technical approach to validate that the proposed price is commensurate with the proposed technical approach and reasonable for conducting the tasks outlined in the PWS.

A Cost/Price realism analysis of the Offeror's proposal may be conducted for such purposes as determining an Offeror's understanding of the solicitation's requirements or assessing risk associated with an Offeror's proposal.

Cost/Price proposal shall be fully supported by detailed information to include all labor and materials that make up the cost/price. Offerors shall provide sufficient detail to facilitate the Government's reasonableness and potential realism evaluations. Included in detailed information provided should be the Offeror's estimation methods,

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judgmental factors, and assumptions in developing the Offeror's pricing, including the rationale and basis for the proposed level of effort and labor mix to perform the PWS tasks.

Specifically, the proposal shall include labor categories and associated hours and material mix and amounts broken down by task requirement area and linked to the Performance Work Statement.